

PSY101 – FINALTERM MERGED HANDOUTS & PPT (LECTURE 24 TO 44)

Introduction to Psychology

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Final Term Syllabus

Lecture 24 to 44

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“VU Global Study Group”

**PSY101 – Interlocution to Psychology
HANDOUTS**

Lesson 24 to Lesson 44

Final Term Syllabus

Introduction to Psychology (PSY101)

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MEMORY II

Long-Term Memory

This system stores information on a permanent or relatively permanent basis; the information thus stored may or may not be retrieved easily. The term *long-term memory* is somewhat broad meaning phrase because it can refer to facts learned a few minutes ago, personal memories many decades old, or skills learned with practice. Generally, however, long-term memory describes a system in the brain that can store vast amounts of information on a relatively enduring basis. When you play cricket, remember what you had for dinner last night, recall your first birthday party, and remember how to play a board game, or to sing along to a favorite song; for all this you draw on information and skills stored in long-term memory.

Psychologists have different theories about how information enters long-term memory. The traditional view is that information enters short-term memory and, depending on how it is processed, may then be transferred to long-term memory. However, another view is that short-term memory and long-term memory are arranged in a parallel rather than sequential fashion. That is, information may be registered simultaneously in the two systems.

There seems to be no finite capacity to long-term memory. People can learn and retain new facts and skills throughout their lives. Although older adults may show a decline in certain capacities, for example, recalling recent events. They can still have new experience even in old age. For example, vocabulary increases over the entire life span. The brain remains plastic and capable of new learning throughout one's lifetime, at least under normal conditions. Certain neurological diseases, such as Alzheimer's disease, can greatly diminish the capacity for new learning.

Psychologists once thought of long-term memory as a single system. Today, most researchers distinguish three long-term memory systems: episodic memory, semantic memory, and procedural memory: memory related to incidents, language, and procedures.

The Components of Long-term Memory

Also known as modules of long- term memory:

- Declarative memory
- Procedural memory
- Semantic memory
- Episodic memory

Declarative Memory

The module responsible for factual data, dates, faces, names etc.

Procedural Memory

The memory center for skills and habits e.g., playing cricket, driving a car etc. It refers to the skills that humans possess. Tying shoelaces, riding a bicycle, swimming, and hitting a football are examples of procedural memory. Procedural memory is often contrasted with episodic and semantic memory. Episodic and semantic memory are both classified as types of declarative memory because people can consciously recall facts, events, and experiences and then verbally declare or describe their recollections. In contrast, non-declarative, or procedural, memory is expressed through performance and typically does not require a conscious effort to recall.

Semantic Memory

Storehouse for facts and general knowledge about the world e.g., historical facts, or scientific formulae. It refers to our general knowledge of the world and all of the facts we know. Semantic memory allows a person to know that the chemical symbol for water is H₂O, or that cats have a tail etc.

Episodic Memory

Memory for information pertaining to life events, episodes, biographical details. It refers to memories of specific episodes in one's life and is what most people think of as memory. Episodic memories are connected with a specific time and place.

Associative Module

A strategy or process whereby material is recalled by thinking about related information e.g., whose son was the Mughal Emperor "Shah Jehan"?

Priming and long-term memory

If an individual is exposed to a piece of information earlier, then later on it is easier for him to recall the same or similar pieces of information.

Explicit and Implicit Memory

Explicit Memory

Recollection of memory that is intentional and conscious e.g., date of your interview, or the day when your course started. Explicit memory refers to the deliberate, conscious recollection of facts and past experiences. If someone asked you to recall everything you did yesterday, this task would require explicit memory processes. There are two basic types of explicit memory tests: recall tests and recognition tests.

Implicit Memory

There are many pieces of information or memories that are stored in our system but which we are not aware of at the conscious level. These can and do affect our behavior and performance later e.g., childhood fear of blood hindering the performance of becoming a doctor.

Implicit memory refers to using stored information without trying to retrieve it. People often retain and use prior experiences without realizing it. For example, suppose that the word 'ostentatiously' is not part of your normal working vocabulary, and one day you hear the word used in a conversation. A day later you find yourself using the word in conversation and wonder why. The earlier exposure to the word primed you to retrieve it automatically in the right situation without intending to do so.

Single Process Approach

Some psychologists do not agree with the idea of three sequential stages of memory; they had a single process approach i.e., levels-of- processing theory.

Levels-of- Processing Theory

- What is most important in the process of memorizing or learning material is the degree to which the material is mentally analyzed.
- How much of new information will be remembered will be determined by the extent or amount of information processing that takes place when new material is encountered.
- The information will be more and more likely to be remembered as the initial processing becomes more and more intense.

Information that is paid little attention will not be processed thoroughly, will remain at the shallow level of memory, and will be forgotten soon.

On the other hand information that receives greater attention is processed thoroughly; such information enters the deeper level of memory.

Shallow Level Memory

This is the physical and sensory aspect of information.

Deepest Level Memory

- Meaning of information is considered important in analysis of information.
- A wider context, rather than a limited or restricted, perspective may be taken.

- Associations between what this information means, and the already existing and known broader networks of knowledge, may be drawn

Recall and Retrieval of Long-term Memory

- Why do we remember certain events very vividly even after a very long time?
- Why can't we remember something that we thought we know very well?
- Are the memories that we can recall very clearly, really exact reproductions or distorted, or may be exaggerated?

Flashbulb Memories

- The memories that are as clear and vivid as a snapshot.
- Such memories are of events that are important for us, are specific, or surprising or astonishing e.g., an interview room, an accident, first day at class.
- Although flashbulb memories are vivid, clear and detailed, still they may be lacking many important details.
- At times the recall may be very different from the actual event whereas the person believes he is remembering right.
- Many elements may be missed and many added.

Tip-of-the-tongue Phenomenon

Inability to recall events, details, or information that we thought we knew very well.

Causes of the Tip-of-the-tongue Phenomenon

- Information overload
- Accumulation of pieces of similar and confusing information
- Preoccupation]

Retrieval Cues Are Helpful!!!

Autobiographical Memory: The Episodic Aspect of Long-term Memory

- Autobiographical memories are recollections of past life events, circumstances, episodes, and happenings.
- These memories, although they seem perfect to us, may also be distorted, exaggerated, changed.
- **Schemas: According to Bartlett information is remembered in terms of schemas i.e., general themes in memory that contain relatively little specific detail.**
- Irrelevant or unimportant details are discarded.
- People's understanding of the situation, their expectations, and awareness of others' motivation affect memory and recall.

Forgetting

Forgetting is the inability to retrieve or recall information from the long- term memory.

Forgetting may occur due to a number of factors such as:

- Information discarded / decayed due to the non- use of the learned material.
- At times we know something, but are unable to access and retrieve it properly.
- Hermann Ebbinghaus, a German psychologist, was the first one to study the phenomenon of forgetting by using himself as a subject for his own experiments.
- Used non- sense syllables; in sets of three; one vowel in between and the two consonants at the ends e.g. FIW, BOZ etc. His experiments, indicated that forgetting occurred systematically; in the initial stages (the first hour and, in all the first nine hours) forgetting was most rapid, but then, the rate of forgetting decreased and slowed down and did not change much with the passage of time.

- Forgetting should not be understood as a problem alone, since it has some advantages too: it helps us discard irrelevant information and enables us to learn new things.

Theories of Forgetting

Why do we forget???

- Several theories have been posed to explain this phenomenon:
 1. Decay theory of forgetting
 2. Interference theory of forgetting
 3. Repression

Decay Theory of Forgetting

The oldest concept which states that stored information decays and is disintegrated with the passage of time.

Criticism against the decay theory

- 1) It can be taken as a general explanation of how forgetting takes place, but can not be accepted as the actual reason, i.e. the process of forgetting.
- 2) At times recollection of memories occurs after a long passage of time, which is an experimentally proven fact, and this theory fails to explain this phenomenon.

As decay theory could not explain the actual process of forgetting, another theory emerged, called interference theory of forgetting.

Interference Theory of Forgetting

- Interference results when the recall of the learned phenomena is blocked/ displaced by other information.
- John Jenkins and Karl Dallenbach (1924) were the first ones to experimentally prove the role of interference in forgetting.
- Experiments on students showed that they recalled non- sense syllables relatively well after waking from the sleep, than when they were fully awake for the whole day.
- Maximum forgetting occurred when they were awake than when they were allowed to sleep which showed that interference of the activities and happenings has a relationship with forgetting.
- Interference, however, sometimes is found to be helpful in the sense that it improves our ability to remember important events of life; not the exact and true characterization, but the important portions/ episodes

Retrieval of the learned phenomena may be influenced by the two processes:

- i. Proactive interference
- ii. Retroactive interference

Proactive interference

Proactive interference occurs when the already stored memories come in between or interfere with the recall of the newly learned material e.g., you learned psychology this semester and sociology in the last semester. When exam was taken in psychology, material from sociology, rather than that of psychology, kept coming into mind.

Retroactive interference

Retroactive interference occurs when the newly learned material interferes with or blocks our ability to recall the previously learned material

In the psychology-sociology example, when examinations were taken, the memories of newly learned material (psychology) may interfere with recall of the formerly learned material (sociology).

Repression

- Repression: putting the undesirable thoughts, events and fears into the unconsciousness and trying not to remember it again.
- Repression is a Freudian concept”. In spite of being sent into the unconscious, unpleasant feelings continuously influence person’s attitudes and behaviors.

Improving Memory and Recall

- Elaborative rehearsal
- Mnemonics
- Method of loci
- The Keyword technique: Pairing foreign word with a similarly sounding common word in native language in order to remember the foreign word e.g., pairing English “amaze” with “maze’ in Urdu.

The Encoding Specificity Phenomenon

Information is best learnt and remembered at a time and place, or environment, similar to or same as the one where it was initially learnt.

Mental Organization of Text Material

When the material is being read for the first time, it should be in the memory.

Taking Notes

- Take cue notes instead of taking down everything.
- Listen and think too.

Other Strategies for Better Memory

- Keeping distractions away
- Chunking
- Rhyming
- Reading aloud
- Sleep intervals

Improving Memory: Memory Skills for Students

Over learning

- Learning the material over and over again.
- Very effective in case of natural science, mathematics or history.

Mnemonics

Short devices that help encode information better. These provide ready cues for quick and accurate retrieval.

S - Q - 3R

- A five- step approach. S-Q-3R is an approach for improved learning and recall of material to be learned.

S = Surveying and skimming.

Q = Questioning: asking questions about what one has read.

3R's:

Read: Going through the surveyed material thoroughly.

Recite: Recitation makes learning active than passive and adds activity to learning.

Review: Reviewing and going through the learnt material from time to time

REMEMBER!!!! THE SEQUENCE OF THESE STEPS IS IMPORTANT

MEMORY III

Memory Disorders/Dysfunctions

Memory loss has long been considered as a result of aging, but now physicians believe that there must be some pathological reasons as well, that cause memory impairment: “Memory Cognitive Impairment” (MCI). Although the suffering individuals are capable of doing every day tasks independently, they also heavily rely on using diaries, calendars, reminders etc.

Amnesia

- Greek word which means “forgetfulness”.
- Loss of memory with other mental difficulties
- In old age, people are unable to retrieve memories.
- Includes two types of amnesia.
 1. Retrograde amnesia
 2. Anterograde amnesia]

Dementia

- Severe and rare disease, mostly affects people in their 60’s and the risk is doubled every 5 years after 60; at 80, one out of every five people develops dementia.
- Symptoms may include repeating things several times, speech problems, intellectual problems, and inability to remember the skills learned early in life e.g., dressing up, tying shoe laces, using knives and forks etc.
- The sufferer is irritable, withdrawn, rude, facing anxiety, depression, suspicious attitude, and aggressive.

Alzheimer’s disease

- Common cause of memory loss in elderly people.
- Progressive loss of memory including effect on person’s language and recognition of people; also losing way, inability to plan things, difficulty in organizing daily tasks etc.
- Major symptoms include depression, lack of motivation, laziness, agitation, physical and verbal aggressiveness; recent events are forgotten; delusions and hallucinations may also occur.

Huntington’s disease (HD)

- Fatal disease that results in involuntary movements and cognitive impairment.
- Genetic disease that runs in families.
- It mainly affects important brain regions such as basal ganglia that have vital role in co-ordination and movement.

Amyotrophic Lateral Sclerosis (ALS)

- Occurs mostly in adults, and is common in men than women.
- Develops in the age of 40- 70 but also occur in the twenties and thirties as well
- Affects motor neurons of the body (that bring messages from the brain to the different muscles of the body).

Korsakoff’s Syndrome

- Severe and often permanent loss of memory.
- Addicted/ long-term alcoholics are the sufferers of this syndrome.
- Memory impairment occurs because of brain damage caused by thiamine deficiency and alcoholism.
- Major symptoms; hallucinations, Impaired dietary intake, repeating information several times, and inability to remember things.

Motivation

Ask Yourself

- Why do some students want to get an “A” in every exam, and some don’t even bother about it?
- Why do the skydivers love to dive down from hundreds of feet above the ground?
- Why do some people some people run after money and some refuse even the most attractive job offers?
- Why do some people leave their country for earning money and some are contented and happy with whatever is available to them at home?
- Why people become doctors, engineers, social workers, pilots, army men etc?
- And to ourselves, why we want to learn/ know about the particular subject?

The answer to all these questions can be explained by understanding the concept of motivation.

What is Motivation?

Motivation is a desire, drive, instinct or need that speeds up our behavior towards some goal.

Psychologists are interested in observing and evaluating the factors that motivate or direct human behavior.

A **MOTIVE** is a need or a want that causes us to act. **Motivation** involves **goal-directed behavior**, an interesting field of study that investigates what motivates us to initiate or take action to pursue a goal. Early theories focused on **instincts**, inherited automatic species-specific behaviors, and **drives**, physiological compulsions that we need to satisfy such as hunger (food).

Basically, there are two types of motives that are essential for the proper human functioning

- I. Primary/ unlearned/ physiological/ biological motives**
- II. Secondary/ learnt/ psychological motives**

I. Primary Motives

Concerned with all the biological/ physiological needs of the body. They are also known as unlearned motives because they entirely include the basic drives such as hunger, thirst, need for sleep, air, excretion etc that do not need any sort of learning.

II. Secondary Motives

Also known as the psycho- social motives because they involve people’s appreciation or appraisal in order to live successfully in any society.

- Also important in the sense that they are the means of satisfying the primary motives such as work and salary, good marks and appreciation from the parents.
- These are a source of mental satisfaction as well.
- The important and interesting fact about motivation is that organisms keep trying to achieve the desired goal. When the goal is achieved, the motivation regresses and when not achieved, the person keeps on trying to achieve it.
- When one motive is satisfied, we start to move forward to satisfy the next need and so on.
- This is how struggle continues throughout our lives.

Primary/Unlearned/ Physiological/ Biological Motives

- The major primary motives are:
- Hunger
- Thirst
- Fatigue and sleep
- Pain
- Sex
- Excretion
- Air
- Warmth and cold

Hunger

- The most urgent and desirable need in all organisms including human and animals.
- Eating is related to the homeostatic mechanism of the body.
- Psychologists have done research on animals to observe the level of motivation, which showed that internal system, not only regulates the quantity of food intake but also the kind of food that has been taken.
- The systems that involved in when to eat and how much to eat is a complicated phenomena.
- It is a proven fact that hunger is not only related with the empty stomach; people whose stomach has been removed still experienced the sensation of it.
- The animals have taken larger amount of food when it contains low level of nutrients as compared to highly nutritive diet; showing that both animals and humans are sensitive about the nutritive value of food.
- Chemical secretions in blood: chemicals/ hormones secreted by the endocrine gland also play a crucial role in the hunger drive e.g., studies showed that when glucose (sugar) was injected in rats, they felt less hunger as compared to when they were given insulin which resulted in hunger pangs.

What Role Does Brain Play in Hunger Drive?

- Hypothalamus, brain's vital organ concerned with hunger and operates on it by regulating its activity i.e., food intake.
- In case of injury in hypothalamus, the most apparent change will occur in eating behaviors.
- Studies showed that when the rats' lateral hypothalamus was removed; it resulted in starving to death and they refused to eat anything when given food.
- When the rat's ventromedial hypothalamus was removed, it resulted in the opposite i.e. extreme overeating behavior; increase in weight by 400 percent than the actual weight.
- It is a known fact that hypothalamus regulates the hunger drive yet it is not clear as to how it operates.

External Factors in Eating Behavior

- Societal rules and conventions.
- Learn eating patterns from past experiences.

Eating-related disorders

- Obesity
- Anorexia nervosa
- Bulimia

Weight Set point

- A certain weight level that the body strives to maintain.

Metabolism

- The rate at which food converts into energy and then is expended by the body.

Managing and reducing weight

- Reducing weight is a tough task so try not to gain too much of weight.
- Eat wisely and do not be tempted.
- Slow eating helps.
- Avoid junk/ fad food.
- Do regular exercise; aerobics do help
- Make realistic goals and pursue them strictly.
- If you failed in achieving the set goal try again and do not feel guilty.
- Reward yourself off and on.... after considerable intervals.

Thirst

- We can live without food for several days or even months, but without water, we can hardly live for few days.
- More than 75% of our body weight is due to the presence of water/ liquid.

- The thirst drive is largely internal and includes three basic mechanisms: when salt concentration in our body becomes high, and then it triggers hypothalamus to act, resulting in thirst drive.
- Secondly, when fluid volume decreases in the circulatory process, then this drive is stimulated e.g. after injury when large amount of blood is wasted.
- Another important factor is the increase in the body temperature.
- The replenishment of water is very rapid; that is why we feel thirsty after a very short time.

Fatigue and Sleep

- Rest and proper sleep is very important for the effective functioning of species.
- After the hard work of the day, person becomes fatigued and needs adequate amount of rest in order to be fresh again next day.
- What causes fatigue is yet unknown, but chemical changes in the body that effect muscles may be the factor causing it; one cause is the excess amount of lactic acid in the muscles.

Fatigue Can Be Psychological As Well As Physical

- Frustration, anxiety, tension, worry, boredom etc are factors other than the physical ones causing fatigue in persons.
- In physical exertion, rest brings freshness and relief, but when the person is in stress or anxiety, rest will not be helpful.
- Partly, sleep is also related with the chemical activity in the body in which the brain, nerves, and muscles are all involved.
- The pattern of sleep is culturally based also; people make adaptation with the successive changes in lightness and darkness; e.g. Norway.

Pain

What makes pain a drive?

- Avoiding injury, scar or wound is what makes it an important drive.
- When pain persists for a longer period of time, then it become a drive to get rid of it.
- Some people respond quickly to pain; more than the normal healthy individual would do and some are less responsive to pain.

Pain is a blessing in disguise!!!!!!

Sex Drive

- Very essential drive for the survival and continuation of species.
- Societal and religious conventions, laws, and restrictions make it a more powerful drive for human beings.
- In humans, past experience, emotions and ways/ manner of expression play a very important role.
- In males, gland is a testis; androgens and other middleman hormones of pituitary are responsible for its expression.
- In females, organ is an ovary; estrogens (arousal) and progesterone (pregnancy) are important and vital glands for its expression.
- Sexual drive is largely dependent on the chemical secretions in the body, which is a proven fact by using animals as subjects.

Excretion: Bowel and Bladder Tensions

- The body has to get rid of the waste regularly.
- The satisfaction of this drive is very important to children as compared to adults for whom it has less importance.
- Toilet training in early childhood plays a very crucial role in the development of personality of the child later in life.

- Harsh/ severe and too early toilet training causes sense of insecurity in the child;
- Withholding of bowel movements even when not required may result from it.
- Important thing to know about is that the bowel and bladder muscles are the last ones that come under the child's control, so the toilet training should be given when the child is physically strong enough and prepared for it.

Air

- Oxygen = Most instant and essential requirement of human body.
- Oxygen deficiency/ hunger can be felt when there is excess of carbon dioxide.
- Continual supply of oxygen to the brain is very important because, although brain uses very small amount of it but deprivation for only few seconds may result in anoxia or neural damage.
- Even during birth of the child, sufficient amount of oxygen is required; otherwise it may result into mental retardation or brain damage.
- At high altitudes oxygen deprivation may result into strange behaviors; person loses control over himself i.e. laugh or burst into tears very rapidly, memory becomes impaired, sense organs do not work properly or in severe cases, brain damage may occur after prolonged deprivation.

Warmth and Cold

- Every one, whether animals or humans is affected by weather; either it is cold or hot.
- Avoiding the extremes becomes the basic motive for all living beings.
- Our skin has separate receptor cells for warmth and cold.
- Body temperature is regulated by complex mechanism as it largely depends on two factors;
 - i. The temperature in the external environment
 - ii. The internal body temperature
- Brain's organ, hypothalamus, as with other basic drives, regulates the temperature of the body with respect to the external environment.
- Below 57 degree of normal temperature, body becomes stimulated; adrenaline and thyroxin are secreted, blood pressure rises, muscular activity increased, blood is transferred to the internal organs and tissues of the body rather than on the surface of the skin.
- On the contrary, in hot weather, the reaction of the body is entirely opposite; i.e. body become slow in its activity, perspiration appears, blood vessels dilates so that more blood become available on the surface of the skin for cooling, blood circulation is increased.

Servo Control: For warmth and cold

- To keep the body at constant temperature, many other adjustments also take place such as use of blankets, heaters, hot drinks etc in cold season and air- conditioners, fans, cold drinks etc in summer season____ such use is called servo control; also partly controlled by hypothalamus and exhibited by the animals also.

Lesson 26

SECONDARY/ LEARNT/ PSYCHOLOGICAL MOTIVES

- Besides the basic biological needs, the expression of psychological needs is also of great significance: through society and culture in which one lives.
- Non-satisfaction of these motives may lead to mental illness.

Main psychological needs are:

- i. **Achievement**
- ii. **Curiosity**
- iii. **Need for appraisal**
- iv. **Need for affiliation**
- v. **Need for power**
- vi. **Work as motive**

1. Achievement

- Self- actualization or attaining excellence in relevant domain is the characteristic feature of this motive.
- The need to achieve something, some object of desire, a goal, or position/status.
- The source of satisfaction is not just the achievement of the goal, but the very act of striving for it too.
- The level of the need for achievement varies from person to person.
- Some are high and some low achievers.
- Competition is an important element of this need.
- Achievement motivation is a significant variable in a competitive society.
- People with high motivation: Take and overcome challenges in order to succeed rather than finding an easy ways of achieving success.
- People with low motivation: Tends to avoid failure, finding easy way outs, not desire to take difficult tasks.
- Methods of measuring achievement motivation:
- **Thematic Apperception Test (TAT)** is used; series of ambiguous pictures are presented to the person and ask him to write a story on it.
- Instructions are given as that the story must have a beginning, middle and an end along with the title; who are the people, what they are thinking, feeling, wanting etc; what is going on and what will happen____ all depicts the needs, desires and motivation to succeed/ achieve.
- In short the subject describes the past, present and future along with the description of characters and their thinking and motivation.
- Factors Contributing to the Need for Achievement
 - Parents who are warm for their child as well as make high standards for their child; encouragement is given on becoming independent.
 - Siblings who are high achievers in their own domain.

2. Curiosity

- Think why a little child always wanting to break toys and things?
- Why children always asks questions of things they saw on TV, read it or listen from any one
- It is all their curiosity and need to explore in order to find answers of these puzzles.
- It is a significantly inborn but learned also: found in both humans and animals.
- Parents encourage their children's curiosity by satisfying their inquisitiveness.
- School also plays an important role; the teaching methodology adopted may encourage or discourage curiosity.

3. Symbolic Reward/ Appraisal

- Appraisal is a powerful motive for everyone; especially for children and animals
- Praising words, petting after doing well etc all serve as symbolic reward for the learner.
- The presence and the attitude of the more liked serves as a social reward for the learner e.g. child with his mother, dog with its caretaker etc.

- Parent's approval and disapproval, liking and disliking towards the child all are included in symbolic rewards.

4. Need for affiliation

- Urge/ desire to main a relationship with other people; making friends, social contact with other people.
- Less desire to be isolated or alone.
- Studies showed that females spend a larger span of time among friends and peers as compared to males.
- Although the need for affiliation is a universal phenomenon, cultural differences do exist in its expression; some cultures have more group cohesiveness than others.

5. Need for Power

- Desire to influence, hold or ruling over others in order to be recognized as powerful individual.
- These types of people prefer to work in big organizations, businesses and other influential professions.
- There also exists gender differences among males and females; men are more apt to take challenges and respond quite aggressively irrespective of women who are socially restrained and traditional in her behavior.

6. Work

- Most of the people spent large span of time in their life at work; for this reason, psychologists take it as another powerful motive
- Work serves as a powerful motive because it satisfies other motives also such as biological motives of hunger, shelter etc, sense of achievement, affiliation and decision-making.

Theories/Explanations of Motivation

- i. Instinct Approaches
- ii. Drive-reduction Approaches
- iii. Arousal Approaches
- iv. Incentive Approaches
- v. Cognitive Approaches
- vi. Maslow's Hierarchy of Needs

Different approaches that focus upon the biological, social, cognitive, and psychological factors.

i. Instinct Theory

- There is a biologically determined behavior pattern that we are born with, known as instincts; biological variables drive our behavior.
- All humans and animals exhibit certain innate tendencies, which are fixed, predetermined, and unlearned.
- Different behaviors are respective responses to specific instincts.
- Instincts are essential for the survival of the species.
- Instincts provide energy for action thus directing organism's behavior.

Shortcomings of Instinct Approaches

- Which are the primary and power instincts...18 as William Mc Dougall said or 5759 as Bernard proposed.
- If instincts are the main driving force, then how come different people behave differently under the influence of the same instinct? e.g. hunger or heat.

ii. Drive- Reduction Theory

- Our biological/ physiological needs create an aroused or tension producing state that motivates us to fulfill them.
- When the basic biological requirements are lacking, and the need is unfulfilled, then drive/ arousal is produced in the organism energizing it to obtain the requirement in order to satisfy the need.

Drive: An arousal or motivational tension that provides energy for action or behavior.

Homeostasis: A stable, well-maintained state of internal biological balance is required for the proper functioning of the body; homeostasis is the process whereby this balance is maintained.

Primary and Secondary Drives

Primary Drives: Entirely biological in nature i.e. hunger, thirst, sleep, sex, air etc
Primary drives are satisfied by reducing the underlying need e.g. hunger followed by food, thirst followed by water.

Secondary Drives: These are psychological as well as social in nature. Prior learning and experiences are what brings about such needs e.g. academic achievement.

Criticism

- What about behaviors directed towards maintenance, and even heightened, arousal?
- What about curiosity or risk-taking behavior?
- Is our behavior guided by drive-reduction alone, or by our goals too?

iii. Arousal Theory

- A certain level of arousal and excitement is needed by our system.
- We try to maintain that level of stimulation and activity; the maintenance may require off and on reduction or increase in the existing level, depending upon the circumstances.
- When our arousal state becomes too high, it needs to come down for optimal functioning and vice versa.
- Too high a motivational arousal may affect performance negatively; it may produce anxiety and irritability in the organism.
- Similarly too low an arousal may also have adverse effect e.g. performance of a person suffering from depression.
- A consistent, well balanced, and leveled arousal is needed for the optimal functioning e.g. in case of exams, athletics, interviews.

iv. Incentive Theory

- Motivational state of the organism is understood and explained in terms of positive or negative environmental stimuli.
- As opposed to the drive- reduction and arousal theory, this theory explains motivation in terms of external events that stimulates/ energizes the organism's behavior, rather than innate instincts or drives.
- Characteristics of the external environment are important variables in a person's motivation.
- Incentives are rewards that energize and drive our behavior.
- Any thing that provides us with a reward triggers motivation for action.
- Incentives may generate behavior even in the absence of an active, un satisfied, instinct or drive e.g. how can we eat a bowl full of ice cream even.
- When our stomach is full after a heavy meal?
- But in many cases drives and incentives go together and have a deeper effect.

Criticism

- What about the motivational process when an individual tries to fulfill needs although there is no apparent incentive present? E.g. working even when paid very little, and the job too is not matching with one's aptitude.
- What about behaviors when there is no apparent incentive available, in fact threat is involved? e.g. helping the people stranded in a house on fire.

v. Cognitive Theory of Motivation

Theories that give importance to the cognitive processes of the individual in explaining motivational process; thoughts, feelings, expectations, understanding and evaluating all are important when explaining the motivation of the person.

Expectancy- Value Theory

Two types of cognitive processes underlie human behavior.

I. Expectation: The expectation, hope, or anticipation that our behavior will help us attain a certain goal.

II. Value: The perception, appraisal, or understanding of the value of the goal to us.

- The level of expectation and the value attached to it, together, determine the level of motivation.
- In case of a high expectation along with high value, the motivation will also be higher.
- But if any of the two is weak or low, the corresponding motivation will also be low
- Can you see the two operating in your exam and study behavior????

Intrinsic and Extrinsic Motivation

Cognitive theories differentiate between intrinsic and extrinsic motivation.

Intrinsic motivation: Motivation from within, or Internal motivation that energizes the person to satisfy or accomplish the goal; the goal is to attain enjoyment and personal satisfaction, in which no external tangible reward is involved e.g. altruistic behavior.

Extrinsic motivation: Revolves around the tangible rewards such as money, social contacts.

- Studies shows that intrinsic motivation produces better results in terms of performance than the extrinsic motivation.
- When extrinsic motivation is more for the desired behavior than the intrinsic one, and it remains like that for long, then the intrinsic motivation eventually declines.
- e.g. a child has been given reward for doing neat and tidy homework, as he is less intrinsically motivated; eventually the entire focus of the child in getting reward after doing tidy homework rather than doing it otherwise.
- Teachers and parents should use extrinsic motivation, but with great care.
- Extrinsic motivation can be used to enhance the motivational level but not at the cost of intrinsic motivation.

vi. Abraham Harold Maslow: 1908-1970

- American psychologist and leading exponent of humanistic approach
- Gave comprehensive theory of motivation.
- Found the prevalent psychology to be too pessimistic and negatively oriented.

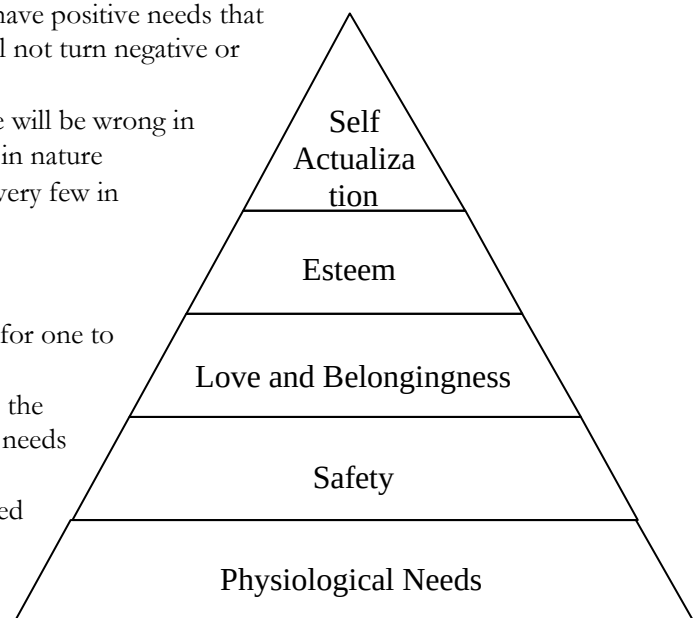
Key Points of Maslow's Theory

- Psychology and the psychologist should look at the positive side of the human beings.
- There must be more to living than just being battered by a hostile environment, or by depraved instincts---- which may actually be leading to self-destruction.
- People's needs are not low level and base. We have positive needs that may become neutral in the worst cases, but will not turn negative or base.

- Human behavior does respond to needs but we will be wrong in saying that all our needs are only physiological in nature
- Needs motivate human action; such needs are very few in number.

Maslow's Hierarchy of Needs

- Basically a stage theory.
- The needs at one level have to be met in order for one to move on to higher order.
- The needs at the lowest/primary/base level are the physiological needs, whereas the highest order needs are the self-actualization needs.
- **Self-Actualization:** Most advanced human need based on the desire to grow and utilize one's potential up to the optimal level.



Categories of Needs

Metaneeds: based on a desire to grow rather than for meeting a deficiency: expressed in the need for self actualization

Deficiency needs: The absence of the underlying requirements triggers these needs e.g. physiological needs, love needs, or esteem needs

Interactions and needs of Behavior

Physiological needs: Fulfilled through: hunger/food; Pathology associated=over eating, anorexia

Safety needs: Fulfilled through: profession, job; Pathology associated= Phobias

Love and belongingness Fulfilled through: Marriage, friendship; Pathology associated= Antisocial personality

Esteem needs: Fulfilled through: Awards, honors, scholarships; Pathology associated =Depression

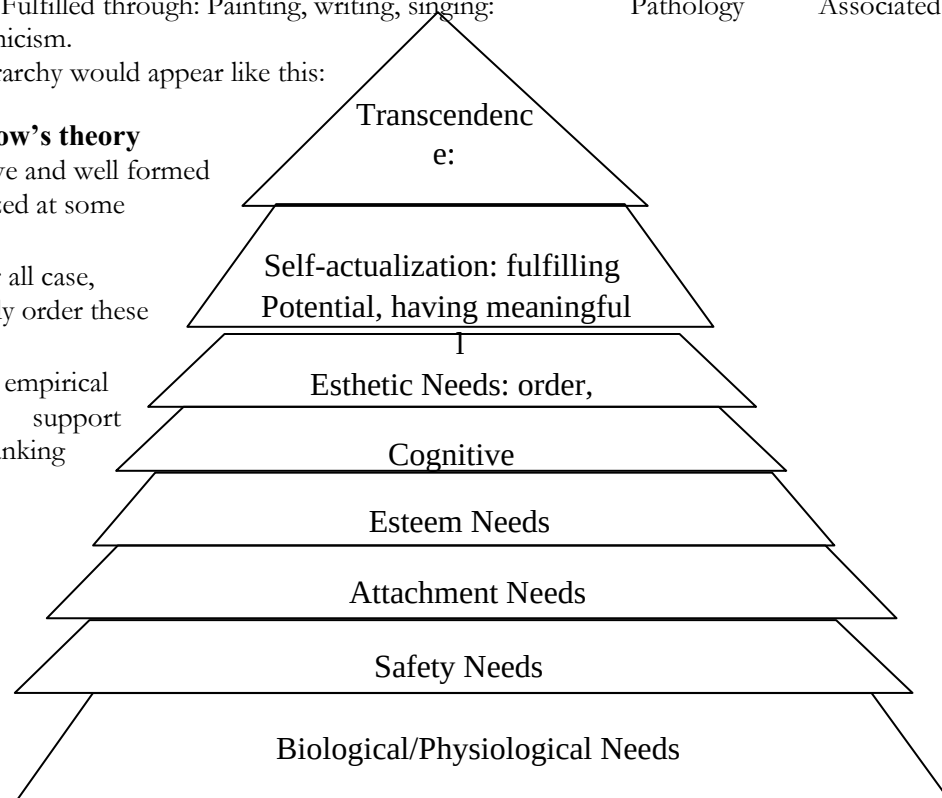
Self-actualization needs: Fulfilled through: Painting, writing, singing: Pathology Associated
=Isolation, alienation, cynicism.

If expanded, Maslow's hierarchy would appear like this:

Criticism against Maslow's theory

Although a comprehensive and well formed theory, it has been criticized at some points:

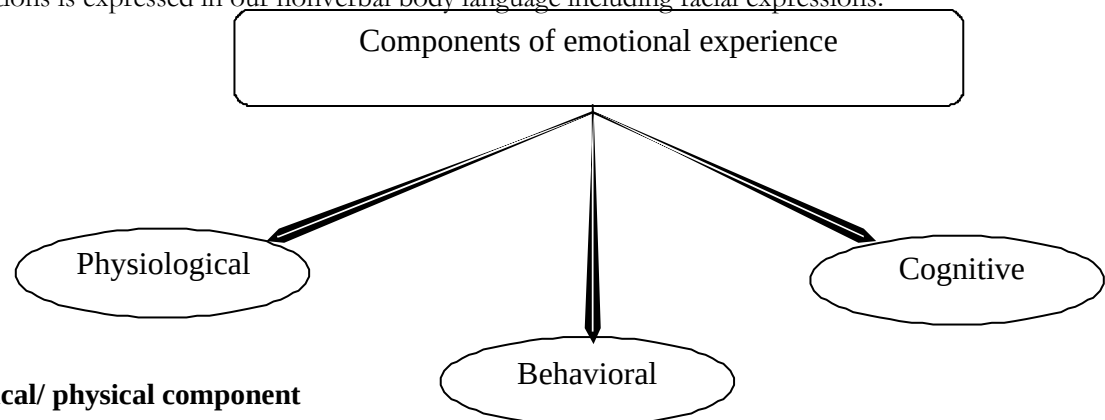
- Can we actually, for all case, distribute and neatly order these needs?
- There is little empirical evidence to support Maslow's way of ranking needs.



EMOTIONS I

Defining Emotions

- Derived from the Latin word “**Emovere**” emotion means “**to excite, stir up or agitate**”.
- A response that includes feelings such as happiness, fear, sadness, grief, sorrow etc: it is the amalgamation of:
 - Physiological arousal,
 - Expression of and
 - The conscious or the cognitive experience of the situation that influence behavior.
- In psychology, emotion is considered a response to stimuli that involves characteristic physiological changes—such as increase in pulse rate, rise in body temperature, greater or less activity of certain glands, change in rate of breathing—and tends in itself to motivate the individual toward further activity.
- As human beings, we have all experienced a range of emotions from happiness, sadness, love, hate, to indifference along with many other emotions.
- Emotions are feelings, highly subjective personal tendencies to respond to internal and external variables. Emotions have ~~cognitive, physiological and behavioral components~~. Our cognitive appraisals and evaluations of events in our lives are key determinants to our emotional responses. Emotions are accompanied by physiological arousal of the automatic nervous system that leads to physical symptoms such as increase in respiration and heart rate. The behavioral component of emotions is expressed in our ~~nonverbal body language including facial expressions~~.



Physiological/ physical component

The physical component of emotion is a **psychological arousal** that usually accompanies the emotion the body is feeling. If the body did not experience this arousal, the intensity of this emotion would be greatly decreased. During the arousal, the body experiences a surge of powerful feelings known as emotions. People who can detect changes in their arousal level experience their emotions much more intensely than those who cannot detect the changes in their arousal level.

Behavioral component

This component has been called the outward expression of our emotions. Body gestures, posture, facial expressions, and our tone of voice display what emotions we are feeling. Many of our facial expressions are universal. For instance, if somebody has a mad look on their face, it doesn't matter what language they speak or where they are from, chances are... [they're mad](#). However, some emotional expressions are influenced by our cultures and society's rules for displaying emotions. For example, the guards outside of Buckingham Palace are not allowed to display any emotion on their face. Some people have described them as looking mad when in reality they are not.

Cognitive component

The cognitive component is how we interpret certain situations or stimulations. This determines which emotion our body will feel. For example; if you are alone, sitting in the dark, watching a scary movie, and you hear a loud noise, you may become scared... fearing that there is an immediate threat or that you are in danger. This emotional response to this imaginary threat is just as powerful as it would be to a real threat. Our perception to the imaginary threat is what makes it feel real to us and causes the emotion in our body.

Mc Dougall divided “Emotions” into three types:

1. **Primary:** Simple emotions as fear, happiness, anger, disgust etc.
 2. **Secondary** Mixture of various instincts as curiosity, escape etc.
 3. **Derived** Learn through experiences such as sadness, boredom etc.
- Have you ever thought why your heart pounds and you burst into tears on hearing dreadful news?
 - Why people become afraid on seeing the snake?
 - Why heart beats at a very high speed while you do jogging/ exercise?
 - Why people face reddened on hearing the good news?

YOU THINK IT IS A SIMPLE PHENOMENON AND YOU KNOW THE ANSWERS BUT IT IS NOT THAT SIMPLE!!!!

- Our heart may pound even at the sight of someone we hate. Also, before an interview or presentation. Or when we are scared.
 - AND
 - Our face may blush and turn red when we are self conscious, or embarrassed, or angry.
- So!!!!
- Same bodily changes may be manifest in different emotional experiences.

What makes the experience different is the accompanying expression and cognition.

Assessing Emotions: Expressions in Emotions

- Vocal, verbal and facial expressions play a vital role in determining emotions as well as our interaction with the physical world. In interpreting emotion, the important things are;
- **Acquaintance** with the other person.
- **Type of association** with that person.
- **Deriving clues from own emotional experiences** in order to understand other's experiences as well.
- Facial expressions clearly reflect the intensity of emotional experiences such as happiness, sadness, anger, and sorrow: all can be understood from emotional expressions. However these vary from person to person.

An adult's voice pattern clearly reflects their emotions. Variations in pitch or loudness express different types of emotions.

Emotions and Cognition: Understanding Emotions

- Some psychologists believed that memory and cognition are entirely separate systems that functions independently; some argue that we emotionally respond to the situation and then understand it cognitively.
- On the other hand, other believed that we first cognitively evaluate the situation and then exhibit the emotional experience we are facing at that time.
- Both point of view are somehow supported by the studies and the debate is still unresolved___ this may be due the fact that emotional experiences varies from situation to situation and so the cognitive appraisal and the emotional experiences.

Role of Emotions in Every Day Life

- Emotions make our life bright and enlightened, because without the experience of emotions, our life would be dull, uninteresting, gloomy and without any purpose

- Psychologists identified number of functions of emotions that have a vital role in our daily life. They are:
- **Stirred up for the fight or flight action** After seeing a snake or after an unusual incident as natural disaster; the body is prepared to stir up our bodies to face and deal with them.
- **Modifying the future responses and behavior** Learning takes place after the emotional state that prepares us to manifest appropriate behaviors in the future, i.e., strategies should be adopted to minimize the aftermath of disasters and avoid us to face the snake.
- **Social interactions are enhanced** As emotions are both verbal and non-verbal so they help people to better understand the responses whether they are being expressed or not.

Common Emotions: Range of Emotions

There are number of basic emotions that have been identified by people in instances. They are:

- Happy
- Anger
- Fear
- Disgust
- Surprise
- Sadness.

J.B. Watson talked about three main emotions:

- Anger
- Love
- Fear

He proved in his experiments that infants are capable of these three primary emotions.

Psychologists have also identified other emotions or related expressions such as **contempt, shame, and startle**.

Robert Plutchik (1984)

Robert Plutchik (1984) became able to determine the basic emotions and their relationship through his studies; identified eight fundamental emotions. They are; **joy, anger, sadness, surprise, fear, acceptance, disgust and anticipation**. He named it as Plutchik's emotional wheel.

- The emotions nearer to one another are closely related, while those that are opposite to each other are conceptually opposites.
- The existence of only these emotions is the major shortcoming of this model; believed that emotions can better be understood by breaking them into components/ elements.

Physiological component of emotions

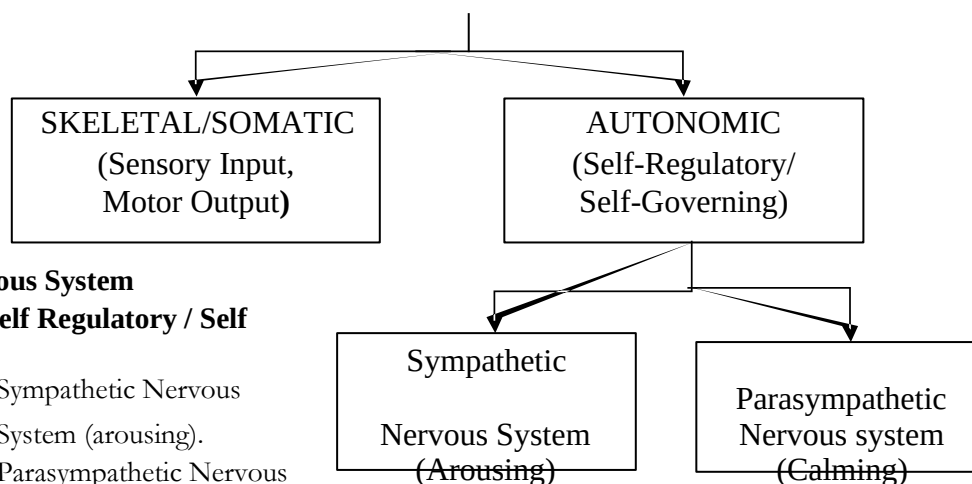
Many physiological changes take place in an emotional state, in the form of changes in:

- Heart rate
- Pulse rate
- Respiration
- Blood pressure
- Digestion and appetite
- Muscular activity
- Body temperature
- Perspiration
- Endocrine and neurotransmitter secretions
- Blood sugar
- Salivary gland activity
- Pupil dilation.

Role of the Nervous System in Emotions

- All the emotional experiences and their expressions are controlled and regulated by the autonomic or visceral nervous system.
- A.N.S. is an integral part of peripheral nervous system.

PERIPHERAL NERVOUS SYSTEM



A. Peripheral Nervous System

i. Autonomic (Self Regulatory / Self Governing)

- Sympathetic Nervous System (arousing).
- Parasympathetic Nervous system (calming).

ii. Skeletal/Somatic (Sensory Input, Motor Output)

B. Autonomic Nervous System (A.N.S)

- Considered as the “self governing or self-regulatory mechanism” because of our involuntary control over it.
- Control the glands and muscles of our internal organs such as heart (heart beat), stomach (digestion) and glandular activity.
- A.N.S. has a dual function; i.e. both arousing and calming.

Sympathetic Nervous System (S.N.S)

- This part of A.N.S. arouses us for defensive action.
- If something alarms or enrages someone, the sympathetic nervous system will accelerate heart beat, slow digestion, raises the sugar level in blood, dilates the arteries and cool the perspiration, making one alert and ready for action.

Parasympathetic Nervous System

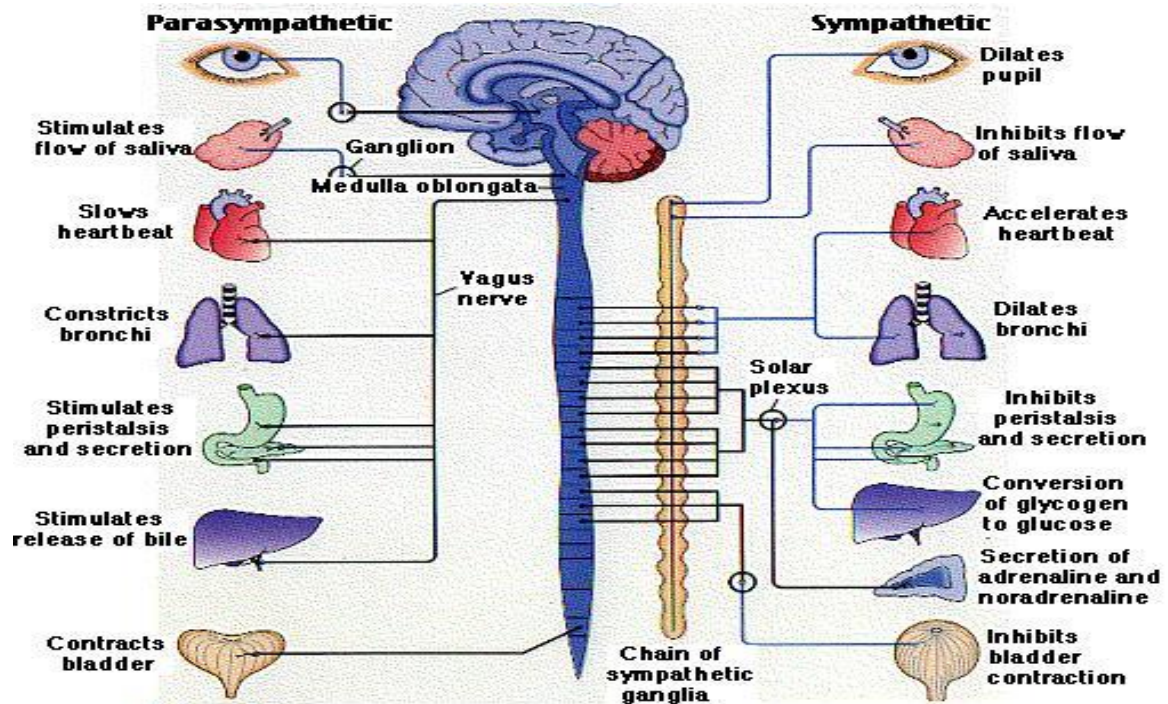
When the stressful situation subsides, parasympathetic nervous system started its activity.

- It produces opposite effect to that of sympathetic nervous system.
- It conserves energy by decreasing heartbeat.
- Lowering blood pressure.
- Lowering blood sugar and so on.
- In daily life situation, both sympathetic and parasympathetic systems work together to keep us in steady internal state (homeostasis).

Areas for Emotions in the Brain

- A.N.S. is the only center of emotions in P.N.S.
- Cortical and sub- cortical structures are involved in emotions.
- Cerebral cortex is involved in relating the present and past emotional experiences; others involved in speed up the heart beat and, in learning emotional experiences
- Whereas sub- cortical regions are involved in organizing the emotional expressions.

- Besides that, the right hemisphere is the most active in recognizing and expressing the emotions we are feeling. It also responds to emotions being conveyed by another person's body language or tone of voice. For example, an employer sarcastically says to an employee who comes to work late, "Glad to see you could make it today". If the employee had damage to his right hemisphere, he may only understand the words and not the sarcastic undertones, whereas a person, whose right hemisphere is functioning normally, would usually have a sarcastic response.
- The right hemisphere helps in our expression through our tone of voice and by controlling our facial expression. Since the right hemisphere controls the left side of the face, the left side usually portrays stronger emotion than the right side of the face. Research continues to accumulate information showing the mechanisms in the brain responsible for negative emotions reside in the right hemisphere, while the left hemisphere is believed to control positive emotions. Research has shown that patients who suffer from manic depression or major depression have decreased activity in the left prefrontal cortex where the positive emotions are produced.
- Much of the frontal lobes consists of areas that are involved with motivation, thinking, positive emotion, impulse control, and other emotional responses. Any damage that occurs to the frontal areas usually produces deficiencies in the ability to anticipate the results of our actions.



Lesson 28

EMOTIONS II

Physiological Changes during Emotions

- Psychologists believed that emotions can be measured **quantitatively** by observing different physiological responses of the individual = indicators of emotions.
- They also believed that they **themselves are different emotional states**.
- The major physiological changes that occur during emotions are:

1. Respiratory Changes

- The most apparent and obvious change during emotions.
- During emotions, respiration increases; occurs also when the person is happy or excited.
- But the production and secretion of saliva decreases as the process of respiration increases.

2. Pupillometrics

- Darwin was the first person who identifies this phenomenon.
- Pupil of the eye is very responsive during emotions.
- Dilates at favorable stimulus and contracts at unfavorable stimulus.
- Our pupil is also very responsive to favorable and unfavorable sounds and taste.

3.Changes in Blood Pressure and heart rate

- **Variations in blood pressure occur during emotions; usually increases during an emotional state.**
- **Heartbeat also increases during emotions.**
- **Increased blood pressure and heightened heart rate for prolonged periods may lead to coronary heart disease.**

4.Glandular Responses

- **During strong emotional states such as anger or fear, excessive amounts of hormones adrenaline and nor adrenaline are secreted into the blood stream.**
- **Due to this secretion; Liver secretes excessive amounts of glucose directly into the blood stream that causes the blood to clot rapidly in case of injury or damage.**
- Blood pressure and sugar level rises, pulse become fast, air passage of the lungs enlarges and causes more air into the lungs, pupil enlarge, sweat appears all over the body particularly on hands, and temperature of the skin rises.
- Nor adrenaline helps to constrict the blood vessels, thus making it available to other parts of the body in case of injury.
- Pituitary and thyroid glands are also responsive to emotional states.
- All the glandular responses help to cope physically with the emotional as well as emergency situations.

Gastrointestinal Function

- **Stomach and intestines are also very responsive to emotional states.**
- **They either start working at a very high rate or stop entirely.**
- **During strong emotional arousal, its working speed decreases and flow of blood is more towards the brain and the skeletal muscles rather than these organs.**

6.Neural Reactions

- **Besides affecting visceral organs of the body, emotions also bring changes in the neural/ nerve activity.**
- **Autonomic division of the P.N.S. is more effective in this regard in which sympathetic and parasympathetic nervous systems work successively.**

7. Galvanic Skin Response

- When perspiration appears during emotions, two important changes occur in skin's electrical stimulation:

O Rapid generation of electromotive energy

O The electrical resistance of skin changes

- These changes can be measured through a measurement of “Galvanic Skin Response (GSR)”; formally called “Psycho Galvanic Response (PGR).

• Galvanic skin response, in combination with the respiration and blood pressure, is used for lie detection with people who are supposed to be guilty of some crime.

8. Emotional Intensity and State of Arousal

- Most of the times we are aware of our emotional states such as angry, excited or afraid, in all these states, the physiological conditions are the same e.g., heart beat increases, face blushes or becomes pale.
- That is why we are unable, at times, to differentiate between different emotions and the associated arousal.

Other Common Bodily Changes during Emotions

- Dryness of throat and mouth,
- Muscle tension,
- Weakness or fainting,
- Trembling, and
- Sinking feeling in heart or stomach

Theories of Emotions

- Psychologists have attempted to define and explain the emotional arousal and physiological conditions that accompany them.
- A number of theories have been developed in this regard, but the most famous ones are;
- *James- Lange theory of emotions*
- *Cannon- Bard theory of emotions*
- *Schachter- Singer theory of emotions; (Cognitive Labeling Theory/ Two Factor Theory)*

Other important theories include:

- **Cognitive Appraisal Theory: Richard Lazarus**
- **Cognitive Theory: Magda Arnold**
- **Opponent- Process Theory: Solomon and Corbit**
- **Activation Theory: Lindsley**
- **Theory of Emotions: Albert Ellis**

1. James- Lange Theory of Emotions

- Earliest theory of emotion; now considered as the classical approach to emotions.
- Given by American psychologist, William James and Danish psychologist, Carl Lange in the 19th century.
- Emotional experience occurs in reaction to instinctive bodily events that take place as a result of an external situation: “...we feel sorry because we cry, angry because we strike, afraid because we tremble” (James, 1890).
- Physiological changes create specific sensations, and our brain interprets these sensations as different emotions.

- Theory asserts that awareness of the physiological responses determines our emotional arousal.

Sequence of Events in James-Lange Model

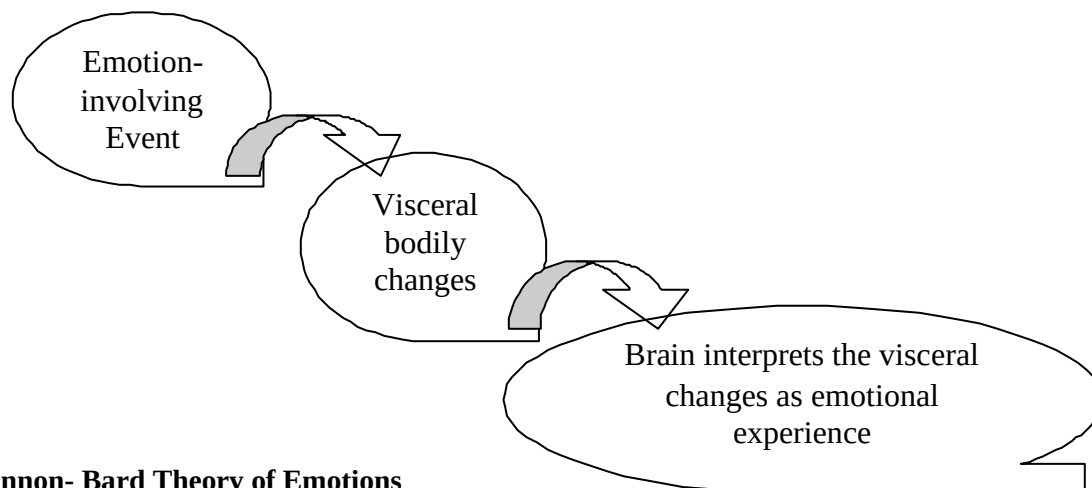
Experience of an emotion-involving situation i.e., find oneself in a waiting room before first job interview.

Physiological responses take place; visceral bodily changes are activated e.g. pounding heart, sweaty palms, respiration increases and other physiological states.

Brain interprets these physiological changes as emotional experience. In the case of the interview situation, fear.

Criticism against James-Lange model

- On most emotional occasions we experience the emotion quite immediately after a triggering stimulus e.g. hearing loud bang and being startled. Are our visceral responses that fast??
 - Many a time we don't even have enough time for this intermediate part. We feel the emotion immediately after the stimulus is registered e.g. feeling disgust at the very sight of someone we hate.
 - The most basic criticism of this theory is emotional experience (fear) results from the physiological responses after evaluating the situation.
 - On many occasions we experience physiological changes but no emotion e.g. patients of hypertension experience raised blood pressure but do not experience associated emotion. It is the case with joggers whose heart does pound but there is no emotional experience.
- How do same physiological changes create different emotional experiences???

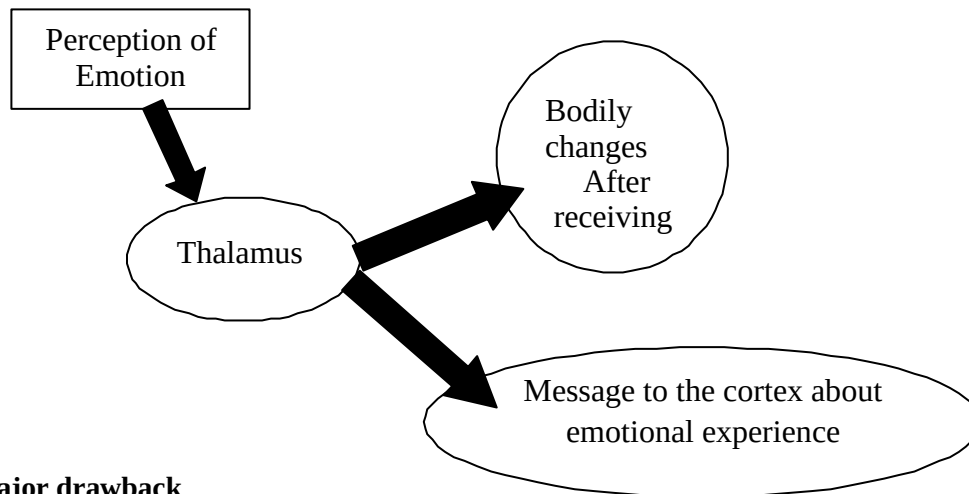


2. Cannon- Bard Theory of Emotions

- Given by Walter Canon and Philip Bard in 1920's
- The theory assumes that emotional states and the physiological reactions work independently, but are triggered by the same nerve impulse simultaneously.

SEQUENCE OF EVENTS IN CANNON-BARD THEORY

1. Perception of the emotion-inducing stimulus
2. Thalamus is activated.
3. Thalamus sends messages to two sites i.e.,
 - a) The Autonomic nervous system, thereby producing a visceral response.
 - b) The cerebral cortex receives a message regarding the nature of emotion being experienced.

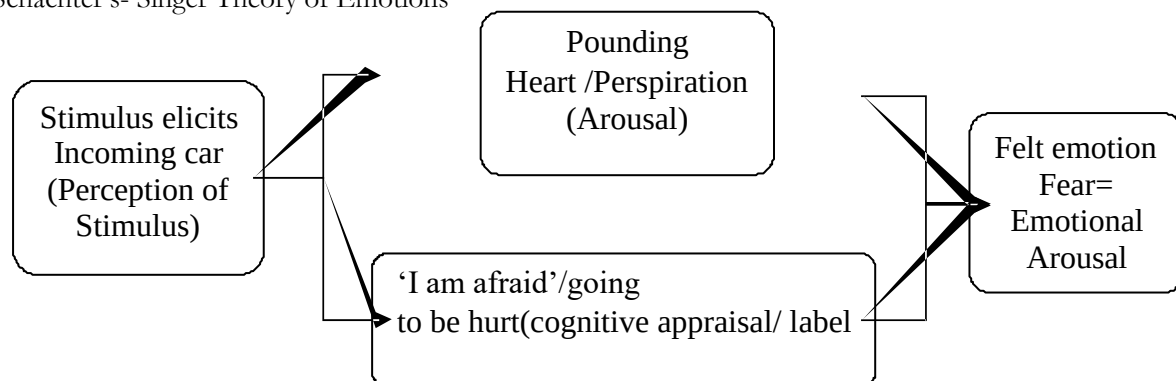
SEQUENCE OF EVENTS IN CANNON-BARD THEORY**Major drawback**

- Research has shown that it is the hypothalamus and the limbic system that are responsible for emotional experience, and not the thalamus!!!!

3.Cognitive Labeling Theory/ Two Factor Theory of Emotions

- **Pioneer: Stanley Schachter and Jerome Singer (1962).**
- **According to them, emotions result from the physiological arousal as well as the cognitive appraisal (evaluation) of the situation.**
- **Arousal comes first and is general in nature.**
- Two forces jointly determine emotional experience:
 - Nonspecific kind of physiological arousal, and
 - The interpretation of the experience based on environmental cues.
- Both of these factors jointly determine the title/label of the emotion, and the meaning of one's reaction.
- We observe the environment and compare ourselves with others, and this help us identify our emotion.

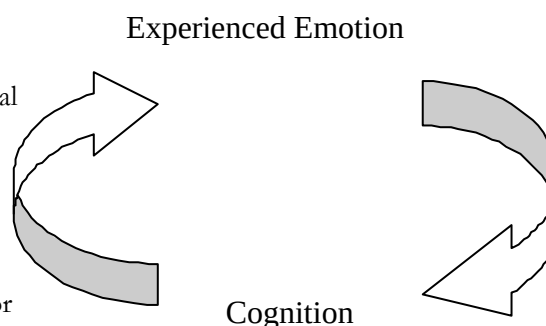
Schachter's- Singer Theory of Emotions



- Labeling of the emotional experience is entirely dependent on the experiencing individual__ one's own evaluation.
- Believed that physiological arousal determines the intensity of emotions.
- Theory includes the physiology cognitive processes as well as social psychology to investigate the nature of emotions. **Cognitive Appraisal Theory: Richard Lazarus**
 - **Given by Richard Lazarus and his colleagues in 1968**
 - **The theory maintains that emotional experience cannot solely be understood of its own, but understanding the environment is to be evaluated.**
 - **Appraisal involves cognition, bodily responses and memory.**

Richard Lazarus Gave two basic types of appraisal

- i. **Primary appraisal:** Evaluate whether the situation is threatening or not
- ii. **Secondary appraisal:** Involve alternatives in order to deal with the perceived threat
- iii. **Reappraisal:** Includes re evaluating the situation and alternative in order to see whether the judgment is true or not.



5. Cognitive Theory: Magda Arnold

- Given in 1966; one of the first ones who use the concept of cognition in emotional state
- His model is known as **“Sequential model”**.
- The steps involved in emotions are;
 1. **Perception;** how the situation is being perceived.
 2. **Appraisal;** Stimulus is beneficial or harmful to ourselves.
 3. **Determining emotions** with regard to the prevailing situation.
 4. **Expressing emotions;** Also accompanying physiological responses.
 5. Finally, they all work to give idea whether to approach a situation or withdraw from it.

6. Opponent- Process Theory: Solomon and Corbit

- Given by Richard Solomon and John Corbit in 1974.
- Theory maintains that every emotional arousal has an opposite, i.e. when one type of emotion is elicited, and then there must be an opposite that is there to suppress or cancel it.
- In this way the emotional arousal remain at some base- line.

Base line ➡ State A ➡ State B ➡ Baseline

7. Activation Theory: Lindsley (1951, 1957)

- Based his theory by observing that reticular system activates the cortex region in the brain that ultimately aroused/ excites the organism.
- Believed that emotional arousal activate the reticular part of the brain present in the brain stem, that ultimately sends impulses to the thalamus and cortex and bring about emotional arousal.
- And if the reticular part is at rest, then the emotional state is calm and relaxing.

8. Theory of Emotions: Albert Ellis

- Given by American psychologist, Albert Ellis.
- According to him, emotions do not result from a single cause, but originate from three different ways.

OFirst, sensory- motor; involves brain and muscles.

OSecond, bio chemical stimulation; function to mediate processes of ANS, hypothalamus and sub- cortical regions.

OThird, cognitive and thinking processes.

To him, emotions do not exist independently but involves several stages in order to take place.

COGNITION AND THINKING

Cognition is the

- **The process of knowing, as well as what is known.**
- **Cognition refers to the higher mental processes.**
- **It is through these mental processes that humans understand the world, process information, make judgments and decisions, and communicate knowledge to others.**
- **Memory, intelligence, and language are important aspects of cognition.**

The term **cognition** is used in several different loosely related ways. In psychology it is used to refer to the **mental processes** of an individual, with particular relation to a view that argues that the mind has internal mental states (such as beliefs, desires and intentions) and can be understood in terms of **information processing**, especially when a lot of abstraction or concretization is involved, or processes such as involving knowledge, expertise or learning for example are at work. It is also used in a wider sense to mean the act of knowing or knowledge, and may be interpreted in a social or cultural sense to describe the emergent development of knowledge and concepts within a group that culminate in both thought and action.

Cognitive Psychology

The branch of psychology that studies cognition, and related areas and issues.

Cognition is commonly known as Thinking

What Is Thinking???

- **The information that our mental faculties receive or generate is in the form of mental representations.**
- **These mental representations may be in various forms e.g. in terms of words, visual images, or may be sounds.**
- Woodworth said that thinking is mental exploration and according to Ruch, thinking is a behavior in which at least some of the subjects that are dealt with are not physically present to sense but are represented by symbols, or in other words, thinking is actually a series of symbolic activities which represent previous learning experiences and an individual's thought processes.
- Most of the time, human beings are thinking. Even when they stop thinking or reading or writing, ones thought wanders on something else and this can be past, present or future or pleasant or un pleasant, or it may be even day- dreaming; they all are the part of thinking phenomena.

Thinking

Thinking is the process whereby these mental representations are manipulated. The process of thinking transforms these representations into a new and different form. The transformation may be made:

- **For finding answers to questions**
- **For finding solutions to problems**
- **For finding facts and exploring reality**

Try to see what different types of mental images we experience!!!!

Close your eyes for a moment and try to:

- **See the face of your mother?**
- **Hear the voice of your favorite singer?**
- **Feel how your favorite food tastes?**
- **Feel how your favorite perfume smells?**
- **Imagine how the prick of an injection feels?**

Probably You Will Be Able To Do All of These Things

The stimulus is not there but we can feel it... mentally

How do all these mental processes take place???

- **Thinking is a subtle, and continuous process.**
- **Our brain is the most intricate, complex, sophisticated, and yet ‘quite’ machine in the universe.**

Fundamental Elements of Thinking

- **Mental Images**
- **Concepts**

1. Mental Images

- **Mental images are an integral part of the thinking process; in fact a major part of our thinking consists of these images.**
- **These are mental representations of the objects and events that we are, or we have been, in contact with.**
- **These images are not necessarily visual in nature; images can be related to all sorts of sensory experiences.**
- **Psychologists have developed exercises for enhancing people’s ability to work on their mental images, in order to sharpen their thinking capacity and thinking skills e.g. problem solving skills, brain teasing, creative thinking exercises.**
- **There is no dearth of research available suggesting that mental images, when used as mental rehearsals can be very helpful in improving other skills, besides thinking, as well e.g. jogging, athletics, dancing, and public speaking.**
- **Images are also used as important tools in** Interventions designed for handling psychological problems e.g. Relaxation exercises, or meditation.

2. Concepts

- **Objects, events, or people sharing common characteristics and properties are categorized and classified as one.**
- **This categorization is known as “concepts”.**
- **Different categories are different concepts e.g. the concept of different objects, people, or events.**

If someone asks you, “Who are your class fellows?” How will you respond?

You might say:

- **“One short girl, one medium height boy, a tall girl and a tall boy, one girl with long hair, and one boy who is bald”.**
- **OR**
- **“Three boys and three girls”**

This is a type of categorization based upon the concept of gender.

The same applies to other objects and event e.g. furniture, fruit, clothes etc.

Use of “Concepts” In Thinking World

- Concepts are the categorization of objects, events, or people that share common properties
- When someone asks you, what is the main thing required for your room? One may start talking in terms of items like, bed, chairs, curtain, carpets etc, or ‘furniture and fixtures’ as a single category.
- These small categories reflect the operation of concepts and the process of cognition
- Concepts make possible the establishment of cognitive categories, and enable us to organize complex phenomena into a simpler and conveniently manageable form.
Imagine how many things we are in contact with in the world around us. Do we, or can we name all of them all the time??????
- Newly encountered objects easily fit into our cognitive structure, if the conceptual category is already there. Therefore,
- If you know what a doctor does, then you can recognize any type of doctor, in any part of the world, in any type of attire, in any type of treatment setting.
- Imagine if we did not have concepts of fruit, vegetables, grains, furniture, or weather!!!!
- Concepts help to define, explain and elaborate complex phenomena into simple, understandable and usable categories, and they also include data from past experiences.
- There are three main types of concepts;
 - a) Artificial concepts
 - b) Natural concepts
 - c) Prototype concepts

a) Artificial Concepts

- Concepts that have a unique set of traits and features.
- These concepts are easy to define and elaborate. e.g. a rectangle has two opposite sides equal, if it is not the case, then it is not a rectangle.
- But our every day concepts are much more difficult than these concepts. e.g. what is the definition of an animal?

b) Natural Concepts

Known, familiar and relatively simple concepts that have rather loose features to define and explain them.

- These concepts are not universal in nature____ such as rectangle has two opposite sides equal, if it is not, and then it is not a rectangle. This is not the case in natural concepts.

People e.g. defining an animal can vary •They in their definition and explanation?**Simple and complex concepts**

- Some concepts are simple in the sense that they are clearly defined.
- When a concept is clearly defined, it is easy to distinguish an example from a non-example e.g the concept of a square, or an equilateral triangle.
- But some concepts are rather difficult to define. These are defined in a variety of ways, and marked by a set of complex features.
- These may be ambiguous, overlapping, and even abstract e.g. a bird, or a chair.

- Most of the things that we are in contact with in our everyday life are not as easy to define as a square or a triangle e.g. defining **‘obsessive- compulsive disorder’**.
 - These may also involve the subjective experience of the person.
- And what about the concept of a comfortable chair or an easy or a difficult task?????In such situations we need specific, and rather exact examples of a concept i.e., prototypes.

c) Prototypes

- **Examples of a concept that is typical and highly representative of a concept.**
- **Prototypes are used to define and explain objects and ideas that cannot be defined in a clear-cut and straightforward manner.**
- **E.g. the prototype of a table can be the ‘dining table’, or the prototype of a bird can be a ‘crow’.**

Agreement on prototypes

- **Usually people in a society, or those belonging to a particular discipline, are unanimous about the prototypes of a concept e.g. if we have to give a prototype of a vehicle, then we will talk about a ‘car’ and not an escalator or ‘elevator’, although these also move and take us from one place to another.**
- **On the other hand, if we are talking about varieties of ‘stairs’, then we can probably take escalators as an example.**
- What will happen if psychologists do not have a common definition of mental illness?
- How do concepts help in thinking?
- We live in a complex world of objects, ideas, and relationships. It is with the help of concepts that we understand this world.
- Concepts make it possible to communicate ideas, thoughts, and feelings even when the object of interest is not actually present.

Thinking and Reasoning

- **Only humans can think in ways, which no other organism is able to, as they have the ability to contemplate, analyze, recollect, and plan out, and carry out those behaviors.**
 - **Humans are the only ones who are capable of using *foresight* as well as *hindsight***
 - Even when we are not thinking, our mind wanders on present, past, future, pleasant or unpleasant, favorite or not so favorite things.
- And even we are dreaming or day- dreaming; actually it is also a form of thinking.

Reasoning

- **It is the ability to use reason, logic, past experience, and learnt information for mental processing.**
- **For decision-making, and problem solving etc.**

Deductive Reasoning

- **Deductive reasoning is the process whereby logical conclusions, inferences, and implications are drawn by using a set of assumptions. These inferences are then generalized over, or applied to, specific cases.**
- **The assumptions or premises that are used for drawing conclusions are thought to be true and base upon reality. In many cases they are considered to be unchallengeable.**

But at times these premises may turn out to be false when tested in reality e.g. all men are brave, or men do not cry.

- Evaluating syllogisms is a technique used for studying deductive thinking.
- A syllogism contains a series of two assumptions or premises that are used for drawing conclusions e.g.
- All women are talkative.
- Anna is a woman.
- Therefore, Anna is talkative.

- This type of deductive thinking is largely influenced by cultural backgrounds of people.
- It has been seen that people from more developed societies use rather.
- Abstract and logical thinking, which they acquired through their experience, and learning.
- People from less developed cultures mostly rely on **concrete modes of reasoning.**
- This sort of difference is largely due to the type of high quality, sophisticated education, variety of learning experiences, and the pressure to think logically and independent.

THINKING, REASONING, PROBLEM- SOLVING AND CREATIVITY

Inductive Reasoning

- *Specific cues are used for drawing inferences.*

Inductive Thinking

- *By using observation, knowledge and experience, different sets of inferences are drawn about the phenomenon of interest.*
- *Small bits of information are used to draw the general conclusion.*
- *The person uses his or her observation, knowledge, and experience of specific case and infers general rules.*
- *For example, if you have been noticing that your teacher is very soft spoken, usually marks you present even when you are late for the class, and forgives your mistakes, then you might take chance for late submission of your assignment.*
- *The major shortcoming of this type of thinking is that the conclusions may be biased, or the evidence used for drawing conclusions may be invalid, insufficient, or may be just a chance occurrence. Whereas appropriate conclusions have to be unbiased.*

Thinking and Decision- Making

- *It is one of the most complicated forms of thinking.*
- *Cognitive psychologists are still focusing on the components and processes that underlie this type of thinking.*
- *An important area in which the cognitive psychologists are most interested.*
- *For many years psychologists have attempted to explain the processes involved for decision making, and solving problems. The most famous examples of which are*
- *Thorndike's trial and error problem solving.*
- *Kohler's insight problem solving.*

Mental shortcuts

Algorithms and Heuristics

- *Algorithms: A rule, if it is applied, ensures the solution to the problem.*
- *Algorithms are always accurate.*
- *We do not necessarily understand their logical basis e.g. algebraic formula*

Algorithms and Heuristics

Algorithms: A rule, if it is applied, ensures the solution to the problem.

- *Algorithms are always accurate.*
- *We do not necessarily understand their logical basis e.g. algebraic formula*

Heuristics: A rule of thumb that if used can be effective in finding solutions to problems, but may not ensure or guarantee this.

- *For example rules for playing naught and crosses, or preparing only the 'important' parts of the course for the exam.*

Problem Solving

- *Thinking for the sake of finding solutions to problems.*
- *Three major steps are involved in solving a problem.*
 - Preparation for finding the solutions*

- ii. Producing the solutions
- iii. Appraisal of the solutions that have been generated

There are two types of problems:

- a. Well- defined problem.
- b. ill- defined problem.

Well- Defined Problem

- *Clear, definite and well-formed problem. Means of solving that problem are available. E.g. when solving the mathematical equation in which the problem is rather difficult, but the method used to solve it is direct and available.*

Ill- Defined Problem

- *Indefinite, unclear, ill- formed problem whose nature is not specifically defined and the ways for solving them is also difficult__ how do we build the morale of the crowd when their own team is losing the match???*
- *. In this, the problem is not specific and so the solution.*
- *The main shortcoming of this type of problem is that it is not possible to draw the immediate and absolute conclusions and solutions of the problem.*

Kinds of Problems

Problems are typically of three types

- *Arrangement problems*
- *Problems of inducing structures*
- *Transformation problems*

Arrangement Problems

- *Solutions of the problems require rearrangement and re evaluation of the components so that certain criterion will be satisfied*
- *There are a number of solutions to certain problems, but there are only specific solutions that fulfill certain requirements e.g. solving mathematical equation*

Problems of Inducing Structures

- *It refers to the problems whose solution depends on the relationship of components among them so that new construct which has a relationship can be developed.*
- *e.g. see and tell what number comes next? 221- 412- 321- 512- 421- 612-. Firstly, one has to consider the existing relationship between these numbers then make out the other relationship*
- ***521-712*

Transformation Problems

- *Problems that require understanding and that should be solved in a manner that involves series of methods so that the initial problematic state can be changed into the goal attaining state.*

Steps for Problem Solving

Four steps are important for solving certain problem.

- **Means- ends analysis: Repeated testing of the behavior in order to lessen the distance between the goal and current existence e.g. taking instructions or asking questions for solving puzzles**
- **Sub goals: Divide the problem into small element and then solve them in steps or sequentially**
- **Insight: Instant awareness of the relationship among the existing components, which**

formally seems independent of each other. A German psychologist Wolfgang Kohler was one of the first psychologists who observed this phenomenon, especially when facing challenges.

- *Evaluation of solution:* **The final step in problem solving which involves appraising the existing solution of the problem to demonstrate whether the solution is adequate or not.**

Impediments to Problem Solving

There are various factors, which serve as an obstacle in finding solutions. They are;

- **Functional fixedness.**
- **Mental set.**

Functional fixedness **is the capacity to think about certain phenomena in its most typical use or form.**

Mental set **is the tendency to think of a solution in a most old patterned ways___ old means of finding and answering problems.**

Creativity and Thinking

- *The word “creative” is derived from the Latin word “creare” means “to make” or Greek word “Krainein” means, “ to fulfill”.*
- **Creativity may be defined as the innovative, novel responses and ideas into a harmonious whole/ form.**
- **Creativity can be flourished by two means;**
- **Inspiration.**
- **Hard work.**

Stages in Creative Thinking:

- **Creativity mainly involves four steps;**
- **Preparation,**
- **Incubation,**
- **Illumination,**
- **Verification,**
- **Revision.**

- Preparation:** It includes assembling or combining the material and think thoroughly about it.
- Incubation:** After thinking intensely about the certain problem, the person lets his mind free by putting the problem aside and let its solution incubate in the mind.
- Illumination:** It also refers to as “ insight” when innovative ideas are instantly generated___ a sudden flash comes to mind when one is brainstorming at it. Scientific innovations are one of the examples.
- Verification:** Evaluation of the problem to find out whether the solution is correct or not
- Revision:** This involves the whole of the above steps involved that should be used in order to reach on some solution

Factors Associated With Creativity

- **The main factors that may contribute in enhancing creativity is**
 - Divergent thinking,*
 - Convergent thinking.*
- **Divergent thinking involves varied thoughts and solutions to a certain problem.**

- **Convergent thinking includes various thoughts and solutions for a particular problem.**

Creative thinking is ‘GOING BEYOND’

Creative thinking is going beyond:

- **The obvious**
- **The defined**
- **The laid out**
- **The conventional**
- **The common**
- **The usual**

Creative thinking is:

The ability to generate a variety of unusual solutions to a problem

Creative thinking is:

- **Open**
- **Original**
- **Imaginative**
- **Uninhibited**
- **Exciting**
- **Fulfilling**
- **Lateral**
- **May be stray and wild at times**

Lateral versus Vertical Thinking

Lateral thinking

- **Finds new ways of looking at things**
- **Avoids looking for what is “right” or “wrong”.**
- **Analyzes ideas to generate new ideas**
- **Considers the irrelevant**
- **Progresses by avoiding the obvious**

Vertical thinking

- **Tries to find absolutes**
- **Seeks continuity**
- **Finds what is right: seeks “yes” or “no” justifications**
- **Looks for stability: rejects irrelevant information.**
- **Uses established patterns & considers the obvious.**

Lateral Thinking

A deliberate process and set of techniques for generating new ideas by changing an individual's or team's way of perceiving and interpreting information; as opposed to vertical thinking i.e., a logical step-by-step process of developing ideas by proceeding continuously from one bit of information to the next.

Lateral/Creative Problem Solving

- **Reversal Technique:** examining problem and turning it completely around; inside out or upside down.
- **Analogy Technique:** developing a statement about similarities between objects, persons, or situations.
- **Cross Fertilization Technique:** asking experts from other fields (totally different fields) to view the problem and suggest methods for solving it from their own area of interest.

Assessing and Examining Creative Thinking

- Name all the things you can think of that are round/circular in shape.
- List as many white, edible things as you can.
- List all the uses that you can think of a watermelon.
- List all the possible uses of a pencil.
- Mr. X has been told a number of times to be in office at 9 ~~but~~ he is always late. WHY???
- Ms. Q has a number of clothes, ~~but~~ she has been wearing the same dress for the last two weeks. WHY???
- Your rich client comes to your office. You offer him coffee ~~but~~ he refuses. WHY???

“You can become what you imagine yourself to be!!!”

As If:

Appearing for an interview

Imagine

As if you were to introduce yourself to an interviewing board.

Think

As if you were introducing yourself to an interviewing board.

Act

As if you were actually introducing yourself to an interviewing board

Thinking Loud

Your friends Z and X have stopped talking to each other, whereas they have to work together in the same office.

a: Think of as many reasons as you can of the dispute.

b: Think of as many ways as you can of a possible patch up planned by you.

Clearly thinking about all possible aspects of the issue before taking decisive action

The cases of shop lifting in a super market rose to such a high level that they had to increase the prices. They hired guards but that did not work much. However they finally found a solid solution.

How???

Concept Challenge

Challenging the established ideas for new solutions

- **Women should do house work.**
- **In solving disputes all parties should have equal part in decision-making.**

Cognitive Complexity

Persons who are high in cognitive complexity are interested in use of more philosophical and abstract ways of thinking___ these types of people give preference in using complex, abstract and intricate stimuli and thinking patterns. Comedy, jokes and humor is its best example

PERSONALITY I

What do we mean when we say:

- *She has got a gorgeous personality!*
- *He has a powerful personality!*
- *He has a dull personality!*
- *She has a vibrant personality!*

In saying so, and while describing someone's personality, what are we referring to?

- The looks of a person? or
- The overall impression that one leaves on our mind? or
- The way a person behaves with us? or
- The way, as we feel, others treat a person?

And what about when we say:

- *He has a strong personality.*
- *She is a good decision maker.*
- *He has a strong will power.*
- *She is too sensitive.*
- *He is very aggressive.*

Here while describing someone's personality, we are talking about:

- *The feelings,*
- *Emotions,*
- *Cognitions, and*
- *Psychological make up of a person.*

Definition of Personality

Personality can be defined and understood in a number of ways:

- *Personality is the sum total of characteristics on the basis of which people can be differentiated from each other.*
- *Personality is the stability in a person's behavior across different situations.*
- *It can also be seen as the characteristic ways in which people behave.*
- *Personality consists of characteristics that are relatively enduring, and that make us behave in a consistent and predictable way.*

Theories of Personality

- *Also known as approaches to personality*
- *These are the approaches to understanding the "WHAT", "HOW", and "WHEN" of characteristics and features that make up an individual's personality.*

“WHAT”: This part consists of:

- *What is personality?*
- *What are the different aspects of personality?*
- *What variables account for these characteristics?*
- *What factors can have an impact, positive or negative, on our personality?*

“HOW”: This part consists of:

- *How do these characteristics develop?*
- *How do we become what we are?*
- *How can we manipulate, improve, or modify our own, or someone else's personality?*

“WHEN”: This part consists of:

- *When does personality develop?*
- *When is personality more pliable, and modifiable?*
- *When can others influence a person's personality more?*

1. Psychodynamic Approach

Approach that focuses upon the unconscious determinants of personality i.e., psychologists belonging to this approach believe that unconscious forces determine our personality.

Unconscious

- **The part of personality, which we are not aware of.**
- **Unconscious contains instinctual drives:**
 - **Infantile wishes,**
 - **Desires,**
 - **Demands, and**
 - **Needs**

These instinctual drives are hidden in the unconscious, and do not surface at the conscious level.

At the same time the person seeks satisfaction and fulfillment of these drives, as they can be a source of pleasure and satisfaction.

Why are these drives hidden then?

- Because they can cause conflict and pain if they became an obvious or overt part of our lives.
- Therefore, they are pushed into the depths of our unconscious.

Sigmund Freud, the most influential figure in the history of psychology, founded psychodynamic Approach. According to this approach the basis of motivation and behavior lies in inner forces: forces that are predetermined...forces over which humans have little control... which the person is not aware of i.e., these are the unconscious **determinants of behavior. Significance of Psychodynamic Approach**

It was the most influential theory of the 20th century.

- It affected psychology and related disciplines in a revolutionary manner.
- It gave an entirely new perspective to the understanding of behavior and mental processes, as well as mental illness.
- It was the first theory to raise the awareness that not all behavior is rational, well thought of, and planned.
- Besides giving an impressive, broad based, therapeutic approach, it provided a basis for understanding everyday life phenomena e.g. interpersonal relationships, aggression, and prejudice.

- Many other approaches built their paradigms on this approach, some by refining it, some by deviating from it. **Foundations of Psychodynamic Approach**

Psychic Determinism

All behavior is determined i.e., it has a cause that lies in the mind/psyche.

Role of Unconscious

A significant part of our behavior is generated by unconscious forces.

Structure of Consciousness

Conscious

Contains thoughts and feelings which one is immediately aware of

Subconscious

Mind level below the level of conscious awareness

Preconscious

Part of the sub conscious that can be accessed by deliberate choice.

Unconscious

Part of the sub conscious that cannot be accessed directly, although impulses, ideas, and feelings may permeate out through other sources e.g. dreams, slips of tongue etc.

Dreams in Freudian Approach

Dreams reflect unconscious needs, desires, and impulses.

Symbolism

- **Dreams have two levels or types of content: manifest content and latent content.**
- **The manifest content is in a symbolic form, converted into this form by the ‘dream censor, a mechanism that ensures that sleep is not disturbed by unconscious desires, and those desires are presented in a socially acceptable form.**

Psychodynamic Model of Personality

The structure of personality consists of Id, Ego, and super ego.

Id

The source of basic drives; operates under the ‘pleasure principle’ i.e., wants immediate gratification of needs.

Ego

Mediates the link of the self with the outside world, the ‘real world’, as well as between the id and superego; ego operates under the “reality principle” or the demands of the environment.

Super Ego

- Governed by the moral constraints
- Opposes the id and represents the moral
- Demands of the family and society; it is the ‘moral self’ or the ‘conscience’ of a person.

Oedipal Conflict and Electra complex

Oedipal conflict

(Also known as Oedipus complex). During the phallic stage, the male child begins to develop love and positive feelings for the mother: whereas negative feelings for the father since he is seen as a rival. But as the father is seen as too strong and powerful, the child fears retaliation and ultimately begins to develop ‘identification’ with the father.

Electra complex

The female child feels the same way toward the father, as the male felt for mother in Oedipal conflict, but ultimately chooses ‘identification’ with the mother.

Anxiety

- **An emotional state experienced as a result of felt threat to the self.**
- **Anxiety arises when ego cannot cope too much of:**
- **Demands of the id,**
- **Demands of the ego,**
- **External danger**
- **In order to protect itself against anxiety and threat, ego uses defense mechanism.**

Defense Mechanisms

Ego defense system that may be distorting reality. A number of defense mechanisms may be used by us for coping with anxiety:

i. Repression

Blocking unpleasant/ unacceptable thoughts by pushing them into the unconscious e.g. forgetting events of the painful childhood.

ii. Regression

Reverting back to a stage that was satisfying e.g. a boss showing temper tantrums like a child; or acting like a baby.

iii. Displacement

Redirecting the expression of unwanted desires or impulses to a substitute rather than the actual target e.g. beating children when a wife cannot express anger toward husband

iv. Rationalization

In order to justify one’s behavior, one develops a socially acceptable explanation or reasoning e.g. going for a second marriage saying that the first wife was quarrelsome.

v. Denial

Refusing to acknowledge or accept anxiety provoking thoughts or impulses e.g. being a heavy smoker but saying ‘I am an occasional smoker’.

vi. Projection

Attributing one’s unwanted thoughts and impulses to others e.g. a person takes bribe and blames the organization for paying him not enough salary.

vii. *Sublimation*

Converting unwanted impulses into socially approved thoughts, feelings and actions e.g. disliking the in-laws but behaving in a very friendly manner, or becoming a stamp collector to overcome the impulse to steal.

Criticism against Freudian Psychodynamic Theory

- 1. There is no scientific proof that many psychodynamic constructs, e.g. unconscious, exist.**
- 2. Psychic Determinism: Freudian approach is deterministic and leaves not much room for conscious, rational, decision making or personal will to act.**
3. It emphasizes the early childhood experiences too much.
4. It ignores the external variables and the environment.
5. Mostly criticized for its interpretation of the relationship between the two genders.
- 6. The therapy based upon this theory is too time consuming and therefore expensive.**

The Psychoanalytic Approach after Freud

The Neo Freudian

The theorists who belonged to the Freudian school and supported it, but later digressed on some issues and differed from Freud.

Basis of Neo-Freudian's disagreement with Freud

- **Their emphasis on the functions of ego, and the control that it had over routine-life activities.**
- **The impact of social variables.**
- **Their emphasis on the role of society and culture on personality development.**
- **Freud's idea of the primary importance of sexual urges.**

The neo-Freudians emphasized, more than Freud, the following:

1. The role of current social environment.
2. Life experiences have a continuing influence and childhood alone should not be of prime importance.
3. Positive interpersonal relations of love, and social motivation have a significant role.
4. Ego functioning is more significant rather than id.
5. Development of self-concept is important.
6. **Self-esteem is important.** Significant neo Freudians

Carl Gustav Jung

The founder of the analytical school of psychology, Jung was mystical in his understanding and description of personality. He had a positive approach toward one's ability to control one's destiny.

Jung's disagreements with Freud

He disagreed with Freud on:

- The understanding and description of the genders.
- The nature of unconscious.

Major Goal of Life

Unification of all aspect of our personality:

Main concepts

Conscious and Unconscious

Introversion (inner directed), extroverted (outer directed).

Libido**Energy for personal growth and development****Types of Unconscious**

Personal: Similar to Freudian view

Collective: ideas, and influences beyond personal experience, inherited from all generations of our ancestors and common to all humanity.

Archetypes

Part of collective unconscious; universal forms and patterns of thought. These include themes that can be seen in myths e.g. masculinity, femininity, good, evil opposites, motherhood.

Archetypes are the universal representations of a particular person, object, or experience, e.g. archetypes of mother, good, or evil.

Adler's Approach

Disagreed with Freud's emphasis on the significance of sexual needs.

Main concepts: Esteem, inferiority complex, birth order, will to power and style of life.

i. We are a product of the social influences on our personality.

ii. Goals and incentives drive us more than drives and instincts.

iii. Our goal in life is to achieve success and superiority.

Primary human motivation

- **Striving for superiority; achievement of self-improvement, and perfection, and not superiority over others.**
- **Inferiority complex, on the other hand, is the state when people feel that they have not been able to conquer, as adults, the feelings of inferiority that they developed as children.**
- **Inferiority complex: the feeling of being less able than others.**

Motivating Forces of Human Life

i. Feeling of inferiority, and a desire to overcome it through striving for superiority.

ii. People are primarily motivated to overcome inherent feelings of inferiority.

Birth Order

Has effect on personality.

Sibling Rivalry**Karen Horney 1885-1952**

She agreed with Freud on the levels of unconscious, anxiety, and repression. She emphasized childhood experiences, social interaction and personal growth.

Disagreement with Freud

Differed from Freud on primary impulses; impulses are not the main motivating force

Disagreed on Freudian position regarding the biological basis of differences between the males and females.

Horney's Main Concepts**Basic Anxiety**

A major concept: if the environment is hostile and the child feels lonely and isolated, then this type of anxiety develops. It can be overcome by proper parental nurturing

Basic Hostility

Children develop such hostility if parents are over strict, punishing, indifferent, or inconsistent.

Children feel very aggressive and hostile but cannot express it. Repressed hostility leads to anxiety.

Social Interaction and Interpersonal Styles

The ways in which people interact with each other is important. There can be three consequences:

- Moving away from others: seeking self-sufficiency and independence.
- Moving toward others: being compliant and dependant.
- Moving against others: trying to gain control, power, and independence.

Neuroses

Arise from emotional conflicts that arise from childhood experiences, and disturbances in interpersonal relationships in later life

Erik Erickson's Theory of Psychosocial Development

- **Student and follower of Sigmund Freud.**
- Broke with his teacher over the fundamental view of what motivates/ drives human behavior.
- For Freud, it was 'biology' or more specifically the biological instincts of life and aggression (Eros and Thanatos)
- For Erikson, the most important force that drives human behavior and which helps in the development of personality was **"social interaction"**
- His developmental theory of the **"Eight Stages of Man"** (Erikson, 1950) was unique and different in the sense that it covered the entire lifespan rather than **'childhood'** and **'adolescent development'**
- He believed that **social environment** combined with **biological maturation** results in a set of **"crises"** that must be resolved
- The individual passes through the "sensitive period" in different stages, which has to be resolved successfully before a new crisis is presented. The results of the resolution, whether successful or not, passed on to the next crisis and provide the foundation for its resolution
- He proposed eight stages of psychosocial development that have been discussed in detail in the section on cognitive development in lecture 11.

2. Trait Approaches

- **Approaches that propose that there are certain traits that form the basis of an individual's personality.**
- **These approaches seek to identify the basic traits necessary to describe and understand personality.**

Traits

- **Enduring dimensions of personality characteristics that differentiate a person from others.**
- **Trait theories do not imply the absence or presence of different traits in different people i.e., either/or situation. These do not say that if one trait exists the other does not.**
- These theories assume that some people are relatively high on some traits whereas, some are low on the same traits.
- The difference in people in terms of these traits is a matter of degree or extent to which the traits exist and manifest them. Some people have more of one trait and less of other ones.
- The degree to which a trait is present can be quantified e.g. "depression" is a tendency found in people; some have it more and some less
- Similarly "friendliness"; some are more friendly and some less
- The point to remember is that all traits may exist in all but these vary in the degree of impact

Allports' Trait Theory

- **After skimming an unabridged dictionary, Gordon Allport identified 18000 separate terms that**

could be used to describe personality.

- After eliminating synonyms he came up with a list of 4500 descriptions

But the important question was that: WHAT WERE THE MOST BASIC TRAITS?

Allports' Basic Traits Categories

There are three classic categories of traits:

- **Cardinal traits**
- **Central traits**
- **Secondary traits**

Cardinal Traits

- **A single personality trait directing most of the person's behaviors and activities e.g. affection, affiliation, kindness, greed.**
- **The person's whole life, or behavior, is influenced by this trait. A person who served the poor and the weak all his life may have a very high degree of " kindness" or " nurturance".**
- **Or a person who likes to hoard things, people, and wealth may be ruled by a high degree of " greed", or perhaps " inferiority".**

Central Traits

- **Do all people possess cardinal traits that encompass all aspects of their being?**
- **PERHAPS NOT!!!**
- **Most people develop a group or set of traits rather than a single one, that form the core of their personality.**
- **Central traits are those major characteristics that make up the core of someone's personality.**
- Central traits usually number from 5- 10 in a person.e.g., affection, love for humanity, and nurturance will form one type of personality.
- Inferiority, need for control, and greed may give a different shape to personality.

PERSONALITY II

Allports' Trait Theory

As said earlier, after skimming an unabridged dictionary, Gordon Allport identified 18000 separate terms that could be used to describe personality. After eliminating synonyms he came up with a list of 4500 descriptions. BUT the important question was that what were the most basic traits?

Allport's Basic Traits Categories included three classic categories of traits:

- **Cardinal traits**
- **Central traits**
- **Secondary traits**

Cardinal Traits refer to a single personality trait directing most of the person's behaviors and activities e.g. affection, affiliation, kindness, and greed. The person's whole life, or behavior, is influenced by this trait. A person who served the poor and the weak all his life may have a very high degree of "kindness" or "nurturance". Or a person who likes to hoard things, people, and wealth may be ruled by a high degree of "greed", or perhaps "inferiority".

Central Traits refer to those major characteristics that make up the core of someone's personality. Most people develop a group or set of traits rather than a single one, that form the core of their personality.

- Central traits usually number from 5- 10 in a person.
- e.g., affection, love for humanity, and nurturance will form one type of personality.
- Inferiority, need for control, and greed may give a different shape to personality.

Secondary Traits, qualities or characteristics that do have an effect on our personality but are much less influential than cardinal or central traits.

These affect fewer life situations as compared to the cardinal or central traits, for example preferring to wear certain colors, or a liking for specific tastes or smells.

Trait Theories Based Upon Factor Analysis

A number of trait theories are based upon factor analysis.

Factor analysis: a statistical method whereby relationships between a large number of variables are summarized into fewer patterns. These patterns are more general in nature. The extensive list is **For example: A researcher prepares a list of traits that people may like in an ideal man then administered to a large number of people, who are asked to choose traits that may describe an ideal man.**

Through the factor analysis, the responses are statistically combined and the traits associated with one another in the same set (or person) are computed. Thus the most fundamental patterns are identified. These patterns are called factors.

Psychologists Raymond B Cattell, and Hans Eysenck presented trait theories based upon factor analysis

Raymond Cattell's Sixteen Personality Factors

After using factor analysis Cattell proposed that two types of characteristics form our personality:

- **Surface traits, and**
- **Source traits**

Surface traits

- *Cattell's factor analysis showed that there are 46 surface traits or clusters of related behavior.*
- *These traits are the characteristics that we can observe in a given situation.*
- *The frequently quoted example in this regard is that of a friendly, gregarious librarian, who is so helpful that he might go out of his way to help you; as a result of your interaction with him it can be decided that he possesses the trait of sociability. His sociability is a surface trait in Cattell's terms.*
- BUT surface traits may not necessarily represent the traits that actually underlie the personality of a person; Surface traits are what we directly observe, and these are based upon our perceptions and representations of personality. These may not be the true descriptions of the actual underlying dimensions of someone's personality.
- The characteristics that form the actual roots and basis of all behavior may be different, and fewer in number.

Source Traits

- **In order to go beyond the surface traits, Cattell carried out further factor analysis.**
- **He could identify 16 traits that represent basic dimensions of personality.**
- **He called these traits, source traits.**

16 Pf: Sixteen Personality Factor Questionnaire

Cattell developed a measure that provided a score for each of the 16 source traits.

Factor symbol	High scorer	Low scorer
A	Outgoing	Reserved
B	More intelligent	Less intelligent
C	Stable Emotional	
E	Assertive	Humble
F	Happy-go-lucky	Sober
G	Conscientious	Expedient
H	Bold	Shy
I	Tender-minded	Tough-minded
L	Suspicious	Trusting
M	Imaginative	Practical
N	Shrewd	Forthright
O	Apprehensive	Placid
Q1	Experimenting	Traditional
Q2	Self-sufficient	Group-tied
Q3	Controlled	Casual
Q4	Tense	Relaxed

Eysenck's Dimensions of Personality

According to Eysenck, personality can be understood and described in terms of just two major dimensions:

- **Introversion-extroversion,**
- **Neuroticism-stability.**

On the first dimension, people can be rated ranging from introverts to extroverts: the rest of the traits fall in between.

The second dimension is independent of the first one, and ranges from being neurotic to being stable.

Introversion-extroversion

Introvert

Quiet, passive, and careful people.

Extroverts

Outgoing, sociable, and active people.

Neuroticism-stability

Neurotics

Moody, touchy, and anxious people.

Stable

Calm, carefree, and even-tempered people.

Eysenck evaluated a number of people along these dimensions. Using the information thus obtained, he could accurately predict people's behavior in a variety of situations.

The Recent Approach to Understanding Personality Traits

The “Big Five”:

Five broad trait factors lie at the core of personality:

1. Surgency: Extroversion and sociability
2. Neuroticism: Emotional stability
3. Intellect
4. Agreeableness
5. Conscientiousness

3. Learning Approaches to Personality

- **Approaches that focus upon the “observable” person rather than the inner drives, instincts, motives, thoughts, or traits.**
- **For the learning theorists:**

Personality is the aggregate of a person's learned responses to the external environment.

- **Variables considered most important by the learning theorist are the features of a person's environment.**

Learning approaches are primarily based upon the principles of:

- **Classical Conditioning**
- **Operant Conditioning**
- **Cognitive Learning**

B. F. Skinner's Approach

- **Personality is a collection of learned behavioral patterns.**
- **Patterns of reinforcement that have been received in various situations in the past cause similarities in responses across different situations, when same or similar situations are encountered.**
- **For example a student tries to make a good presentation every time he has to present because he has been receiving positive reinforcement for good presentations in the past...not because of an inborn drive or a trait of being a hard working or industrious person. Similarly, a person who is never aggressive may be so because he was always punished for aggressiveness and rewarded for being polite.**

For learning theorists

- **Consistencies in behavior across different situations are not as important as the strategies for modifying behavior are.**

Learning theorists are more optimistic in their approach, as compared to the psychodynamic theorists; they believe in the potential for change, and do not believe in the passivity of psychic determinism.

4. Social Cognitive Approach to Personality

- **The approaches that lay emphasis upon the role of people's cognitions in determining their personalities.**
- **Cognitions include: people's thoughts, feelings, expectations, and values.**
- **These approaches consider the "inner" variables to be important in determining one's personality.**

These approaches emphasize the reciprocity between individuals and their environment.

There exists a web of reciprocity, consisting of the interaction of environment and people's behavior. Our environment affects our behavior, and our behavior in turn influences our environment and causes modifications in the environment. The modified environment in turn, affects our behavior.

Albert Bandura

According to him, we possess the ability to foresee the probable consequences of certain of our behaviors in a given setting, without actually having carried out those behaviors or actually being in those settings. This so happens primarily as a result of "observational learning" i.e., having seen the outcomes of others (models) performing the same behaviors in same or similar situations.

For example, this is how we learn to be aggressive, sociable, or industrious.

Bandura also emphasized

- **Self-efficacy, and**
- **Reciprocal determinism**

Self-efficacy

- **Self-efficacy consists of learned expectations that one is capable of performing a certain behavior, or producing a desired outcome.**
- **Self-efficacy is the underlying variable in people's faith in their ability to carry out a particular behavior.**
- **The higher the sense of self-efficacy in a person the greater will be the persistence in his behavior, and also the greater will be the likelihood of his success.**

Reciprocal Determinism

- **According to Bandura, the key to understanding behavior lies in reciprocal determinism.**
- **We can understand the personality and behavior of a person by understanding the interaction between the environment, behavior, and the individual; and how this interaction causes people to behave in the manner they do.**
- **Environment affects behavior and the behavior in turn affects the environmental factors.**

For example

- **A woman likes to make friends. She gets an opportunity to make friends at parties. She in turn arranges parties herself and invites people she likes, or those she thinks are potential friends. Her desire for finding friends is satisfied as a result, at the same time she becomes confident that she can achieve what she wants by working on it. This causes persistence in her behavior.**

5. Humanistic approach to Personality

- **The humanistic approach stresses that people possess a basic goodness, and have a natural tendency to grow to higher levels of functioning.**
- **They have a conscious, self-motivated ability to change and improve.**
- **The basic goodness, and the natural tendency to grow, along with their unique creative impulses form the core of personality.**

Carl Rogers

- **All people require be loving and respecting. This is a universal phenomenon that is reflected in their need for positive regard.**
- **This love and regard comes to us from other people. When other people provide for this basic need, we become dependent on them. We begin to rely on others' values and evaluate and judge ourselves through the eyes of others.**

Self-concept and conflicts

- **Our self-concept and others' opinions are related.**
- **At times there may be discrepancies or conflicts between our self-concept (self-impression) and our actual experiences.**
- **Minor discrepancies lead to minor problems, whereas deeper conflicts lead to psychological disturbances in daily functioning e.g. frequent obsessions or anxiety.**

Unconditional positive regard

- **A person's conflicts can be resolved if he receives unconditional positive regard from another person.**
- **Unconditional positive regard means an attitude of total acceptance and respect from another person without any conditions. No matter what you say or do, the person accepts it.**

- As a result of this acceptance, a person gets an opportunity to evolve and grow cognitively as well as emotionally, and to develop a more realistic self-concept.

Self-actualization

- According to the humanistic approach, self-actualization is the ultimate goal of personality growth (see Rogers and Maslow).
- Self-actualization is a state of self-fulfillment in which people realize their optimal potential.
- Self-actualization occurs when our everyday life experiences and our self-concept match closely.
- Self-actualized people accept themselves the way they are in reality. This enables them to achieve happiness and a feeling of fulfillment.

6. Biological Approaches to Personality

- Approaches that emphasize the significance of biological variables and inherited personality characteristics.
- These approaches propose that important components that constitute our personality are inherited or genetically determined e.g. temperament.

Temperament

- Temperament is one of the main ingredients of personality.
- Temperament is the basic, innate disposition that emerges early in life.
- Even very young infants show signs of different dispositions e.g. some smile, some frown even when otherwise at ease, some are irritable, some calm, some shy, and some restless.
- Such behaviors persist and at an early stage in their life the children are labeled as stubborn, shy, restless etc.

Inhibited children

- According to Jerome Kagan children who are unusually fearful of the sight of unfamiliar adults, and fret when confronted with unfamiliar objects or new settings are the inhibited children.
- Such children are labeled as “shy” by their parents and teachers by the age of 3-4 years.
- They are consistently shy and emotionally restrained and noticeably quite in unfamiliar situations.
- They constitute around 10% of all children.

There are biological differences between the inhibited and uninhibited children:

- At age 5 muscle tension (especially in the vocal cords and the larynx) is higher in inhibited children.
- They differ in the heart beat pattern too. They experience more of rapid resting heartbeat. In case of confronting a new situation their heart beat increases more.
- Hormonal differences and variations in the excitability of the limbic system of the brain have also been seen to be different in the two groups.

Kagan concluded that these differences can be explained in terms of an inborn characteristic of the inhibited children i.e., their greater physiological reactivity.

Twin studies supporting the genetic argument

- A number of studies on twins reared together and reared apart have supported the biological approach to understanding personality.
- Study by Auke Telegen and colleagues (1988):

A sample of 350 pairs of twins was studied. They included 44 genetically identical twins who were reared apart.

- **The subjects were given a battery of tests, including one that measured personality traits.**

The results showed that

- **The twins were quit similar in their personality, in major respects.**
- **There are certain traits that are more influenced by heredity than others.**
- **Genetic component was found to be particularly strong in case of social potency and traditionalism.**
- **Genetic component was relatively weak in case of achievement and social closeness.**

PERSONALITY III

Assessment of Personality

1. Interview
2. Observation and behavioral assessment
3. Psychological tests
4. Self-report measures
5. Projective tests

Interview

- Interview refers to direct face-to-face encounter and interaction.
- Verbal as well as non-verbal information is available to the psychologist.
- Interviews are usually used to supplement information gathered through other sources.
- Skill of the interviewer is very important since the worth and utility of the interview depends on how well he can draw relevant information from the interviewee.

Behavioral Assessment

- Direct observation measure for studying and describing personality characteristics.

Psychological Tests

- In order to objectively assess personality and behavior standard measures are devised. These measures are called psychological tests.
- Psychological tests have to be valid and reliable. Besides they need to be based on norms.

Self-report measures

- Measures wherein the subjects are asked questions about a sample of their behavior. These are paper and pencil tools or tests.

MMPI (Minnesota Multiphasic Personality Inventory)

- The most frequently used personality test. It was initially developed to identify people having specific sorts of psychological difficulties. But it can predict a variety of other behaviors too.
- It can identify problems and tendencies like Depression, Hysteria, Paranoia, and Schizophrenia for example.
- At the same time it has been used to predict if college students will marry within 10 years, and whether they will get an advanced degree.

Projective Tests/ Techniques

Tests in which the subject is first shown an ambiguous stimulus and then he has to describe it or tell a story about it.

The most famous and frequently used projective tests are:

- i. **Rorschach test, and**
- ii. **TAT or Thematic Apperception Test**

Rorschach test
The test consists of Inkblot presses. These have no definite shape.

The shapes are symmetrical, and are presented to the subject on separate cards.
Some cards are black and white and some colored.

Procedure of Rorschach administration

The subject is shown the stimulus card and then asked as to what the figures represent to them?

The responses are recorded.

Using a complex set of clinical judgments, the subjects are classified into different personality types.

The skill and the clinical judgment of the psychologist or the examiner are very important.

Thematic Apperception Test/ TAT

A series of ambiguous pictures is shown to the subject, who has to write a story. This story is considered as a reflection of the subject's personality.

The subject is asked to describe whatever is happening in it just like forming a story.

The subject has to tell what is happening in the scene, what the antecedent conditions were, who the characters are, what are their thoughts and wishes, and what is going to happen next.

In short the subject describes the past, present and future along with the description of characters and their thinking and motivation.

Intelligence

Have you ever consciously considered these questions?

- **Am I an intelligent person?**
- **How intelligent am I?**
- **How do we judge if someone is intelligent or not?**
- **How can we measure the intelligence of a person?**

There can be other questions too:

–What is intelligence?

–Is intelligence how one deals with others? or

–Is intelligence how precisely we learn a new task? or

–Is it how good we are in our studies? or

–Is intelligence how well we can solve problems? or

–Is it how we accurately judge people? or

–Is intelligence all of this, or even more than all this?

–Different people may understand intelligence differently.

If you think intelligence is all of this or even more than all this, then you are right.

Intelligence

- **According to Feldman “intelligence is the capacity to understand the world, think rationally, and use resources effectively when faced with challenges”**
- **Intelligence is an all-encompassing concept.**
- **It is not restricted to just one aspect of one’s life, or to just one faculty or ability.**

Part of intelligence is inherited and part of it is learned.

Intelligence refers to the ability to adapt, to reason, to solve problems, and think in an abstract manner; it also includes learning and experiencing new things and understanding from the past experiences.

Intelligence or the intellectual ability of a person is based upon a constant and ongoing interaction between environmental factors and inherited potentials in order to have better understanding of how to ‘use’ and ‘apply’ the potentials in a meaningful manner.

- **Modern psychology considers both environment and heredity and their interaction to be influential.**

Development of Intelligence

Psychologists have attempted to uncover the factors operative in the development of intelligence; through studies, it is now evident that intellectual development does not take place in a smooth and straight fashion; it comes in intermittent bursts, and the pattern is different from person to person.

The physiology of intelligence

In early times, Greeks believed that the soul dwelt in the brain of the person and intellect is somewhere in the lungs.

This belief prevailed even till the 18th century.

Recent research has revealed that the flow of blood in the lateral prefrontal cortex of the brain is most when an individual indulges in solving some kind of puzzles.

- Another finding revealed that, in the elderly people, the flow of blood is much less in the areas that are concerned with alertness and memory as compared to the other areas.
- Levy's research revealed, after studying the two hemispheres of the brain, that the left hemisphere is most active and involved in analytical functioning and language.
- Whereas, the right hemisphere is more involved while doing visual and spatial skills and it tends to work more holistically.

Theories of Intelligence

- Theories or viewpoints on the understanding, explanation, and measurement of intelligence.
- These include psychometric approaches that are used to measure intelligence qualitatively and quantitatively.

Francis Galton

Cousin of Charles Darwin.

He was born in the family of geniuses and he himself was a genius having an IQ of more than 200.

He was a geographer, meteorologist, tropical explorer, “**founder of differential psychology**”, inventor of fingerprint identification, pioneer of statistical correlation and regression, convinced of hereditarianism, eugenicist, proto-genetics and a best selling author.

He gave the concept of “hereditary genius”.

According to Francis Galton ("Hereditary Genius, 1869)___ “gifted individuals” tended to come from families, which had other, gifted individuals. He went on to analyze biographical dictionaries and encyclopedias, and became convinced that talent in science, the professions, and the arts, ran in families. His was the first systematic attempt to measure intelligence by investigating the role of **heredity** and its impact on **intellectual abilities**.

He further attempted to measure human trait quantitatively in order to determine the distribution of heredity in it.

For this he used “word association test”, and “mental imagery”.

Galton argued that it would be "quite practicable to produce a highly gifted race of men by judicious marriages during several consecutive generations".

Eugenics: "The study of the agencies under social control that may improve or repair the racial qualities of future generations, either physically or mentally."

For Galton:

"What Nature does blindly, slowly, and ruthlessly, man may do providently, quickly, and kindly"

"Intelligence must be bred, not trained".

Such arguments appealed many and some people took this approach to extremes; this way of thinking had drastic social consequences and was used to support apartheid policies, sterilization programs, and other acts of withholding basic human rights from minority groups.

James McKeen Cattell

- American psychologist who gave more importance to the mental processes.
- First ever to use the term “mental test” for devices used to measure intelligence.
- Developed tasks that were aimed to measure reaction time, word association test, keenness of vision and weight discrimination.
- These tests were proved to be a failure as they were not comprehensive and complex enough to measure intelligence.
- Psychometric Approaches: Spearman's Theory of Intelligence
- British psychologist, Charles Spearman gave his theory in the early 1900s
- His theory laid the foundations for the later theories.
- He observed that people who scored high on one mental test also tend to score on the other as well. The same applies to the low scorers.
- He developed a statistical technique known as “factor analysis” on the basis of which he proposed two factors that can account for the individual differences

The first one he named as “g” factor or “general intelligence” and the other as “s” factor or “specific intelligence”

According to Spearman, ‘g’ factor can account for the general ability that is common in all people: as observed from the mental tests.

Whereas ‘s’ factor can account for the specific abilities that are different in different people; and also different tests required particular abilities from people

Spearman and his followers gave more importance to the ‘g’ factors and suggested that ‘g’ measured the ‘mental power’ or ‘mental energy’

Thorndike’s Theory: Social Intelligence

According to Thorndike:

“Intelligence and abilities are in the series”.

He criticized Spearman on the fact that correlation, which he studied, was not enough to define and explain intelligence.

He disagreed with Spearman on this, arguing that instead of only one ‘g’ factor, there are a number of factors that make and influence intelligence; factors that cannot generally be found out but that are expressed in human actions.

He divided intelligence into three main divisions

- i. Social intelligence; one’s ability to understand and manage relationships
- ii. Abstract intelligence; one’s ability to understand and manage ideas e.g. Mathematics, algebra or abstract concepts
- iii. Concrete intelligence; one’s ability to manage concrete and mechanical concepts and ideas e.g. Economics, architecture, banking etc.

Thurstone’s Theory of Intelligence: Primary Mental Abilities

American psychologist Louis L. Thurstone, believed that intelligence is not a general factor, but it is composed of small independent factors or elements, which he named as “primary mental abilities”.

For the identification and verification of these abilities, Thurstone and his wife, prepared a set of 56 tests, which they administered to 240 college students and analyzed the results through factor analysis.

From the analysis, he identified seven primary mental abilities:

- i. **Verbal comprehension:** An ability to understand and define words
- ii. **Word fluency:** An ability or speed of thinking of verbal material such as rhyming, or naming words in a given category
- iii. **Spatial visualization:** Ability to recognize and manipulate objects or things in three dimensions such as drafting and blue print reading
- iv. **Perceptual speed:** An ability to quickly perceive and detect the visual details and differentiate between the similarities and differences between designs
- v. **Reasoning/ inductive reasoning:** A logical ability of deriving general ideas from specific information
- vi. **Numbers/ Arithmetic ability:** Capability of doing work easily on numbers such as doing simple arithmetic tasks fast and rapidly
- vii. **Memory:** An ability or capacity of remembering and retaining the material such as words, letters and ability to recall and associate different words

R.B Cattell and J.L Horn’s Theory; Crystalline and Fluid Intelligence

The theory was given by American psychologists Raymond Cattell and John Horn in 1960s, in which they applied different forms of factor analysis and came up with two kinds of intelligence:

- I. Crystalline intelligence (gc)
- II. Fluid intelligence (gf)

Although these forms of intelligence are conceptually different, these are correlated forms of intelligence

- i. **Crystalline intelligence** is the capability of using information that has been learnt: this type of intelligence is largely influenced by education and culture. It keeps on increasing with the learning experiences of a person; such as vocabulary or knowledge.

ii. Fluid intelligence is largely influenced by biological factors; it is the capability of solving novel problems which depends more on the neurological development of a person such as reasoning and memory, which decline with age

Guilford's theory of the Structure of Intellect (SOI) It is a model of intelligence according to which intelligence is the result of the interaction of operations, contents and products

He believed that intelligence is a much more complex phenomenon than one thinks of it; it is difficult to define it as a 'g' factor or in terms of 'primary mental abilities'

Developed a cube- shape model of intelligence, which is made up of 120 separate factors, known as "the structure of intellect". Guilford recently expanded his model and it now includes 180 factors.

The different components of intelligence are:

Operations: it is the potential of different ways of thinking including:

- Evaluation
- Convergent thinking Divergent thinking
- Memory retention
- Memory recording
- Cognition

Contents: A potential of what we think about something. It includes;

- Visual
- Symbolic
- Semantic

Behavioral Products: The results obtained by applying certain operations to certain contents, or the ability of thinking in a certain manner about a certain thing.

It includes:

- Units
- Classes Relations
- Systems Transformation
- Implication

Lesson 34

INTELLIGENCE**Gardner's Theory of Multiple Intelligences**

- Given by Howard Gardner in 1985
- He entirely disagreed with the theorists who gave importance to factor analysis, except one thing that is common and found in his and the former theories is that intelligence is the composition of many specific abilities
- The theory maintains that intelligence consists of eight independent intelligences that vary in degree among individuals; it includes the potential of solving the problems or creating products that can be valued within one or more than one cultural settings
- Maintains that humans possess around eight kinds of intelligences, they are
 - Linguistics
 - Logical mathematical
 - Spatial intelligence
 - Musical intelligence
 - Bodily kinesthetic
 - Interpersonal intelligence
 - Intrapersonal intelligence
 - Naturalistic intelligence

Sternberg's Triarchic Theory of Development

- Given by American psychologist, Robert Sternberg, in 1980s, which is similar to Gardner's theory of intelligence.
- He observed that mental tests sometimes do not accurately predict success or failure in reality.
- According to his triarchic or three- dimensional theory, intelligence consists of three main components:
 - Analytic intelligence
 - Creative intelligence
 - Practical intelligence

Piaget's View of Intelligence: Intellectual development can be defined in terms of qualitative changes in thinking which are clearly apparent in children of particular age.

- His theory is more concerned with the universal patterns of intellectual development and functioning. He maintained a comprehensive theory that emphasized on 'how' children acquire knowledge and use it to solve logical problems
- He was more interested in how children exhibit intelligence in different stages of life as he proposed the four stages of cognitive development, which he termed as universal and invariant (occurring in the same sequence). The stages are: sensorimotor, preoperational, concrete operational and formal operational.

Hierarchical Theory of Intelligence

- Given by Carroll 1986 and Vernon 1971.
- Their hierarchy consists of three levels.
- At the highest level, there is general intelligence or "g" factor as given by Spearman.
- In the second level, are the factors of primary mental abilities as identified by Thurstone.
- At the third level, there are specific abilities or factors underlying intelligence

Measurement of Intelligence

- **Intelligence of a person can be measured through intelligence tests. The use of a battery of tests is preferred over a single test**

The History of Measurement of Intelligence

- **Man was always interested in knowing their own ability level, or in other words the intelligence, as well as that of others.**
- **People looked for capable persons while searching for life-mates, companions, workers, architects, artists, and poets or authors.**
- **Even in case of chefs, tailors, carpenters, or barbers, those with the fine skill and the ability to generate unique ideas were preferred.**

- In the olden times, observation and previous experience were the only ways to judge a person's capability or intelligence level. The prevalent, modern, approaches to measure intelligence are based upon the contribution of Alfred Binet.
- The first formal measure of intelligence was developed by French psychologist Alfred Binet and Theodore Simon, in 1905 in France.
- The test or the scale was developed in order to assist the education ministry and department in identifying “dull” students in the Paris school system, so that they could be provided remedial aid.

The main idea was that intelligence can be measured in terms of performance of a child

- If performance on certain tasks, that were the test items, improved with age then it could be taken as an indication of intelligence of a person.
- By developing tasks on which people's performance improved with age, a measure could be devised which could distinguish intelligent people from those not intelligent.
- Using the same concept Binet developed the first intelligence test in 1905. The test could identify more intelligent children within a particular age group. It could differentiate intelligent children from the less intelligent ones.
- The test was devised for locating the ‘dullest’ students in the Paris school system so that remedial assistance could be provided to them before they were denied instruction.

The Testing Procedure Adopted by Binet and Simon

- Initially Binet developed a number of tasks.
- Then he took groups of students who were categorized or labeled as ‘dull’ or ‘bright’ by their teachers.
- The tasks were presented to them. The tasks that could be completed by the ‘bright’ students were retained; the rest were discarded.
- The idea was to retain tasks that could be completed by the bright students, as these were considered to be indicative of the child's intelligence.
- With further work, dull or bright children could be identified with reference to their age.
- The scale could, thus, identify bright or dull students within particular age groups.

Here are some sample items from Simon-Binet Test (1911)

Three years

Shows nose, eyes and mouth. Repeats two digits. Describes objects in a picture. Gives family name. Repeats a sentence of six syllables.

Four years

Gives own sex. Names key, knife, and penny. Repeats three digits. Compares the length of two lines.

Five years

Compares two weights. Copies a square. Repeats a sentence of ten syllables. Counts four pennies.

Six years

Distinguishes between morning and afternoon. Defines objects in terms of their use. Copies a shape. Counts 13 pennies. Compares faces from the aesthetic point of view.

Seven years

Identifies right hand and left ear. Describes a picture. Follows precise directions. Names four colors.

Eight years

Compares two remembered objects. Counts from 20 to 0. Indicates omissions in pictures. Gives day and date. Repeats five digits.

Fifteen years

Repeats seven digits. Gives three rhymes. Repeats a sentence of 26 syllables. Interprets a picture. Solves a problem from several facts

Later Revisions

- The original Binet- Simon scale was revised a number of times.
- The American psychologist, Lewis Terman gave the first Stanford revision of the scale in 1916.
- These revision comparison American standards from age 3 to adulthood.
- Further revisions were made in 1937 and 1960.
- Stanford- Binet is one of the most widely used tests even today.

The Concept of Mental Age

- Children taking the Binet- Simon test were assigned a score that corresponded to the age group they belonged to.
- This score indicated their “mental age”.
- Mental age referred to the average age of children who secured the same score.
- Mental age can be understood as the typical intelligence level found for people at a given chronological age.

Mental age of a person can be different from his or her chronological age i.e., it can be above or below that. It could reflect whether or not a child was performing at a level at which his age mates were.

BUT it gave rise to a problem. How could we compare people belonging to different age groups?

Will a 20 year old with a mental age of 23 be equally intelligent as an 8 year old having a mental age of 11? The problem becomes more serious when we are talking of comparing those below their mental age e.g. comparing an 8 year old operating at a mental age of 5, with a 20 year old operating at a mental age of 17

The Concept of Intelligence Quotient or IQ

- As a result of problems with depending merely on mental age, a solution was devised in terms of intelligent quotient, a concept whereby the chronological age of the person is also given due consideration.

Intelligence Quotient: IQ

- An indicator or measure of intelligent that considers a person’s mental as well as chronological age.
- The formula for IQ:

$$\text{IQ score} = \text{MA} / \text{CA} \times 100$$

Using This Formula Means That

- If the mental and chronological age of a person is the same, then he or she will have an IQ of 100. If one is below his chronological age then the IQ will fall below 100 and vice versa.

Deviation IQ Scores

- Psychologists kept working on the concept of IQ and made amendments in the primary concept of IQ.
- Today, although the main concept of calculating IQ is still adhered to, we talk in terms of Deviation IQ scores.
- *Deviation IQ scores* are based upon sophisticated statistical techniques. The average score of a group is kept in mind, and the IQ of the person taking an IQ test is stated with reference to the points with which he deviates from the average.

Available Statistics Show That

- **Around two- thirds or 66% of the population fall within a range of 15 IQ points from the average IQ score of 100 i.e., the majority of people have an IQ between 85 and 115.**

The Meaning of IQ Test Scores

IQ score	Rating
< 70	Retarded
85	Borderline
100	Average
Above 115	Superior
Above 140	Gifted

IQ Tests

Today a number of IQ tests are available.

- **But before choosing a test for assessing a subject's intelligence, and considering it a trustworthy device, the psychologist has to make sure that the test is:**
- **Valid**
- **Reliable, and**
- **Standardized**

Stanford-Binet Test (4th Ed.)

- **Last revised 1985.**
- **Contains a series of test items that vary in nature according to the subject's age.**
- **For example a child may be asked to copy figures or answer questions about everyday life activities.**
- **Older subjects solve analogies, explain proverbs, or describe similarities underlying sets of words.**

Administration of Stanford-Binet test

- **Individual-oral administration.**
- **The examiner begins from a mental level at which he finds out the subject to be.**
- **Items from succeeding levels are asked.**
- **The test ends when they reach a level where no items are successfully attempted.**
- **IQ is computed by examining the pattern of correct and incorrect responses of the subject.**

Wechsler Adult Intelligence Scale-Revised

WAIS-R and Wechsler Intelligence Scale for children-III: WISC-III

WAIS-R and WISC-III

- **Psychologist David Wechsler developed both.**
- **The two tests consist of two major parts:**

- i. *Verbal part/scale*
- ii. *Performance or non verbal part/scale*

Verbal scale

- **Consists of more conventional types of problems involving vocabulary definition, and comprehension of various concepts.**
- **The subscales include:**
 - **Information**
 - **Comprehension**
 - **Arithmetic**
 - **Similarities**

Performance Scale

- **It involves assembling small objects and arranging pictures in a logical order.**
- **The subscales include:**
 - **Digit symbol**
 - **Picture completion**
 - **Object assembly**

Administration of WAIS and WISC

- **Individual administration.**
- **Time consuming.**

Group Intelligence Tests

- **Considering the time consuming nature of the administration of Stanford-Binet test, WAIS-R, and WISC-III, psychologists have developed a number of group intelligence tests.**

Cultural Biases and Intelligence Tests

- **Tests used to assess people's intelligence have been frequently criticized for being biased against particular groups of people.**
- **Culture-fair IQ tests are developed and used for overcoming this problem. These tests do not discriminate against any minority or cultural group.**

Alternative Formulations

- **Psychologists may also use other tools for assessing the ability and capacity of a person. They believe in measuring other aspects of a person's ability, besides just relying upon the I.Q. of a person. These aspects include:**
 - **Moral intelligence**
 - **Social intelligence**
 - **Emotional intelligence**

Moral Intelligence

- **Given by Coles (1997) and Hass (1998)**
- **It is the ability to differentiate between right and wrong**
- **More comprehensively, it is the capacity of making right decisions that are not only beneficial for one self but to others as well**

Social Intelligence

- **Also known as SQ.**
- **It is the ability to understand and deal with people; salesmen, politicians, teachers, clinicians, and religious leaders exhibit this type of intelligence**
- **It is also the ability to understand and deal one's own self by identifying one's thoughts, feeling, attitudes and behaviors**
- **The approach has been given and supported by Hough, 2001, Riggio, Murphy, & Pirozzolo (2002).**

Emotional intelligence or EI (indicated by EQ)

- **It is the type of social intelligence which is the ability to cope with one's own and other's emotions; to differentiate between them and use information for guiding one's thoughts and actions.**
- **Indicated by the EQ of a person. It includes these aspects:**
 - Self-awareness
 - Managing emotions
 - Empathy
 - Handling relationships.

Lesson 35

PSYCHOPATHOLOGY

- Psychological illness, psychological disorders, or mental illness are referred to as psychopathology.
 - The term is used to describe abnormal behavior.
- Psychopathology is the area of study in psychology that primarily focuses upon the origin, development and manifestation of behavioral and mental disorders.
- Abnormal psychology is that branch of psychology that studies, describes, explains, and identifies abnormal behavior.
- The observable behavior and mental experiences of an individual may be indicative of a mental or psychological disorder. The overt behavior and other experiences provide cues to the development of mental or psychological disorders.
 - Psychiatrists and clinical psychologists treat mental disorders.
 - Besides, they are also interested in studying and conducting research on the nature and role of the events that cause these disorders e.g. past history of a person and other variables that contribute to mental illness.

Historical Perspective of Psychopathology

Human life existed, presumably, on earth even more than three million years ago, but the records of only few thousand years are available to us.

One of the earliest traces in history, about the treatment of mental disorders date back to the stone age that was more than half a million years ago.

- Headaches, convulsive attacks, and other brain related problems were treated, at that time, with stone instruments in which a hole of about 2 cm was made in the skull of a person. This process was called **“trephining”**.

- Evidence is available that trephining was performed even 4000 to 5000 years ago.
- It is believed that the Stone Age people used to perform this procedure.
- The ancient explanations centered on the supernatural: gods, evil spirits, demons etc. It was thought that the supernatural forces caused strange and troubled behaviors, as they resided within the body of the person. The practice of trephining was based upon the idea that the evil spirits needed a hole to get out of human body, thus ridding the person of its impact. In the ancient societies the influence of evil spirits was the main focus:

Herodotus, the ancient Greek historian, wrote about a king who was driven insane by evil spirits.

According to the legend of Hercules, he killed his own children because evil spirits drove him mad.

- Trephining was done in order to allow the evil spirits to escape the body of a person so that he returned to sanity. This view prevailed throughout the rich civilizations of that time such as the Egyptian, Hebrew, Greek, and Chinese civilizations.
- One of the most primitive ways of treating abnormality, and freeing the person of the “evil” possession was the use of **“exorcism”**.
- Various techniques were used for casting the evil spirits out of the body of the afflicted person; these included black magic, noisemaking, chanting, prayer, and the use of potions.
- In extreme case flogging and burning the patient were also used.
- This treatment were done by the **“Shaman or the medicine man”**, who was regarded a very special person possessing unusual healing powers.
- In Egypt and Greece, the tasks of treating the ill were eventually taken over by the priests.
- These priests were a combination of a priest, physician, psychologist and a magician.
- The cure however still depended on magic.
- But not all ancient thinkers believed in this approach.
- In the Golden Age of Greece, temples for healing the ill were maintained.
- This was like a revolution that took place at a time when knowledge about human anatomy and physiology was very limited, and insufficient.
- During that time, Hippocrates, “ the father of modern medicine”, made his contributions in various fields.

Hippocrates

Believed in the rational knowledge for understanding and treating the psychological disorders and denied the interventions that was used at that time for treating the abnormal.

He that all illnesses, including mental disorders, had a natural cause and required natural treatments for curing the patient.

He maintained that brain is the central organ for all activities including the intellectual abilities and that disorders are the result of brain pathology.

- He also believed that abnormality was caused by the imbalances of some humors or liquids within the body of a person.

- He gave theory of “humors” that account for the basic human activity. He was of the view that the perfect health is the result of the proportionate mixture of these humors

- Maintained that four humors led to four temperaments forming one’s personality. The temperaments were:

a) Sanguine (Cheerful and Active)

b) Melancholic (Sad)

c) Choleric (Angry and Aggressive)

d) Phlegmatic (Calm and Passive)

Hippocrates maintained that epilepsy is caused by insufficient air carried by the veins to the brain and limbs.

Socrates

For him, soul is most important it should be properly looked after.

It is not just one faculty or any particular material entity; rather it is a broad area comprising intelligence and character, or man’s conscious personality.

He maintained that thought and reasoning are the building blocks of personal worth and happy life.

It can be concluded that problems with the soul led to abnormal behavior.

Plato

Believed in the humane treatment of mentally ill patients

He was of the view that disorders developed when the conflict arises between emotions and reason

But despite having modern thoughts, he still believed that mental disorders were partly treated by the divine powers too.

Aristotle

He mainly followed Hippocrates’ theory of mental disorders.

Gave different ideas about many emotional states such as anger, fear, envy, hatred, courage etc.

Asclepiades

First ever to distinguish between the acute and chronic mental disorders

Also differentiated between delusions, hallucination and illusions.

The movement of atoms in the body caused disease.

He advocated the practice of baths, dieting, exercise, and massage for curing disease.

Galen

- Most outstanding physician of antiquity after Hippocrates.
- He regarding the anatomy and nervous system of human beings as he was performing dissections on animals made influential contributions.
- The following many centuries did not see a major breakthrough regarding the understanding and treatment of abnormality.

In The Middle Ages

- In the middle ages of Europe, religious thought, dominated the understanding and treatment of mental disorders.
- This period lasted from the fall of the Roman Empire in 5th century AD till the 15th century.
- It was thought that the mentally ill were possessed by demons or the devil.
- They were accused of being witches who could infect others with madness.
- As a result the mentally ill were treated not humanely but with religious inquisition and barbaric treatment.

- However, many thinkers even at that time believed that mental disorders did have a physical cause.
- They believed that the imbalance in the four basic humors, grief, and poor diet caused such illnesses.

In The Islamic World

- The mentally ill were treated in a good and humane manner by the physicians during the rule of Muslim caliphs. Asylums were made for the mentally ill.
- **Special baths, diet, medication, music, and pleasant environment were provided to the patients.**
- Such asylums began in 8th century AD.
- The first one was established in Baghdad, and was followed by others in Cairo, Damascus and Fez.

In Europe

- The mentally ill underwent inhuman treatment for long.
- The situation prevailed even in the 17th and 18th century AD.
- They were either left to wander in wilderness, or kept isolated in institutions.
- Many hospitals in Britain, France and Italy became notorious for their callous way of treating the mentally ill.
- However with growing public awareness the need for a change was felt.
- La Bicetre hospital in Paris, France, was a pioneer in this regard.
- Pussin was the superintendent of a ward for incurable mental patients.
- He released patients from shackles, and the staff was forbidden from beating them.
- Philippe Pinel who became the chief physician of La Bicetre hospital continued this practice.

Developments in 17th And 18th Centuries

The period of 17th century is regarded as the “age of reason”, and that of 18th century as the “age of enlightenment”.

Great changes as well as discoveries were made at that time in a number of fields including psychology

Franz Friedrich Anton Mesmer (1734- 1815)

- Mesmer proposed that a power similar to magnetism existed in humans.
- **This, animal magnetism, exercised a powerful influence on our body.**
- He asserted that animal magnetism had a medicinal value.
- He believed that maintaining and balancing the magnetic field could cure mental illnesses.
- He was of the view that hand movements and their magnetic force were enough to produce dramatic changes in the ill persons.

He introduced mesmerism, a trance like state, as a curative technique. Mesmerism was similar to today's hypnosis. He made more use of magnetism than suggestion.

The major breakthrough took place in the later half of the 18th century.

Philippe Pinel: (1745-1826)

- His contributions began with the revolutionary changes at the La Bicetre hospital in Paris. He believed that abnormal behavior is caused by some hereditary defects or nervous system defects. He was of the view that mental patients should be treated with great care as he severely condemned chaining and shackling of the mentally ill.
- **Due to his efforts, La Bicetre and Salpetriere hospitals were regarded as the first modern hospitals for caring and curing the insane.**

- Pinel gave the concept of Moral Treatment. He said that the mental patients should be treated with kindness and sensitivity. Cruelty and violence were discouraged and forbidden.

William Tuke

Tuke established the York Retreat in rural England in 1796. Here, mental patients were provided compassionate treatment. This retreat became a model for others to follow.

Wilhelm Griesinger:(1817-1868)

He believed that the best way to understand mental disorders is to assume that they are caused by brain pathology. He firmly believed on the organic origin of mental disorders rather than the psychological cause.

Kraepelin: (1856-1926)

- Published the first system of classification of mental diseases.
- This system could help to label different disorders as psychosis and neurosis.
- Kraepelin clearly differentiated between ‘dementia praecox’ (now known as schizophrenia) and ‘depressive psychosis’; severe mental disorders.
- These severe disorders were thought to be developed out of the organic reasons.

Jean M. Charcot: (1825- 1893)

- **French neurologist.**
- Charcot was most interested in observing and treating hysterical patients.
- He noticed that these patients had strange beliefs about their own bodily functions. He developed techniques for treating hysterical patients as well as also being able to induce hysterical symptoms in normal individuals

Defining Abnormality

What is abnormal behavior?

Abnormality can be defined in a number of ways.

People identify, understand and explain abnormality according to their past experience, common information, cultural tradition, societal attitude, and/or professional knowledge. Definitions of Abnormal Behavior

1. Statistical definition

People deviating from the norm are considered “abnormal”.

- BUT what if majority of people indulge into erratic behavior?
- What about the creative artists who did not go along the norm?

2. Abnormality as deviation from “Ideal” Ideal refers to the standard toward which most people strive.

- Abnormality, according to this definition, is not striving toward the ideal.
- BUT what about those for whom the ‘ideal’ is not the ‘ideal’?
- **For example a student who is a very good painter and does not want to pursue conventional education.**

ABNORMAL BEHAVIOR I

Definitions of Abnormal Behavior

3. A Sense of Personal Discomfort Seen As Abnormality

- A person is seen as abnormal if his thoughts and behavior are a source of discomfort for him.
- Discomfort can be in the form of anxiety, distress, or guilt.

4. Inability to function effectively

- People, who cannot function and perform as effectively as they ought to, are seen as abnormal.
- This definition includes adjusting, and adapting to the social requirements.

5. The Legal Definition of Abnormality

- Laws in different countries define abnormality according to their legal standards.
- It is primarily needed for differentiating sanity from insanity.
- Abnormality may be viewed as not being able to foresee and understand the consequences of the criminal act.
- Or it can be taken as inability to control one's own thoughts and behaviors.
- Or it can be the ability to see right as different from wrong.

Perspectives on Abnormality

- Approaches to studying, describing, understanding, explaining, and predicting abnormality.
- These approaches affect the way a mental patient will be treated.

1. Medical Perspective

- Psychological problems are caused by physiological factors.
- These can be the biological processes and systems, genetic factors, the nervous system and the neurotransmitters, hormonal changes, or external variables affecting the biology of a person.

2. Psychodynamic Perspective

- Childhood experiences are the root cause of mental disorders.
- Unconscious determinants are significant.

3. Behavioral Perspective

- Abnormal behavior is learned.
- Abnormality is a learned response.
- It results from our interaction with the external world.

4. Cognitive Perspective

- The factors causing mental disorders are a person's cognitions, thoughts, and beliefs.

5. Humanistic Perspective

- People's need to self-actualize, and their responsibility for their own actions, play a central role

in abnormality behavior.

6. Sociocultural Perspective

- **The social milieu in which one lives, the family and the people around, the society, and the culture at large are of primary importance in the onset, and later treatment, of mental illness**

Classification of Mental Disorders

- **Kraepelin gave the first classification system of mental disorders.**
- **A number of classification systems followed afterwards.**
- **The purpose was to assist the clinicians diagnose mental disorders, as well as to determine the extent of the problem.**

Classification Systems

- **DSM- IV- TR**
- **ICD**

DSM-IV-TR

- **Diagnostic and statistical manual of mental disorders is the classification system compiled by the American Psychiatric Association.**
- **This is the most widely used classification system all over the world.**

ICD: International Classification

- **For decades, mental health professionals in Western Europe and a major part of the world used this classification system.**
- **The World Health Organization developed ICD.**
- *ICD is a comprehensive classification system of all kinds of diseases, including psychological or psychiatric illnesses.*
- **For a number of years ICD9 remained a popular diagnostic system.**
- **Research, in the last more than a decade, reflected that the revised and improved versions of DSM had an edge over ICD in many respects.**
- **Besides, there were no major differences as such in the two systems.**
- **Also, the need for a single universally accepted system was intensely felt.**
- **Therefore today DSM-IV-TR is recognized as a universally accepted diagnostic system.**

DSM-IV-TR

- **The first DSM was published in 1917.**
- **It originated from a project of the American Medico-Psychological Association, now known as American psychiatric Association and United States Bureau of the Census.**
- **In order to collect uniform data on hospitalized mental patients, they developed a list of 59 mental illnesses.**
- **The list was further expanded with the publication of the first DSM in 1952.**
- **The first DSM included a list of 106 mental illnesses.**
- **DSM-II was published in 1968.**
- **DSM-III was published in 1980.**
- **DSM-III-R was published in 1987.**

- **DSM-IV was published in 1994.**
- **DSM-IV was developed after a special 27-member task force of experts worked for five years.**
- **More than 1000 psychiatrists contributed and advised in deciding about the diseases and other information to be included in DSM-IV.**
- **DSM-IV-TR was published in 2000.**

DSM-IV-TR contains definitions of more than 200 mental disorders.
These disorders are organized into 17 major categories.

Multi Axial System of DSM-IV-TR

- **DSM-IV-R also contains five axes, or five types of information, that have to be considered in the diagnosis of a patient.**

Axes of DSM-IV-TR

- **Axis I: Clinical disorders**
- **Axis II: Long standing problems that are frequently overlooked in the presence of disorders listed in axis mental retardation, personality disorder, and I.**
- **Axis III: General medical conditions that may be relevant to a psychological disorder.**
- **Axis IV: Psychosocial or environmental problems that a person is facing.**
- **These problems may affect the diagnosis, treatment, or the course of the mental disorder.**
- **Axis V: Global Assessment Of Functioning.**

Major Categories of Disorders in DSM-IV-TR

1. **Anxiety disorders**
2. **Somatoform disorders**
3. **Dissociative disorders**
4. **Mood disorders**
5. **Schizophrenia**
6. **Personality disorders**
7. **Sexual disorders**
8. **Substance-related disorders**
9. **Delirium, dementia, amnesia, and other cognitive disorders.**

Anxiety Disorders

- **Disorders in which anxiety becomes an impediment in a person's routine functioning.**
- **Anxiety is a reaction to real or imagined threat that may hamper the daily functioning and results in uneasiness, worry, and apprehension.**
- **In anxiety disorders, anxiety occurs without an obvious external cause, to an extent that it affects routine functioning of the person.**

• Stress is the part of daily routine in a person's life but the reactions to stress vary from individual to individual.

• Anxiety is one of the various reactions to stress.

• Whether or not one will develop anxiety, and to what extent, will depend on the nature of stress faced, family history, and fatigue or over work, and the person's coping strategies.

Major symptoms of stress include

- Sleeplessness
- Headaches
- Twitching and trembling

- Dry mouth
- Memory problems
- Nightmares
- Irritability
- Fatigue
- Sweating
- Muscle tension
- Insomnia

Common causes are

- Imagined threat
 - Grief
 - Physical or emotional stress
 - Use of drugs
 - Withdrawal from drugs.
- #### **Subcategories of Anxiety Disorders**
- **Generalized anxiety disorder**
 - **Panic disorder**
 - **Phobic disorder**
 - **Obsessive compulsive disorder**
 - **Post- traumatic stress disorder**

Treatment can be done through

- Finding the actual cause of anxiety.
- Avoid becoming dependent on mood altering drugs.
- Avoid stimulants such as caffeine, nicotine, alcohol etc.
- Biofeedback and relaxation therapy.
- Aerobic exercises.
- Avoid the effects that have been produced due to anxiety, if anxiety is cured, the other symptoms will be resolved automatically. **Generalized Anxiety Disorders**
- **The disorder marked by long-term, persistent, anxiety and worry.**
- **It refers to the long- term anxiety in which there is continual and exaggerated state within the person due to which he/ she is continually tense, apprehensive and in automatic nervous system arousal.**
- **Chronic form of anxiety disorders.**

Causes include

- Hereditary causes,
- Or this disorder begins at very early age and the revealing of the symptoms is gradual not burst.

Treatment involves

- Medications and use of psychotherapy,
- Exposure therapy,
- Behavioral therapy and cognitive behavioral therapy.

Symptoms involve

- People with this disorder are unable to relax,
- Insomnia
- Trembling,
- Muscle tension,

- Head aches, sweating,
 - Twitching,
 - Trembling,
 - Feel tiredness,
 - Depression etc.
- Panic Disorder**
- **Disorder in which anxiety is manifested in the form of panic attacks lasting from a few seconds to many hours.**
 - **Panic attacks are unpredictable; resulting from vague anxiety and that may accompany physiological manifestations.**
 - **Symptoms include:**
 - **Dizziness and/or fainting**
 - **Sweating**
 - **Trembling**
 - **Palpitation**
 - **Nausea**
 - Choking
 - Fear of dying
 - Fear of being out of control
 - Skin blushing or flushing
 - Chest pain and discomfort
 - Sleep disturbances
 - Agitation
 - Facial paralysis etc.
- Causes involve**
- Use of drugs and stimulants.
 - As a result of some incident or risk factor.
 - The exact cause of panic attacks is still not known; may result due to temporal dysfunction of the brain or may have been learnt through past experiences.
 - More frequent in women than men.

Prognosis: The disorder is difficult to treat and long- lasting as well, but behavioral therapies and use of drugs can minimize the symptoms.

Phobias

- **Phobias are the particular, persistent, irrational and intense paralyzing fear of some objects and situations that they are unable to explain and overcome; and that may occur without any actual cause.**

Symptoms include

- Perspiration
- Frustration,
- Rapid heart beat
- Headaches etc.

ABNORMAL BEHAVIOR II**Causes of Phobias may include:**

- Result of some traumatic event or disaster
- Hereditary component,
- Prevalent equally in men and women,
- Anxiety,
- Panic attacks.

SOME COMMON PHOBIAS

	Fear of:
Acrophobia	Height
Aerophobia	Flying
Agoraphobia	Entering public places
Claustrophobia	Closed spaces
Hydrophobia	Water
Mikrophobia	Germes
Nyctophobia	Darkness
Ophidiophobia	Snakes
Phonophobia	Speaking out loud
Pyrophobia	Fire
Thanatophobia	Death
Xenophobia	Strangers

Treatment of Phobias includes

- Use of behavior therapy especially behavioral- modification therapy.
- Procedure of systematic desensitization is used.
- Biofeedback is also helpful.

Obsessive- Compulsive Disorder

Obsession is an unwanted, recurrent and persistent thought that continuously recurs, and that can be intrusive and inappropriate

A **compulsion** is the uncontrollable urge to perform an apparently strange and unreasonable act repeatedly.

Symptoms include

- Distress,
- Frustration,
- Anxiety etc

Causes include:

- Risk factor,
- Stereotype behaviors,
- Brain abnormalities,
- Unpleasant thoughts,
- Some incident etc.

Prognosis: It is a chronic illness in which total removal of symptoms is not possible, but improvement through medication and therapy is possible

Somatoform Disorders

A disorder in which psychological problems take the physical (somatic) form without any apparent physical cause; a state where there are physical symptoms present but no explicable medical cause.

Symptoms include

- Blurred vision,
- Dizziness,
- Vomiting,
- Difficulty in swallowing etc

There are **two types** of somatoform disorders.

1) Hypochondriasis**2) Conversion disorders****Hypochondriasis**

Type of somatoform disorder in which the person experiences a persistent fear of illness, and is preoccupied by health concerns.

Even minor pains and aches may be interpreted as a symptom of some serious disease.

Symptoms involve

- In this disorder doctor shopping is very frequent.
- Sympathy may exaggerate these complaints.
- Patient undergoes surgery and regularly takes medication.
- Patient focuses closely on normal physiological states such as rapid heartbeat, sweating, palpitations etc; patient interprets it as some severe disease.
- Minor health problems may become severe as a result of persistent stress and discomfort

Conversion Disorders

Disorder in which the persons undergoes an actual, genuine and specific, physical problem and disturbance. The problem has a purely psychological reason and there is no biological cause involved.

The problem manifests itself suddenly, without any prior indication.

Unexplainable neurological symptoms appear at once when no testable cause is present.

Symptoms include

- Partial blindness.
- Loss of voluntary control over motor and sensory functions.
- Inability to hear and talk.
- Sudden display of emotions: and at times there is no emotion.
- Symptoms may be exaggerated by stress.

The sufferers frequently do not show a natural concern about the symptoms.

Causes include

- Hereditary component, and observational learning
- A state of severe stress
- People who have other organic problems may develop conversion disorder
- A prior knowledge of the disease and symptoms is there.

Dissociative Disorder

A disorder in which critical personality facets, that is normally integrated and working together, become separate.

This allows stress avoidance and anxiety reduction by way of escape.

The person uses defense mechanisms for avoiding stress and to deal with traumatic experiences

At a time, two or more personalities, may exist within the person

Symptoms include

1. Auditory or visual illusion,
2. Feeling of confusion and disorientation,
3. Severe anxiety attacks,
4. Suicidal attempts,
5. Inflicting self-injuries

Causes involves

- High state of stress

Treatment includes

- Psychotherapy
- Self- induced trance
- Minimize stress

Types of Dissociative Disorder**Dissociative Amnesia**

A state when a selective loss of memory occurs.

The person is unable to recall specific events often as a result of extreme stress.

Significant memory loss occurs about personal information that is not due to an organic cause.

This disorder vanishes abruptly as it begins and rarely re-occurs.

Dissociative Fugue:

Fugue means, “ flight”

Fugue is a form of amnesia.

The sufferer takes sudden impulsive trips, at times assuming a new identity.

Dissociative fugue includes forgetting as well as fleeing from one’s home for days and weeks, also being unable to remember one’s identity.

Unconscious wandering in which the person has limited social contacts.

In some instances, person may take over another personality that is more sociable than the previous one.

Dissociative identity disorder/Multiple personality

- Rare disorder in which the person may take over two or more personalities that are entirely different from one another
- The first one is usually restrained, restricted and dull but the other one is entirely different from the previous one; one’s mannerisms, vocal, movements are entirely different from one another

Mood Disorders

Psychological and affective disturbances characterized by emotional extremes that are enough to produce troubles in daily living

The emotional response is disturbed and so strong that it disturbs everyday living.

Mood disorders mainly include:

1. Major depression
2. Mania
3. Bipolar disorder

i. Major Depression

Previously known as “ melancholia”

Major depression is a severe form of depression.

Common form of mood disorders

A disorder characterized by lack of concentration, decision- making, sociability, withdrawal from others, and a feeling of worthlessness and inadequacy.

Depression is labeled as depressive disorder when it persists for long and hampers daily life.

Symptoms include

1. Concentration problems,
2. Irritability and restlessness,
3. Persistent sadness, anxious and empty mood
4. Fatigue,
5. Appetite changes
6. Feeling of agitation
7. Sleep disturbances
8. Hopelessness and pessimism
9. Loss of interest in activities, which are pleasurable
10. Suicidal thoughts.

Causes include

4. Hereditary cause,
5. Stress,
6. Chemical imbalances in the brain; the sufferer however has the belief that it is a medical illness rather than a psychological one
7. Most commonly occurs in people with low self-esteem
8. Women are twice as likely to develop major depression as men.
9. Learning experiences may contribute to the development of depression
10. Serious loss in business or some other disaster,
11. Relationship problems, financial setbacks etc

Treatment includes

Use of medication,
 Psychotherapy,
 Behavioral therapy.

ii. Mania

Mania is the opposite state of depression.

It is an extended state of intense wild elation.

iii. Bipolar Disorder:

- Bipolar disorder is a combination of depression and mania.
- The sufferer alternates between periods of extreme euphoria and elation i.e., mania, and bouts of depression.
- Side Effects of Mood Disorders
- The height of elation may lead to high creative output, although it does not ensure high quality of the creative output.
- The manics are often reckless and end up with self-injury.

Causes

Psychodynamic explanation: Feeling of loss that can be real or potential.

Hereditary factor: These disorders appear to be running in families.

The role of neurotransmitters: Serotonin and nor epinephrine have been found to be related to these disorders.

Alterations in the level of these chemicals have a role to play in mood disorders

Behavioral explanation: Lack of, or reduction in, positive reinforcement leads to mood disorders.

Cognitive explanation: the sufferers of depression believe that they are life's losers; they are failures, inadequate, and not meant to be the 'winners' in life. They have a pessimistic view of life.

Evolutionary psychology explanation: Considering the impact of the genetic factors, it states that depression is an adaptive response to unattainable goals.

Schizophrenia

- **Schizophrenia is a category of mental disorders marked by severe distortion of reality.**
- There is a deep division between the real world and the schizophrenic's world.
- What makes schizophrenia different from other disorders?
- Significant decline from a previous level of functioning.
- Disturbances of thought and language

Symptoms in**Schizophrenia**

Delusions,
 Hallucinations,
 Emotional disturbances,
 Withdrawal.

Delusions

Unshakable, firm, and deeply believed in beliefs are held by the schizophrenic.

Delusions can be about one's being grand, or being persecuted by others, or others planning against him, or one's thoughts being relayed to others who are out of physical reach.

Hallucinations and Perceptual Disorders

The schizophrenic has sensory experiences that ordinary people do not have.

They may hear voices, see people or objects, and/or smell things that others find to be non-existent.

The hallucinations mean reality to the schizophrenic.

Hallucinations are usually based on the delusions.

The sense of own body is also affected in schizophrenia.

Emotional Disturbances

Overall the schizophrenics show a flat, blank, and bland emotional response.

Also, their emotional responses are inappropriate.

Withdrawal

Schizophrenics live in an isolated world of their own.

Schizophrenics withdraw from others.

They avoid socializing.

They are not interested in others.

In extreme cases they are oblivious of the presence of others.

Types of Schizophrenia**a. Disorganized or hebephrenic type**

Marked by inappropriate emotion: inappropriate giggling, laughter, silliness, incoherent speech, infantile behavior, and strange and at times obscene behavior.

b. Paranoid Schizophrenia

- The patient experiences delusions and hallucinations of his own greatness.
- Behavior is unpredictable, and erratic.
- Sense of judgment is lost.

c. Catatonic Schizophrenia

- Catatonic schizophrenia is marked by disturbances in the motor activity and muscular control.
- Major disturbances occur in movement.
- At times all motion stops and the patient just freezes in one position.
- This frozen posture may last for hours and even days.
- In some phases the patient exhibits wild, free floating, and even violent movement.

d. Undifferentiated Schizophrenia

This variety of schizophrenia involves a combination of the major symptoms found in other varieties.

This diagnosis is used when patients do not fit into any one of the major categories of schizophrenia.

e. Residual Schizophrenia

Residual schizophrenia consists of minor signs of schizophrenia after a major, more serious, episode.

PSYCHOTHERAPY I

When does one need to go to a psychotherapist?

According to Engler and Goleman (1992) the following list of symptoms can serve as a guideline for people in determining whether they need outside intervention, and whether it can be of help:

- *Long-term feelings of distress that interfere with your sense of well-being, competence, and ability to function effectively in daily life activities.*
- *Occasions in which you experience overwhelmingly high stress, accompanied by feelings of inability to cope with the situation.*
- *Prolonged depression or feelings of hopelessness, particularly in the absence of a cause.*
- *Withdrawal from other people.*
- *A chronic physical problem for which no physical cause can be determined.*
- *A fear or phobia that prevents you from engaging in everyday activities.*
- *Feelings that other people are out to get you or are talking about you or plotting against you.*
- *Inability to interact effectively with others, preventing the development friendships and loving relationships.*

Varieties of psychotherapies

- About 400 psychotherapies are available to us.
- Psychotherapists can choose any of these depending upon their theoretical orientation, as well as the nature of the problem they are dealing with.
- Also, depending upon their prior knowledge if any, patients and sufferers may choose a psychotherapeutic approach that they prefer to undergo.

Psychotherapeutic Orientations

- *Although a variety of psychotherapies are available, the primarily stem from four major psychological approaches:*
- *Psychodynamic approach*
- *Behavioral approach*
- *Cognitive approach*
- *Humanistic approach*

Who can be a psychotherapy provider?

- *Clinical Psychologists*
- *Counseling Psychologists*
- *Psychiatrists*
- *Psychoanalysts*
- *Licensed Professional Counselors or Clinical Mental Health Counselors*
- *Clinical or Psychiatric Social Workers*

Clinical Psychologists

- *They are psychologists with a Ph.D. or Psychology. Degree.*
- *They have to complete a postgraduate internship prior to practicing.*
- *They have a specialization in the assessment and treatment of psychological problems.*

Counseling Psychologists

- *Counseling Psychologists have Ph.D. or Ed.D. Degree.*
- *They typically provide help in solving routine-life adjustment problems.*

- *Their usual workplace is a university mental health clinic.*

Psychiatrists

- *Psychiatrists are primarily medical doctors.*
- *They have an M.D., M.B.B.S. or any other qualifying medical degree according to the law of any nation.*
- *They undergo post-graduate training in dealing with mental disorders.*
- *Being medical doctors, psychiatrists can prescribe medicine.*
- *Although there are no restrictions on which they can treat, psychiatrists generally treat severer cases of mental disorders.*

Psychoanalysts

- *These professionals specialize in psychoanalysis.*
- *They can be medical doctors or psychologists trained in psychoanalysis.*

Licensed Professional Counselors or Clinical Mental Health Counselors

- *These professionals hold a master's degree, with a national or state certification in therapy.*
- *They provide therapy to suffering individuals, couples, and families.*

Clinical or Psychiatric Social Workers

- *These professionals are basically master's degree holders.*
- *They undergo specialized training.*
- *As a result of their training, they can provide therapy regarding family and personal problems.*

Psychotherapies based upon the psychodynamic approach

- *These therapies are primarily based upon the psychodynamic model introduced by Freud.*
- *This approach is based upon two assumptions:*
 - *Unresolved past conflicts is the root cause of abnormal behavior.*
 - *Unacceptable unconscious impulses will enter consciousness.*

Psychoanalysis

- *Introduced by Sigmund Freud.*
- *Therapy is based on the premise that the roots of all abnormal behavior reside as unresolved conflicts in the unconscious.*
- *To avoid these conflicts, people use defense mechanisms*
- *•Psychoanalysis is an intensive, long term, psychotherapeutic procedure.*
- *•Requires long sessions over extended periods----- may be years*
- *•Better suited to intelligent individuals.*
- *•Involves a special relationship between the therapist and the patient*

Target: To explore unconscious motivation, conflicts, desires

Goal: Establishing intra psychic harmony by developing awareness of the role of the id, reducing over compliance with super ego, and by strengthening the ego

Understanding of 'repression': The therapy gives central importance to the understanding of the manner in which the person uses repression to handle conflict

Interventions used in Psychoanalysis

Free association

Kept in a comfortable position, the patient is asked to talk aloud and say whatever comes to his mind without considering whether or not it is relevant, rational, or sensible

The patient is asked to reveal even the most undesirable and strong thoughts that have been repressed. This leads to emotional release, called ‘catharsis’.

Dreams in Freudian Approach

Dreams reflect unconscious needs, desires, impulses.

With the help of these dream analysis, psychotherapists seek an understanding about the unconscious motives, desires and conflicts

There are two parts of dreams.

Dream content

1. **Latent content**
2. **Manifest content**

Manifest part is what the patient describes about the dream and its surface interpretation

Latent part is the hidden part or the “true message” within the dream.

Symbolism

The manifest content is in a symbolic form.

Converted into this form by the ‘dream censor, a mechanism that ensures that sleep is not disturbed by unconscious desires, and those desires are presented in a socially acceptable form.

Analysis of Resistance

- **At times patient feels inhibitions, and is unable or unwilling to express some thought or feeling i.e., barriers between conscious and unconscious. The psychoanalyst aims to break down such resistances so that the patient is enabled to face the unpleasant thoughts, impulses, and events.**

Dream Analysis

- **The therapist tries to uncover the latent content of dreams and decipher the symbolism involved**

Analysis of Transference & Counter Transference

Transference: The patient’s emotional response toward the therapist is often an indication of the patient’s relationship with a person who had been the center of the conflict. It may be negative or positive

Counter Transference: The therapists’ emotional reaction toward the patient.

Criticism Against Freudian Psychodynamic Theory

There is no scientific proof that many psychodynamic constructs, e.g. unconscious, exist

Psychic Determinism: Freudian approach is deterministic and leaves not much room for conscious, rational, decision making or personal will to act.

It ignores the external variables and the environment.

It emphasizes the early childhood experiences too much.

Mostly criticized for its interpretation of the relationship between the genders

The therapy is too time consuming and therefore expensive.

Behavioristic Approach to Treatment

A psychological approach that considers the relationship between behavior and environmental stimuli as the focus of study; observable behavior is what psychology should be studying, understanding and explaining.

An approach that dominated psychology for most of the 20th century.

What do the Behaviorists Study?

Observable/ overt behavior

Specific measurable responses

How particular types of behaviors are controlled by particular types of environmental stimuli

Method of investigation: **Data typically collected under controlled laboratory conditions, employing technological assistance.**

What the Behaviorists Are Not Interested in:

- **Unconscious**
- **Inner motivation**
- **Biochemical processes**

Behavior Therapy Is More Directed Towards

- **Behavioral therapists focused on unlearning the mal- adaptive patterns of behavior which people acquired through learning; and abnormal behavior would be treated by making them to relearn new experiences**
- **Changing individual's behavior and make them able to function more efficiently definitely solves the problem**

Classical Conditioning and Behavior Therapy:

Behavior therapy uses various classical conditioning strategies for modifying the mal- adaptive behavior. These important techniques are;

- *Systematic desensitization*
- *Aversive therapy*
- *Flooding and implosive therapy.*

Systematic Desensitization

- *A procedure used in behavior therapy in which a stimulus that generates pleasant feeling is repeatedly paired with the anxiety provoking stimulus in hope to alleviate anxiety. In the beginning the therapist prepares a hierarchical list of the fear provoking stimuli, beginning from the least fear provoking.*
- *Before using this procedure, the patient has to learn muscle- relaxation techniques by the therapist.*
- *The patient is kept in a relaxed state. Then the therapist utters the names of the fear provoking stimuli, one by one. He keeps taking the names till the patient shows signs of discomfort and anxiety. The process is repeated the next time, till a stage when the patient can tolerate the whole list, showing that his fear is gone.*

Aversive therapy

- *Developed by Ivan Pavlov.*
- *A technique used to unlearn the unwanted habits by associating it with the unwanted impulses. An aversive stimulus is*

attached with an undesirable behavior, e.g. adding an emetic substance to the cigarettes of the patient.

This technique is frequently used with alcoholics, smokers and substance abusers. Implosive Therapy:

- **Making the patient directly encounter the fear-provoking stimulus.**

Observational Learning

- *Learning the experiences through modeling and imitation. The patient observes others perform the behavior that he finds difficult to perform. This can be done with live modeling or with video recordings.*
- *Behavior therapists used modeling for teaching the patient new skills and habits and alleviate their anxieties, fears and phobias.*

Operant Conditioning

- *Operant conditioning approaches uses the rewards and punishment strategies for modifying behaviors.*

Token Economy/ Token System

- *The person is rewarded with some form of a token every time a desired behavior is exhibited.*
- *The token can be play money/token or a chip representing money; it can be the silver or gold stars earned by the child; parents can give different colored paper tokens for good behavior.*
- *After a specific number of token have been earned, they can be exchanged for something desirable.*

Contingency Contracting

- *A written contract is held between the client and the therapist, specifying all goal-behaviors as well as consequences: parents and teachers can also use it.*
- *The contract is followed strictly no matter if the consequences of behavior are negative, and the client may in fact dislike them; the purpose is to promote target behavior.*
- *e.g. if an over-eater fails to refrain from confectionary throughout the week, he will have to send a donation cheque for drinks in a marathon; the cheques are prepared at the beginning of the program*

Usefulness of Behavior Therapy

- *Found to be effective with 50-90% of patients especially treating phobias, anxiety disorder and other unwanted behaviors such as smoking and drug abuse.*
- *Also helpful in the sense that it can also be employed by non- professionals too.*
- *Cheap and economical as it directly focused on the problem to be unlearn.*

Cognitive Approach in Behavior Modification

- *Negative and unacceptable behavior is modified through constructive strategies.*
- *According to this theory, person's beliefs and attitudes effect the motivation and behavior of a person.*
- *In order to modify the behavior, reinforcement techniques are used.*
- *For attaining the desired goal, realistic strategies are used with continuous feedback.*

Behavior Modification

- *A therapeutic/intervention strategy used for modifying behavior in such a manner that the frequency of desired behavior is increased up to the optimal level, and the frequency of undesired behavior is brought down to the minimum...or to extinction level.*
- *The intervention is based upon the principles of learning.*

Steps in Behavior Modification

- *Identification of goals in terms of target behavior.*

- *Recording the preliminary/background information concerning the behavior in question.*
- *Designing the intervention, issues involved and deciding its components*
- *Implementation of the planned program as well as careful monitoring.*

Altering the Belief System

Psychologists are of the view that psychological problems arise due to the way people perceive themselves in relation with the people they interact.

Main focus of the therapist is to alter the irrational belief system of a person.

Cognitive Therapy for Depression

- *Aaron Beck formulated the therapy for depression patients.*
- *Therapist helps the depressive person to change the faulty patterns of thinking through problem- solving techniques.*
- *Believed that depression reoccur in depressive patients because the negative thoughts occur automatically from which they are unaware.*

Rational-Emotive Behavior Therapy

- *Developed by Albert Ellis (1962, 1977).*
- *Focused on altering the irrational beliefs into more acceptable way.*
- *Clients are forbidden to use “should”, “must”, “ought” etc.*
- *Confrontation techniques are used which focused on changing the attitudes through rational reasoning.*

PSYCHOTHERAPY II

Behavior Modification

A therapeutic/intervention strategy used for modifying behavior in such a manner that the frequency of desired behavior is increased up to the optimal level, and the frequency of undesired behavior is brought down to the minimum...or to extinction level.

The intervention is based upon the principles of learning, specifically shaping.

Steps in Behavior Modification

- Identification of goals in terms of target behavior.
- Designing the intervention, issues involved and deciding its components.
- Implementation of the planned program as well as careful monitoring.

Shaping

- Successive approximations of a required/-desired response are reinforced until that response is fully learnt.
- In the beginning each and every success is reinforced with a reward, no matter how small the success.
- Once the desired response is learnt the reinforcer immediately follows it, every time it happens.
- Once learnt the behavior, in many cases, the organism may not need reinforcement since many behaviors are self-reinforcing e.g. learning to play a musical instrument.

Usefulness of Behavior Therapy

- **Found to be effective with 50-90% of patients especially treating phobias, anxiety disorder and other unwanted behaviors such as smoking and drug abuse.**
- **Also helpful in the sense that it can also be employed by non- professionals too.**
- **Cheap and economical as it directly focused on the problem to be unlearn.**
- Criticized also because it also give importance to the overt observable behavior and does not evaluate the inner thoughts and experiences.

Cognitive Approach in Behavior Modification

Negative and unacceptable behavior is modified through constructive strategies

- According to this theory, person's beliefs and attitudes affect his motivation and behavior.
- In order to modify the behavior, reinforcement techniques are used.
- For attaining the desired goal, realistic strategies are used with continuous feedback.

Steps in Behavior Modification

- Identification of goals in terms of target behavior;
- Recording the preliminary/background information concerning the behavior in question;
- Designing the intervention, issues involved and deciding its components.
- Implementation of the planned program as well as careful monitoring.
- Recording the events, progress, and problems during the implementation phase
- Evaluating the program and making alterations if required.

Altering the Belief System

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- Main focus of the therapist is to alter the irrational belief system of a person.

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- **Therapist helps the depressive person to change the faulty patterns of thinking through problem- solving techniques.**
- **Believed that depression reoccur in depressive patients because the negative thoughts occur automatically from which they are unaware.**

Four tactics are used by the therapist that unfounded depression

- i. Challenging the patient's ill beliefs.
- ii. Evaluating the cause of depression.
- iii. Attributing the cause to the environmental situation/ event not to the person's in competencies.
- iv. Finding the alternative and effective solutions for the complex problems.

Rational-Emotive Behavior Therapy

- **Developed by Albert Ellis (1962, 1977)**
- **Focused on altering the irrational beliefs into more acceptable way.**
- **Clients are forbidden to use “should”, “must”, “ought” etc.**
- **Confrontation techniques are used which focused on changing the attitudes through rational reasoning.**
- Task is to protect the self worth, potential to be self-actualized by blocking the irrational thinking patterns.

Humanistic Therapies

Central Themes of Humanistic Approach

- Human beings are capable of shaping their own destiny.
- They can think and design their course of action and can follow it in the way they like
- People can overcome or minimize the environmental, and intrinsic influences.
- “Here and now” is important.
- “Wholeness” or “completeness” of the personality is important rather than its separate, disintegrated, structural parts.

Humanistic approach emphasizes

- Individual's freedom in directing his future.
- Capacity for personal growth.
- Intrinsic worth, and,
- Potential for self-fulfillment.

Rogers' Approach

- Primarily a clinical theory, based on years of Roger's experience dealing with his clients.
- In its richness and maturity his theory matches that of Freud; a theory well thought-out and logical, having broad application.
- The theory emphasizes on a single factor “force of life” which he calls the actualizing tendency i.e. built-in motivation present in every life form to develop its potentials to the fullest extent possible.

To be **fully functioning** means experiencing:

- i. Optimal psychological adjustment.
- ii. Optimal psychological maturity.
- iii. Complete congruence (a feeling of integration when the self and the ideal self match; incongruence is a feeling of conflict or unease experienced in case of a mismatch between the two).
- iv. Complete openness to experience.

Humanistic Approaches to Therapy

- **The approach suggesting that people are in control of their life.**
- **The person or the self, and personal growth and development are emphasized by the humanistic psychologists.**
- **The humanistic approach includes a number of other theories with the same or similar**

orientation e.g., ‘existential’ and ‘phenomenological’ psychology.

- Rogers maintained that the therapist must possess the following qualities:
 - i. **Congruence** -- genuineness, honesty with the client.
 - ii. **Empathy** -- the ability to feel what the client feels.
 - iii. **Respect** -- acceptance, unconditional positive regard towards the client

Carl Roger’s Psychotherapy

- **Carl Rogers is best known for his contributions to therapy known as “person- centered/ Client-centered therapy/ Non- directive therapy.**
- **Also known as the Rogerian Therapy”.**
- **His main technique “Reflection” __ mirroring of emotional experiences.**
- **Therapy in which the therapist’s role is to reflect back the patient’ statement in such a manner that the patient finds solutions to his/ her problems.**
- **Aim of the therapy: To help a person grow and self-actualize.**

Other Approaches

Existential Therapy

- **Contrary to humanistic approach to therapy that focuses on unique freedom and potential as positive force, existential therapy is based on the notion that when an individual becomes unable to deal with his/ her freedom, it will result in anguish, fear, and concern.**
- **The goal of life, according to this therapy is to properly grip and use one’s freedom effectively with value systems one has in his/ her life.**
- **Therapy also focuses on the individual’s responsibilities that he must take in order to make free choices about his lives.**
- **Therapist’s job is to probe and challenge the patient’s views of the world around.**
- **In the same way, a strong bond is developed between the patient and the therapist so that the patient feels comfortable while interacting. Also both are experiencing the same difficulties and apprehensions.**

Gestalt Therapy

- **The Gestalt therapy focuses on integrating the patient’s thoughts and feelings into an integrate whole**
- **This can be done through by placing oneself in that position as mother or father and then his/ her own position in order to experience different parts of a conflict.**
- **The aim of the therapy is to express whatever is frustrating and conflicting to the person such as kicking the things around when angry, yelling out while frustration etc**
- **This sort of activities encouraged the person to act out/ express the things that are conflicting and frustrating.**

Group Therapy

- **A form of therapy in which people discuss problems and difficulties with the group members.**
 - **Several unrelated people work with the therapist and discuss their psychological problems with him; in some cases the therapist is active and directed towards the problem and sometimes the group takes up an issue and determines how to proceed.**
 - **People often discuss issues like smoking, alcoholism, or lacking social skills.**
- The other members of the group provide emotional support and suggestions so that the it can also be helpful to them in similar circumstances.

Family Therapy

- **The therapy, which involves members of the family for finding solutions to problems.**
- **Therapist considers family members as a unit in which each member serves as a contributor in order to gain an understanding of “ how” they interact with one another.**
- **Family therapists believe that problems arise in the family due to the rigid roles and conventions exhibited by the family and the solution of their problems lies in adopting new and constructive roles and patterns of behavior.**

Biomedical Therapies

- **Medication**
- **Psychosurgery**
- **Lifestyle changes**

POPULAR AREAS OF PSYCHOLOGY

- Psychology today is the most popular social science.
- American Psychological Association has 55 divisions.

Most popular areas of psychology today:

- Clinical psychology.
- Health psychology.
- Organizational psychology.

Major Sub fields of Psychology:

- **Behavioral Neuroscience**
- **Clinical psychology**
- **Clinical Neuropsychology**
- **Cognitive Psychology**
- **Counseling Psychology**
- **Cross-cultural Psychology**
- **Developmental Psychology**
- **Educational Psychology**
- **Environmental Psychology**
- **Evolutionary Psychology**
- **Experimental psychology**
- **Forensic Psychology**
- **Health Psychology**
- **Industrial/Organizational Psychology**
- **Personality psychology**
- **Program Evaluation**
- **Psychology of women**
- **School Psychology**
- **Social Psychology**
- **Sport psychology**

SOCIAL PSYCHOLOGY

The branch of psychology concerned with how people's thoughts, feelings and actions are affected by others.

- How will we behave if we do not have any people around us?
- What would we be like if we had never ever come across another human being

Attitudes, Behavior and Persuasion

Attitudes

Learned predispositions to respond in a favorable or unfavorable manner to a particular object.

Attitudes can also be defined as our learned evaluations that we hold about other people, objects, events, behaviors, and beliefs.

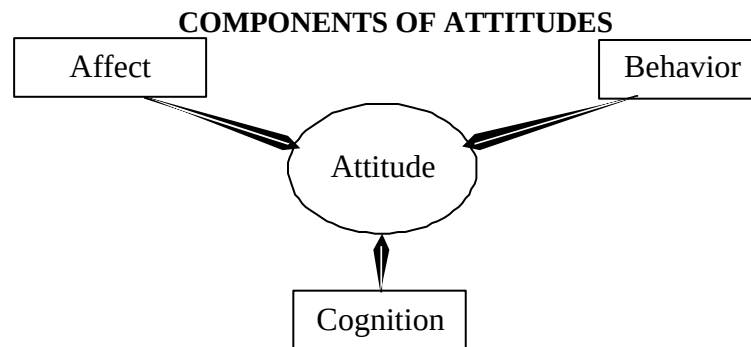
ABC MODEL

The model suggesting that an attitude has three components: affect, behavior and cognition.

Affect component: That part of an attitude encompassing how one feels about the object of one's attitude.

Behavior component: A predisposition to act in a way that is relevant to one's attitude.

Cognition component: The beliefs and thoughts held about the object of one's attitude.



Forming and Maintaining Attitudes Classical Conditioning and Attitudes

Advertisers make use of the principals of classical conditioning of attitudes by attempting to link a product they want consumers to buy with a positive feeling or event.

Operant Conditioning Approaches to Attitude Acquisitions

Attitudes that are reinforced, either verbally or nonverbally, tend to be maintained.

Vicarious Learning of Attitudes: Learning by observing others.

For example, even if they have never met a blind person, children whose parents say that “blind people are incompetent’ may adopt such attitudes themselves. We also learn attitudes vicariously through television, films and other media. (e.g. violence).

Agents of Attitude Formation

- Family
- School
- Peer group
- Neighborhood
- Social Environment
- Mass Media

Persuasion

Persuasion is the process through which people's attitudes are changed.

Factors affecting attitude change

- Message source
- Characteristics of the message
- Characteristics of the target audience

Message Source

The characteristics of the attitude communicator or the deliverer of the message are important.

Characteristics that make a difference:

- Ø Physical attractiveness
- Ø Social attractiveness
- Ø Expertise
- Ø Trustworthiness
- Ø Audiences belief that the communicator does not have any ulterior motive.

Characteristics of the Message

Two-sided messages are more effective than one-sided messages.

Two-sided messages are the ones in which both sides of the argument are discussed.

Fear-producing messages are effective when it is accompanied by a solution to the problem as well.

If the message is too fear provoking, and without a solution too, then it may be ignored.

Characteristics of the Target

Less intelligent people are less resistant and the more intelligent are more resistant to persuasion.

Persuasion is relatively easier with women when in public settings, and when they have less knowledge about the issue under question.

There are no gender differences in case of a change in private attitudes.

Types of Information Processing used by the Target for Interpreting the Message

- a. Central Route Processing
- b. Peripheral Route Processing

Central Route Processing

Interpretation of messages involving thoughtful consideration of the issues and arguments used to persuade.

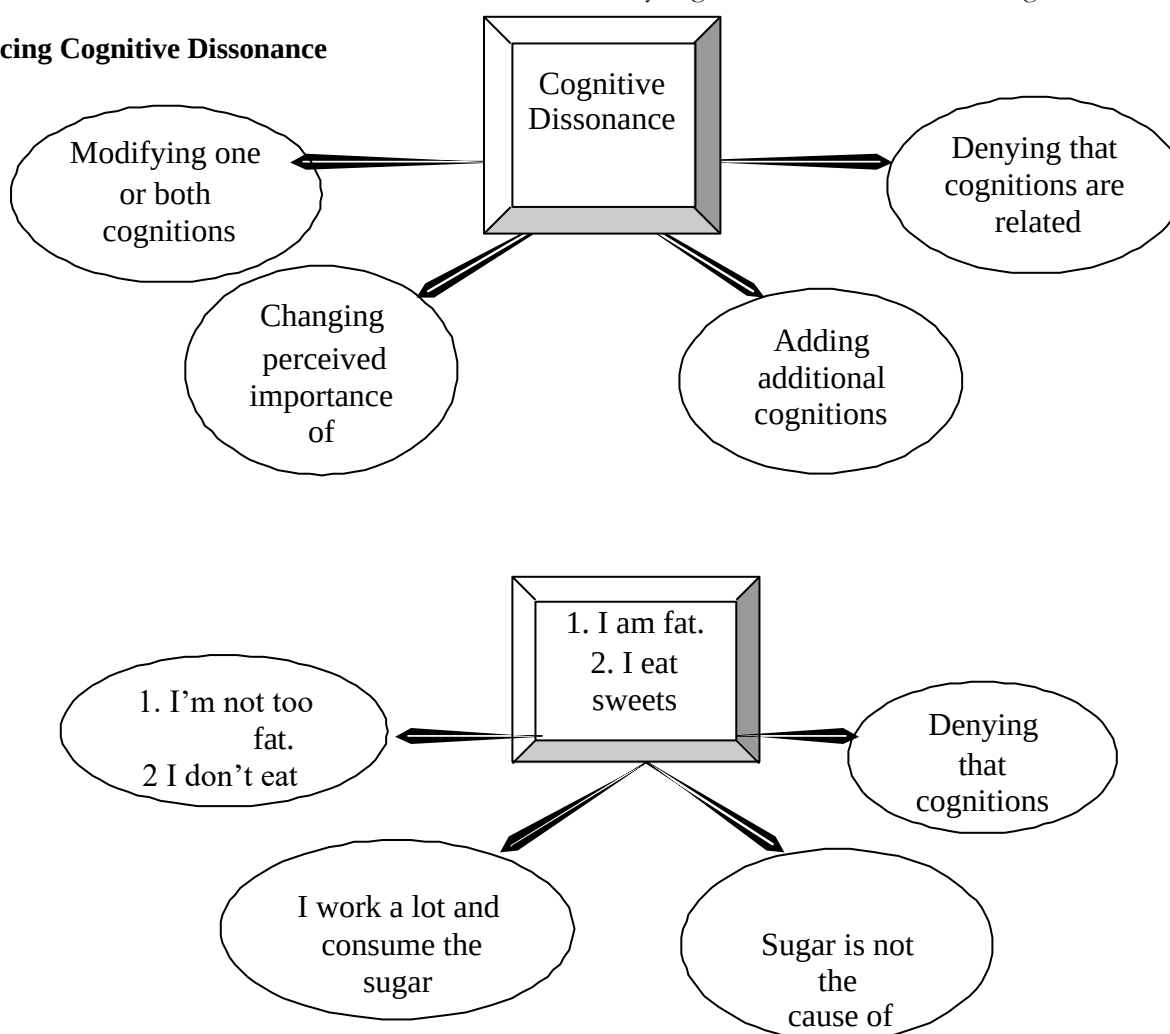
Peripheral Route Processing

Interpretation of messages involving consideration of the message source, and related general information, instead of considering the message itself.

Cognitive Dissonance

- Cognitive dissonance is a conflict experienced by a person.
- This conflict arises when an individual holds two contradictory cognitions i.e., attitudes or thoughts.

Reducing Cognitive Dissonance



Social Cognition

The processes that underlie our understanding of the social world.

- We use social cognitions to understand and make sense of others and of ourselves.

Schemas

- Research shows that we have highly developed schemas.

Schemas are sets of cognitions about people and social experiences.

Impression formation

The process by which an individual organizes information about another individual to form an overall impression of that person.

Central traits

The major traits considered in forming impressions of others.

Theories of social cognition describe how people form an overall impression other's personality traits.

Attribution Processes

- The processes of attributing causes to other people's behavior

Attribution Theory

- The theory of personality that explains how we decide, about the specific causes of an individual's behavior on the basis of samples of that person's behavior.

The Nature of Perceived Causes of Behavior*Situational causes*

A perceived cause of behavior that is based on environmental factors.

Dispositional causes

A perceived cause of behavior that is based on internal traits or personality factors.

Biases in Attribution*1. The fundamental attribution error*

People have a tendency to attribute the behavior of other people to dispositional causes; they give less importance to the situational causes.

2. The halo effect

- If our initial perception of a person is positive then we tend to expect that the person has other uniformly positive characteristics too.

3. Assumed similarity bias

- We have a tendency to think that others are similar to us. This may happen even when we see someone for the first time.

Social Influence

The process through which our behavior is affected by the actions of another individual or a group.

- Social influence can be seen in terms of:
- Conformity,

- **Obedience,**

Conformity

- *Conformity is going along with people.*
- *People are said to be conforming when their behavior or attitudes change as a result of a desire to follow the beliefs or standards of other people.*
- *The following variables may determine whether or not one will conform:*
 - *The characteristics of the group*
 - *The situation in which the individual is responding*
 - *The kind of task*
 - *Unanimity of the group*

Obedience

- *It is a form of conforming behavior that results from the commands of others.*

Compliance

- **A form of conforming behavior that results from direct social pressure.**
- **Different tactics are used by lay people, organizations, and salespersons to make others comply:**
 - **The foot-in-the-door technique,**
 - **The-door-in-the-face technique,**
 - **The that's-not; all technique,**
 - **The not-so-free sample.**

Prejudice and Discrimination**Prejudice**

Prejudice refers to the negative or positive expectations about social groups and their members.

Discrimination

Negative behavior toward members of a particular group.

Prejudice and discrimination may result from stereotypes:

Stereotype

A kind of schema in which beliefs and expectations about members of a group are held simply on the basis of their membership in that group.

Stereotypes can be positive as well as negative

Ingroup -Outgroup Bias

The tendency to hold less favorable opinions about groups to which we do not belong, while holding more favorable opinions about to which we do belong.

Out groups

Groups to which people feel and believe they do not belong.

In-groups

Groups to which people feel and believe they do belong.

Self-fulfilling prophecy

An expectation about the occurrence of an event or behavior that increases the likelihood that the event or behavior will happen.

HEALTH PSYCHOLOGY

- **The branch of psychology that focuses upon the role of psychological factors in the development and prevention of illness, in coping with disease, and in health promotion.**
- **The branch of psychology that studies the relationship between psychological variables and well-being of a person.**

According to Brannon and Fiest (2000)

- **Health psychology is the branch of psychology that concerns individual behaviors and lifestyles affecting a person's physical health.**
- **Health psychology includes psychology's contribution to the enhancement of health, the prevention and treatment of disease, the identification of health risk factors, the improvement of the health care system, and the shaping of public opinion with regard to health.**

A health psychologist works in the following areas:

Enhancement of health
Prevention of disease
Treatment of disease
Identification of risk factors
Improvement of the health care system
Shaping public opinion regarding health

Understanding Health

In order to understand the modern concept of health, we need to consider some questions:

- **How would you define health?**
- **Who is a healthy person?**
- **Can lifestyles be healthy and unhealthy?**

According to the World Health organization (WHO):

HEALTH is “ a state of complete physical, mental, and social well – being and not merely the absence of disease or infirmity”.

•The modern approach towards health is more prevention - oriented rather than treatment – oriented.

Emergence of Health Psychology

- **American Psychological Association established its Division 38, Health Psychology, in 1978.**
- **This was the formal establishment of health psychology as a separate discipline.**

Facts leading to this newer definition and approach

- a. The leading causes of death today are no more infectious diseases, but lifestyle – related diseases: implying that they are preventable.
 - b. The escalating costs of health care have led to the realization that these costs could be controlled, even reduced, by preventive strategies.
 - c. The dissatisfaction with the existing medical model has been on a constant increase.
- Dissatisfaction is primarily based on the observation that a person, even after taking proper medication and receiving full medical care may still not be feeling healthy.
 - This implies that something else may also be contributing to one's well-being.
- d. Research on the role of psychological factors in health and illness:**
- **It was observed that psychological factors have a role in physical illness.**

- **Also, psychological interventions are helpful in altering physiological conditions and symptoms.**
- **Today there is a growing consensus that health is essentially an individual responsibility’.**

No community or state can give health BUT the required facilities.

No improvement in the health care system can be efficacious unless the citizens assumes responsibility for their own well – being.

Different psychological theories and models have tried to explain health – related beliefs and behaviors.

- The Health-Belief Model
- The Protection Motivation Theory
- The Theory of Planned Behavior
- Self – Regulatory Model
- Self – Efficacy Theory

All explanations agree on one point i.e.

A person’s health, as well as the feeling of being healthy, depends to a large extent, on how much control does one feel over one’s life, circumstances and health.

Health or a State of Complete Well-Being has to be earned and maintained by the Individual himself, who must accept this Responsibility

Some Psychological Variables Affecting Health

Health Locus of Control

Who is responsible for an individual’s degree of health or illness? ...The doctors, family/ friends, fate, or the person himself?

Perceived Control

The tendency to feel or experience responsibility about the personal health related acts and behaviors, as well as the ability to get the desired outcomes.

Self Perceptions of Wellness

The individual’s personal feeling and evaluation of experiencing well-being, especially in comparison with others. •Wellness dimensions include; emotional, intellectual, physical, social, spiritual, and general dimensions.

Learned Helplessness

Proposed by Martin Seligman.

It is a belief that one has no control over the environment.

People conclude that unpleasant or aversive stimuli cannot be controlled.

This pattern becomes so ingrained that they do not try to remedy the aversive circumstances even if they actually can exert some influence.

Health and Principles of Behavior Change

- **Classical conditioning**
- **Operant conditioning**
- **Observational learning**

Classical conditioning

- **Many health-related behaviors are a result of classical conditioning:**
- **Fear of going to a doctor or dentist**
- **Fear of surgical procedures**
- **Fear of swallowing medicine**
- **Bulimia and Anorexia Nervosa: Fear of gaining weight after eating even in small amounts.**

Operant conditioning

- A number of health-related behaviors depend on operant conditioning:
- Children tend to over-eat when mothers encourage them for eating.
- Teenagers may begin drug abuse or smoking when peers dare them to try a drug or a cigarette.
- Many people exercise regularly because others appreciate their physical fitness.

Observational Learning

- Drug addiction usually results from observational learning.
- People whose peer's exercise regularly tend to indulge in regular exercise.
- Children may go for surgical procedures without fear if they observe other children doing the same.

Stress, Health, and Illness

- *Stress* is the response of an organism to events that are threatening or challenging.

STRESSORS: circumstances that produce threats to our well-being (e.g. death of a loved one, marriage, exams, illness).

- *Uplifts:* Minor positive events that make one feel good e.g. seeing an old friend, feeling healthy, completing a task.
- *Daily Hassles* are also a source of stress.

The Nature of Stressors

There are three general classes of events that can be seen as stressful:

- A. Cataclysmic Events
- B. Personal Stressors
- C. Background Stressors

A. Cataclysmic Events

Strong stressors that occur suddenly affecting many people at once e.g., natural disasters. Once they are over people can look forward to the future, knowing that the worst is behind them. But at times the victims may experience POST-TRAUMATIC STRESS DISORDER i.e., Re-experiencing the original stress event and associated feelings in flashbacks or dreams

B. Personal Stressors

• Major life events that have immediate negative consequences, which generally fade with time (e.g. death of a loved one, loss of job, broken love affair).

C. Background Stressors

Daily hassles causing minor irritations but have no long-term ill effects unless they continue or are compounded by other stressful

Manifestations of Stress

- A. **Psychological:** tension, anxiety, depression, and helplessness.
- B. **Physiological:** Psychosomatic disorders i.e., medical problems caused by an interaction of emotional, psychological, and physical difficulties (allergies, aches and pains, flu).

Impact of Stress

a. Direct Physiological Effects

- High blood pressure

- **Decreased functioning of the immune system**
- **Elevated hormonal activity**
- **Psycho physiological conditions**

b. Harmful Behaviors

- **Increased unhealthy behaviors e.g. smoking, alcoholism, and/or drug abuse**
- **Poor nutrition**
- **Sleep disturbances.**

c. Indirect Health-related Behaviors:

- **Poor compliance with medical advice.**
- **Delayed medical consultation and diagnosis.**
- **Avoiding going to a doctor**

Stress and Illness

Stress and Recovery from Disease

- **The patients' emotional response may partially determine the course of their disease.**
- **A positive state of mind has been found to be associated with longevity of life.**

Cancer and Stress

Research evidence supports the hypothesis that stressors have a role to play in the onset of cancer.

Smoking

Cigarette smoking has been found to be associated with stress, which in turn is associated with heart disease, cancer, and a number of other diseases

Stress and Recovery From Disease

The patients' emotional response may partially determine the course of their disease.

A positive state of mind has been found to be associated with longevity of life.

Measuring Stress

- **Major life events may be used as indicators of the stress one experienced.**
- **The Social Readjustment Rating Scale by Holmes and Rahe is one of the assessment tools that can indicate the stress related risk that one may be facing.**

Understanding Stress

General Adaptation Syndrome Model

A model explaining the course of stress. Proposed by Hans Selye.

It suggests that a person's response to stress consists of three stages:

- a) **Alarm and mobilization stage:** a person's initial awareness of the presence of a stressor.
- b) **Resistance stage:** coping with the stressor.
- c) **Exhaustion stage:** failure to adapt to a stressor leading to physical, psychological, and emotional problems.

Coping With Stress

The efforts to control, reduce, or learn to tolerate the threats that lead to stress.

Strategies for Coping with Stress

Psychodynamic view

Defense Mechanisms: unconscious strategies people use to reduce anxiety by concealing its source from themselves or others. COPING CAN BE

a. Emotion-focused

b. Problem-focused

Coping styles (hardiness) and social support play a significant role.

Practical Strategies for Coping with Stress

1. Turn threat into challenge.

2. Make a threatening situation less threatening.
3. Change your goals
4. Take physical action.
5. Preparing for stress before it happens i.e., stress inoculation.

Psychological Interventions for Helping People cope with Stress:

- Relaxation techniques
- Biofeedback
- Behavior therapy
- Cognitive therapy
- Hypnosis
- Meditation

Behavioral/Psychological Factors in Major Disorders and Risky Behaviors

Coronary Heart Disease

Cancer

Diabetes

AIDS

Hepatitis

Accidents

Obesity

Eating Disorders

Addictive behavior

INDUSTRIAL/ORGANIZATIONAL PSYCHOLOGY

- **The branch of psychology that investigates the psychology of the workplace.**
- **Industrial/organizational psychologists use the scientific methods and knowledge for studying the affects, cognitions, and behaviors of people in the work settings.**

Major focus of interest

How best to fit the right person to a given job.

How best to fit the job to the person

How best to fit the right person to a given job

What does “doing a good job,” mean?

Personnel selection: How to select people who do the job well?

Training: How to train them so that they do the job well?

Motivation: How to motivate them so that they do the job well?

How best to fit the job to the person?

Quality of work life

Job satisfaction

Worker safety

Fitting the right person to the job

It involves the following:

a. Job analysis

b. Personnel selection

c. Personnel training

d. Worker’s motivation

a. Job analysis

- **The first step in selecting the right person for a job is to do a job analysis.**
- **Job analysis is to prepare a specific description of a job. It encompasses the qualities and behaviors required of a person to do the job properly.**
- **It is “the systematic study of the tasks, duties, and responsibilities of a job and the knowledge, skills, and abilities needed to perform it” (Riggio, 1990).**
- **The purpose of job analysis is to find the best person for the job.**
- **For this we have to be very clear about the requirements of the job.**
- **Also, these requirements have to be translated into specific measurable behaviors.**

A complete job analysis is a two-step process

Step 1

Preparing a detailed description of what a person to be selected for a particular job is expected to do.

- **The job analysis has to be specific.**

- **Instead of stating general duties it should describe actual behaviors that the person has to perform.**
- **In general the duties and the responsibilities have to be specified.**

Step 2

- **Determining *the performance criteria* needed for the proper performance of a job.**
- **The specified duties and responsibilities have to be translated into measurable personal characteristics.**
- **Here, the Industrial/Organizational psychological has to outline the exact behaviors and characteristics that a person ought to have in order to perform the job in the best possible manner.**

‘Hard’ Criteria and ‘Soft’ Criteria

Hard Criteria

These are the objective criteria.

These criteria are obtained from the available data e.g. salary, number of units sold, number of absentees etc.

Soft Criteria

These are the subjective criteria.

Soft criteria have a personal touch and require a degree of judgment i.e., sense of humor, congeniality, creativity etc.

- **For example the best student of your college may be selected on the basis of her grades, or her interaction with fellow students, or both i.e., soft as well as hard criteria.**

b. Personnel Selection

- **Once job analysis is complete, the next task is to select the right person for the job.**
- **Personnel selection includes:**
- **Devising ways of selecting the best applicant.**
- **Making decisions regarding retention.**
- **Making decisions regarding promotion.**
- **Making decisions regarding termination.**

Personnel Selection: Functions of an Application Form

- **It can be a rough screening device.**
- **It can supplement, or provide cues for, interviewing.**
- **The information contained in the application form may be used as a predictor of future performance e.g. academic record and job history can be indicative of a person’s ability and potential.**

Personnel Selection: Employment Interview

- **Employment interviews can be structured or unstructured.**
- **Structured interviews are preferred over the unstructured interviews.**
- **These consist of carefully phrased, prescribed, uniform, and fixed-ordered questions for all**

applicants.

- Structured interviews are considered more valid than the unstructured ones.

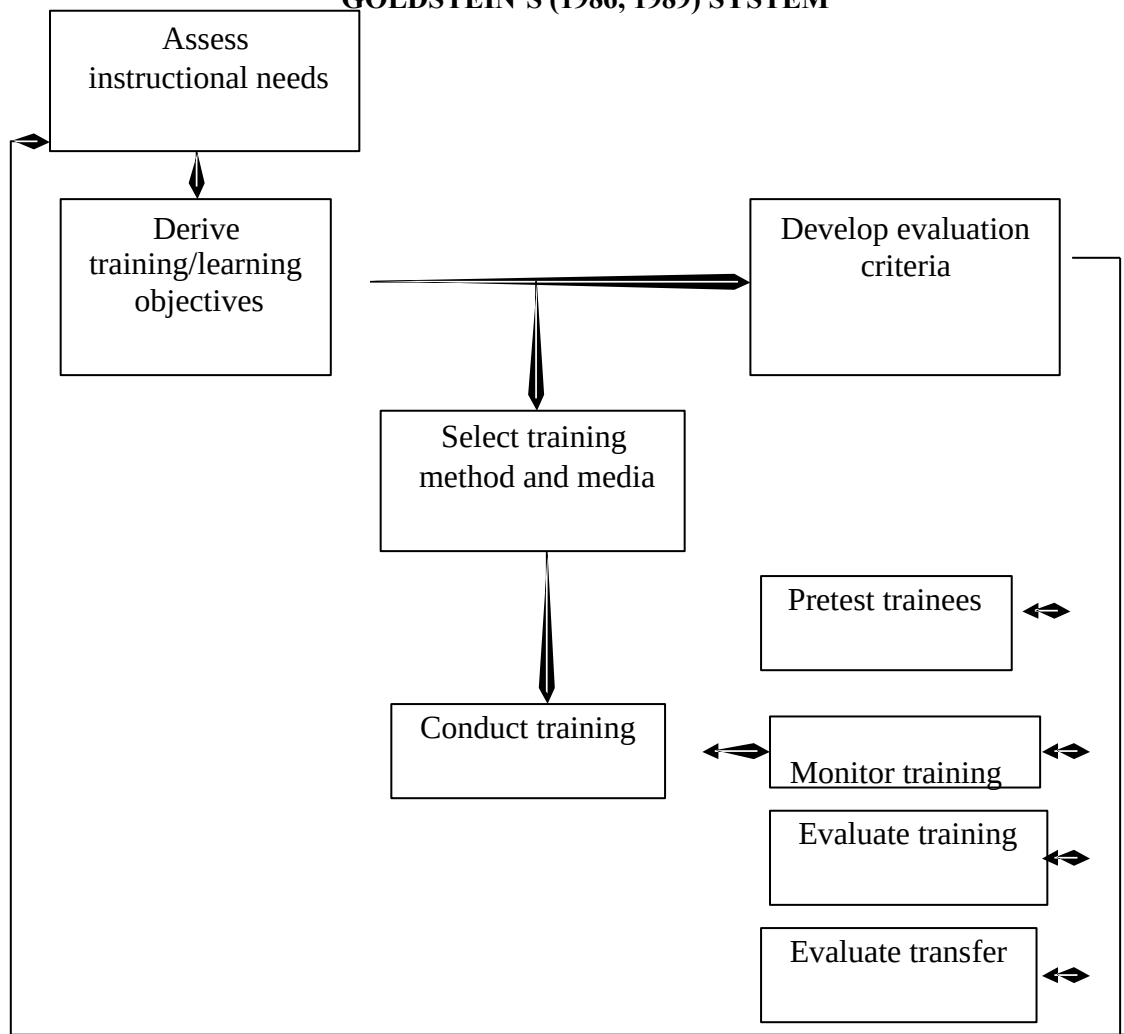
Personnel selection: Use of Psychological Tests

- At times the data obtained through application form and interviews may need to be supplemented by psychological assessment.
- Intelligence, ability, aptitude, achievement, or personality tests may be used.
- The tests of cognitive functioning (e.g. ability or achievement) have been found to be most useful.
- The use of I.Q tests for screening purposes is an issue of dispute.

c. Training the Selected Personnel

- Proper training is a requirement and a partial guarantee that the selected person will do the job well
- Training refers to a systematic and intentional process of altering the behaviors of employees to increase organizational effectiveness (Gerow, 1997).

ASSESSING TRAINING NEEDS GOLDSTEIN'S (1986, 1989) SYSTEM



Evaluating training Effectiveness

- It can be done in various ways;
 - Taking trainees' ratings
 - Assessment by the organization i.e., measuring effectiveness with reference to training objectives

d. Workers' Motivation

- Workers' motivation affects efficiency and productivity of the organization.
- A team comprising unmotivated workers will not be able to attain the desired goals.

Motivation to Work

- For an organizational psychologist, what motivates a person to carry on or not his work is much dependent on three explanations. They are;
- Need theories
- Cognitive theories and
- Reinforcement theories

Need theories are primarily based upon Maslow's hierarchy of needs in which the most basic needs are easily fulfilled while complex needs may be difficult to meet or may remain unfulfilled.

In order for higher order needs to be met; the basic and lower level needs have to be met first.

- When this theory relates to work, it maintains that initially workers are more concerned with salary and job security but when these requirements are met, and then they move towards the more sophisticated and complex needs.
- In this way workers strive to fulfill their requirements by doing work

Cognitive Theories of Work Motivation

- Theories that focus on the cognitive aspects of motivation; i.e. about the job and the work place as the motivational force
- How people think, feel, understand and expect about the job are the cognitions that affect motivation.

Expectancy Theory

- Workers make logical choices to do what they believe will result in their attaining outcomes of highest value.

Equity Theory

- Workers are motivated to match their inputs and outcomes with those of fellow workers in similar positions.
- This can be assumed to have a basis in vicarious learning.

Reinforcement Theories of Work Motivation

- The theory is based on the learning principles and maintains that motivation is increased or decreased by the level and type of reinforcement that is given to the workers
- Positive reinforcement increases in production or optimal production
- Punishment suppresses motivation.

- **No reinforcement discourages motivation.**

Goal setting as a motivational technique

- **Setting goals can motivate workers:**
- **Set difficult but achievable goals.**
- **Goals should be specific and focused rather than general and vague.**
- **Feedback regarding achievement of goals should be regularly provided.**
- **Employees should be aware of the specific goals, and should accept them as reasonable.**
- **Cultural concerns should be kept in mind.**

In collective cultures, where working together is practiced, involvement in goal setting is more important than in individualistic cultures.

Fitting the Job to the Person

- **Molding the job and the workplace in such a way that workers put in optimal efficiency and productivity.**

Job Satisfaction

- **An attitude toward or a collection of positive feelings about, one's job or job experience**

Factors of Job Satisfaction:

- **In job satisfaction, decision- making is very important.**
- **Usually two types of decision- making may take place:**
 - **Decentralized decision- making and**
 - **Centralized decision- making**
- **In decentralized decision- making the power extends throughout the organization**
- **In centralized decision- making power and authority rests in the hands of just few individuals at the top**
- **Job satisfaction is higher in organizations following decentralized decision-making.**

Some Facts about job Satisfaction

- **Younger workers tend to be more dissatisfied with their job.**
- **Older workers are more jobs dissatisfied by the end of their careers.**
- **There are no significant gender differences in terms of job satisfaction.**
- **There is a positive relationship between job satisfaction and perceived level or status of one's job.**

Nature and Type of Job and Job Satisfaction

- **It depends on two things**
- *Job clarity*

The rules and what is being expected of the person from the particular job are explicit to the worker.

- *Role conflicts*

It has a negative effect on job satisfaction.

It arises when the person is unable to perform the job adequately and optimally because the roles and responsibilities are not clearly defined.

- **Role conflict may make it easier to hide mistakes.**
- **But it also leaves little room for one's contribution being clearly identified.**

Ways of Enhancing Job Satisfaction

- **Industrial and organizational psychologists are concerned with ways of enhancing job satisfaction, as it is beneficial both for the organization and the workers.**
- **There can be three main approaches to affecting job satisfaction:**
- **Changing the job**
- **Changing the person in the job**
- **Matching the person to the job**

Worker Safety

- **Job satisfaction can be improved if the organization is concerned about the safety and security of the workers, and the workers are also aware of the fact.**
- **Practical and explicit steps toward maintaining employees' protection have a positive effect.**

CONSUMER PSYCHOLOGY

Try to find answers to these questions

- Why do manufacturers advertise their products?
- Why do advertisers use models in the advertisements?
- Why do advertisers use cartoons rather than human models in the advertisements of children's products?
- Why do manufacturers introduce only one new model in a year?
- How will we decide what to buy if there were no advertisements?

Consumer Psychology

- The branch of psychology that studies consumers' buying behavior and the effect of advertisement on these behaviors.
- Consumer psychology focuses upon consumers' decision making and their behavior in the market place.
- The effect of advertisement on people's attitude and buying habits is an area of special interest for a consumer psychologist.
- Consumer psychology is division 23 of apa.

Focus of Interest

What Do Consumer Psychologists Study?

- The responsive attitude of humans towards the product and service related information and experiences.
- Responses such as emotions, feelings, attitudes, beliefs and judgment as well as purchase decisions and consumption practices.
- Information about the product and the related service information are very important.
- Variables such as advertisements, product packaging, package labels, coupons, consumer magazines, and word- of - mouth communication with the near ones that affect consumer behavior.
- Also brand loyalty, product preference, price, marketing and selling strategies are

Aim or Goal

- The main aim or goal of consumer psychology is to describe, predict, influence, and/ or explain consumer responses.

Consumer Psychologist

- Consumer psychologists are the psychologists who are educated and trained in understanding consumer habits and the influence of advertisements on consumers' attitudes, thoughts and behavior.

Other tasks of consumer psychologist

- Consumer psychologists are educators, trainers, researchers as well as administrators.
- Understanding and predicting consumer behavior is not an easy task.

- Even the experienced salesmen are not able to accurately read the consumer's mind.
- The main task of a consumer psychologist is to conduct consumer research.

History of consumer psychology

- j b. watson was the first ever-prominent psychologist to apply principles of psychology in the field of advertising.
- watson believed that psychology could not be recognized as a scientific discipline until its practical utility is proved by its application and demonstration in real life situations.

According to watson

"If psychology would follow the plan i suggest, the educator, the physician, the jurist, and the businessman could utilize our data in a practical way."

he himself designed ads for johnson and johnson's baby powder.

in that ad, he not only targeted the emotions and anxieties of mothers, but also used the experts' recommendations and the impact of using the product.

Focus of interest

- The main focus of interest of consumer psychology is:
- To emphasize the consumer's problems, likes, dislikes and preferences
- Develop strategies for making ads, influencing purchasing decisions, innovative marketing, brand and product techniques.

Sub- fields of consumer psychology

- Advertising
- Perception
- Life stages: time, experiences and preferences have a physical as well as psychological effect.
- Motivation
- Psychology of price
- Market research

Important terminology of consumer psychology

- Consumer behavior
- Personality
- Sales
- Marketing
- Product choice
- Product preference
- Brand loyalty

Learning principles used for attracting consumers

Classical conditioning

Advertisers use models to create a positive feeling and attraction for the product.

Operant conditioning

Manufacturers announce prizes, prize coupons, and extra amounts of product, upsized products at same price, price reductions, and lucky draws for consumers.

Observational learning

advertisements show renowned people using the product under question so that potential consumers feel like following their footsteps.

Some factors affecting product preference

- personality type and brand loyalty
- peer pressure
- product price
- reputation of the product
- packaging: color, wrappers, shape, size, captions etc
- quality of the product
- advertisement
- need

Advertising appeals

Advertisements are to develop a positive image about the product by using attractive models and positive feelings like happiness, excitement, curiosity, and desire.

Two main techniques are used to make advertisements:

- i. soft sell appeals
- ii. hard sell appeals

i. Soft sell appeal

An approach used to selling the product in which the softer aspects of the product are promoted. the product is associated with an image related to the product's use.

The soft sell delivers the message that the potential buyers will obtain or radiate a desirable image through the use of the product.

The purpose is to attract the potential buyer through the peripheral route.

Here the image rather than the product is sold.

Examples of the soft sell appeal

- **Advertisements promoting cosmetics talk about the beauty of the model.**
- **As of men's products deliver messages about the manliness of the male model, or how people are impressed by his masculinity e.g. advertisements of cigarettes.**

ii. Hard sell

The hard sell approach is the opposite of the soft sell appeal.

It focuses on the qualities of the product itself.

The advertisers promote the function of the product

Qualities like taste, smell, speed, durability, efficiency, dependability, and/or its nutritional value are emphasized.

It is stressed as to how much improved the consumers' lives will be because of using the product being advertised.

Examples of the hard sell appeal

Advertisements of automobiles, joggers, mechanical instruments, exercisers, metal products, air conditioners etc.

Research findings about advertising appeals

- **Children's products are sold through soft sell appeal.**
- **In case of women's products soft sell appeal is used in advertisements of cosmetics, toilet soaps, and garments.**

- **But in case of women's perfumes or shoes hard sell appeal is preferred.**
- **Advertisements of household linen and paints use the hard sell appeal.**

Self- monitoring and buying behaviors

Self monitoring has been found to be an important variable in one's buying behavior and experimenting with new products.

Self monitoring is the tendency to consider our own behavior and changing it in order to present us well in particular social situations

High self- monitors are flexible in nature and adjust their behavior from situation to situation in an attempt to present them in an effective manner.

They are concerned about the image they project and are apt to show conspicuous variations in their behavior from one social context to the next

In contrast, low self- monitors are relatively more "fixed" in their general attitude.

They are much less sensitive to the demands of different social settings, and their behavior is more indicative of their attitudes, values and beliefs.

The behavior of low self- monitors is considerably more consistent across situations, as compared to that of high self- monitors

High self-monitors are affected more by advertisements in general and soft sell appeal in particular.

Psychographics: investigating the psychology of the consumers

When the manufacturer launches a product, or the advertiser plans a campaign, they have to be aware of the needs of the potential buyers.

Research has shown that people belonging to different backgrounds and different socioeconomic classes have different buying needs and behaviors.

It has been identified that people with different lifestyles and needs go for different varieties of products.

Researchers have used different psychological profiles of people based upon their lifestyles to predict and target their needs.

Psychographics is a technique for dividing people into life style profiles that are related to purchasing patterns.

Such profiles take into account characteristics as marital status, race and ethnic background, and educational level, as well as the kinds of activities that potential buyers engage in.

One of the most popular and widely used general classification systems for consumers is VALS or values and lifestyles (Mitchell, 1983).

the VALS approach divides consumers into four major groups.

VALS nine American lifestyles

Need driven consumers include?

Survivors

Sustainers

Outer- directed consumers include

• belongs

emulators

achievers

Inner- directed consumers include

i- am- me

experientials

Societal conscious

Combined outer- and inner- directed group

integrateds

research findings show that members of particular subcultures will have distinct purchasing patterns.

Different groups have significant differences in values and interests, besides the socio- economic variations.

•awareness of these differences enables the sellers to attempt to target products and advertisements toward certain groups.

a. Need- driven consumers

they make purchases primarily to satisfy basic, elemental needs.

Outer- directed consumers:

they make purchases in order to increase their own self- awareness.

the outer- and inner- directed consumers have similar social and self- needs

like maslow's hierarchy, the first few levels in the vals hierarchy classify groups of individuals whose needs are largely related to two particular lifestyles,

The survivors and the sustainers

both of these are small, impoverished groups whose basic preoccupation is to simply 'living'.

these groups have limited material resources.

the range of products that are advertised through advertisers is quite limited for the sustainers and the survivors.

the manufacturers, sellers, and the advertisers are keener to work upon the groups that have the financial means to purchase the products that they are there to sell.

there are two other categories:

the i- am-me's (young, egocentric people confused about their goals in life).

the experientials (former i-am- me 's who become surer of their goals and more outward looking) and

socially conscious (mature, well informed individuals with a strong interest in social issues such as politics or the environment).

Psychographic profiles can be used for many reasons

- **Identifying specific needs of people coming from specific backgrounds i.e., where to sell tissue papers, and where to paste the advertising posters.**
- **Also, these help in deciding the medium of advertising.**
- **For example, in areas without electricity tv ads would be meaningless.**
- **Instead radio could be an apt medium since most people will be listening to a transistor.**

Sub cultural differences in consumer behavior

consumer research has revealed a lot about the understanding of differences that exist between sub groups within the general population.

the way different groups make decisions regarding purchase are different e.g. in our culture taking loans for buying household goods is generally not considered desirable.

studies on sub cultural differences in consumer behavior have revealed that clear differences exist between different racial and ethnic groups.

Consumer cognitive processing: decision making in the marketplace

how do you decide which product to buy, which brand to stick to, and which new brand or product to buy?

Micro marketing is a technique of targeting marketing efforts to particular buyers on a neighborhood or even individual store level.

Problem recognition:

the first process in the model is to recognize that some types of needs exist.

in the case of the cell phone that continually breaks down, the need is to buy a new one. in the other cases, such as buying a music system, the need is not fully clear, and a certain amount of cognitive efforts is required before one can make decision.

Information search:

once the problem is clear; the consumer gathers relevant information through different sources.

Evaluations of alternatives:

different alternatives gathered after research are compared and contrasted.

their pros and consequence are evaluated.

Brand beliefs:

consumers also make decisions regarding their brand beliefs and brand loyalties.

brand beliefs are the assumptions held by consumers of a product, based on their knowledge of the reputation of the manufacturer.

prior experience with a brand also plays a role in taking a decision.

Purchase

finally the purchase is made.

when purchasing a product the consumer tries to keep the risk to the minimum.

he may seek a guarantee or assurance.

post purchase evaluation

Last stage in the decision buying process

In this stage, we assess the decision to buy, including the choice of the product.

Advantages and qualities are also evaluated.

In case of an expensive choice, the consumer may experience cognitive dissonance

Cognitive dissonance reduction strategies may be used here

- **However if the re evaluation suggests that in spite of being expensive the product was worth buying, then there may be no cognitive dissonance.**

SPORT PSYCHOLOGY

Have You Ever Thought About These Issues?

- Why don't cricketers show a consistent performance?
- Why do they come back to the pavilion when we expected them to make a century?
- Why do sportsmen end up with a sprain or a pulled muscle when they had to play a crucial match?
- Why do spectators cheer up their favorite team and hoot the other one? Why do some players become aggressive, agitated, irritable, and even refuse to play when hooted or teased by the crowd?
- The answers to these questions may be found in sport psychology.

Sport Psychology

- Exercise and sport psychology is Division 47 of APA.
- The branch of psychology that studies, understands, describes, and predicts the impact of psychological variables on athletic and sport performance.

Sport psychology is the application of the principles, knowledge, training, and understanding of psychology for the understanding of factors affecting sport performance, with an aim to improve it, and to make the sportspersons feel stronger and more confident.

- “The application of psychological principles to sport and physical activity at all levels of skill development” (Brown and Mahoney, 1984).

History of Sport Psychology

- Although man was always interested in sports, in improving sport performance, in sport training, and in negatively affecting the performance of the opponent's team, the history of scientific research is not very old.
- Research in psychological aspects of sport originated in Europe and then flourished in the U.S.
- The earliest proper sport research was reported by Norman Triplett in 1897.
- He analyzed the performance of cyclists under conditions of social facilitations.
- For this purpose he used field observation and secondary data.
- He reported from his research that the presence of other competitors could facilitate better cycling performance.

Coleman Roberts Griffith

- Known as the father of Sport psychology in North America.
- He established the first sport psychology laboratory at the University of Illinois in 1925.
- He studied the nature of psychomotor skills, motor learning, and the relationship between personality variables and physical performance.

The formative years of Sport psychology

- The period from 1950 to 1980 is considered as the formative years of sport psychology.
- Sport psychology emerged as a separate discipline distinct from exercise physiology and motor learning.

The Subject Matter of Sport Psychology

- **Education**
- **Training**
- **Research**
- **Sport performance**

Psychological Characteristics of Athletes

- **One of the areas of special interest to the sport psychologists is the study of personality characteristics of athletes and sportspersons.**
- **One could predict sport performance and choose good athletes if one knew the psychological indicators of sport performance**
- **One has to be clear about the different connotations of the words ‘athletes’ and ‘sportsmen’.**
- **Although the two terms are used differently in everyday life vocabulary, to a sport psychologist these two do not mean much different.**

Some Research Findings

- **Although not very strong empirical evidence is available on this issue, some interesting facts are available in research literature.**
- **As compared to the non athletes, the athletes usually score higher on tests of: assertion**
- **Dominance**
- **Aggression, and**
- **Need for achievement**

Athletes score lower on

- **Anxiety level**
- **Depression, and**
- **Fatigue.**
- **These findings stand truer when the athletes are at a high skill level.**
- **Athletes in some sports e.g. hockey and football are more tolerant of pain as compared to athletes in other sports e.g. bowling or golf.**
- **But there is some evidence suggesting that the pain tolerance may be an outcome of an athlete’s success rather than a cause.**

Enhancing and Maximizing Sport Performance

- **One of the major tasks of a sport psychologist is to assist the trainers, the coaches, and the team managers as well as the sportspersons themselves in raising the level of performance.**
- **In this regard, a major portion of research evidence pertains to the arousal level of the sportsperson.**
- **Sport psychologists have also worked in the following areas:**
- **Enhancing motivation**
- **Learning relaxation techniques**
- **Stress management**
- **Pain control and management**
- **Overcoming anxiety**

- **Enhancing stamina**
- **Improving performance**

Arousal level

- **Sport psychology suggests that sport performance can be improved by manipulating arousal level of the**
- **one playing in the field.**
- **Arousal level has been found to be related with motivation of the sports person.**

A certain level of arousal is good, in fact essential, for sport performance and competition. However, too much or too little arousal has negative effects.

What is Arousal?

- **Arousal is a neutral physiological phenomenon or state accompanied by:**
- **Increased heart rate**
- **Elevated Blood pressure**
- **Rapid respiration**
- **Increased metabolism**
- **More than usual hormone secretion**
- **The state of arousal is an active sympathetic nervous system (SNS) state.**
- **It is like a fight or flight response.**

How much Arousal is Good for Sport Performance?

- **Although no standard can be fixed for it, research shows that different levels of arousal may be involved in optimal performance on different tasks.**
- **According to Cox (1990):**
 - **Making a long putt in golf requires a low level of arousal,**
 - **Blocking a shot in volley ball requires a slightly higher level,**
 - **Making a tackle in football an even higher level, and**
 - **A bench press in weight lifting requires a very high level of arousal.**

Arousal Level and the Role of a Sport psychologist

- **A sport psychologist makes the sportspersons aware of, and sensitive to, their arousal level.**
- **They are trained to maintain arousal at a certain appropriate level in a manner that the task being performed remains concentrated upon.**
- **The knowledge of biofeedback is applied here.**
- **They are trained in being sensitive to arousal related indicators e.g. blood pressure, respiration rate, heart and pulse rate, muscle tension etc.**
- **There is abundant evidence available that suggests, and that has proved, that human beings can gain control over even their involuntary functions.**

Mental Practice: Helping sportspersons give “Peak Performance”

- **Sport psychologists have devised strategies for helping athletes and others in reaching and maintaining peak performance.**
- **Research suggests that mental practice involving “imagery” accompanied by actual physical**

practice is helpful in learning skills, improving and maintaining performance.

Mental Practice and Sport Performance

- Rehearsing the task mentally, imagining one's self in the actual field.

Functions of Mental Practice

- The task at hand is rehearsed mentally. This provides a vivid image of the scenario to occur later.
- Negative thoughts that may interfere with performance are reduced.
- The sportsperson can rehearse his/her part in a team sport.
- It helps in setting realistic goals.

The Phenomenon of Home Field Advantage

- It is a common observation that athletes and sportspersons perform better and are more relaxed when playing in home ground.
- Psychological research does not provide a 100% solid support to this observation.
- Mixed findings are available in this regard.
- Research has shown that frenzied, yelling, screaming hometown fans may raise arousal levels of the home team beyond the point of maximum efficiency.
- This reveals the negative effects of playing on the home ground.
- One possible fallout may be that the athletes feel more under pressure as they feel they are expected to perform at their best, and no one is going to forgive a mistake.
- According to Mahoney, while commenting on Olympic athletes:

“At this level of competition the difference between two athletes is 20 percent physical and 80 percent mental”.

Mental practice can help overcome this problem.

Other strategies used by sport psychologists

Overcoming self consciousness

- The same tactics as those used for assertiveness training are used.

Principles of learning and sport performance

- Operant conditioning approach involving positive reinforcement is effective in raising motivation to performance at peak level.
- In learning a sport observational learning is most beneficial.
- Vicarious learning can take place through direct, live observation, or through video recordings.

Stress management and overcoming anxiety

- Relaxation techniques
- Good nutrition
- Developing optimism through cognitive interventions
- Self Talk

**PSY101 – Interlocution to Psychology
Merged PPTs**

Lecture 24 to Lecture 44

Final Term Syllabus

Lecture 24

Recap

- Memory
- Short- term memory

Long-term Memory

- This system stores information on a permanent or relatively permanent basis; the information thus stored may or may not be be retrieved easily

The Components of Long-term Memory

Also known as modules:

- Declarative memory
- Procedural memory
- Semantic memory
- Episodic memory

Components of Memory

Declarative Memory: The module responsible for factual data, dates, faces, names etc.

Procedural Memory: The memory center for skills and habits e.g playing cricket, driving a car etc.

Semantic Memory: Storehouse for facts and general knowledge about the world e.g historical facts, scientific formulas

Episodic Memory: Memory for information pertaining to life events, episodes, biographical details

Associative Module:

A strategy or process whereby material is recalled by thinking about related information e.g Whose son was the mogul emperor Shah Jahan?

Priming:

If an individual is exposed to a piece of information earlier, then later on it is easier for him to recall the same or similar pieces of information

Explicit and Implicit Memory

Explicit Memory :

**Recollection of memory that is intentional and conscious
e.g date of your interview, or the day when your course
started**

Implicit Memory :

**There are many pieces of information or memories that are
stored in our system but which we are not aware of at the
conscious level. These can and do affect our behavior and
performance later e.g childhood fear of blood hindering the
performance of a would be doctor**

Single Process Approach

- Some psychologists do not agree with the idea of three sequential stages of memory; they had a single process approach
- Levels-of- processing theory

Levels-of- Processing Theory

- What is most important in the process of memorizing or learning material is the the degree to which the material is mentally analyzed
- How much of new information will be remembered will be determined by the extent or amount of information processing that takes place when new material is encountered
- The information will be more and more likely to be remembered as the initial processing becomes more and more intense

Information that is paid little attention will not be processed thoroughly , will remain at the shallow level of memory, and will be forgotten soon

On the other hand information that receives greater attention is processed thoroughly ; such information enters the deeper level of memory

Shallow Level Memory :

- ❑ Physical and sensory aspects of information

Deepest Level:

- ❑ Meaning of information is considered important in analysis of information
- ❑ A wider context, rather than a limited or restricted, perspective may be taken
- ❑ Associations between what this information means, and the already existing and known broader networks of knowledge may be drawn

Recall and Retrieval of Long-term Memory

- Why do we remember certain events very vividly even after a very long time?
- Why can't we remember something that we thought we know very well?
- Are the memories that we can recall very clearly, really exact reproductions or distorted, or may be exaggerated?

Flashbulb Memories

- The memories that are as clear and vivid as a snapshot
- Such memories are of events that are important for us, are specific, or surprising or astonishing e.g an interview room, an accident, first day at class

❑ Although flashbulb memories are vivid, clear and detailed, still they may be lacking many important details

❑ At times the recall may be very different from the actual event whereas the person believes he is remembering right

❑ Many elements may be missed and many added

Tip-of-the-tongue Phenomenon

- Inability to recall events, details, or information that we thought we knew very well

Causes of the Tip-of-the-tongue Phenomenon

- Information overload
- Accumulation of pieces of similar and confusing information
- Preoccupation

RETRIEVAL CUES ARE HELPFUL

Autobiographical Memory : Another Aspect of Long-term Memory

- Autobiographical memories are recollections of past life events, circumstances, episodes, and happenings
- These memories, although they seem perfect to us, may also be distorted, exaggerated, changed

Schemas

- According to Bartlett information is remembered in terms of schemas i.e., general themes in memory that contain relatively little specific detail
- Irrelevant or unimportant details are discarded
- People's understanding of the situation, their expectations, and awareness of others' motivation affect memory and recall

Forgetting

- ❑ **The inability to retrieve or recall information from the long-term memory**
- ❑ **Forgetting may occur due to a number of factors such as;**
 - **Information discarded/ decayed due to the non-use of the learned material**
 - **At times we know something, but are unable to access and retrieve it properly**

- ❑ **Hermann Ebbinghaus, a German psychologist, was the first one to study the phenomenon of forgetting by using himself as a subject for his own experiments**
- ❑ **Used non- sense syllables ;in sets of three; one vowel in between and the two consonants at the ends e.g. FIW, BOZ etc**
- ❑ **His experiments, indicated that forgetting occurred systematically; in the initial stages (the first hour and, in all the first first nine hours) forgetting was most rapid, but then, the rate of forgetting decreased and slowed down and did not change much with the passage of time**

- ❑ **Forgetting should not be understood as a problem alone, since it has some advantages too: it helps us discard irrelevant information and enables us to learn new things**

Theories of Forgetting

- ❑ **Why do we forget???**
- ❑ **Several theories have been posed to explain this phenomenon:**
 - i. Decay theory of forgetting**
 - ii. Interference theory of forgetting**
 - iii. Repression**

Decay Theory of Forgetting

- ❑ **The oldest concept which states that stored information decays and is disintegrated with the passage of time**

❑ Criticism against the decay theory:

i) it can be taken as a general explanation of how forgetting takes place, but can not be accepted as the actual reason, i.e. the process of forgetting

ii) At times recollection of memories occurs after a long passage of time, which is an experimentally proven fact, and this theory fails to explain this phenomenon

- ❑ **As decay theory could not explain the actual process of forgetting, another theory was emerged, called interference theory of forgetting**

Interference Theory of Forgetting

- ❑ **Interference results when the recall of the learned phenomena is blocked/ displaced by other information**
- ❑ **John Jenkins and Karl Dallenbach (1924) were the first ones to experimentally prove the role of interference in forgetting**

- ❑ Experiments on students; they recalled non- sense syllables relatively well after waking from the sleep than when they were fully awake for the whole day
- ❑ maximum forgetting occurred when they were awake than when they were allowed to sleep which showed that interference of the activities and happenings has a relationship with forgetting

- ❑ Interference, however, sometimes is found to be helpful in the sense that it improves our ability to remember important events of life; not the exact and true characterization, but the important portions/ episodes
- ❑ Retrieval of the learned phenomena may be influenced by the two processes;
 - i) Proactive interference
 - ii) Retroactive interference

I. Proactive interference

- ❑ Proactive interference occurs when the already stored memories come in between or interfere with the recall of the newly learned material e.g. You learned psychology this semester and sociology in the last semester. When exam was taken in psychology , material from sociology kept coming into mind rather than that of psychology**

II. Retroactive interference

- ❑ **Retroactive interference occurs when the newly learned material interferes with or blocks our ability to recall the previously learned material**
- ❑ **In the psychology-sociology example, when examinations were taken, the memories of newly learned material (psychology) may interfere with recall of the formerly learned material (sociology)**

Repression

- ❑ **Repression: putting the undesirable thoughts, events and fears into the unconsciousness and trying not to remember it again**
- ❑ **A Freudian concept”**
- ❑ **In spite of being sent into the unconscious, unpleasant feelings continuously influence person’s attitudes and behaviors**

Improving Memory and Recall

- Elaborative rehearsal
- Mnemonics
- Method of loci
- The Keyword technique: pairing foreign word with a similarly sounding common word in native language, in order to remember the foreign word e.g pairing English “amaze” with “maze’ in Urdu

The Encoding Specificity Phenomenon

- Information is best learnt and remembered at a time and place, or environment, similar to or same as the one where it was initially learnt

Mental Organization of Text Material

- when the material is being read for the first time, it should be in the memory

Taking Notes

- Take cue notes instead of taking down everything
- Listen and think too

Other Strategies for Better Memory

- Keeping distractions away
- Chunking
- Rhyming
- Reading aloud
- Sleep intervals

Improving Memory: Memory Skills for Students

I. Over learning:

- Learning the material over and over again
- Very effective in case of natural science, mathematics or history

II. Mnemonics:

- Short devices that help encode information better. These provide ready cues for quick and accurate retrieval

III. SQ3R:

- **A five- step approach for improved learning and recall of material to be learned**
 - i. **S= Surveying and skimming**
 - ii. **Q= Questioning; asking questions about what one has read**
 - iii. **3 Rs:**
 - **Read: Going through the surveyed material thoroughly**
 - **Recite: Recitation makes learning active than passive, and adds activity to learning**
 - **Review: Revwing and going through the learnt material from time to time**

**REMEMBER!!!! THE SEQUENCE OF THESE
STEPS IS IMPORTANT**

Lecture 25

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Recap

- Long-term memory
- Forgetting
- Improving memory and recall
- Memory skills for Students

Memory Disorders/ Dysfunctions

Memory loss has long been considered as a factor of aging, but now physicians believe that there must be some pathological reasons as well that cause memory impairment;

“ Memory Cognitive Impairment (MCI)”

Although the suffering individuals are capable of doing every day tasks independently, they also heavily rely on using diaries, calendars reminders etc

Amnesia

Greek word which means “ forgetfulness”
Loss of memory with other mental difficulties
In old age, people are unable to retrieve

memories

Includes two types of amnesia

Retrograde amnesia

Anterograde amnesia

Dementia

- Severe and rare disease ,mostly affects people in 60's and the risk is doubled every 5 years after 60; at 80, one out of every five people develops dementia
- Symptoms may include repeating things several times, speech problems, intellectual problems, and inability to remember the skills learned early in life e.g dressing up, tying shoe laces, using knives and forks etc
- The sufferer is irritable, withdrawn, rude, facing anxiety, depression,suspicious attitude,and aggressive

• Alzheimer's Disease

Common cause of memory loss in elderly people

- progressive loss of memory including effect on person's language and recognition of people; also losing way, inability to plan things, difficulty in organizing daily tasks etc

- **Major symptoms include depression, lack of motivation, laziness, agitation, physical and verbal aggressiveness, recent events are forgotten; delusions and hallucinations may also occur**

HUNTINGTON'S DISEASE (HD)

Fatal disease that results in involuntary movements and cognitive impairment
Genetic disease that runs in families
It mainly affects important brain regions such as basal ganglia that have vital role in co- ordination and movement

Amyotrophic Lateral Sclerosis (ALS)

- ❑ Occur mostly in adults, and is common in men than women
- ❑ Develops in the age of 40- 70 but also occur in the twenties and thirties as well
- ❑ Effect motor neurons of the body
(that bring messages from the brain to the different muscles of the body)

Korsakoff's Syndrome

- **Severe and often permanent loss of memory**
- **Addicted/ long-term alcoholics are the sufferers of this syndrome**
- **Memory impairment occurs because of brain damage caused by thiamine deficiency and alcoholism**
- **Major symptoms; hallucinations, Impaired dietary intake, repeating information several times, and Inability to remember things**

MOTIVATION

Ask Yourself

- ☐ Why do some students want to get an “A” in every exam, and some don’t even bother about it?
- ☐ Why do the sky divers love to dive down from hundreds of feet above the ground
- ☐ Why do some people some people run after money and some refuse even the most attractive job offers?
- ☐ why do some people leave their country for earning money and some are contented and happy with whatever is available to them at home?

- ☐ **Why people become doctors, engineers, social workers, pilots, army men etc?**
- ☐ **And to ourselves, why we want to learn/ know about the particular subject?**

**THE ANSWER TO ALL THESE QUESTIONS
CAN BE EXPLAINED BY UNDERSTANDING
THE CONCEPT OF MOTIVATION**

What is Motivation?

- ❑ **Motivation is a desire, drive, instinct or need that speeds up our behavior towards some goal**
- ❑ **Psychologists are interested in observing and evaluating the factors that motivate or direct human behavior**

of motives that are
essential for the proper
human functioning

i. Primary/ unlearnt/
physiological/
biological
motives

ii. Secondary/ learnt/

I. Primary Motives; Concerned with all the biological/ physiological needs of the body. They are also known as unlearnt motives because they entirely include the basic drives such as hunger, thirst, need for sleep, air, excretion etc. which do not need any sort of learning

II. Secondary Motives; Also known as the psycho- social motives because they involve people's appreciation or appraisal in order to live successfully in any society .

- ❑ Also important in the sense that they are the means of satisfying the primary motives such as work and salary, good marks and appreciation from the parents
- ❑ These are a source of mental satisfaction as well

- ☐ The important and interesting fact about motivation is that organisms keep trying to achieve the desired goal. When the goal is achieved, the motivation regresses and when not achieved, the person keeps on trying to achieve it
- ☐ When one motive is satisfied, we start to move forward to satisfy the next need and so on
- ☐ This is how struggle continues throughout our lives

Primary/ Unlearnt/ Physiological/ Biological Motives

❑ The major primary motives are;

I. Hunger

II. Thirst

III. Fatigue and sleep

IV. Pain

V. Sex

VI. Excretion

VII. Air

VIII. Warmth and cold

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I. Hunger

- ❑ The most urgent and desirable need in all organisms including human and animals
- ❑ Eating is related to the homeostatic mechanism of the body
- ❑ Psychologists have done research on animals to observe the level of motivation which showed that internal system not only regulates the quantity of food intake but also the kind of food that has been taken

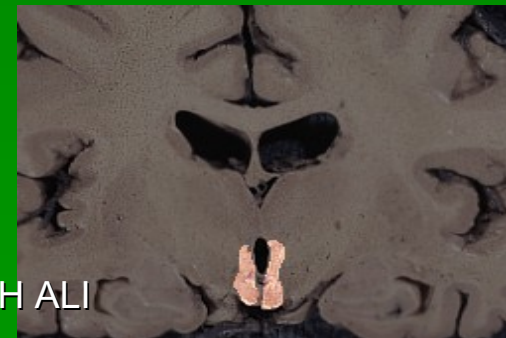
- ❑ The systems that involved in when to eat and how much to eat is a complicated phenomena
- ❑ It is a proven fact that hunger is not only related with the empty stomach; people whose stomach have been removed still experienced the sensation of it

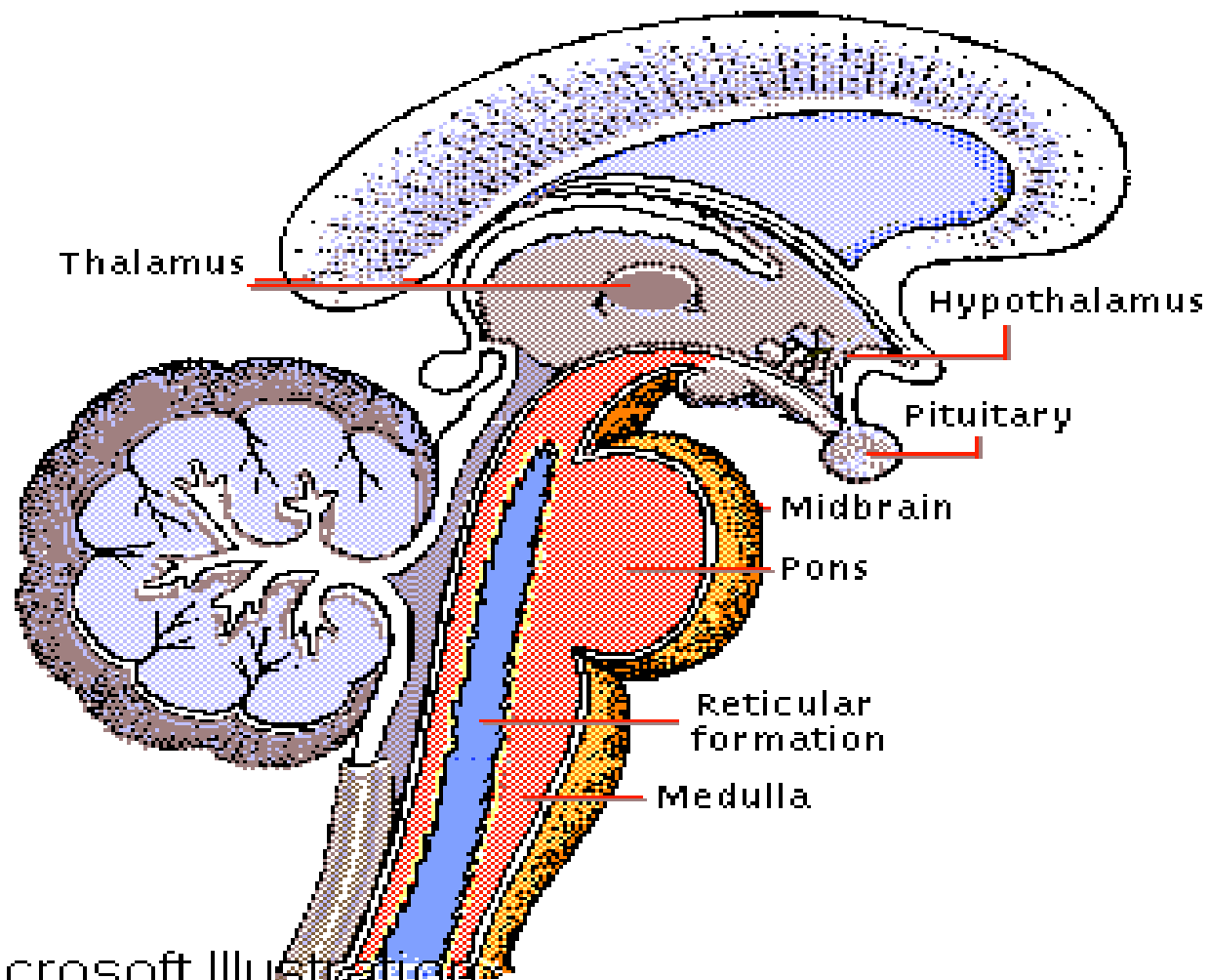
❑Larger amount of food have been taken by the animals when it contains low level of nutrients as compared to highly nutritive diet; showing that both animals and humans are sensitive about the nutritive value of food

❑ Chemical secretions in blood: chemicals/ hormones secreted by the endocrine gland also play a crucial role in the hunger drive.e.g.studies showed that when glucose (sugar) was injected in rats, they felt less hunger as compared to when they were given insulin which resulted in hunger pangs

What Role Does Brain Play in Hunger Drive

- ❑ Hypothalamus, brain's vital organ concerned with hunger and operates on it by regulating its activity.i.e. food intake
- ❑ In case of injury in hypothalamus, the most apparent change will occur in eating behaviors





- ❑ Studies showed that when the rats lateral hypothalamus was removed; it resulted in starving to death and they refused to eat anything when given food
- ❑ When the rat's ventromedial hypothalamus was removed, it resulted in the opposite.i.e. extreme overeating behavior; increase in weight by 400 percent than the actual weight
- ❑ It is a known fact that hypothalamus regulates the hunger drive yet it is not clear as to how it operates

➤ External Factor in Eating Behavior:

- **Societal rules and conventions**
- **Learn eating patterns from past experiences**

Eating-related disorders

- Obesity
- Anorexia nervosa
- Bulimia

Weight Set point

- A certain weight level that the body strives to maintain

Metabolism

- The rate at which food converts into energy and then is expended by the body

Managing and reducing weight

- Reducing weight is a tough task so try not to gain too much of weight
- Eat wisely and do not be tempted
- Slow eating helps
- Avoid junk/ fad food
- Do regular exercise; aerobics do help
- Make realistic goals and pursue them strictly
- If you failed in achieving the set goal try again and do not feel guilty
- Reward yourself off and onafter considerable intervals

II. Thirst

- **We can live without food for several days or even months, but without water, we can hardly live for few day**
- **more than 75% of our body weight is due to the presence of water/ liquid**

- **The thirst drive is largely internal and includes three basic mechanisms; when salt concentration in our body becomes high, then it triggers hypothalamus to act, resulting in thirst drive**
- **Secondly, when fluid volume decreases in the circulatory process, then this drive is stimulated.e.g. after injury when large amount of blood is wasted**

- **Another important factor is the increase in the body temperature**
- **The replenishment of water is very rapid; that is why we feel thirsty after a very short time**

III. Fatigue and Sleep

- **Rest and proper sleep is very important for the effective functioning of species**
- **After the hard work of the day, person becomes fatigued and needs adequate amount of rest in order to be fresh again next day**
- **What causes fatigue is yet unknown, but chemical changes in the body that effect muscles may be the factor causing it; one cause is the excess amount of lactic acid in the muscles**

FATIGUE CAN BE PSYCHOLOGICAL AS WELL AS PHYSICAL

- **Frustration, anxiety, tension, worry, boredom etc are factors other than the physical ones causing fatigue in persons**
- **In physical exertion, rest bring freshness and relief, but when the person is in stress or anxiety, rest will not be helpful**

- Partly, sleep is also related with the chemical activity in the body in which the brain, nerves, and muscles are all involved
- The pattern of sleep is culturally based also; people make adaptation with the successive changes in lightness and darkness;e.g. Norway

IV. Pain

- **What makes pain a drive?**
- **Avoiding injury, scar or wound is what makes it an important drive**
- **When pain persists for a longer period of time, then it become a drive to get rid of it**
- **some people respond quickly to pain; more than the normal healthy individual would do and some are less responsive to pain**

**PAIN IS A BLESSING IN
DISGUISE!!!!!!**

V. Sex Drive

- ❑ **Very essential drive for the survival as well as continuation of species**
- ❑ **Societal and religious conventions, laws, and restrictions make it a more powerful drive for human beings**
- ❑ **Sexual drive is largely dependent on the chemical secretions in the body which is a proven fact by using animals as subjects**

❑ In humans, past experience, emotions and ways/ manner of expression play a very important role

VI. Excretion: Bowel and Bladder Tensions

- The body has to get rid of the waste regularly
- The satisfaction of this drive is very important to children as compared to adults for
- Toilet training in early childhood plays a very crucial role in the development of personality of the child later in life
- Harsh/ severe and too early toilet training causes sense of insecurity in the child;
- withholding of bowel movements even when not required may result from it

- **Important thing to know about is that the bowel and bladder muscles are the last ones that come under the child,s control, so the toilet training should be given when the child is physically strong enough and prepared for it**

VII. Air

- **Oxygen= most instant and essential requirement of human body**
- **Oxygen deficiency/ hunger can be felt when there is excess of carbon dioxide**
- **Continual supply of oxygen to the brain is very important because, although brain uses very small amount of it but deprivation for only few seconds may may result in anoxia or neural damage**

- **Even during birth of the child, sufficient amount of oxygen is required, otherwise it may result into mental retardation or brain damage**
- **At high altitudes oxygen deprivation may result into strange behaviors; person loses control over himself, i.e. laugh or burst into tears very rapidly, memory becomes impaired, sense organs do not work properly or in severe cases, brain damage may occur after prolonged deprivation**

VIII. Warmth and Cold

- Every one, whether animals or humans is affected by weather; either it is cold or hot
- Avoiding the extremes becomes the basic motive for all living beings
- Our skin has separate receptor cells for warmth and cold
- Body temperature is regulated by complex mechanism as it largely depends on two factors;
 - i. The temperature in the external environment
 - ii. The internal body temperature

- **Brain's organ, hypothalamus, as with other basic drives, regulates the temperature of the body with respect to the external environment**
- **Below 57 degree of normal temperature, body becomes stimulated; adrenaline and thyroxin are secreted, blood pressure rises, muscular activity increased, blood is transferred to the internal organs and tissues of the body rather than on the surface of the skin**

- **On the contrary, in hot weather, the reaction of the body is entirely opposite; i.e. body become slow in its activity, perspiration appears, blood vessels dilates so that more blood become available on the surface of the skin for cooling, blood circulation is increased**

Lecture 26

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Recap

- **Memory dysfunctions**
- **Motivation**
- **Definition of primary and secondary motives**

- **Primary motives**
- **Hunger**
- **Thirst**
- **Fatigue and sleep**
- **Pain**
- **Bowel and bladder tension**
- **Air**
- **Temperature: warmth and cold**



Motivation

 The driving force, need, instinct, drive, urge, cause, or reason behind every action

TYPES OF MOTIVES

```
graph TD; A[TYPES OF MOTIVES] --> B[Primary motives]; A --> C[Secondary motives];
```

Primary motives

Secondary motives

Primary Motives

- Primarily biological in nature
- Focused upon the fulfillment of bodily/physiological/biological needs

Secondary Motives;

- ❑ The learnt motives
- ❑ The psycho- social motives
- ❑ Geared towards the satisfaction of psycho- social needs

- **Main include:**

I. Need for praise and reward

II. Work





Need for Achievement

Curiosity

**Praise, appreciation,
reward**

Need for affiliation

Need for power

Work

The need for achievement

- **The need to achieve something, some object of desire, a goal, or position/status**
- **The source of satisfaction is not just the achievement of the goal, but the very act of striving for it too**

- **The level of the need for achievement varies from person to person**
- **Some are high and some low achievers**
- **Competition is an important element of this need**

•Those with high achievement motivation level seek situations involving competition; however they are not indiscriminate in this regard

•Low achievement oriented people tend to avoid situations where there is any risk of failure involved; they prefer to look for an easy way out

Achievement motivation is a significant variable in a competitive society

- **How Can We Measure Achievement Motivation ???**
- **Thematic apperception test (TAT) :**
- **A series of ambiguous pictures is presented to the person who is asked to describe whatever is happening in it just like forming a story**
- **the subject has to tell what is happening in the scene, what the antecedent conditions were, who the characters are, what are their thoughts and wishes, and what is going to happen next**

In short the subject describes the past, present and future along with the description of characters and their thinking and motivation

Factor Contributing to the Need for Achievement

- ❑ **Parental treatment: expression of warmth for the child as well as setting high standards for him/her; providing encouragement on becoming independent**
- ❑ **High achieving siblings**



Curiosity

 **The natural desire and need to explore reality, to seek information, and to make the unknown known**

- 
- **Why do children try to to break toys and other objects into pieces?**
 - **Why children ask questions about what they watch on television, see on roads, read from somewhere, or listen from any one?**

- It is all their curiosity and need to explore in order to find answers to every 'what', 'why', and 'how'
- Curiosity is inborn , found in both humans and animals, but is learned as well
- Parents encourage their children's curiosity by satisfying their inquisitiveness
- School also plays an important role; the teaching methodology adopted may encourage or discourage curiosity

Need for Praise, Appreciation, and Reward

- ☐ Praise is a powerful motive for everyone; especially for children and animals
- ☐ Words and different acts and gestures serve as symbols of praise and reward
- ☐ The presence and the attitude of someone we like also serves as a social reward for the learner
- ☐ Appreciation and reward from parents and teachers determine the quality of learning to a great extent

Need for affiliation

- ☐ Urge and desire to develop a positive relationship with others; making friends, social contacts
- ☐ Isolation is not preferred; in fact avoided
- ☐ Some gender differences have been observed in need for affiliation
- ☐ Females spend a more of their time among friends as compared to males

Although the need for affiliation is a universal phenomenon, cultural differences do exist in its expression; Some cultures have more group cohesiveness than others

Need for Power

- ❑ **Desire to influence, have hold over, or rule others in order to be recognized as a powerful individual**
- ❑ **People with a high need for power prefer professions that provide an opportunity to control and influence other people**
- ❑ **Gender differences do exist; men exhibit and experience more of need for power than women; women tend to be more socially restrained and traditional and less tempted by challenges**

Work

- **People spend a major part of their lives in work, and that is why it is considered to be one of the motivating forces**
- **Work serves as a powerful motive because it satisfies other needs and motives as well; biological needs like hunger, shelter etc; it also satisfies sense of achievement, affiliation, and decision making**

Explanations of Motivation

Instinct Approaches

Drive-reduction Approaches

Arousal Approaches

Incentive Approaches

Cognitive Approaches

Maslow's Hierarchy of Needs

Theories/Explanations of Motivation



- ❑ **Different approaches that focus upon the biological, social, cognitive, and psychological factors**

INSTINCT THEORY

There is a biologically determined behavior pattern that we are born with, known as instincts; biological variables drive our behavior

- ☐ **All humans and animals exhibit certain innate tendencies which are fixed , predetermined, and unlearned**
- ☐ **Different behaviors are respective responses to specific instincts**
- ☐ **Instincts are essential for the survival of the species**

☐ Instincts provide energy for action thus directing organism's behavior

☐ Shortcomings of Instinct Approaches

☐ Which are the primary and power instincts?....18 as William McDougall said or 5759 as Bernard proposed

☐ If instincts are the main driving force, then how come different people behave differently under the influence of the same instinct? e.g. hunger or heat

Drive- Reduction Theory

- ❑ Our biological/ physiological needs create an aroused or tension producing state that motivates us to fulfill them
- ❑ When the basic biological requirements are lacking, and the need is unfulfilled, then drive/ arousal is produced in the organism energizing it to obtain the requirement in order to satisfy the need

Drive:

An arousal or motivational tension that provides energy for action or behavior

Homeostasis:

A stable, well maintained state of internal biological balance is required for the proper functioning of the body; homeostasis is the process whereby this balance is maintained

Primary and Secondary Drives

Primary Drives:

- ☐ Entirely biological in nature i.e. hunger, thirst, sleep, sex, air etc
- ☐ Primary drives are satisfied by reducing the underlying need e.g hunger followed by food, thirst followed by water

Secondary Drives

These are psychological as well as social in nature

□ Prior learning and experiences are what brings about such needs e.g. academic achievement

Criticism

- What about behaviors directed towards maintenance, and even heightened, arousal???
- What about curiosity or risk-taking behavior??
- Is our behavior guided by drive-reduction alone, or by our goals too?

AROUSAL THEORY

- ❑ A certain level of arousal and excitement is needed by our system
- ❑ We try to maintain that level of stimulation and activity; the maintenance may require off and on reduction or increase in the existing level, depending upon the circumstances
- ❑ When our arousal state becomes too high, it needs to come down for optimal functioning and vice versa

- ❑ Too high a motivational arousal may affect performance negatively; it may produce anxiety and irritability in the organism
- ❑ Similarly too low an arousal may also have adverse effect e.g performance of a person suffering from depression
- ❑ A consistent, well balanced, and leveled arousal is needed for the optimal functioning e.g in case of exams, athletics, interviews

INCENTIVE THEORY

- ❑ Motivational state of the organism is understood and explained in terms of positive or negative environmental stimuli
- ❑ As opposed to the drive-reduction and arousal theory, this theory explains motivation in terms of external events that stimulates/ energizes the organism's behavior, rather than innate instincts or drives
- ❑ Characteristics of the external environment are important variables in a person's motivation

- ❑ Incentives are rewards that energize and drive our behavior
- ❑ Any thing that provides us with a reward triggers motivation for action
- ❑ Incentives may generate behavior even in the absence of an active, unsatisfied, instinct or drive
e.g How can we eat a bowl full of ice cream even when our stomach is full after a heavy meal?
- ❑ But in many cases drives and incentives go together and have a deeper effect

☐ Criticism:

- ☐ What about the motivational process when an individual tries to fulfill needs although there is no apparent incentive present? e.g working even when paid very little, and the job too is not matching with one's aptitude
- ☐ What about behaviors when there is no apparent incentive available, in fact threat is involved? e.g helping the people stranded in a house on fire

Cognitive Theory of Motivation

- ❑ Theories that give importance to the cognitive processes of the individual in explaining motivational process; thoughts, feelings, expectations, understanding and evaluating all are important when explaining the motivation of the person

Expectancy- Value Theory

❑ Two types of cognitive processes underlie human behavior

i. Expectation:

The expectation, hope, or anticipation that our behavior will help us attain a certain goal goal

ii. Value:

The perception, appraisal, or understanding of the value of the goal to us

The level of expectation and the value attached to it, together, determine the level of motivation

In case of a high expectation along with high value, the motivation will also be higher

But if any of the two is weak or low, the corresponding motivation will also be low

Can you see the two operating in your exam and study behavior?????

Intrinsic and Extrinsic Motivation

- ❑ Cognitive theories differentiate between intrinsic and extrinsic motivation

- ❑ **Intrinsic motivation:** Motivation from within or Internal motivation that energizes the person to satisfy or accomplish the goal; the goal is to attain enjoyment and personal satisfaction , in which no external tangible reward is involved e.g altruistic behavior
- ❑ **Extrinsic motivation:** Revolves around the tangible rewards such as money, social contacts
- ❑ **Studies shows that intrinsic motivation produces better results in terms of performance than the extrinsic motivation**

- ❑ When extrinsic motivation is more for the desired behavior than the intrinsic one, and it remains like that for long, then the intrinsic motivation eventually declines
- ❑ e.g. A child has been given reward for doing neat and tidy home work as he is less intrinsically motivated; eventually the entire focus of the child in getting reward after doing tidy homework rather than doing it otherwise

- **Teachers and parents should use extrinsic motivation ,but with great care**
- **Extrinsic motivation can be used to enhance the motivational level but not at the cost of intrinsic motivation**

Abraham Harold Maslow

1908-1970

American psychologist and leading exponent of humanistic approach

Gave comprehensive theory of motivation

Found the prevalent psychology to be too pessimistic and negatively oriented

Key Points of Maslow's Theory:

- ❑ Psychology and the psychologist should look at the positive side of the human beings
- ❑ There must be more to living than just being battered by a hostile environment, or by depraved instincts----which may actually be leading to self-destruction

❑ People's needs are not low level and base. We have positive needs that may become neutral in the worst cases, but will not turn negative or base

❑ Human behavior does respond to needs but we will be wrong in saying that all our needs are only physiological in nature

❑ Needs motivate human action; such needs are very few in number

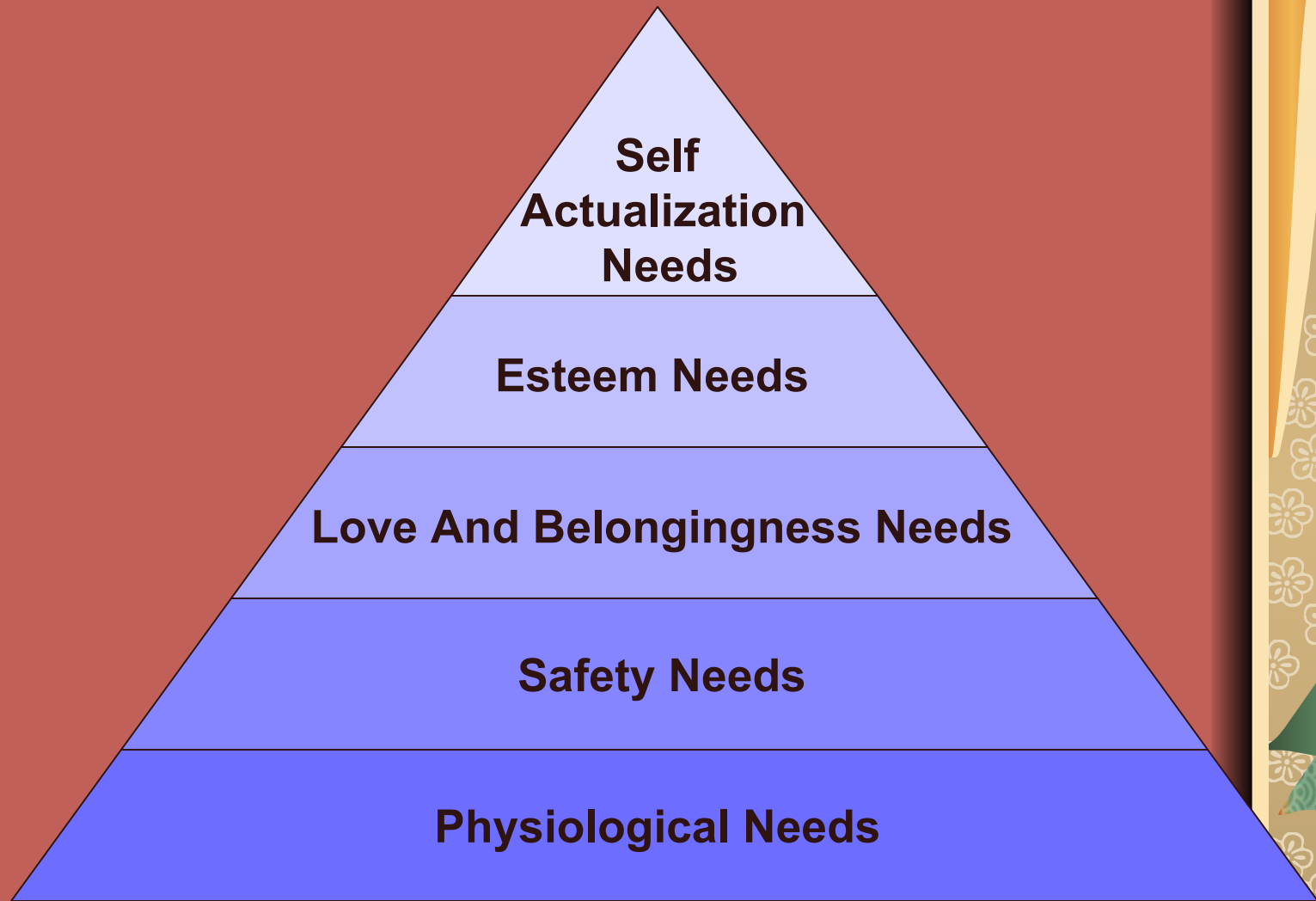
Maslow's Hierarchy of Needs

Basically a stage theory

The needs at one level have to be met in order for one to move on to higher order

The needs at the lowest/primary/base level are the physiological needs, whereas the highest order needs are the self actualization needs

Self Actualization: most advanced human need based on the desire to grow and utilize one's potential up to the optimal level



Categories of Needs

❑ Metaneeds: based on a desire to grow rather than for meeting a deficiency: expressed in the need for self actualization

❑ Deficiency needs: The absence of the underlying requirements triggers these needs e.g. physiological needs, love needs, or esteem needs

Interactions and needs of Behavior

- ❑ Physiological needs: Fulfilled through = hunger/food; Pathology associated=over eating, anorexia
- ❑ Safety needs: Fulfilled through= profession, job; Pathology associated= Phobias
- ❑ Love and belongingness Fulfilled through=marriage,friendship; Pathology associated= Antisocial personality

❑ Esteem needs: Fulfilled through= Awards, honors, scholarships; Pathology associated= Depression

❑ Self actualization needs: Fulfilled through= Painting, writing, singing; Pathology associated= Isolation, alienation, cynicism

**Transcendence:
Spiritual needs**

**Self-actualization: fulfilling
potential, having meaningful goals**

Esthetic Needs: order, beauty

Cognitive Needs

Esteem Needs

Attachment Needs

Safety Needs

Biological/Physiological Needs

Criticism against Maslow's theory

Although a comprehensive and well formed theory, it has been criticized at some points :

Can we actually, for all case, distribute and neatly order these needs?

There is little empirical evidence to support Maslow's way of ranking needs



Lecture 28

Recap

- **Motivation**
- **Motives are the driving force for all voluntary human action**
- **Motives can be primary and secondary**
- **Primary motives are biological in nature**
- **Secondary motives psychological and social also**
- **There are different theories of motivation**



Secondary motives



- **Achievement**
- **Curiosity**
- **Praise and appreciation**
- **Affiliation**
- **Power**
- **Work**

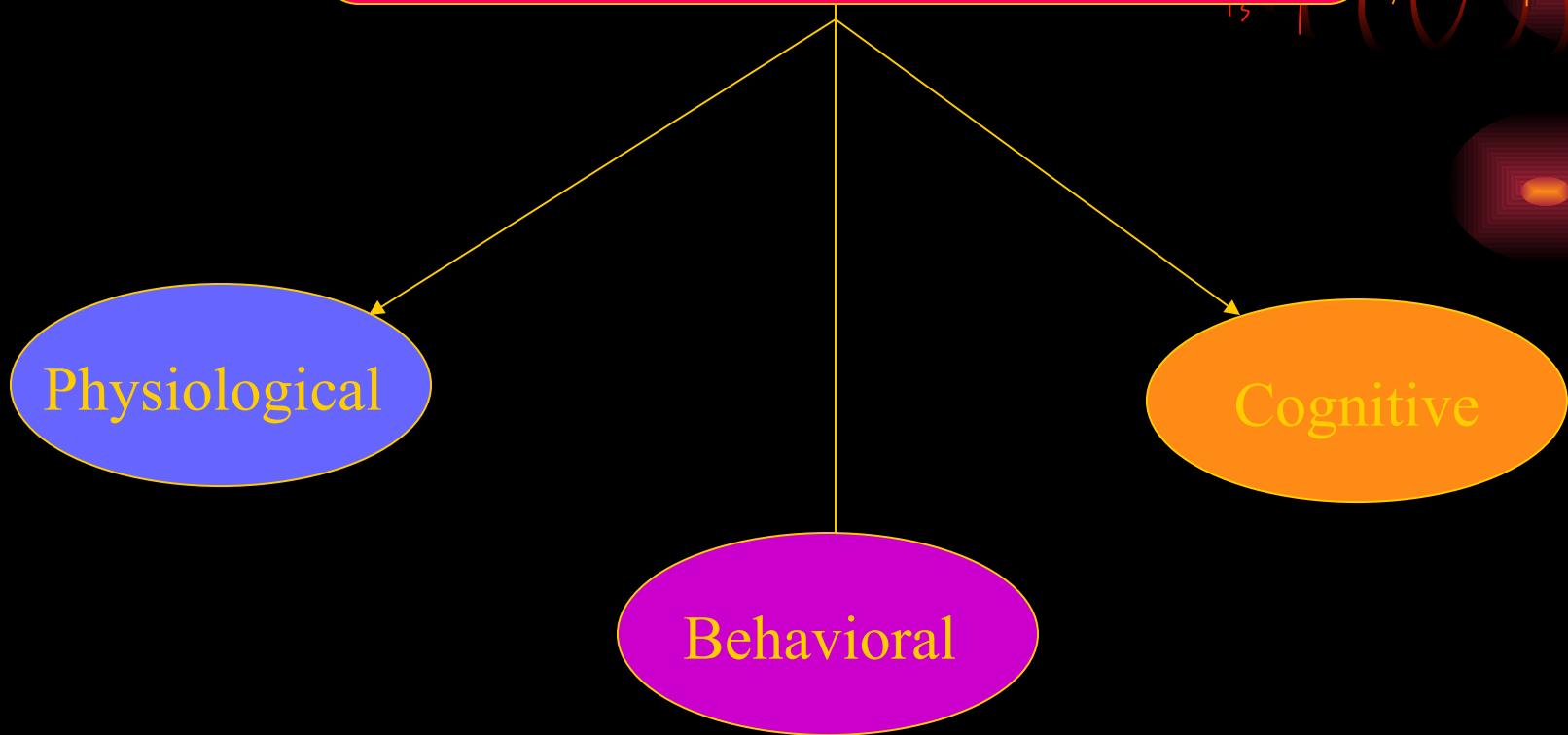
Theories of Motivation

- **Instinct Approach**
- **Drive-reduction Approach**
- **Arousal Approach**
- **Incentive Approach**
- **Cognitive Approach**
- **Maslow's Hierarchy of needs**

Emotions

- **Derived from the Latin word “emovere” means “to excite, stir up or agitate”**
- **A response that include feelings such as happiness, fear, sadness, grief, sorrow etc, is the amalgamation of**
 - **Physiological arousal,**
 - **Expression, and**
 - **The conscious or the cognitive experience of the situation that influence behavior**

Components of emotional experience





❑ **Mc Dougall divided emotions into three types:**

- **Primary**___ Simple emotions like fear, happiness, anger, disgust etc
- **Secondary**___ Mixture of various instincts like curiosity, escape etc
- **Derived**___ Those learned through experiences such as sadness, boredom etc

☐ Have you ever thought:

☐ Why does your heart pound at the sight of someone you like?

☐ Why do you burst into tears on hearing a dreadful news?

☐ What will give you the feeling that you are scared when you see a wild snake?

☐ Why does our face redden on hearing a good news?

**YOU THINK IT IS A SIMPLE PHENOMENON
AND YOU KNOW THE ANSWERS**



BUT IT IS NOT THAT SIMPLE!!!!



- **Our heart may pound even at the sight of someone we hate. Also, before an interview or presentation. Or when we are scared.**

AND

- **Our face may blush and turn red when we are self conscious, or embarrassed, or angry**

So!!!!

❑ Same bodily changes may be manifest in different emotional experiences.

❑ What makes the experience different is the accompanying expression and cognition

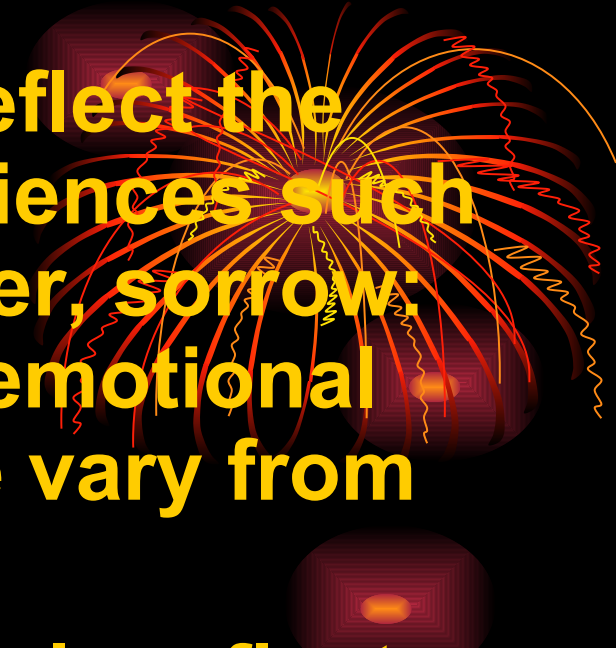
Assessing Emotions



Assessing Emotions: Expressions in Emotions



- ❑ **Vocal, verbal and facial expressions play a vital role in determining emotions as well as our interaction with the physical world. In interpreting emotion, the important things are**
 - **Acquaintance with the other person**
 - **Type of association with that person**
 - **Deriving clues from own emotional experiences in order to understand other's experiences as well**

- 
- ❑ Facial expressions clearly reflect the intensity of emotional experiences such as happiness, sadness, anger, sorrow: all can be understood from emotional expressions. However these vary from person to person.
 - ❑ An adult's voice pattern clearly reflects their emotions. Variations in pitch or loudness express different types of emotions

Emotions and Cognition in Understanding Emotions



- ❑ A persons cognition, or thinking ,plays a very vital role in the emotional experience. that is why the same stimulus may cause joy for one and fright for another

❑ Some psychologists believe that we first cognitively evaluate the situation and then exhibit the emotional experience we are facing at that time.

❑ Some others believe that we experience emotion first and then attach meaning to it i.e., the cognitive component comes later.

Role of the Emotions in Everyday Life



- **Emotions make our life bright and colorful. Our relationships are determined by the emotions we experience for people and situations.**
- **Without the experience of emotions, our life would probably be dull, uninteresting, non arousing and without any purpose**
- **Psychologists have identified a number of vital functions that emotions play in our daily life.**

- Emotions stir up the fight or flight action
After seeing a scary object or after an unusual incident like a natural disaster, the body's combat system is stirred up, and our body is ready for facing and dealing with it.
- Remember the role of the Sympathetic Nervous System ????
- Emotions help us modify the future responses and behavior
- Learning take place after the emotional state that prepares us to manifest appropriate behaviors in future.

- **Social interactions are enhanced**
- **Emotions are expressed both verbally and non-verbally. They help people better understand the responses of others, even when they are not clearly expressed**



Common/ Primary Emotions



- **There are a number of basic emotions that have been identified :**
- **Happiness, joy, elation**
- **Anger**
- **Fear**
- **Disgust**
- **Surprise**
- **Sadness**

J.B.Watson talked about three main emotions:

- **Anger**
- **Love**
- **Fear**

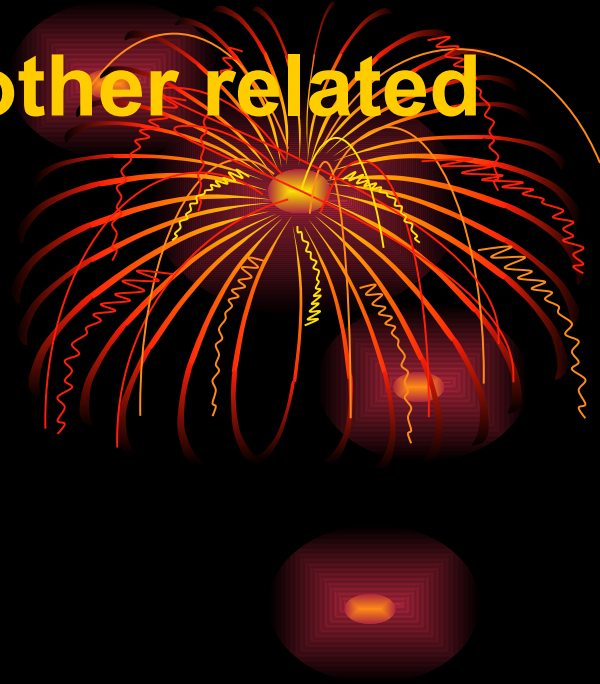
He proved in his experiments that infants are capable of these three primary emotions.

• **Psychologists have identified other related emotions as well :**

☐ **contempt**

☐ **shame, and**

☐ **startle**



- **Plutchik's emotional wheel:**

Robert Plutchik (1984), determined basic emotions and their mutual relationship. He identified eight fundamental emotions:

- **Joy**
- **Anger**
- **Sadness**
- **Surprise**
- **Fear**
- **Acceptance**
- **Disgust ,and**
- **Anticipation**



❑ Adjacent and nearer emotions complement each other(e.g joy and acceptance) whereas those opposite each other clash with each other and are conceptual opposites (e.g disgust and acceptance).

❑ Other researches suggest that there are cultural differences in the expression of emotions

Physiological component of emotions:

- **Many physiological changes take place in an emotional state, in the form of changes in:**
- **Heart rate**
- **Pulse rate**
- **Respiration**
- **Blood pressure**
- **Digestion and appetite**
- **Muscular activity**
- **Body temperature**
- **Perspiration**
- **Endocrine and neurotransmitter secretions**

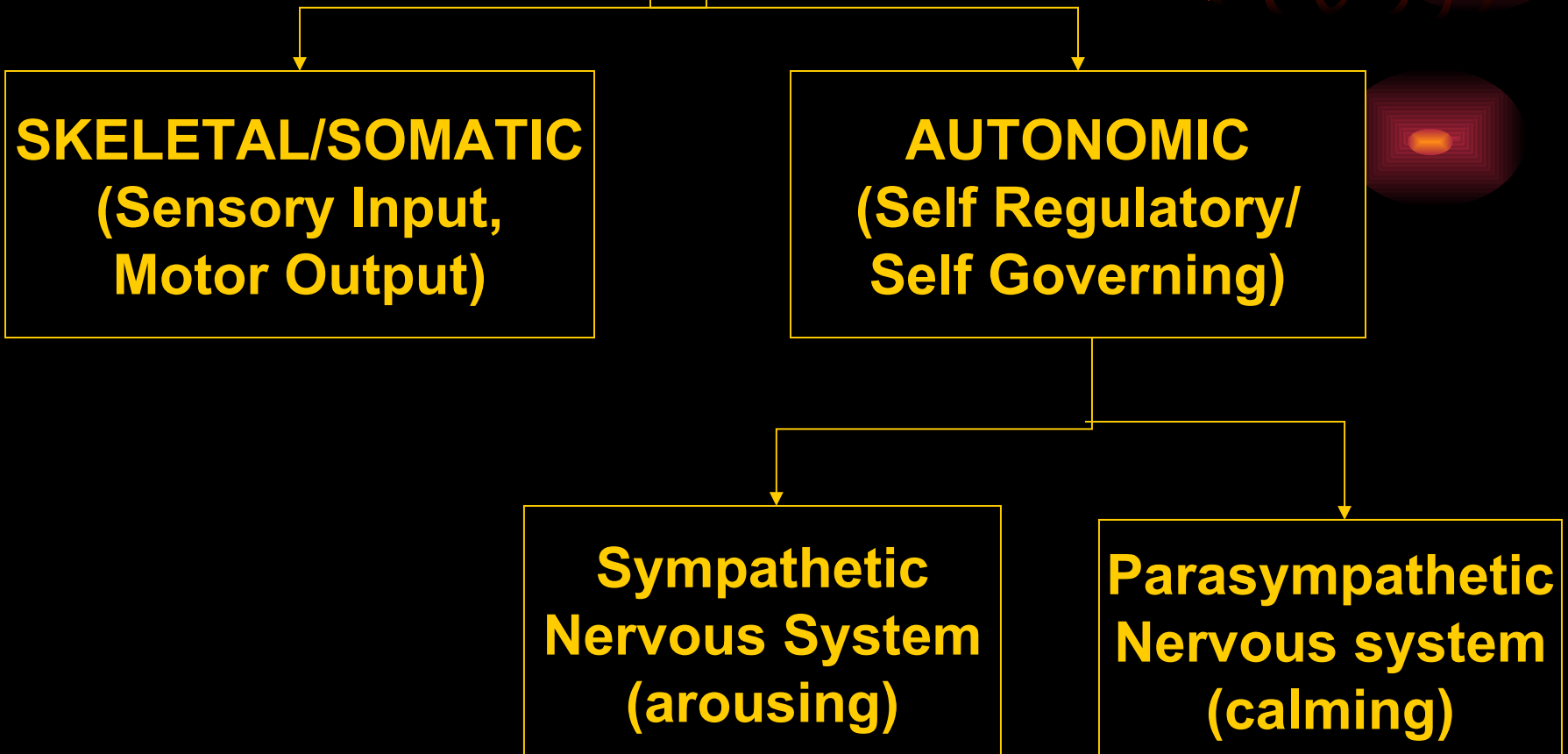
- **Blood sugar**
- **Salivary gland activity**
- **Pupil dilation**



Role of Nervous System in Emotions

- **All the emotional experiences and their expression are controlled and regulated by the autonomic nervous system that is a part of peripheral nervous system**

PERIPHERAL NERVOUS SYSTEM



AUTONOMIC NERVOUS SYSTEM

(A.N.S):

- ❑ Considered as the “self governing or self-regulatory mechanism” because of our involuntary control over it.
- ❑ Controls the glands and muscles of our internal organs such as heart (heart beat), stomach (digestion) and glandular activity.
- ❑ A.N.S.has a dual function; i.e.both arousing and calming.

a. SYMPATHETIC NERVOUS **SYSTEM (S.N.S):**



- ❑ This part of A.N.S. arouses us for defensive action.**
- ❑ If something alarms or enrages someone, the sympathetic nervous system accelerates heart beat, slows down digestion, raises the sugar level in blood, dilates the arteries, and cool the body through perspiration, making one alert and ready for action**

b. PARASYMPATHETIC NERVOUS **SYSTEM:**

When the stressful situation subsides, parasympathetic nervous system takes over.

❑ It produces opposite effect to that of sympathetic nervous system.

❑ It conserves energy by:

❑ Decreasing heart beat

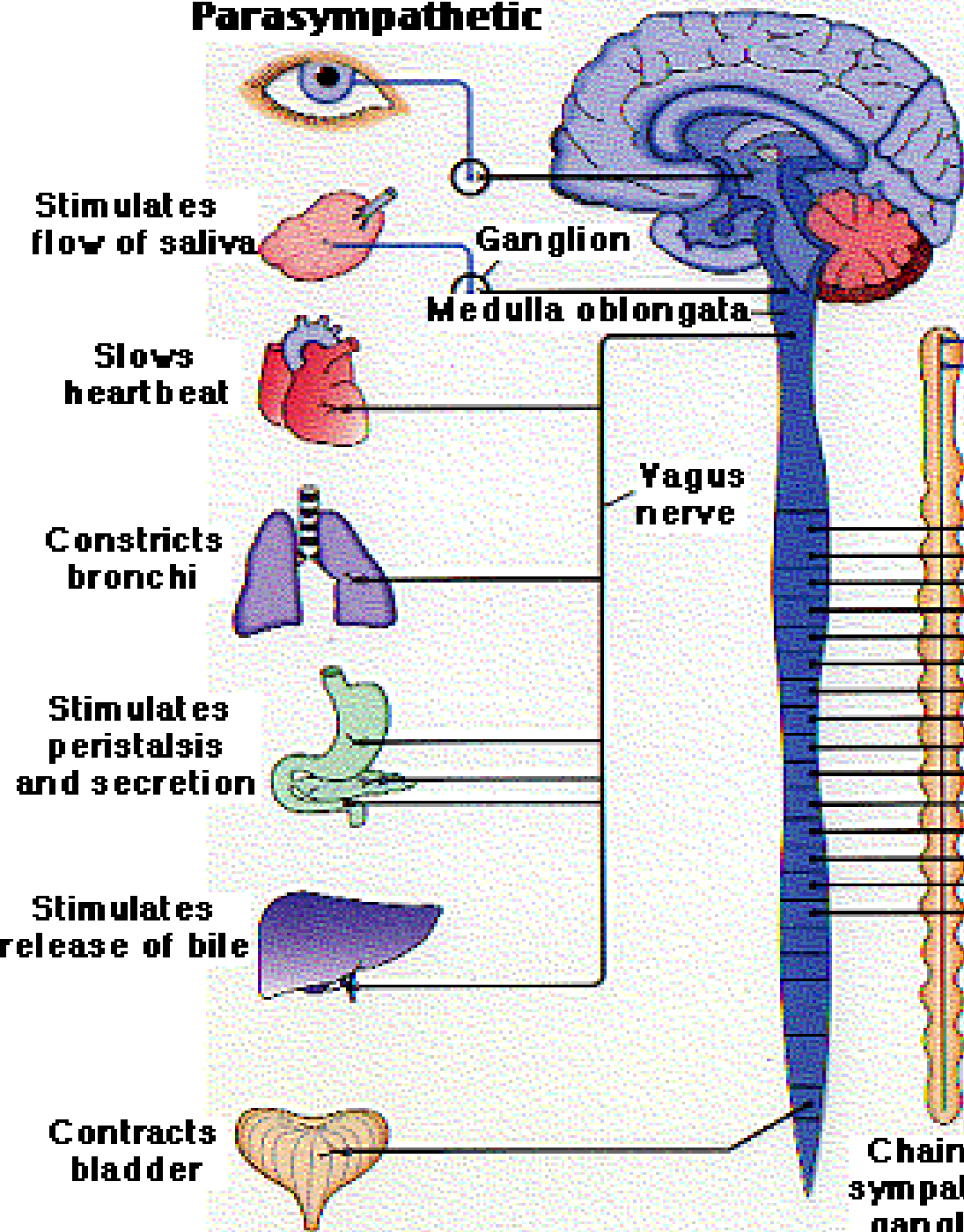
❑ Lowering blood pressure

❑ Lowering blood sugar and so on

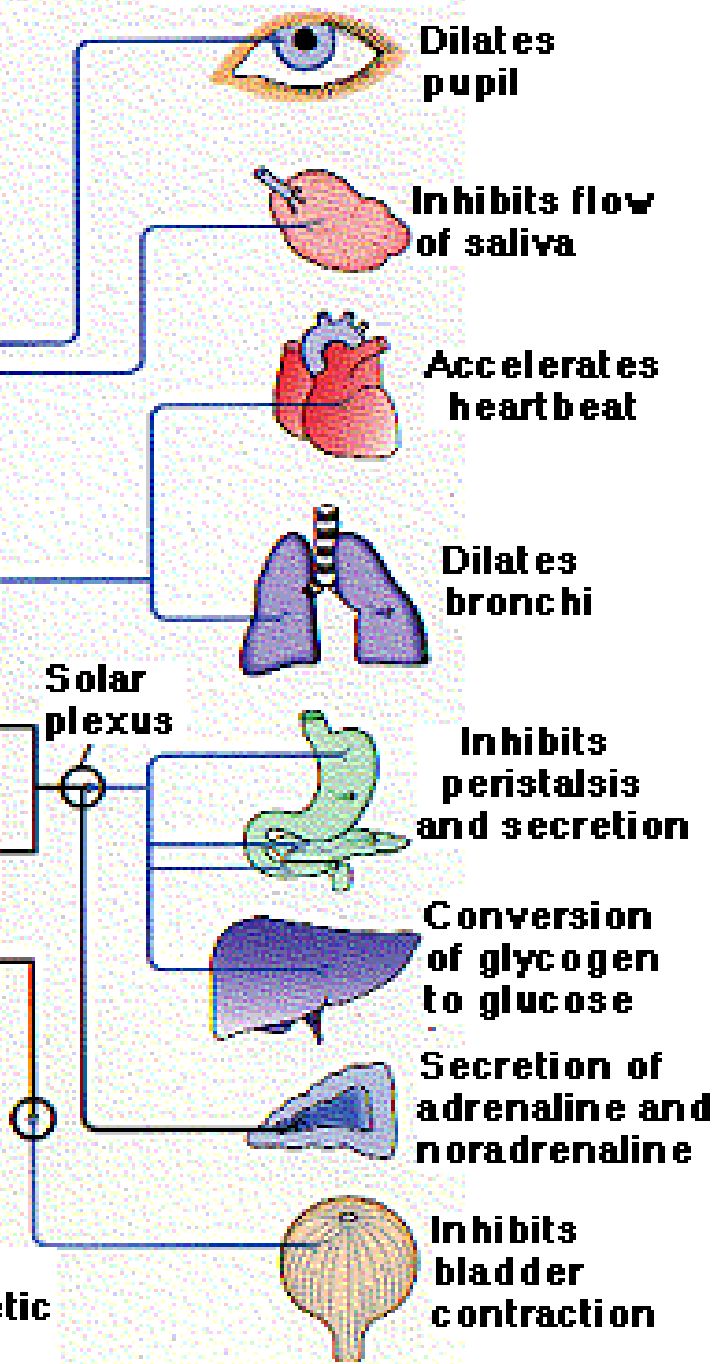
In daily life situation, both sympathetic and parasympathetic systems work together to keep us in a steady internal state of homeostasis.



Parasympathetic



Sympathetic



Areas for Emotions in Brain

- ❑ **A.N.S. is the center of emotions in P.N.S.**
- ❑ **Cortical and sub- cortical structures are involved in emotions.**
- ❑ **Cerebral cortex is involved in relating the present and past emotional experiences; others involved in speeding up the heart beat, and in learning emotional experiences.**
- ❑ **Sub- cortical regions are involved in organizing the emotional expressions.**

Physiological Changes During Emotions

- ❑ Psychologists believed that emotions can be measured quantitatively by observing different physiological responses of the individual i.e., the indicators of emotions
- ❑ The major physiological changes that occur during emotions are:

I. Respiratory Changes:

- **The most apparent and obvious change during emotions**
- **During emotions, respiration increases; occurs more when the person is happy or excited**
- **But the production and secretion of saliva decreases as the process of respiration increases**

II. Pupillometrics



- ❑ Darwin was the first person to identify this phenomenon.**
- ❑ Pupil of the eye is very responsive during emotional states.**
- ❑ Pupils dilate at favorable stimulus and contract at unfavorable stimulus.**
- ❑ Our pupils are also very responsive to favorable and unfavorable sounds and taste.**

Lecture 28

Recap

- Emotions
- Primary emotions
- Physiological component of emotions
- The role of nervous system in emotions
- Major physiological changes during emotions

I. Respiratory Changes

II. Pupillometrics

III.Changes in Blood Pressure **and heart rate:**

- ☐ Variations in blood pressure occur during emotions; usually increases during an emotional state.
- ☐ Heart beat also increases during emotions.
- ☐ Increased blood pressure and heightened heart rate for prolonged periods may lead to coronary heart disease.

IV. Glandular Responses

- During strong emotional states such as anger or fear, excessive amounts of hormones adrenaline and nor adrenaline are secreted into the blood stream.
- Due to this secretion:
- liver secretes excessive amounts of glucose directly into the blood stream that causes the blood to clot rapidly in case of injury or damage.

- **Blood pressure and sugar level rises, pulse become fast, air passage of the lungs enlarges and causes more air into the lungs, pupil enlarge, sweat appears all over the body particularly on hands, and temperature of the skin rises**
- **Nor adrenaline helps to constrict the blood vessels, thus making it available to other parts of the body in case of injury.**

- Pituitary and thyroid glands are also responsive to emotional states.
- All the glandular responses help to cope physically with the emotional as well as emergency situations

V. Gastrointestinal Function:

- **Stomach and intestines are also very responsive to emotional states.**
- **They either start working at a very high rate or stop entirely.**
- **During strong emotional arousal, its working speed decreases and flow of blood is more towards the brain and the skeletal muscles rather than these organs.**

VI. Neural Reactions

- **Beside affecting visceral organs of the body, emotions also bring changes in the neural/ nerve activity.**
- **Autonomic division of the P.N.S. is more effective in this regard in which sympathetic and parasympathetic nervous systems work successively.**

VII. Galvanic Skin Response:

- When perspiration appears during emotions, two important changes occur in skin's electrical stimulation:
 - Rapid generation of electromotive energy
 - The electrical resistance of skin changes
- These changes can be measured through a measurement of “Galvanic Skin Response (GSR)”; formally called “PsychoGlavanic Response (PGR)

- Galvanic skin response, in combination with the respiration and blood pressure, is used for lie detection with people who are supposed to be guilty of some crime.

VIII. Emotional Intensity and State of Arousal

- Most of the times we are aware of our emotional states such as angry, excited or afraid. In all these states, the physiological conditions are the same e.g., heart beat increases, face blushes or becomes pale
- That is why we are unable, at times, to differentiate between different emotions and the associated arousal.

Other Common Bodily Changes During Emotions

- i. Dryness of throat and mouth**
- ii. Muscle tension**
- iii. Weakness or fainting**
- iv. Trembling, and**
- v. Sinking feeling in heart or stomach**

THEORIES OF EMOTIONS

Theories of Emotion

- Psychologists have attempted to define and explain the emotional arousal and physiological conditions that accompany them.
- A number of theories have been developed in this regard, but the most famous ones are;
 - I. James- Lange theory of emotions
 - II. Cannon- Bard theory of emotions
 - III. Schachter- Singer theory of emotions;
(Cognitive labeling theory/ Two Factor theory)

- **Other important theories include:**

IV. Cognitive Appraisal Theory: Richard Lazarus

IV. Cognitive Theory: Magda Arnold

V. Opponent- Process Theory: Solomon and Corbit

IV. Activation Theory: Lindsley

V. Theory of Emotions: Albert Ellis

I. James- Lange Theory of Emotions

- **Earliest theory of emotion; now considered as the classical approach to emotions**
- **Given by American psychologist, William James and Danish psychologist, Carl Lange in the 19th century.**

Emotional experience occurs in reaction to instinctive bodily events, that take place as a result of an external situation: “...we feel sorry because we cry, angry because we strike, afraid because we tremble” (James, 1890).

Physiological changes create specific sensations, and our brain interprets these sensations as different emotions.

- **Theory asserts that awareness of the physiological responses determines our emotional arousal**

SEQUENCE OF EVENTS IN JAMES-LANGE MODEL

- Experience of an emotion-involving situation i.e., find oneself in a waiting room before first job interview
- Physiological responses take place; visceral bodily changes are activated e.g. pounding heart, sweaty palms, respiration increases and other physiological states
- Brain interprets these physiological changes as emotional experience. In the case of the interview situation, fear

Criticism against James-Lange model

- On most emotional occasions we experience the emotion quite immediately after a triggering stimulus e.g hearing loud bang and being startled. Are our visceral responses that fast??
- Many a time we don't even have enough time for this intermediate part. We feel the emotion immediately after the stimulus is registered e.g feeling disgust at the very sight of someone we hate
- The most basic criticism of this theory is emotional experience (fear) results from the physiological responses after evaluating the situation

On many occasions we experience physiological changes but no emotion e.g. patients of hypertension experience raised blood pressure but do not experience associated emotion. Same is the case with joggers whose heart does pound but there is no emotional experience.

How do same physiological changes create different emotional experiences???

.

**Emotion-
involving
Event**

```
graph LR; A[Emotion-involving Event] --> B[Visceral bodily changes]; B --> C[Brain interprets the visceral changes as emotional experience];
```

**Visceral bodily
changes**

**Brain interprets
the visceral changes
as emotional
experience**

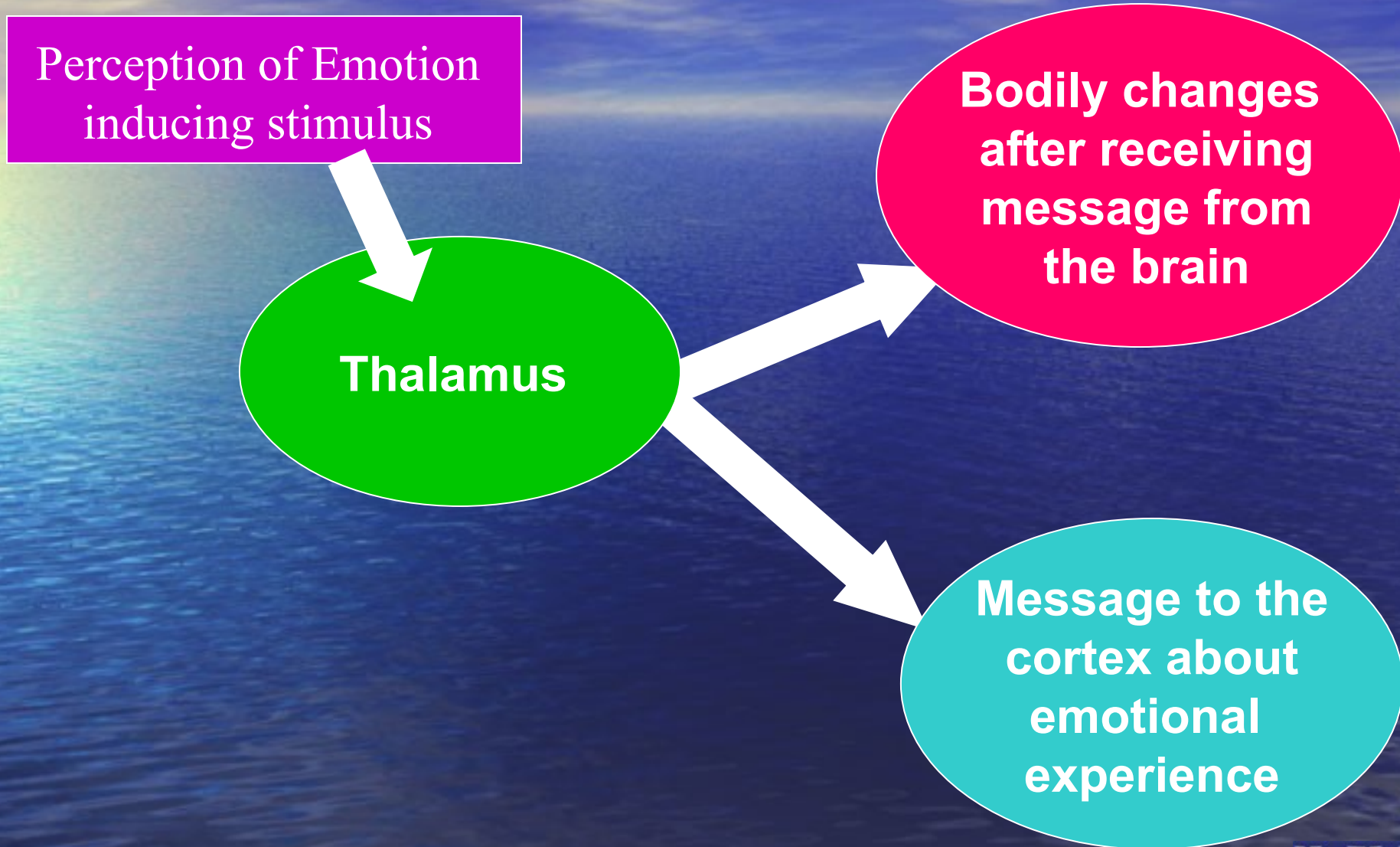
II. Canon- Bard Theory of Emotions

- **Given by Walter Canon and Philip Bard in 1920's**
- **The theory assumes that emotional states and the physiological reactions work independently, but are triggered by the same nerve impulse simultaneously**

- **SEQUENCE OF EVENTS IN CANNON-BARD THEORY**

- 1. Perception of the emotion-inducing stimulus.**
- 2. Thalamus is activated. thalamus sends messages to two sites i.e.,**
 - a) The Autonomic nervous system, thereby producing a visceral response.**
 - b) The cerebral cortex receives a message regarding the nature of emotion being experienced.**

•SEQUENCE OF EVENTS IN CANNON-BARD THEORY



Major drawback:

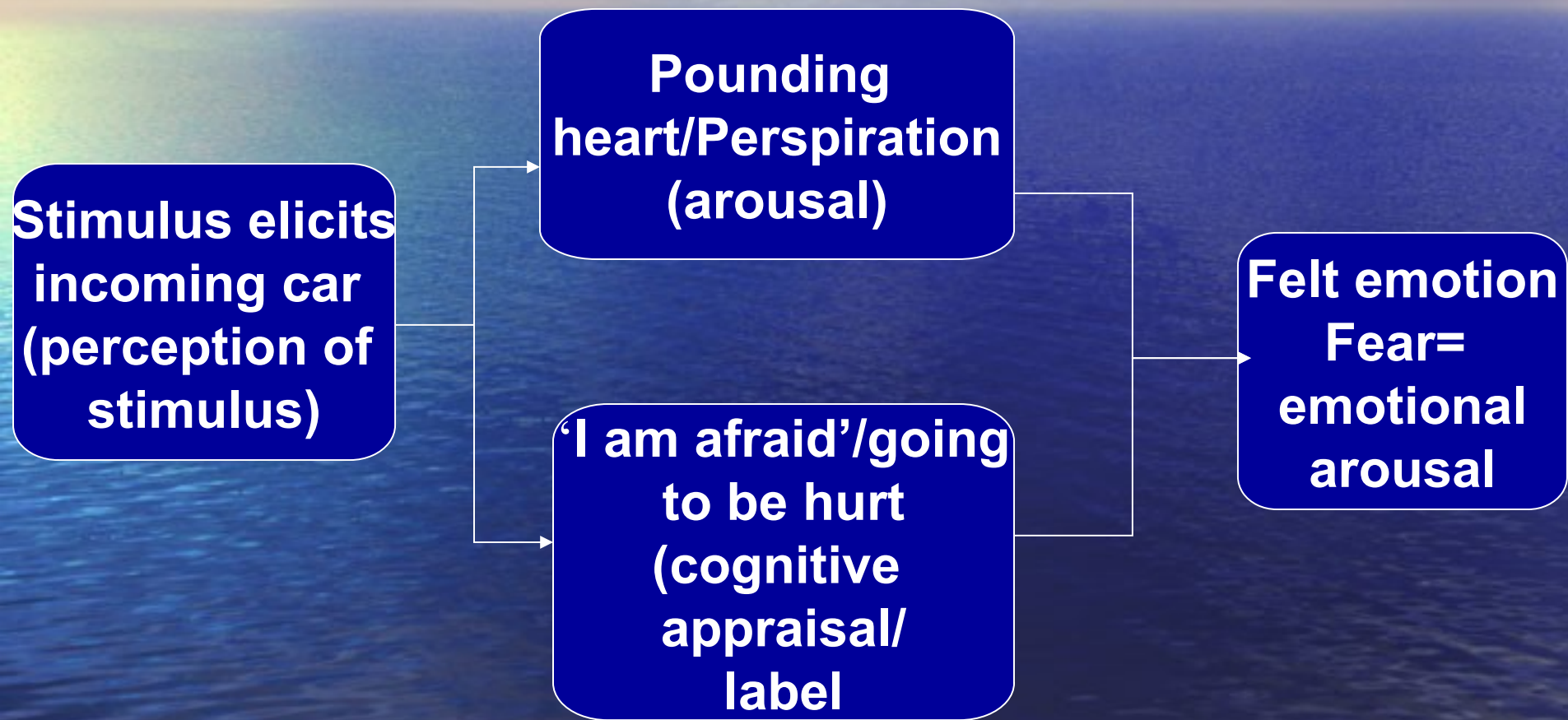
- Research has shown that it is the hypothalamus and the limbic system that are responsible for emotional experience, and not the thalamus!!!!

III. Cognitive labeling theory/ Two Factor theory of Emotions;

- ☐ Pioneer: Stanley Schachter and Jerome Singer (1962)
- ☐ According to them, emotions result from the physiological arousal as well as the cognitive appraisal (evaluation) of the situation
- ☐ Arousal comes first and is general in nature

- ❑ **Two forces jointly determine emotional experience:**
 - a) **Nonspecific kind of physiological arousal, and**
 - b) **The interpretation of the experience based on environmental cues**
- ❑ **Both of these factors jointly determine the title/label of the emotion, and the meaning of one's reaction.**
- ❑ **We observe the environment and compare ourselves with others, and this help us identify our emotion**

Schachter's- Singer Theory of Emotions



- ☐ Labeling of the emotional experience is entirely dependent on the experiencing individual__ one 's own evaluation
- ☐ Believed that physiological arousal determines the intensity of emotions
- ☐ Theory includes the physiology cognitive processes as well as social psychology to investigate the nature of emotions

IV. Cognitive Appraisal Theory:

Richard Lazarus

- **Given by Richard Lazarus and his colleagues in 1968**
- **The theory maintains that emotional experience cannot solely be understood of its own, but understanding the environment is to be evaluated**
- **Appraisal involves cognition, bodily responses and memory**

Experienced Emotion



Cognition

- Gave two basic types of appraisal
 - i. Primary appraisal: Evaluate whether the situation is threatening or not
 - ii. Secondary appraisal: Involve alternatives in order to deal with the perceived threat

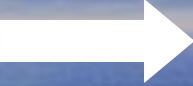
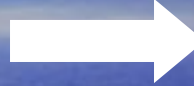
V. Cognitive Theory: Magda Arnold

- **Given in 1966; one of the first ones who use the concept of cognition in emotional state**
- **His model is known as “ Sequential model”**
- **The steps involved in emotions are**

- i. Perception; How the situation is being perceived
- ii. Appraisal; Stimulus is beneficial or harmful to ourselves
- iii. Determining emotions with regard to the prevailing situation
- iv. Expressing emotions; Also accompanying physiological responses
- v. Finally, they all work to give idea whether to approach a situation or withdraw from it

VI. Opponent- Process Theory: **Solomon and Corbit**

- **Given by Richard Solomon and John Corbit in 1974**
- **Theory maintains that every emotional arousal has an opposite, i.e. when one type of emotion is elicited, then there must be an opposite that is there to suppress or cancel it**

- In this way the emotional arousal remain at some base- line
- Base line  State A  State B 
Baseline

VII. Activation Theory: Lindsley (1951, 1957)

- Based his theory by observing that reticular system activates the cortex region in the brain that ultimately aroused/ excites the organism
- Believed that emotional arousal activate the reticular part of the brain present in the brain stem, that ultimately sends impulses to the thalamus and cortex and bring about emotional arousal
- And if the reticular part is at rest, then the emotional state is calm and relaxing

VIII.Theory of Emotions: Albert Ellis

- **Given by American psychologist, Albert Ellis**
- **According to him, emotions do not result from single cause, but originate from three different ways**
 - **First, sensory- motor; involves brain and muscles**
 - **Second, bio chemical stimulation; function to mediate processes of ANS, hypothalamus and sub- cortical regions**

- **Third, cognitive and thinking processes**
- **To him, emotions do not exist independently but involves several stages in order to take place**

Lecture 29

Recap

- ❑ **Physiological changes in emotions**
- ❑ **Theories of emotion**
- ❑ **James-Lange theory**
- ❑ **Cannon- Bard theory**
- ❑ **Schachter-Singer theory**
- ❑ **Cognitive appraisal theory of Lazarus**
- ❑ **Other theories with a cognitive orientation**

Cognition

- ☐ The process of knowing, as well as what is known.
- ☐ Cognition refers to the higher mental processes.
- ☐ It is through these mental processes that humans understand the world, process information, make judgments and decisions, and communicate knowledge to others.
- ☐ Memory, intelligence, and language are important aspects of cognition.

Cognitive Psychology

- ❑ The branch of psychology that studies cognition, and related areas and issues.

Cognition is commonly known
as Thinking

What is thinking???

- ❑ The information that our mental faculties receive or generate is in the form of mental representations.
- ❑ These mental representations may be in various forms e.g in terms of words, visual images, or may be sounds.

Thinking

- ❑ Thinking is the process whereby these mental representations are manipulated.
- ❑ The process of thinking transforms these representations into a new and different form. The transformation may be made :
 - For finding answers to questions
 - For finding solutions to problems
 - For finding facts and exploring reality

Try to see what diff types of
mental images we
experience!!!!

Close your eyes for a moment and try to:

- ☐ see the face of your mother?
- ☐ hear the voice of your favorite singer?
- ☐ feel how your favorite food tastes?
- ☐ feel how your favorite perfume smells?
- ☐ Imagine how the prick of an injection feels?

***PROBABLY YOU WILL BE ABLE TO DO
ALL OF THESE THINGS***

***The stimulus is not there but we can
feel it..... mentally***

How do all these mental processes take place???

- ❑ Thinking is a subtle, and continuous process.

- ❑ Our brain is the most intricate, complex, sophisticated, and yet 'quite' machine in the universe

Fundamental Elements of Thinking

1. Mental Images
2. Concepts

1. Mental Images

- ❑ Mental images are an integral part of the thinking process, in fact a major part of our thinking consists of these images.
- ❑ These are mental representations of the objects and events that we are, or we have been, in contact with.
- ❑ These images are not necessarily visual in nature; images can be related to all sorts of sensory experiences.

Psychologists have developed exercises for enhancing people's ability to work on their mental images, in order to sharpen their thinking capacity and thinking skills e.g problem solving skills, brain teasing, creative thinking exercises.

There is no dearth of research available suggesting that mental images, when used as mental rehearsals can be very helpful in improving other skills, besides thinking, as well e.g jogging, athletics, dancing, and public speaking.

Images are also used as important tools in
interventions designed for handling psychological
problems e.g Relaxation exercises, or meditation.

2. Concepts

- ❑ Objects, events, or people sharing common characteristics and properties are categorized and classified as one.
- ❑ This categorization is known as “concepts”.
- ❑ Different categories are different concepts e.g the concept of different objects, people, or events.

If someone asks you “Who are your class fellows?”

☐ How will you respond?

You might say:

☐ **“One short girl, one medium height boy, a tall girl and a tall boy, one girl with long hair, and one boy who is bald”**

☐ **OR**

☐ **“Three boys and three girls”**

This is a type of categorization based upon the concept of gender.

The same applies to other objects and event e.g furniture, fruit, clothes etc.

USE OF “CONCEPTS” IN THINKING WORLD

- Concepts are the categorization of objects, events, or people that share common properties
- When someone asks you, what are the main thing required for your room? One may start talking in terms of items like, bed, chairs, curtain, carpets etc, or ‘furniture and fixtures’ as a single category.
- These small categories reflect the operation of concepts and the process of cognition

- Concepts make possible the establishment of cognitive categories, and enable us to organize complex phenomena into a simpler and conveniently manageable form.

IMAGINE HOW MANY THINGS WE ARE IN CONTACT WITH IN THE WORLD AROUND US. DO WE, OR CAN WE NAME ALL OF THEM ALL THE TIME??????

- Newly encountered objects easily fit into our cognitive structure, if the conceptual category is already there.

Therefore,

If you know what a doctor does, then you can recognize any type of doctor, in any part of the world, in any type of attire, in any type of treatment setting.

**Imagine if we did not have concepts of fruit,
vegetables, grains, furniture, or weather!!!!**

- Concepts help to define, explain and elaborate complex phenomena into simple, understandable and usable categories, and they also include data from past experiences.
- There are three main types of concepts
 - a) Artificial concepts
 - b) Natural concepts
 - c) Prototype concepts

a) Artificial Concepts

- Concepts that have a unique set of traits and features
- These concepts are easy to define and elaborate. e.g. a rectangle has two opposite sides equal, if it is not the case, then it is not a rectangle.
- But our every day concepts are much more difficult than these concepts. e.g. what is the definition of an animal

b) Natural Concepts

- **Known, familiar and relatively simple concepts that have rather loose features to define and explain them**
- **These concepts are not universal in nature____ such as rectangle has two opposite sides equal, if it is not, then it is not a rectangle. This is not the case in natural concepts**
- **They can be varied in their definition and explanation by people e.g defining an animal?**

Simple and complex concepts:

- Some concepts are simple in the sense that they are clearly defined,
- When a concept is clearly defined, it is easy to distinguish an example from a non example e.g the concept of a square, or an equilateral triangle.

- But some concepts are rather difficult to define. These are defined in a variety of ways, and marked by a set of complex features.

- These may be ambiguous, overlapping, and even abstract e.g a bird, or a chair.

- Most of the things that we are in contact with in our everyday life are not as easy to define as a square or a triangle e.g. defining 'obsessive-compulsive disorder'.

- These may also involve the subjective experience of the person

And what about the concept of a comfortable chair or an easy or a difficult task?????

In such situations we need specific, and rather exact examples of a concept i.e., prototypes.

c) Prototypes:

- ❑ Examples of a concept that are typical and highly representative of a concept.
- ❑ Prototypes are used to define and explain objects and ideas that can not be defined in a clear-cut and straight forward manner.
- ❑ E.g the prototype of a table can be the 'dining table', or the prototype of a bird can be a 'crow'.

Agreement on prototypes:

- ❑ Usually people in a society, or those belonging to a particular discipline, are unanimous about the prototypes of a concept e.g if we have to give a prototype of a vehicle, then we will talk about a 'car' and not an 'escalator' or 'elevator', although these also move and take us from one place to another.
- ❑ On the other hand, if we are talking about varieties of 'stairs', then we can probably take escalators as an example.

What will happen if psychologists do not have a common definition of mental illness?

How do concepts help in thinking?

- ☐ We live in a complex world of objects, ideas, and relationships. It is with the help of concepts that we understand this world.
- ☐ Concepts make it possible to communicate ideas, thoughts, and feelings even when the object of interest is not actually present

Thinking and Reasoning

- ☐ Only humans can think in ways which no other organism is able to, as they have the ability to contemplate, analyze, recollect, plan out and carryout those behaviors.
- ☐ Humans are the only ones who are capable of using foresight as well as hindsight

- Even when we are not thinking, our mind wanders on present, past, future, pleasant or un pleasant, favorite or not so favorite things.
- And even we are dreaming or day-dreaming, actually it is also a form of thinking

Reasoning:

- ☐ It is the ability to use reason, logic, past experience, and learnt information for mental processing.
- ☐ For decision making, and problem solving etc.

Deductive Reasoning

- ❑ **Deductive reasoning is the process whereby logical conclusions, inferences, and implications are drawn by using a set of assumptions. these inferences are then generalized over, or applied to, specific cases.**

The assumptions or premises that are used for drawing conclusions are thought to be true and base upon reality. In many cases they are considered to be unchallengeable.

But at times these premises may turn out to be false when tested in reality e.g all men are brave, or men do not cry.

- Evaluating syllogisms is a techniques used for studying deductive thinking.

- A syllogism contains a series of two assumptions or premises, that are used for drawing conclusions e.g.,

- All women are talkative

- Anna is a woman

- Therefore, Anna is talkative

- This type of deductive thinking is largely influenced by cultural back grounds of people.
- It has been seen that people from more developed societies use rather abstract and logical thinking which they acquired through their experience and learning.
- People from less developed cultures mostly rely on concrete modes of reasoning.
- This sort of difference is largely due to the type of high quality, sophisticated education, variety of learning experiences, and the pressure to think logically and independently.

The system of education has an important role to play in this.

Lecture 30

Recap

- Cognition
- Mental images
- Concepts
- Thinking and reasoning
- Deductive reasoning
- Inductive reasoning

Inductive reasoning

- Specific cues are used for drawing inferences.

Inductive Thinking

- By using observation, knowledge and experience, different sets of inferences are drawn about the phenomenon of interest.
- Small bits of information is used to draw the general conclusion
- The person uses his or her observation, knowledge, and experience of specific case and infers general rules.

For example, if you have been noticing that your teacher is very soft spoken, usually marks you present even when you are late for the class, and forgives your mistakes, then you might take a chance for late submission of your assignment.

- **The major short coming of this type of thinking is that the conclusions may be biased, or the evidence used for drawing conclusions may be invalid, insufficient, or may be just a chance occurrence. Whereas appropriate conclusions have to be unbiased**

Thinking and Decision-Making

- It is one of the most complicated forms of thinking
- Cognitive psychologists are still focusing on the components and processes that underlie this type of thinking
- An important area in which the cognitive psychologists are most interested in.
- For many years psychologists have attempted to explain the processes involved for decision making, and solving problems. The most famous examples of which are
 - i. Thorndike's trial and error problem solving
 - ii. Kohler's insight problem solving

Mental shortcuts: Algorithms and Heuristics

- Algorithms: A rule, if it is applied, ensures the solution to the problem.
- Algorithms are always accurate.
- We do not necessarily understand their logical basis e.g algebraic formula

Algorithms and Heuristics

- Algorithms: A rule, if it is applied, ensures the solution to the problem.
- Algorithms are always accurate.
- We do not necessarily understand their logical basis e.g algebraic formula

Heuristics

- A rule of thumb that if used can be effective in finding solutions to problems, but may not ensure or guarantee this.
- For example rules for playing naughts and crosses, or preparing only the 'important' parts of the course for the exam

Problem Solving

- Thinking for the sake of finding solutions to problems.

- **Three major steps are involved in solving a problem**
 - i. Preparation for finding the solutions**
 - ii. Producing the solutions**
 - iii. Appraisal of the solutions that have been generated**

- **There are two types of problems**
 - a. Well- defined problem**
 - b. Ill- defined problem**

a. Well- Defined Problem:

- **Clear, definite and well formed problem. Means of solving that problem are available. e.g. when solving the mathematical equation in which the problem is rather difficult, but the method used to solve it is direct and available**

b.III- Defined Problem:

- Indefinite, unclear, ill- formed problem whose nature is not specifically defined and the ways for solving them is also difficult__ how do we build the morale of the crowd when their own team is losing the match???
- . In this, the problem is not specific and so the solution
- The main shortcoming of this type of problem is that it is not possible to draw the immediate and absolute conclusions and solutions of the problem

Kinds of Problems

- Problems are typically of three types
 - i. Arrangement problems
 - ii. Problems of inducing structures
 - iii. Transformation problems

i. Arrangement Problems:

- **Solutions of the problems require rearrangement and re evaluation of the components so that certain criterion will be satisfied**
- **There are a number of solutions to certain problems, but there are only specific solution that fulfill certain requirements.e.g. solving mathematical equation**

ii. Problems of Inducing Structures:

- It refers to the problems whose solution depends on the relationship of components among them so that new construct which have a relationship can be developed.
- e.g. see and tell what number comes next? 221- 412- 321- 512- 421- 612-. Firstly, one has to consider the existing relationship between these numbers then make out the other relationship
- **521-712

iii. Transformation Problems:

- **problems that require understanding and that should be solved in a manner that involves series of methods so that the initial problematic state can be changed into the goal attaining state.**

Steps for Problem Solving:

- Four steps are important for solving certain problem
 - i. Means- ends analysis___ Repeated testing of the behavior in order to lessen the distance between the goal and current existence e.g. taking instructions or asking questions for solving puzzles
 - ii. Sub goals__ Divide the problem into small element and then solve them in steps or sequentially

iii. Insight_____ Instant awareness of the relationship among the existing components which formally seems independent of each other

- A German psychologist Wolfgang Kohler was one of the first psychologists who observed this phenomenon, especially when facing challenges

iv. Evaluation of solution_____ The final step in problem solving which involves appraising the existing solution of the problem to demonstrate whether the solution is adequate or not

Impediments to Problem Solving

- There are various factors which serves as an obstacle in finding solutions. They are;
 - a. Functional fixedness
 - b. Mental set
- Functional fixedness is the capacity to think about certain phenomena in its most typical use or form.
- Mental set is the tendency to think of a solution in a most old patterned ways___ old means of finding and answering problems

Creativity and Thinking

- The word “ creative” is derived from the Latin word “ creare” means “ to make ” or Greek word “Krainein” means “ to fulfill”
- Creativity may be defined as the innovative, novel responses and ideas into a harmonious whole/ form
- Creativity can be flourished by two means
 - i. Inspiration
 - ii. Hard work

Stages in Creative Thinking

- Creativity mainly involves four steps
 - i. Preparation
 - ii. Incubation
 - iii. Illumination
 - iv. Verification
 - v. Revision

- i. Preparation: It includes assembling or combining the material and think thoroughly about it
- ii. Incubation: After thinking intensely about the certain problem, the person lets his mind free by putting the problem aside and let its solution incubate in the mind
- iii. Illumination: It also refers to as “insight” when innovative ideas are instantly generated__ a sudden flash comes to mind when one is brainstorming at it. Scientific innovations are one of the examples

- i. **Verification:** Evaluation of the problem to find out whether the solution is correct or not
- ii. **Revision:** This involves the whole of the above steps involved that should be used in order to reach on some solution

Factors Associated With Creativity:

- The main factors that may contribute in enhancing creativity is
 - a. Divergent thinking
 - b. Convergent thinking
- Divergent thinking involves varied thoughts and solutions to a certain problem
- Convergent thinking includes various thoughts and solutions for a particular problem

Creative thinking is
GOING BEYOND

Creative thinking is *going beyond*:

- the obvious
- the defined
- the laid out
- the conventional
- the common
- the usual

Creative thinking is:

The ability to generate a variety of unusual solutions to a problem

tive thinking is:

- open
- original
- imaginative
- uninhibited
- exciting
- Fulfilling
- lateral
- may be stray and wild at times

Lateral versus Vertical Thinking

Lateral thinking:

- ☐ Finds new ways of looking at things
- ☐ Avoids looking for what is “right” or “wrong”
- ☐ Analyzes ideas to generate new ideas
- ☐ Considers the irrelevant
- ☐ Progresses by avoiding the obvious

Vertical thinking:

- ☐ Tries to find absolutes
- ☐ Seeks continuity
- ☐ Finds what is right: seeks “yes” or “no” justifications
- ☐ Looks for stability: rejects irrelevant information
- ☐ Uses established patterns & considers the obvious

Lateral Thinking:

A deliberate process and set of techniques for generating new ideas by changing an individual's or team's way of perceiving and interpreting information; as opposed to vertical thinking i.e., a logical step-by-step process of developing ideas by proceeding continuously from one bit of information to the next.

Lateral/Creative Problem Solving

- ❑ **Reversal Technique:** examining problem and turning it completely around; inside out or upside down
- ❑ **Analogy Technique:** developing a statement about similarities between objects, persons, or situations
- ❑ **Cross Fertilization Technique:** asking experts from other fields (totally different fields) to view the problem and suggest methods for solving it from their own area of

Assessing and Examining Creative thinking

- Name all the things you can think of that are *round/circular* in shape.
- List as many white, edible things as you can.
- List all the uses that you can think of a water melon.
- List all the possible uses of a pencil.

- Mr. X has been told a number of times to be in office at 9 but he is always late.WHY???
- Ms. Q has a number of clothes,but she has been wearing the same dress for the last two weeks. WHY???
- Your rich client comes to your office. You offer him coffee but he refuses. WHY???

**“ You can become what you
imagine yourself to be!!!”**

- ☐ Use the power of your imagination
- ☐ Make a mental movie
- ☐ You are the hero of this movie
- ☐ You can do whatever you like to do
- ☐ SO.....why don't you do it???????

As If:

Appearing for an interview

- ❑ **IMAGINE** *As If* you were to introduce yourself to an interviewing board
- ❑ **THINK** *As If* you were introducing yourself to an interviewing board
- ❑ **ACT** *As If* you were actually introducing yourself to an interviewing board

THINKING LOUD:

Your friends Z and X have stopped talking to each other , whereas they have to work together in the same office.



- ☐ a: Think of as many reasons as you can of the dispute
- ☐ b: Think of as many ways as you can of a possible patch up planned by you

Creative thinking and Problem Solving:

Define the problem

Clearly thinking about all possible aspects of the issue before taking decisive action

- ❑ The cases of shop lifting in a super market rose to such a high level that they had to increase the prices. They hired guards but that did not work much. However they finally found a solid solution. How???

Concept Challenge

Challenging the established ideas for new solutions

- ☐ Women should do house work
- ☐ In solving disputes all parties should have equal part in decision making

Cognitive Complexity:

- Interested in use of more philosophical and abstract ways of thinking___ these type of people give preference in using complex, abstract and intricate stimuli and thinking patterns. Comedy, jokes and humor is its best example

Lecture 31

Recap

- Cognition
- Inductive reasoning: its advantages and shortcomings
- Decision making
- Problem solving
- Creative thinking

Personality

What do we mean when we say:

- She has got a gorgeous personality!
- He has a powerful personality!
- He has a dull personality!
- She has a vibrant personality!

- In saying so, are we referring to:
- The looks of a person? or
- The overall impression that one leaves on our
- mind? or
- The way a person behaves with us? or
- The way, as we feel, others treat a person?

And What About When We Say

- He has a strong personality.
- She is a good decision maker.
- He has a strong will power.
- She is too sensitive.
- He is very aggressive.

Here We Are Talking About

- The feelings,
- Emotions,
- Cognitions, and
- Psychological make up of a person.

Personality

- ❑ The sum total of characteristics on the basis of which people can be differentiated from each other.
- ❑ The stability in a person's behavior across different situations.
- ❑ Characteristic ways in which people behave.
- ❑ Characteristics that are relatively enduring, and that make us behave in a consistent and predictable way.

Theories of Personality

- Also known as approaches to personality.
- Understanding the “WHAT”, “HOW”, and “WHEN” of characteristics and features that make up an individual’s personality.

“WHAT”

- What is personality?
- What are the different aspects of personality?
- What variables account for these characteristics?
- What factors can have an impact, positive or negative, on our personality?

“HOW”

- How do these characteristics develop?
- How do we become what we are?
- How can we manipulate, improve, or modify our own, or someone else's personality?

“WHEN”

- When does personality develop?
- When is personality more pliable, and modifiable?
- When can a person’s personality be influenced more by others.

1. Psychodynamic Approach

- Approach that focuses upon the unconscious determinants of personality i.e., psychologists belonging to this approach believe that unconscious forces determine our personality.

Unconscious

- The part of personality which we are not aware of.
- Unconscious contains instinctual drives:
 - Infantile wishes,
 - Desires,
 - Demands, and
 - Needs.

- **These instinctual drives are hidden in the**
- **unconscious, and do not surface at the conscious**
level.
- **At the same time the person seeks satisfaction**
and fulfillment of these drives as they can be a
source of pleasure and satisfaction.

Why are these drives hidden then?

- Because they can cause conflict and pain if they became an obvious or overt part of our lives.
- Therefore, they are pushed into the depths of our unconscious.

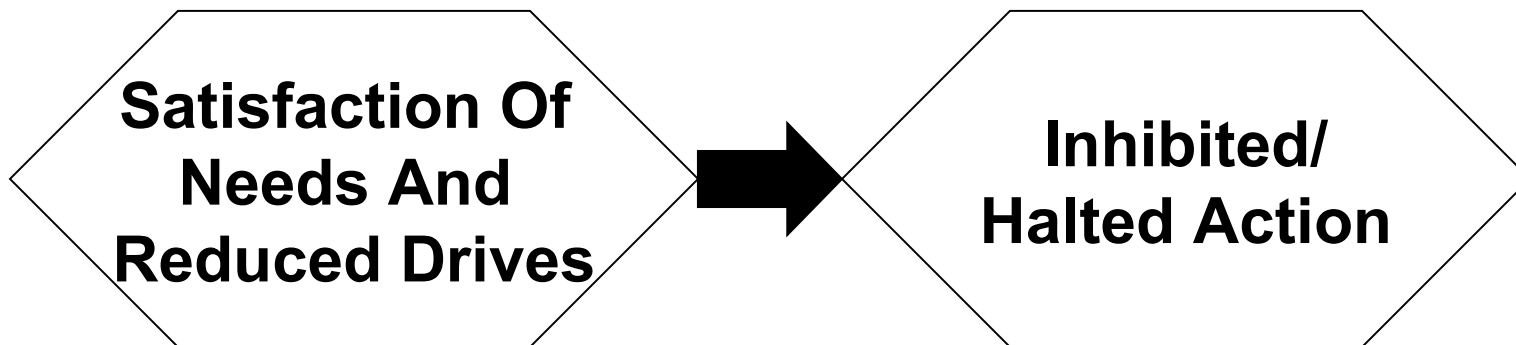
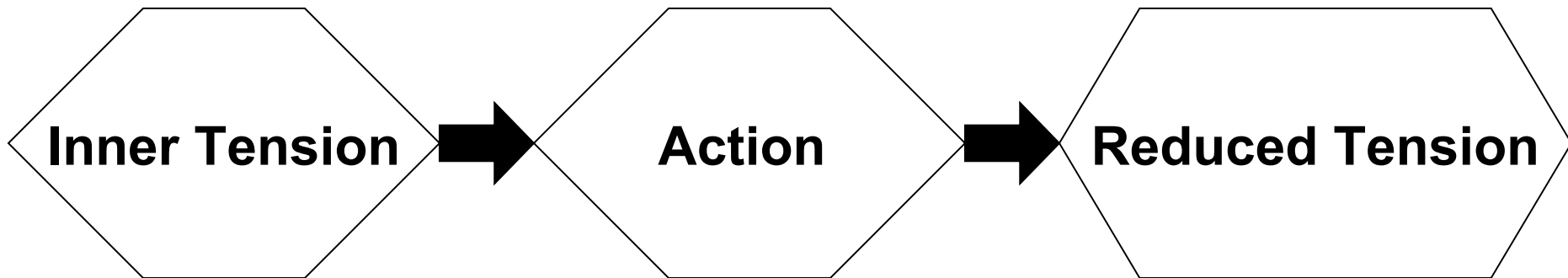
•Psychodynamic Approach was founded by Sigmund Freud, the most influential figure in the history of psychology.

•According to this approach the basis of motivation and behavior lies in inner forces: forces that are predetermined.....forces over which humans have little control..... which the person is not aware of i.e., unconscious determinants of behavior.

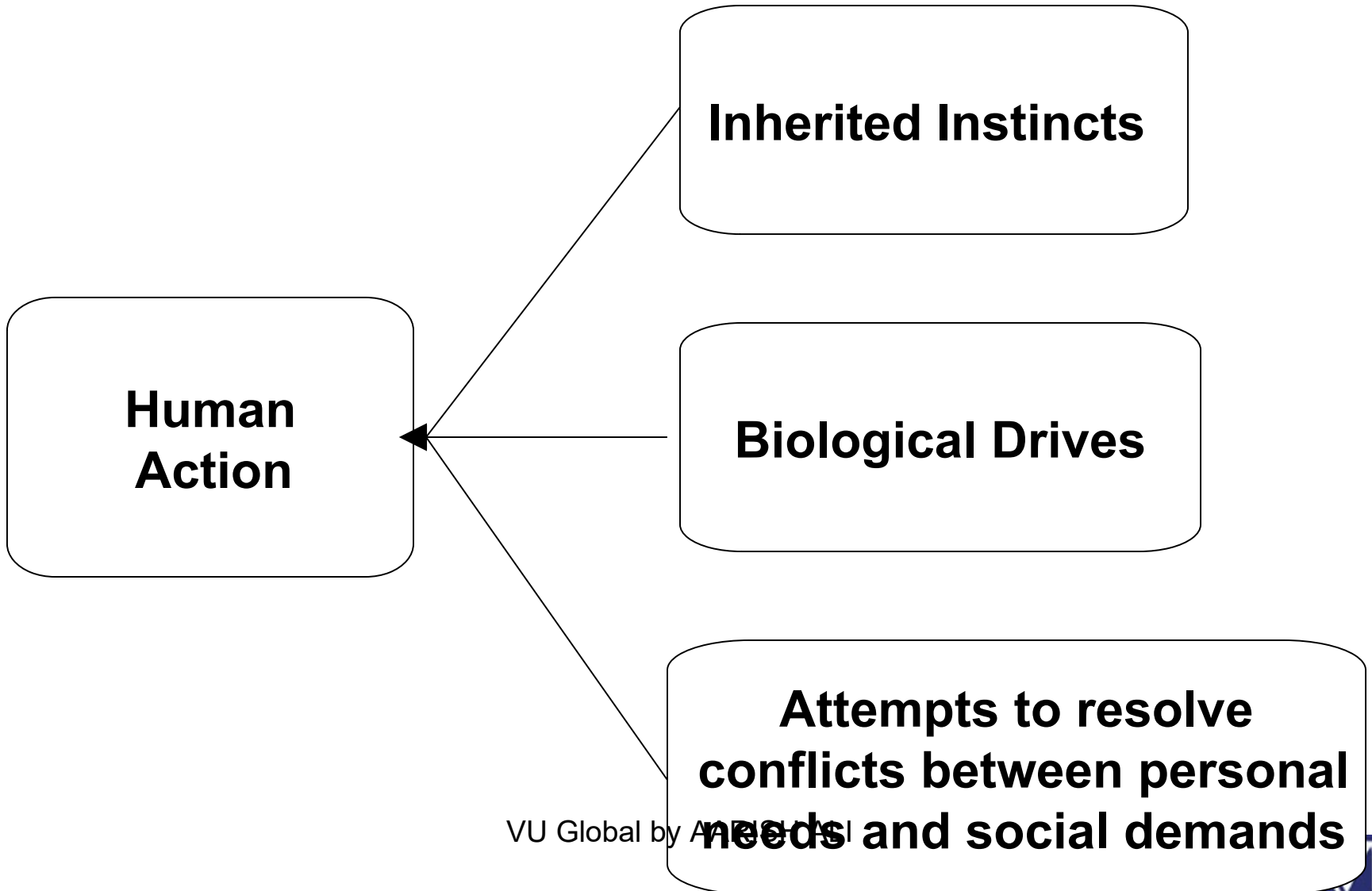
Significance Of Psychodynamic Approach

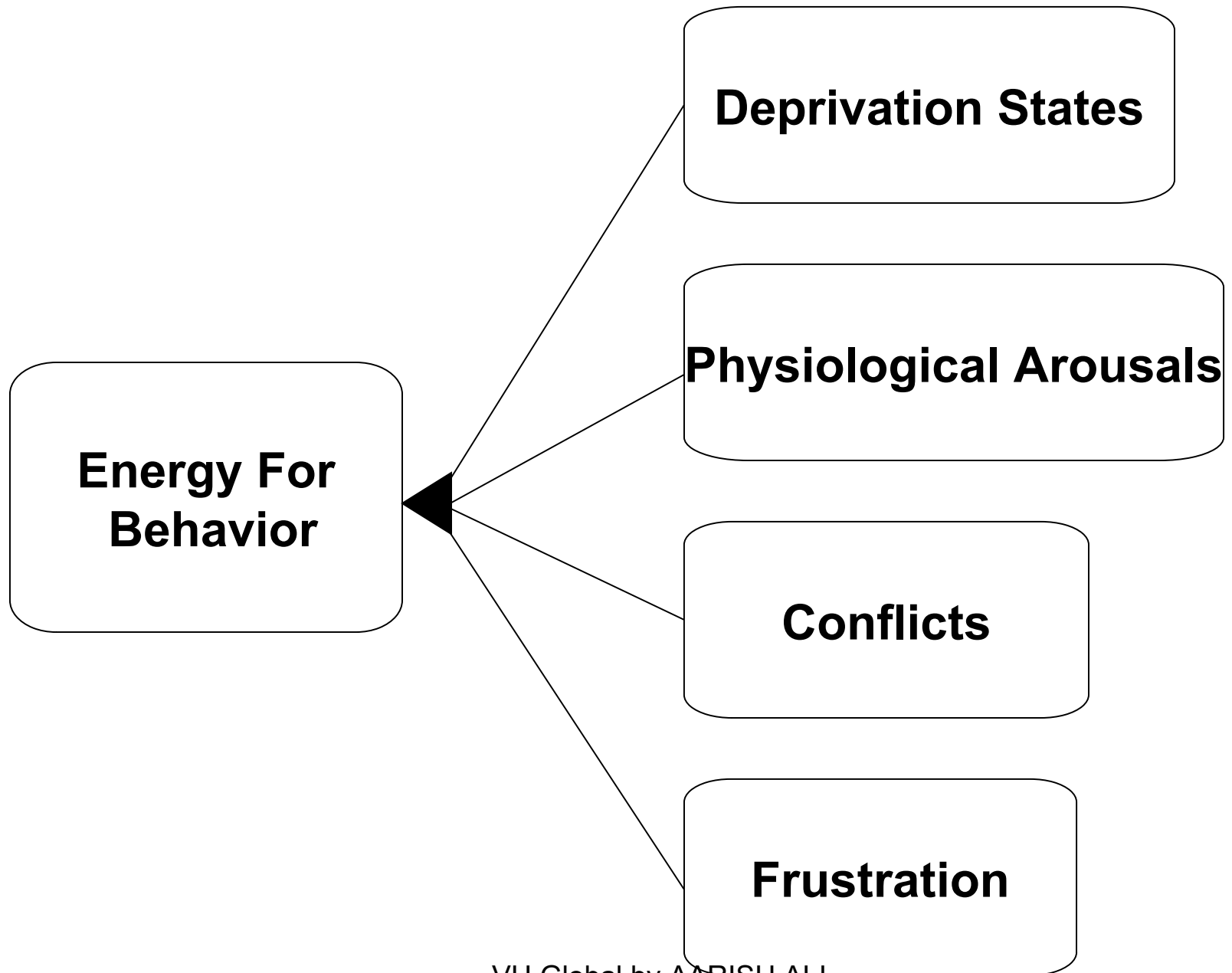
- ❑ The most influential theory of the 20th century.**
- ❑ Affected psychology and related disciplines in a revolutionary manner.**
- ❑ Gave an entirely new perspective to the understanding of behavior and mental processes, as well as mental illness.**

- ❑ The first theory to raise the awareness that not all behavior is rational, well thought of, and planned.**
- ❑ Besides giving an impressive, broad based, therapeutic approach it provided a basis for understanding everyday life phenomena e.g. interpersonal relationships, aggression, prejudice.**
- ❑ Many other approaches built their paradigms on this approach_____some by refining it, some by deviating from it.**



Roots Of Human Action





Foundations of Psychodynamic Approach

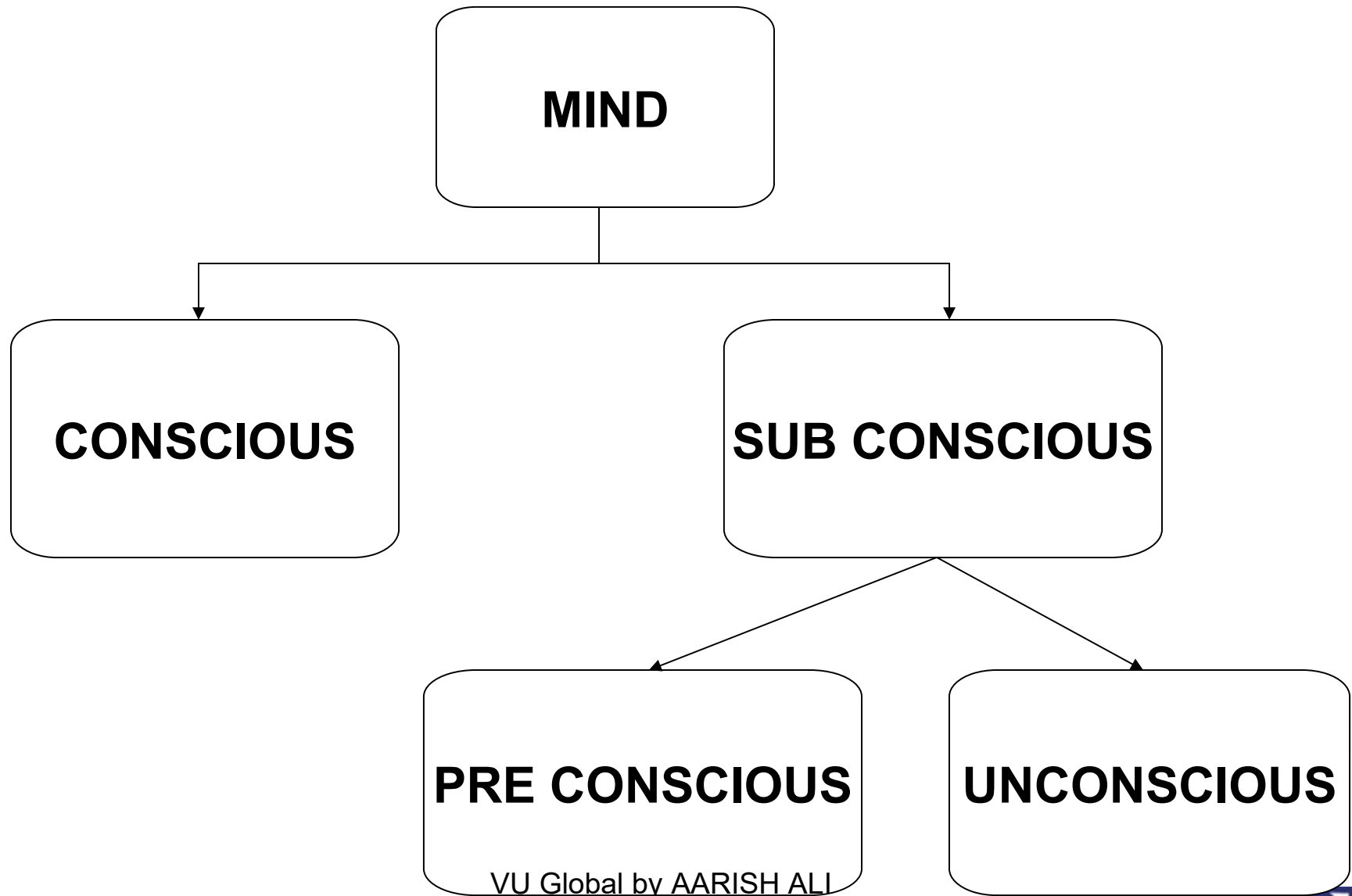
i. Psychic Determinism:

All behavior is determined i.e., has a cause that lies in the mind/psyche.

ii. Role of Unconscious :

A significant part of our behavior is generated by unconscious forces.

Structure of Consciousness



i. Conscious:

Contains thoughts and feelings which one is immediately aware of.

ii. Subconscious:

Mind level below the level of conscious awareness.

iii. Preconscious:

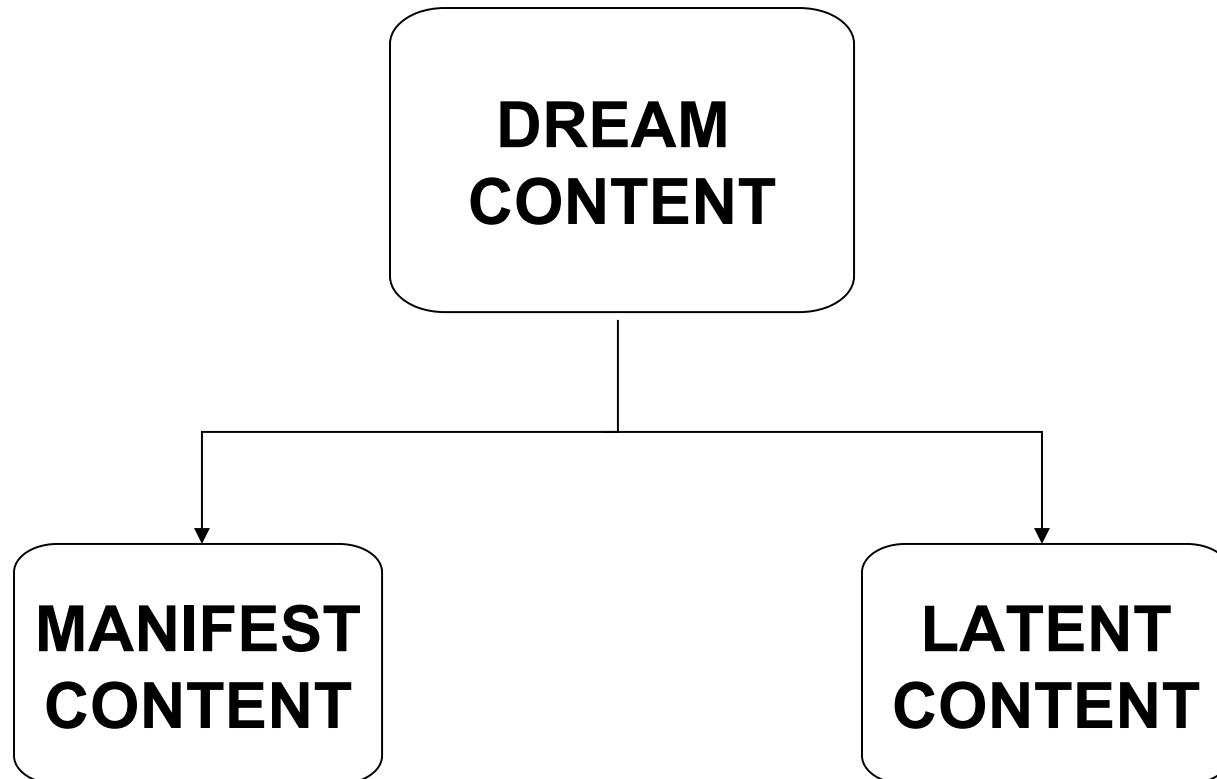
Part of the sub conscious that can be accessed by deliberate choice.

iv. Unconscious:

Part of the sub conscious that cannot be accessed directly, although impulses, ideas, and feelings may permeate out through other sources e.g. dreams, slips of tongue etc.

Dreams in Freudian Approach

Dreams reflect unconscious needs, desires, impulses.



Symbolism:

- ❑ The manifest content is in a symbolic form.**
- ❑ Converted into this form by the ‘dream censor, a mechanism that ensures that sleep is not disturbed by unconscious desires, and those desires are presented in a socially acceptable form.**

Motivating Forces

```
graph TD; A[Motivating Forces] --> B[EROS  
Life affirming force  
expressed in  
creative activities  
including procreation]; A --> C[THANATOS  
Destructive energy  
expressed in self  
destructive activities  
as well as aggression  
towards others];
```

EROS

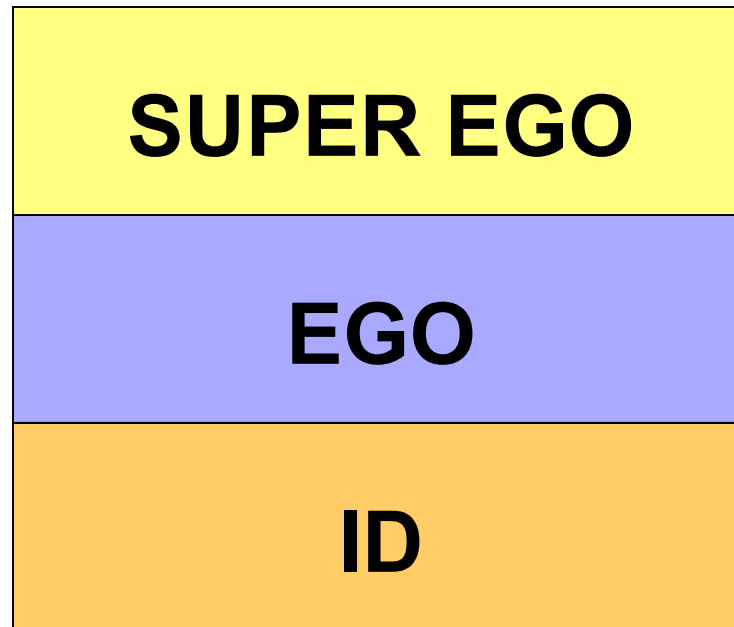
**Life affirming force
expressed in
creative activities
including procreation**

THANATOS

**Destructive energy
expressed in self
destructive activities
as well as aggression
towards others**

Psychodynamic Model of Personality

- ❑ A three part structure:



i. Id:

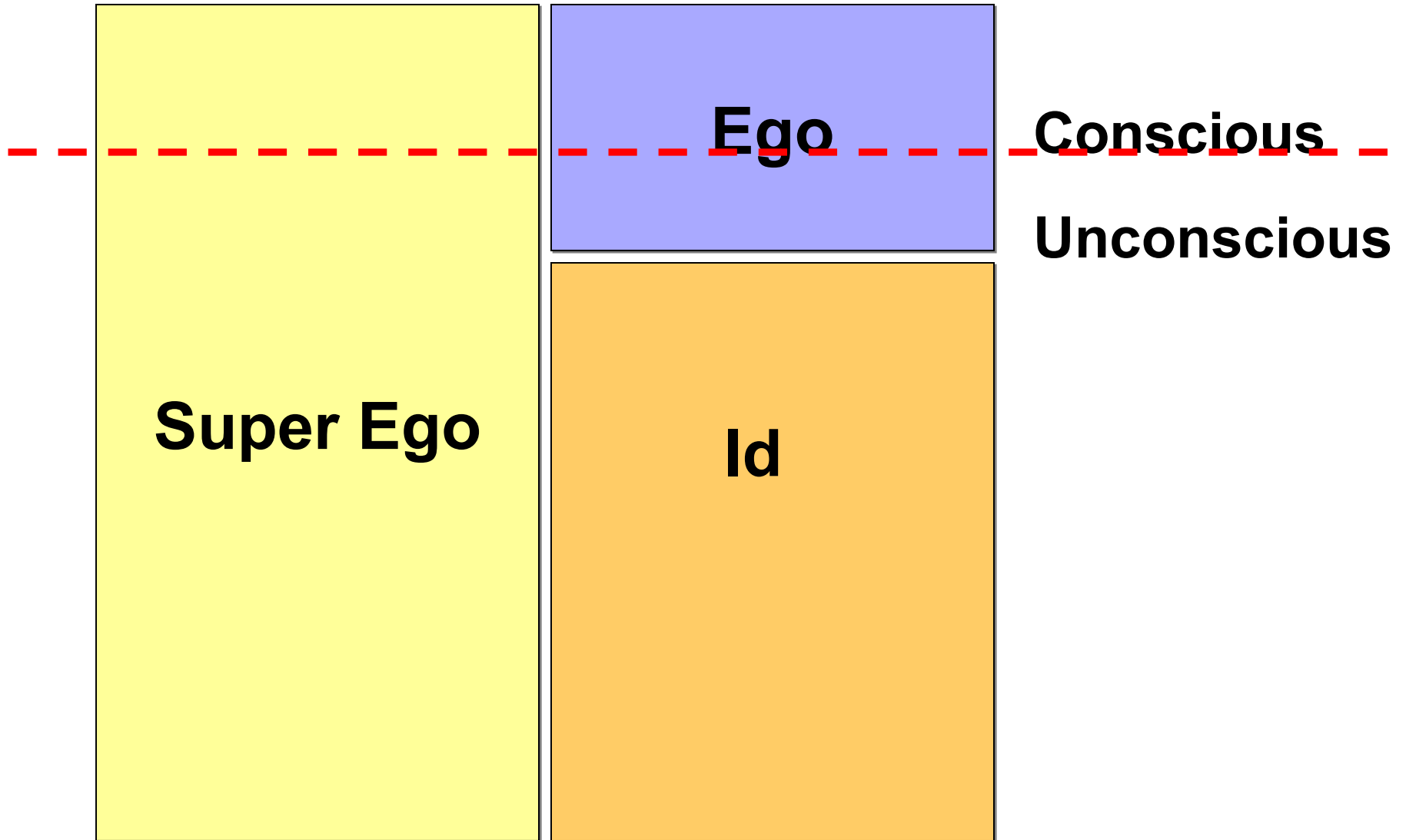
The source of basic drives; operates under the ‘pleasure principle’ i.e., wants immediate gratification of needs.

ii. Ego:

Mediates the link of the self with the outside world, the ‘real world’, as well as between the id and superego; ego operates under the “reality principle” or the demands of the environment.

iii. Super Ego:

- ❑ Governed by the moral constraints**
- ❑ Opposes the id and represents the moral demands of the family and society; it is the ‘moral self’ or the ‘conscience’ of the person.**



Development of Personality: A Stage Approach

Oral Stage	Birth 1 ½ Years	Ego formation begins, weaning begins, delayed gratification is learnt; body image develops.
Anal Stage	1 ½ — 3 Years	Continuation of ego development; toilet training; formation of super ego.

Phallic Stage	3__5 Years	Bodily and genital awareness.
Genital Stage	Puberty_ end of life	Symbolic gratification of drives;secondary process thinking.

Oedipal conflict and Electra complex

- Oedipal conflict:

(Also known as Oedipus Complex). During the phallic stage, the male child begins to develop love and positive feelings for the mother; whereas negative feelings for the father since he is seen as a rival. But as the father is seen as too strong and powerful, the child fears retaliation and ultimately begins to develop ' identification' with the father.

- Electra complex:

The female child feels the same way toward the father, as the male felt for mother in oedipal conflict, but ultimately chooses 'identification' with the mother.

Anxiety:

- ❑ An emotional state experienced as a result of felt threat to the self.**
- ❑ Anxiety arises when ego cannot cope too much of:**
 - i. Demands of the id,**
 - ii. Demands of the ego,**
 - iii. External danger.**
- ❑ In order to protect itself against anxiety and threat, ego uses defense mechanism.**

Defense Mechanisms

Criticism Against Freudian Psychodynamic Theory

- ❑ There is no scientific proof that many psychodynamic constructs , e.g. unconscious, exist.**
- ❑ Psychic Determinism: Freudian approach is deterministic and leaves not much room for conscious, rational, decision making or personal will to act.**

- ❑ It emphasizes the early childhood experiences too much.**
- ❑ It ignores the external variables and the environment.**
- ❑ Mostly criticized for its interpretation of the relationship between the two genders.**
- ❑ The therapy is too time consuming and therefore expensive.**

The Psychoanalytic Approach after Freud

The Neo- Freudian

- ❑ The theorists who belonged to the Freudian school and supported it, but later digressed on some issues and differed from Freud.**

Disagreement with Freud;

- Emphasis on the functions of ego, and the control that it had over routine-life activities.
- The impact of social variables.
- The role of society and culture on personality development.
- Primary importance of sexual urges.

- ❑ The Neo-Freudians emphasized, more than Freud, the following:**
 - i. The role of current social environment.**
 - ii. Life experiences have a continuing influence and childhood alone should not be of prime importance.**
 - iii. Positive interpersonal relations of love, and social motivation have a significant role.**
 - iv. Ego functioning is more significant rather than id.**

iv. Development of self concept is important.

v. self esteem is important.

Significant Neo- Freudians

Carl Gustav Jung

- ❑ **The founder of the analytical school of psychology, Jung was mystical in his understanding and description of personality.**

❑ He had a positive approach toward one's ability to control one's destiny.

Jung's disagreements with Freud:

❑ The understanding and description of the genders.

❑ The nature of unconscious.

Major Goal of Life: Unification of all aspect of our personality:

- ❑ Conscious and Unconscious.**
- ❑ Introversion (inner directed), extroverted (outer directed).**

Libido:

Energy for personal growth and development.

Types of Unconscious:

- ❑ Personal: Similar to Freudian view.**
- ❑ Collective: ideas, and influences beyond personal experience, inherited from all generations of our ancestors and common to all humanity .**

Archetypes:

- **Part of collective unconscious; universal forms and patterns of thought. These include themes that can be seen in myths e.g. masculinity, femininity, good, evil opposites, motherhood.**
- **Archetypes are the universal representations of a particular person, object, or experience, e.g archetypes of mother, good, or evil._**

Adler's Approach

- ❑ **Disagreed with Freud's emphasis on the significance of sexual needs.**

Main concepts: Esteem, inferiority complex, birth order, will to power and style of life.

- i. We are a product of the social influences on our personality.**
- ii. Goals and incentives drive us more than drives and instincts.**
- iii. Our goal in life is to achieve success and superiority.**

Primary Human Motivation

- **Striving for superiority; achievement of self-improvement, and perfection, and not superiority over others.**
- **Inferiority complex , on the other hand, is the state when people feel that they have not been able to conquer, as adults, the feelings of inferiority that they developed as children.**
- **Inferiority complex: the feeling of being less able than others.**

- ❑ **Inferiority complex**: the feeling of being less able than others.
- ❑ **Motivating Forces of Human Life**:
 - i. Feeling of inferiority, and a desire to overcome it through striving for superiority.
 - ii. People are primarily motivated to overcome inherent feelings of inferiority.
- ❑ **Birth Order**:
Has effect on personality.

VU Global by AARISH ALI

- ❑ **Sibling Rivalry**:



Karen Horney 1885-1952

- ❑ She agreed with Freud on the levels of unconscious, anxiety, and repression.**
- ❑ She emphasized childhood experiences, social interaction and personal growth.**

Disagreement with Freud:

- ❑ Differed from Freud on primary impulses; impulses are not the main motivating force.**
- ❑ Disagreed on Freudian position regarding the biological basis of differences between the males and females.**

Horney's Main Concepts

Basic Anxiety:

A major concept; If the environment is hostile and the child feels lonely and isolated, then this type of anxiety develops. It can be overcome by proper parental nurturing.

Basic Hostility:

- ❑ Children develop such hostility if parents are over strict, punishing, indifferent, or inconsistent.**

❑ Children feel very aggressive and hostile but cannot express it. Repressed hostility leads to anxiety.

Social Interaction and Interpersonal Styles:

The ways in which people interact with each other:

- ❑ Moving away from others: seeking self sufficiency and independence.**
- ❑ Moving toward others: being compliant and dependant.**
- ❑ Moving against others: trying to gain control, power, and independence.**

Neuroses:

Arise from emotional conflicts that arise from childhood experiences, and disturbances in interpersonal relationships in later life.

Erik Erikson's Theory of Psychosocial Development

- ❑ Student and follower of Sigmund Freud.**
- ❑ Broke with his teacher over the fundamental view of what motivates/drives human behavior.**

❑ For Freud, it was ‘biology’ or more specifically the biological instincts of life and aggression (Eros and Thanatos).

❑ For Erikson, the most important force that drives human behavior and which helps in the development of personality was “social interaction”.

❑ His developmental theory of the "Eight Stages of Man" (Erikson, 1950) was unique and different in the sense that it covered the entire lifespan rather than 'childhood' and 'adolescent development'.

❑ He believed that social environment combined with biological maturation results in a set of "crises" that must be resolved.

❑ The individual passes through the "sensitive period" in different stages which has to be resolved successfully before a new crisis is presented. The results of the resolution, whether successful or not, passed on to the next crisis and provide the foundation for its resolution.

❑ He proposed eight stages of psychosocial development that have been discussed in detail in the section on cognitive development.

2.Trait Approaches

- **Approaches that propose that there are certain traits that form the basis of an individual's personality.**
- **These approaches seek to identify the basic traits necessary to describe and understand personality.**

Traits

- **Enduring dimensions of personality characteristics that differentiate a person from others.**

- **Trait theories do not imply the absence or presence of different traits in different people.i.e., either/or situation.**
- **These assume that some people are relatively high on some traits whereas, some are low on the same traits.**

- **The difference in people in terms of these traits is a matter of degree or extent to which the traits exist and manifest themselves.**
- **The degree to which a trait is present can be quantified.**

- **e.g. “depression” is a tendency found in people; some have it more and some less.**
- **Similarly “ friendliness”; some are more friendly and some less.**
- **The point to remember is that all traits may exist in all but these vary in the degree of impact.**

Allports' Trait Theory

- **After skimming an unabridged dictionary, Gordon Allport identified 18000 separate terms that could be used to describe personality.**
- **After eliminating synonyms he came up with a list of 4500 descriptions.**

BUT the important question was that:

- **WHAT WERE THE MOST BASIC TRAITS?**

Allports' Basic Traits

Categories:

- **There are three classic categories of traits:**
 - i. **Cardinal traits**
 - ii. **Central traits**
 - iii. **Secondary traits**

i. Cardinal Traits:

- **A single personality trait directing most of the person's behaviors and activities.e.g affection, affiliation, kindness, greed.**
- **The person's whole life, or behavior, is influenced by this trait. A person who served the poor and the weak all his life may have a very high degree of “kindness” or “nurturance”.**

- **Or a person who likes to hoard things, people, and wealth may be ruled by a high degree of “greed”, or perhaps “inferiority”.**

ii. Central Traits:

- **Do all people possess cardinal traits that encompass all aspects of their being?**
- **PERHAPS NOT!!!**
- **Most people develop a group or set of traits rather than a single one, that form the core of their personality.**
- **Central traits are those major characteristics that make up the core of someone's personality.**

- **Central traits usually number from 5- 10 in a person.**
- **e.g., affection, love for humanity, and nurturance will form one type of personality.**
- **Inferiority, need for control, and greed may give a different shape to personality.**

Lecture 32

Recap

- Personality
- The concept of personality
- Personality Theories
- The psychodynamic approach to personality
- Trait approaches
- Allports' trait theory
 - i. Cardinal traits
 - ii. Central traits

iii. Secondary Traits

- Qualities or characteristics that do have an effect on our personality, but are much less influential than cardinal or central traits.
- These affect fewer life situations as compared to the cardinal or central traits.
- For example preferring to wear certain colors, or a liking for specific tastes or smells.

Trait Theories Based Upon Factor Analysis:

- **Factor analysis:** A statistical method whereby relationships between a large number of variables are summarized into fewer patterns. These patterns are more general in nature.

For Example:

- A researcher prepares a list of traits that people may like in an ideal man.

The extensive list is then administered to a large number of people, who are asked to choose traits that may describe an ideal man.

Through the factor analysis, the responses are statistically combined and the traits associated with one another in the same set(or person) are computed.

Thus the most fundamental patterns are identified. These patterns are called factors.

Psychologists Raymond B Cattell , and Hans Eysenck presented trait theories based upon factor analysis.

Raymond Cattell's Sixteen Personality Factors

- after using factor analysis Cattell proposed that two types of characteristics form our personality
- Surface traits, and
- Source traits

Surface traits

- Cattell's factor analysis showed that there are 46 surface traits or clusters of related behavior.
- These traits are the characteristics that we can observe in a given situation.
- The frequently quoted example in this regard is that of a friendly, gregarious librarian, who is so helpful that he might go out of his way to help you; as a result of your interaction with him it can be decided that he possesses the trait of sociability. His sociability is a surface trait in Cattell's terms.

- BUT surface traits may not necessarily represent the traits that actually underlie the personality of a person; Surface traits are what we directly observe, and these are based upon our perceptions and representations of personality. These may not be the true descriptions of the actual underlying dimensions of someone's personality.

- The characteristics that form the actual roots and basis of all behavior may be different, and fewer in number.

Source Traits

- In order to go beyond the surface traits, Cattell carried out further factor analysis.
- He could identify 16 traits that that represent basic dimensions of personality.
- He called these traits, source traits.

16 PF: Sixteen Personality Factor Questionnaire

- Cattell developed a measure that provided a score for each of the 16 source traits.

Factor symbol	High scorer	Low scorer
A	Outgoing	Reserved
B	More intelligent	Less intelligent
C	Stable	Emotional
E	Assertive	Humble
F	Happy-go-lucky	Sober
G	Conscientious	Expedient
H	Bold	Shy
I	Tender-minded	Tough-minded
L	Suspicious	Trusting
M	Imaginative	Practical
N	VU Global by AARISH ALI Shrewd	Forthright

Factor symbol	High scorer	Low scorer
O	Apprehensive	Placid
Q1	Experimenting	Traditional
Q2	Self-sufficient	Group-tied
Q3	Controlled	Casual
Q4	Tense	Relaxed

Eysenck's Dimensions of Personality

- According to Eysenck, personality can be understood and described in terms of just two major dimensions:
 - a) Introversion-extroversion
 - b) Neuroticism-stability
- On the first dimension, people can be rated ranging from introverts to extroverts: the rest of the traits fall in between.
- The second dimension is independent of the first one, and ranges from being neurotic to being stable.

Introversion-Extroversion

- Introverts:

Quiet, passive, and careful people.

- Extroverts:

Outgoing, sociable, and active people.

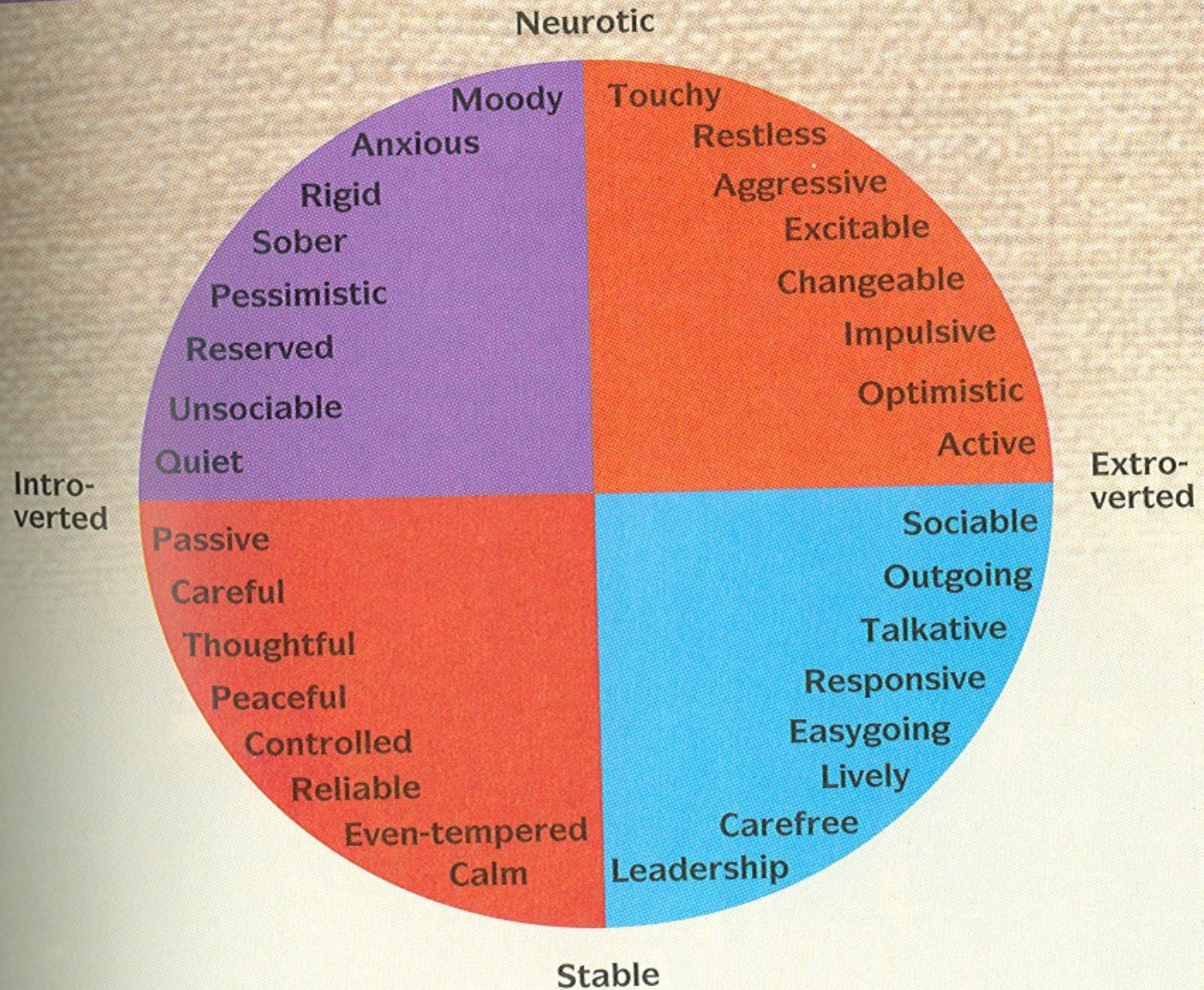
Neuroticism-Stability

- Neurotics:

**Moody, touchy, and
anxious people.**

- Stable:

**Calm, care-free, and
even-tempered
people.**



Eysenck evaluated a number of people along these dimensions. Using the information thus obtained, he could accurately predict people's behavior in a variety of situations.

The Recent Approach to Understanding Personality Traits:

- The “Big Five”:

Five broad trait factors lie at the core of personality:

Surgency: Extroversion and sociability

Neuroticism: Emotional stability

Intellect

Agreeableness

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Conscientiousness

3. Learning Approaches to Personality

- Approaches that focus upon the “observable” person rather than the inner drives, instincts, motives, thoughts, or traits.
- For the learning theorists:
Personality is the aggregate of a person’s learned responses to the external environment.
- Variables considered most important by the learning theorist are the features of a person’s environment.

Learning approaches are primarily based upon the principles of;

- Classical Conditioning
- Operant Conditioning
- Cognitive Learning

B. F. Skinner's Approach

- Personality is a collection of learned behavioral patterns.
- Patterns of reinforcement that have been received in various situations in the past cause similarities in responses across different situations, when same or similar situations are encountered.
- For example a student tries to make a good presentation every time he has to present because he has been receiving positive reinforcement for good presentations in the past.....not because of an inborn drive or a trait of being a hard working or industrious person.

Similarly

- A person who is never aggressive may be so because he was always punished for aggressiveness and rewarded for being polite.

For Learning Theorists

- Consistencies in behavior across different situations are not as important as the strategies for modifying behavior are.

Learning theorists are more optimistic in their approach , as compared to the psychodynamic theorists; they believe in the potential for change, and do not believe in the passivity of psychic determinism.

Social Cognitive Approach to Personality

- The approaches that lay emphasis upon the role of people's cognitions in determining their personalities.
- Cognitions include: people's thoughts, feelings, expectations, and values.
- These approaches consider the "inner" variables to be important in determining one's personality.

These approaches emphasize the reciprocity between individuals and their environment .

There exists a web of reciprocity, consisting of the interaction of environment and people's behavior. Our environment affects our behavior, and our behavior in turn influences our environment and causes modifications in the environment . The modified environment in turn, affects our behavior.

Albert Bandura

- According to him:
We possess the ability to foresee the probable consequences of certain of our behaviors in a given setting, without actually having carried out those behaviors or actually being in those settings.
- This so happens primarily as a result of “observational learning” i.e., having seen the outcomes of others(models) performing the same behaviors in same or similar situations.
- For example, this is how we learn to be aggressive, sociable, or industrious.

Bandura also emphasized;

- Self-efficacy, and
- Reciprocal determinism.

Self-efficacy

- Self-efficacy consists of learned expectations that one is capable of performing a certain behavior, or producing a desired outcome.
- Self-efficacy is the underlying variable in people's faith in their ability to carry out a particular behavior.
- The higher the sense of self-efficacy in a person the greater will be the persistence in his behavior, and also the greater will be the likelihood of his success.

Reciprocal Determinism

- According to Bandura, the key to understanding behavior lies in reciprocal determinism.
- We can understand the personality and behavior of a person by understanding the interaction between the environment, behavior, and the individual; and how this interaction causes people to behave in the manner they do.
- Environment affects behavior and the behavior in turn affects the environmental factors.

For example:

- A woman likes to make friends. She gets an opportunity to make friends at parties. She in turn arranges parties herself and invites people she likes, or those she thinks are potential friends. Her desire for finding friends is satisfied as a result, at the same time she becomes confident that she can achieve what she wants by working on it. This causes persistence in her behavior.

4. Humanistic Approach to Personality :

- The humanistic approach stresses that people possess a basic goodness, and have a natural tendency to grow to higher levels of functioning.
- They have a conscious, self-motivated ability to change and improve.
- The basic goodness, and the natural tendency to grow, along with their unique creative impulses form the core of personality.

Carl Rogers

- All people require to be loved and respected. This is a universal phenomenon that is reflected in their need for positive regard.
- This love and regard comes to us from other people. When other people provide for this basic need, we become dependent on them. We begin to rely on others' values and evaluate and judge ourselves through the eyes of others.

Self-concept and Conflicts

- Our self-concept and others' opinions are related.
- At times there may be discrepancies or conflicts between our self-concept (self-impression) and our actual experiences.
- Minor discrepancies lead to minor problems, whereas deeper conflicts lead to psychological disturbances in daily functioning e.g frequent obsessions or anxiety.

Unconditional Positive Regard

- A person's conflicts can be resolved if he receives unconditional positive regard from another person.
- Unconditional positive regard means an attitude of total acceptance and respect from another person without any conditions. No matter what you say or do, the person accepts it.
- As a result of this acceptance, a person gets an opportunity to evolve and grow cognitively as well as emotionally, and to develop a more realistic self-concept.

Self-actualization

- **According to the humanistic approach, self-actualization is the ultimate goal of personality growth (see Rogers and Maslow).**
- **Self-actualization is a state of self-fulfillment in which people realize their optimal potential.**
- **Self-actualization occurs when our everyday life experiences and our self-concept match closely.**
- **Self-actualized people accept themselves the way they are in reality. This enables them to achieve happiness and a feeling of fulfillment.**

5. Biological Approaches to Personality

- Approaches that emphasize the significance of biological variables and inherited personality characteristics.
- These approaches propose that important components that constitute our personality are inherited or genetically determined e.g temperament .

Temperament

- Temperament is one of the main ingredients of personality.
- Temperament is the basic, innate disposition that emerges early in life.
- Even very young infants show signs of different dispositions e.g some smile, some frown even when otherwise at ease, some are irritable, some calm, some shy, and some restless.
- Such behaviors persist and at an early stage in their life the children are labeled as stubborn, shy, restless etc.

Inhibited Children

- According to Jerome Kagan children who are unusually fearful of the sight of unfamiliar adults, and fret when confronted with unfamiliar objects or new settings are the inhibited children.
- Such children are labeled as “shy” by their parents and teachers by the age of 3-4 years.
- They are consistently shy and emotionally restrained and noticeably quite in unfamiliar situations.
- They constitute around 10% of all children.

There are biological differences between the inhibited and uninhibited children

- At age 5 muscle tension (especially in the vocal cords and the larynx) is higher in inhibited children.
- They differ in the heart beat pattern too. they experience more of rapid resting heart beat. In case of confronting a new situation their heart beat increases more.
- Hormonal differences and variations in the excitability of the limbic system of the brain have also been seen to be different in the two groups.

Kagan concluded that these differences can be explained in terms of an inborn characteristic of the inhibited children i.e., their greater physiological reactivity.

Twin Studies Supporting the Genetic Argument

- A number of studies on twins reared together and reared apart have supported the biological approach to understanding personality.
- Study by Auke Telegen and colleagues (1988):
A sample of 350 pairs of twins was studied. They included 44 genetically identical twins who were reared apart.
- The subjects were given a battery of tests, including one that measured personality traits.

The Results Showed That

- The twins were quite similar in their personality, in major respects.
- There are certain traits that are more influenced by heredity than others.
- Genetic component was found to be particularly strong in case of social potency and traditionalism.
- Genetic component was relatively weak in case of achievement and social closeness.

WHICH THEORY OR APPROACH IS THE BEST?

Lecture 33

Recap

- Trait theories of personality
- Learning theories of personality
- Humanistic approach to personality
- Biological approach to personality

Assessment of Personality

- Interview
- Observation and behavioral assessment
- Psychological tests
 - Self-report measures
 - Projective tests

Interview

- Direct face to face encounter and interaction.
- Verbal as well as non-verbal information is available to the psychologist.
- Interviews are usually used to supplement information gathered through other sources.
- Skill of the interviewer is very important since the worth and utility of the interview depends on how well he can draw relevant information from the interviewee.

Behavioral Assessment

- Direct observation measure for studying and describing personality characteristics.

Psychological Tests

- In order to objectively assess personality and behavior standard measures are devised. These measures are called psychological tests.
- Psychological tests have to be valid and reliable. Besides they need to be based on norms.

Self- report measures

- Measures wherein the subjects are asked questions about a sample of their behavior.
- **MMPI (Minnesota Multiphasic Personality Inventory):**
- The most frequently used personality test. It was initially developed to identify people having specific sorts of psychological difficulties. But it can predict a variety of other behaviors too.
- It can identify problems and tendencies like Depression, Hysteria, Paranoia, and Schizophrenia for example.
- At the same time it has been used to predict if college students will marry within 10 years, and whether they will get an advanced degree.

Projective Tests/ Techniques

- Tests in which the subject is first shown an ambiguous stimulus and then he has to describe it or tell a story about it.
- The most famous and frequently used projective tests are:
 - i. Rorschach test, and
 - ii. TAT or Thematic Apperception Test

Rorschach test

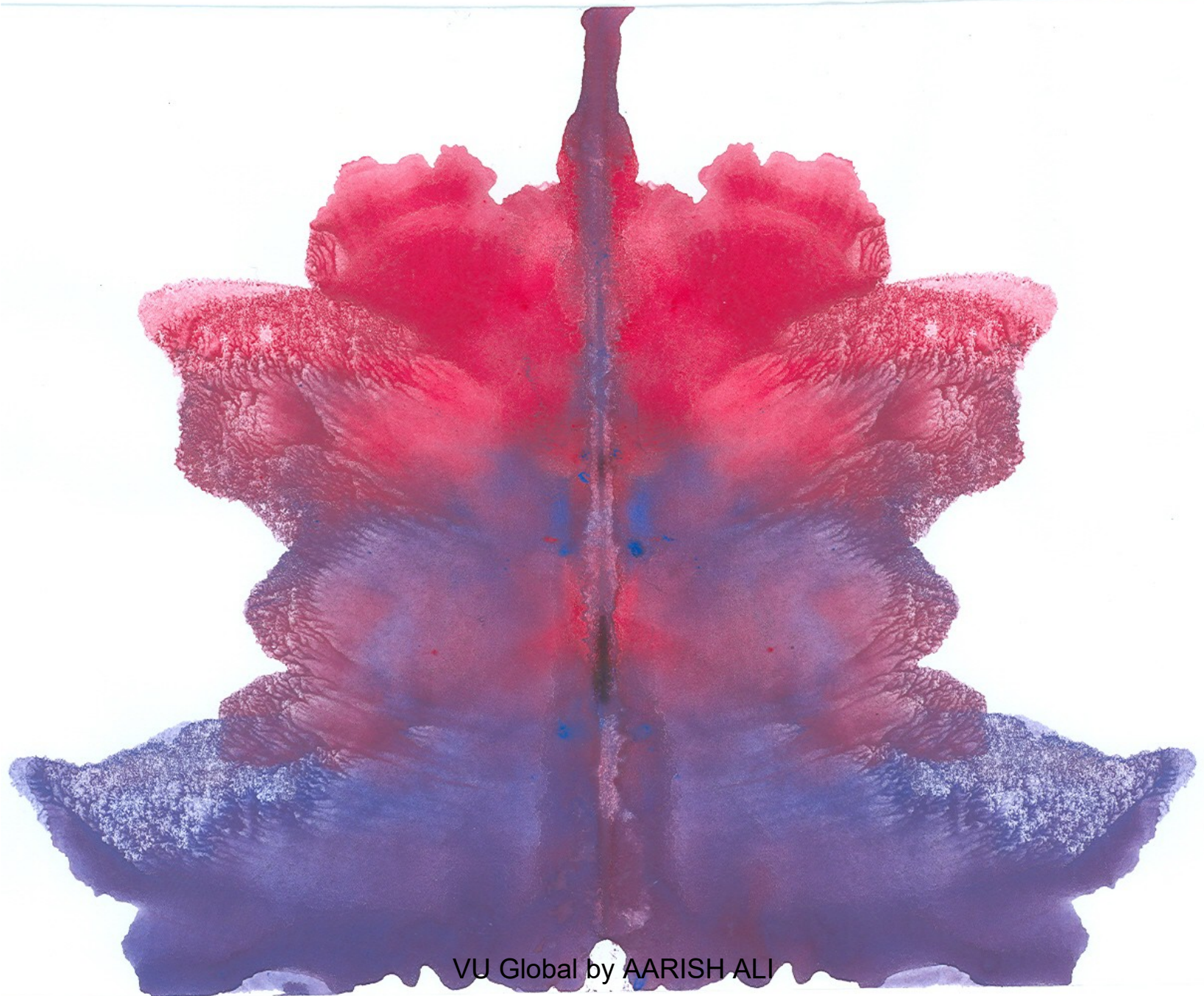
- The test consists of Inkblot presses. These have no definite shape.
- The shapes are symmetrical, and are presented to the subject on separate cards.
- Some cards are black and white and some colored.
-

Rorschach shapes look something like this:



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Procedure of Rorschach administration:

The subject is shown the stimulus card and then asked as to what the figures represent to them?

The responses are recorded.

Using a complex set of clinical judgments, the subjects are classified into different personality types.

THE SKILL AND THE CLINICAL JUDGMENT OF THE PSYCHOLOGIST OR THE EXAMINER ARE VERY IMPORTANT.

Thematic Apperception Test/ TAT:

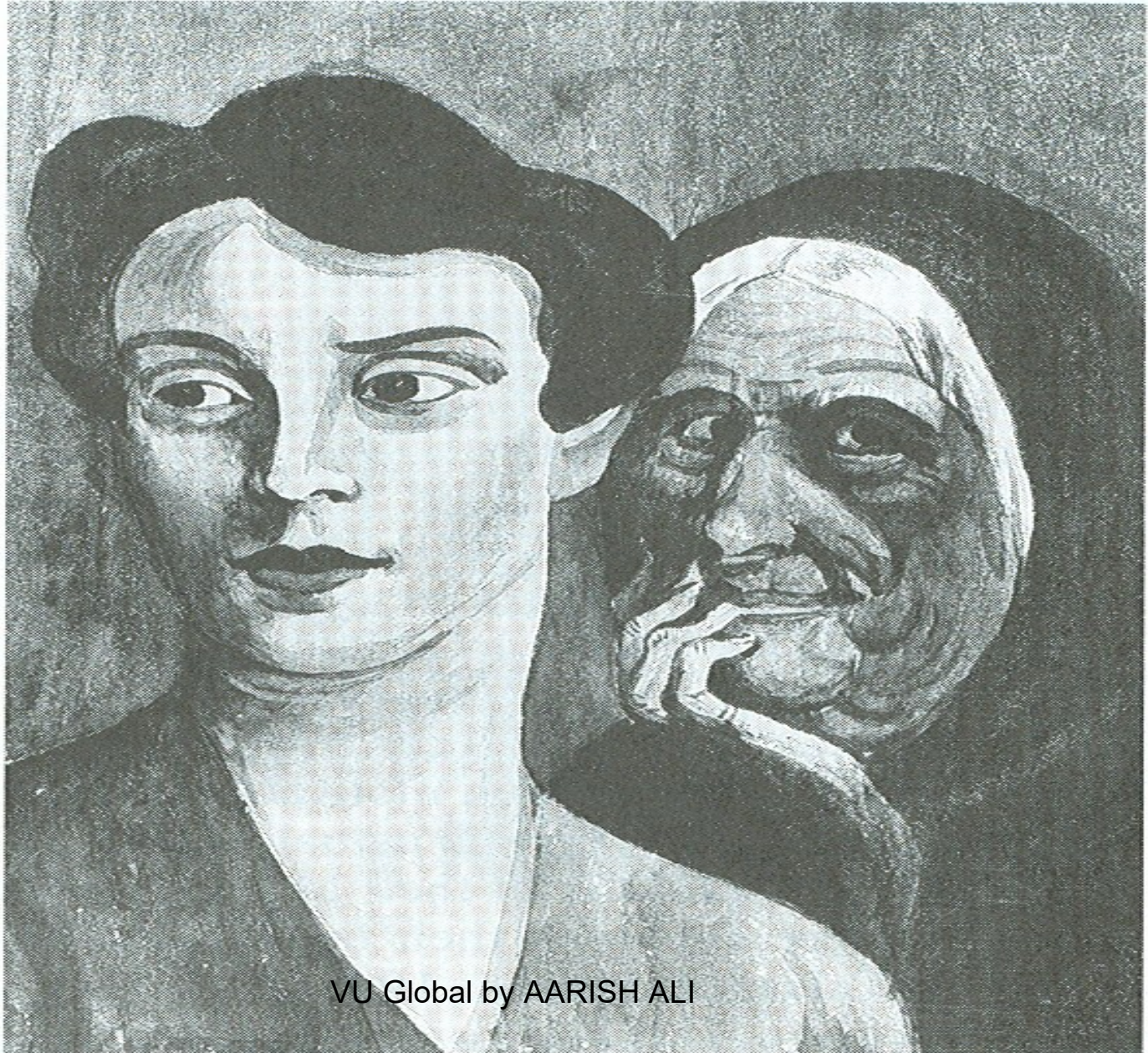
- A series of ambiguous pictures is shown to the subject, who has to write a story This story is considered as a reflection of the subject's personality.

- The subject is asked to describe whatever is happening in it just like forming a story.

- The subject has to tell what is happening in the scene, what the antecedent conditions were, who the characters are, what are their thoughts and wishes, and what is going to happen next.

- In short the subject describes the past, present and future along with the description of characters and their thinking and motivation

TAT pictures look something like this:



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Intelligence

Have you ever consciously considered these questions?

- Am I an intelligent person?
- How intelligent am I?
- How do we judge if someone is intelligent or not?
- How can we measure the intelligence of a person?

- There can be other questions too:
 - **What is intelligence?**
 - **Is intelligence how one deals with others? or**
 - **Is intelligence how precisely we learn a new task? or**
 - **Is it how good we are in our studies? or**
 - **Is intelligence how well we can solve problems? or**
 - **Is it how we accurately judge people? or**
 - **Is intelligence all of this, or even more than all this?**

Different people may understand intelligence differently.

If you think intelligence is all of this or even more than all this, then you are right.

Intelligence

- According to Feldman “intelligence is the capacity to understand the world, think rationally, and use resources effectively when faced with challenges”

- Intelligence is an all encompassing concept.
- It is not restricted to just one aspect of one's life, or to just one faculty or ability.

Part of intelligence is inherited and part of it is learned.

Intelligence refers to the ability to adapt, to reason, to solve problems, and think in an abstract manner; it also includes learning and experiencing new things and understanding from the past experiences.

- Intelligence or the intellectual ability of a person is based upon a constant and ongoing interaction between environmental factors and inherited potentials in order to have better understanding of how to 'use' and 'apply' the potentials in a meaningful manner.
- Modern psychology considers both environment and heredity and their interaction to be influential.

Development of Intelligence

- Psychologists have attempted to uncover the factors operative in the development of intelligence; through studies, it is now evident that intellectual development does not take place in a smooth and straight fashion; it comes in intermittent bursts, and the pattern is different from person to person.

The physiology of intelligence

- In early times, Greeks believed that the soul dwelt in the brain of the person and intellect is somewhere in the lungs.
- This belief prevailed even till the 18th century.
- Recent research has revealed that the flow of blood in the lateral prefrontal cortex of the brain is most when an individual indulges in solving some kind of puzzles.

- Another finding revealed that, in the elderly people, the flow of blood is much less in the areas that are concerned with alertness and memory as compared to the other areas.
- Levy's research revealed, after studying the two hemispheres of the brain, that the left hemisphere is most active and involved in analytical functioning and language.
- Whereas, the right hemisphere is more involved while doing visual and spatial skills and it tends to work more holistically.

Theories of Intelligence

- Theories or viewpoints on the understanding, explanation, and measurement of intelligence.
- These include psychometric approaches that are used to measure intelligence qualitatively and quantitatively.

I. Francis Galton

- Cousin of Charles Darwin.
- He was born in the family of geniuses and he himself was a genius having an IQ of more than 200.
- He was a geographer, meteorologist, tropical explorer, “founder of differential psychology”, inventor of fingerprint identification, pioneer of statistical correlation and regression, convinced of hereditarianism, eugenicist, proto-genetics and a best selling author.
- He gave the concept of “hereditary genius”.

- According to Francis Galton ("Hereditary Genius, 1869) — “gifted individuals” tended to come from families which had other gifted individuals. He went on to analyze biographical dictionaries and encyclopedias, and became convinced that talent in science, the professions, and the arts, ran in families.
- His was the first systematic attempt to measure intelligence by investigating the role of heredity and its impact on intellectual abilities.

- He further attempted to measure human trait quantitatively in order to determine the distribution of heredity in it .
- For this he used “ word association test”, and “ mental imagery”.

- Galton argued that it would be "quite practicable to produce a highly gifted race of men by judicious marriages during several consecutive generations".
- Eugenics: "The study of the agencies under social control that may improve or repair the racial qualities of future generations, either physically or mentally."

For Galton:

- "What Nature does blindly, slowly, and ruthlessly, man may do providently, quickly, and kindly"
- "intelligence must be bred, not trained".
- Such arguments appealed many and some people took this approach to extremes; this way of thinking had drastic social consequences and was used to support apartheid policies, sterilization programs, and other acts of withholding basic human rights from minority groups.

James McKeen Cattell

- American psychologist who gave more importance to the mental processes.
- First ever to use the term “ mental test” for devices used to measure intelligence.
- Developed tasks that were aimed to measure reaction time, word association test, keenness of vision and weight discrimination.
- These tests were proved to be a failure as they were not comprehensive and complex enough to measure intelligence.

Psychometric Approaches:

Spearman's Theory of Intelligence

- British psychologist, Charles Spearman gave his theory in the early 1900s
- His theory laid the foundations for the later theories.
- He observed that people who scored high on one mental test also tend to score on the other as well. The same applies to the low scorers.

- He developed a statistical technique known as “factor analysis” on the basis of which he proposed two factors that can account for the individual differences
- The first one he named as “g” factor or “general intelligence” and the other as “s” factor or “specific intelligence”

- According to Spearman, ‘g’ factor can account for the general ability that is common in all people; as observed from the mental tests.
- Whereas ‘s’ factor can account for the specific abilities that are different in different people; and also different tests required particular abilities from people
- Spearman and his followers gave more importance to the ‘g’ factors and suggested that ‘g’ measured the ‘mental power’ or ‘mental energy’

Thorndike's Theory: Social Intelligence

- According to Thorndike:
“Intelligence and abilities are in the series”.
- He criticized Spearman on the fact that correlation which he studied was not enough to define and explain intelligence.
- He disagreed with Spearman on the arguing that instead of only one ‘g’ factor, there are a number of factors that make and influence intelligence; factors that cannot generally be found out but that are expressed in human actions.

- He divide intelligence into three main divisions
 - i. Social intelligence; one's ability to understand and manage relationships
 - ii. Abstract intelligence; one's ability to understand and manage ideas.e.g. mathematics, algebra or abstract concepts
 - iii. Concrete intelligence; one's ability to manage concrete and mechanical concepts and ideas.e.g. economics, architecture, banking etc

Thurstone's Theory of Intelligence: Primary Mental Abilities

- An American psychologist Louis L. Thurstone, believed that intelligence is not a general factor, but it is composed of small independent factors or elements which he named as “primary mental abilities”.

- For the identification and verification of these abilities, Thurstone and his wife, prepared a set of 56 tests, which they administered to 240 college students and analyzed the results through factor analysis.
- From the analysis, he identified seven primary mental abilities:
 - i. Verbal comprehension: An ability to understand and define words
 - ii. Word fluency: An ability or speed of thinking of verbal material such as rhyming, or naming words in a given category

- iii. Spatial visualization: Ability to recognize and manipulate objects or things in three dimensions such as drafting and blue print reading
- iv. Perceptual speed: An ability to quickly perceive and detect the visual details and differentiate between the similarities and differences between designs
- v. Reasoning/ inductive reasoning: A logical ability of deriving general ideas from specific information

- vi. Numbers/ Arithmetic ability: Capability of doing work easily on numbers such as doing simple arithmetic tasks fast and rapidly
- vii. Memory: An ability or capacity of remembering and retaining the material such as words, letters and ability to recall and associate different words

R.B Cattell and J.L Horn's Theory; Crystalline and Fluid Intelligence

- Given by American psychologists Raymond Cattell and John Horn in 1960s in which they applied different forms of factor analysis and came up with two kinds of intelligence:
 - i. Crystalline intelligence (g_c)
 - ii. Fluid intelligence (g_f)
- Although these forms of intelligence are conceptually different , these are correlated forms of intelligence

- i. **Crystalline intelligence** is the capability of using information that has been learnt: this type of intelligence is largely influenced by education and culture
 - It keeps on increasing with the learning experiences of a person; such as vocabulary knowledge of a teacher
- ii. **Fluid intelligence** is largely influenced by biological factors; it is the capability of solving novel problems which depends more on the neurological development of a person such as reasoning and memory, which decline with age

Guilford's theory of the Structure of Intellect (SOI)

- It is a model of intelligence according to which intelligence is the result of the interaction of operations, contents and products
- He believed that intelligence is a much more complex phenomena than one thinks of it; it is difficult to define it as a 'g' factor or in terms of 'primary mental abilities'

- Developed a cube- shape model of intelligence which is made up of 120 separate factors, known as “ the structure of intellect”.Guilford recently expanded his model and it now included 180 factors
- **Operations:** it is the potential of different ways of thinking including:

Evaluation

Convergent thinking

Divergent thinking

Memory retention

Memory recording

Cognition

- **Contents:** A potential of what we think about something. It includes;
 - i. Visual
 - ii. Auditory
 - iii. Symbolic
 - iv. Semantic
 - v. Behavioral

- **Products:** The results obtained by applying certain operations to certain contents, or the ability of thinking in a certain manner about a certain thing
- It includes:
 - i. Units
 - ii. Classes

iii. Relations

iv. Systems

v. Transformation

vi. Implication

Lecture 34

Recap

- Assessment of personality
- Intelligence
- Definition of intelligence
- Theories of intelligence
- Francis Galton theory of intelligence
- James Mckeen Cattell theory of intelligence
- Spearman's two factor theory of intelligence
- Thorndike's theory of social intelligence
- Thurstone's theory of primary mental abilities
- R. Cattell and JL. Horn's theory of crystalline and fluid intelligence
- Guilford'd theory; Structure of Intellect (SOI)

Gardner's Theory of Multiple Intelligences

- Given by Howard Gardner in 1985
- He entirely disagreed with the theorists who gave importance to factor analysis, except one thing that is common and found in his and the former theories is that intelligence is the composition of many specific abilities
- The theory maintains that intelligence consists of eight independent intelligences that vary in degree among individuals; it includes the potential of solving the problems or creating products that can be valued within one or more than one cultural settings

- iv. Musical intelligence
- v. Bodily kinesthetic
- vi. Interpersonal intelligence

- Maintains that humans possess around eight kinds of intelligences, they are
 - i. Linguistics
 - ii. Logical mathematical
 - iii. Spatial intelligence

vii.Intrapersonal intelligence

viii.Naturalistic intelligence

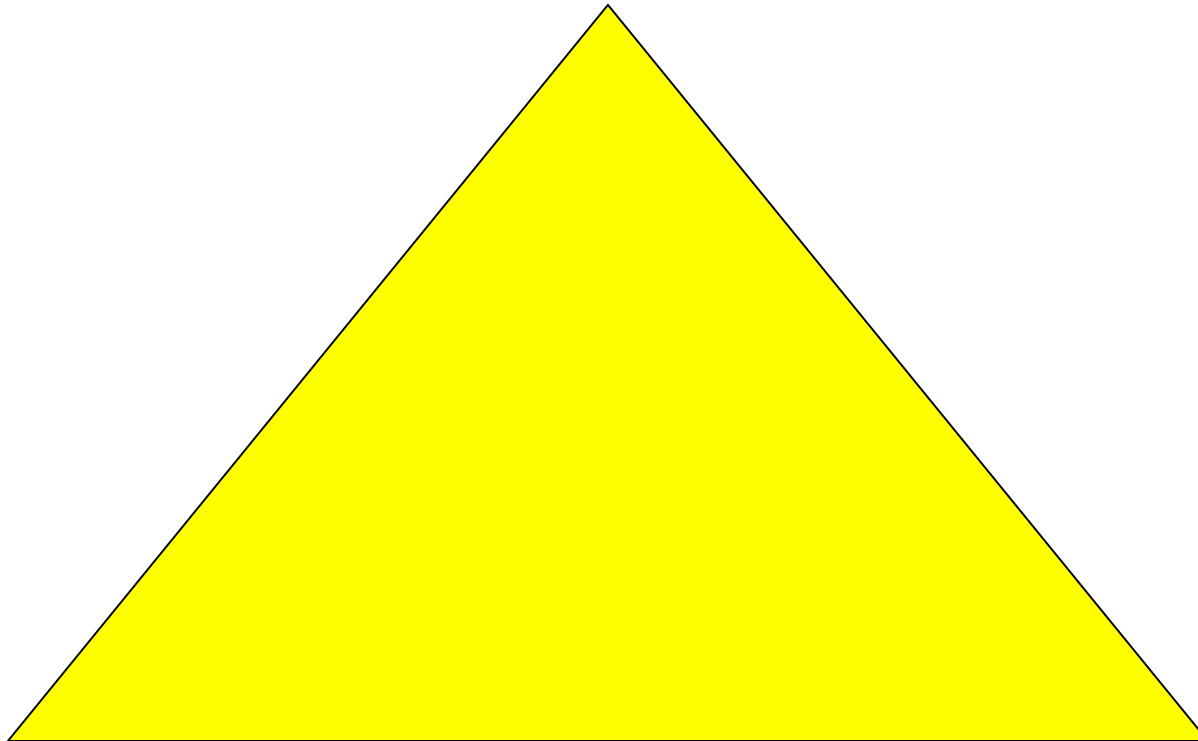
Sternberg's Triarchic Theory of Development

- Given by American psychologist Robert Sternberg in 1980s which is similar to Gardner's theory of intelligence
- He observed that mental test sometimes do not accurately predict success or failure in reality
- According to his triarchic or three- dimensional theory, intelligence consists of three main components:

- i. Analytic intelligence
- ii. Creative intelligence
- iii. Practical intelligence

Triarchic theory of intelligence

Componential aspect of intelligent



Contextual aspect of intelligent

Experiential aspect of intelligent

Piaget's View of Intelligence

- Intellectual development can be defined in terms of qualitative changes in thinking which are clearly apparent in children of particular age
- His theory is more concerned with the universal patterns of intellectual development and functioning. He maintained a comprehensive theory that emphasized on 'how' children acquire knowledge and use it to solve logical problems

- He is more interested in how children exhibit intelligence in different stages of life as he proposed the four stages of cognitive development which he termed as universal and in variant (occurring in the same sequence)

Hierarchical Theory of Intelligence

- Given by Carroll 1986 and Vernon 1971
- Hierarchy consists of three levels
- At the highest level, there is general intelligence or “g” factor as given by Spearman
- In the second level, there are factors of primary mental abilities as identified by Thurstone
- At the third level, there are specific abilities or factors underlying intelligence

Measurement of Intelligence

- Intelligence of a person can be measured through intelligence tests. the use of a battery of tests is preferred over a single test

The History of Measurement of Intelligence

- ❑ Man was always interested in knowing their own ability level, or in other words the intelligence , as well as that of others.
- ❑ People looked for capable persons while searching for life-mates, companions, workers, architects, artists, and poets or authors.
- ❑ Even in case of chefs, tailors, carpenters, or barbers, those with the fine skill and the ability to generate unique ideas were preferred.
- ❑ In the olden times, observation and previous experience were the only ways to judge a person's capability or intelligence level.

The prevalent, modern, approaches to measure intelligence are based upon the contribution of Alfred Binet:

- The first formal measure of intelligence was developed by French psychologist Alfred Binet and Theodore Simon, in 1905 in France.
- The test or the scale was developed in order to assist the education ministry and department in identifying “dull” students in the Paris school system, so that they could be provided remedial aid.

The main idea was that intelligence can be measured in terms of performance of a child:

- If performance on certain tasks, that were the test items, improved with age then it could be taken as an indication of intelligence of a person.
- By developing tasks on which people's performance improved with age, a measure could be devised which could distinguish intelligent people from those not intelligent.

- Using the same concept Binet developed the first intelligence test in 1905. The test could identify more intelligent children within a particular age group. It could differentiate intelligent children from the less intelligent ones.
- The test was devised for locating the 'dullest' students in the Paris school system so that remedial assistance could be provided to them before they were denied instruction.

The Testing Procedure Adopted by Binet and Simon

- Initially Binet developed a number of tasks.
- Then he took groups of students who were categorized or labeled as 'dull' or 'bright' by their teachers.
- The tasks were presented to them. The tasks that could be completed by the 'bright' students were retained; the rest were discarded.
- The idea was to retain tasks that could be completed by the bright students, as these were considered to be indicative of the child's intelligence.
- With further work, dull or bright children could be identified with reference to their age.
- The scale could, thus, identify bright or dull students within particular age groups.

Sample Items From Simon-binet Test(1911)

Three years:

Shows nose, eyes and mouth. Repeats two digits.
Describes objects in a picture. Gives family name.
Repeats a sentence of six syllables.

Four years:

Gives own sex. Names key, knife, and penny.
Repeats three digits. Compares the length of two lines.

Five years :

Compares two weights. Copies a square. Repeats a sentence of ten syllables. Counts four pennies.

Six years:

Distinguishes between morning and afternoon.
Defines objects in terms of their use. Copies a shape.
Counts 13 pennies. Compares faces from the aesthetic point of view.

Seven years:

Identifies right hand and left ear. Describes a picture.
Follows precise directions. Names four colors.

Eight years: Compares two remembered objects.
Counts from 20 to 0. Indicates omissions in pictures.
Gives day and date. Repeats five digits.

The Highest Level

Fifteen years:

Repeats seven digits. Gives three rhymes. Repeats a sentence of 26 syllables. Interprets a picture. Solves a problem from several facts.

Later Revisions

- The original Binet- Simon scale was revised a number of times.
- The American psychologist, Lewis Terman gave the first Stanford revision of the scale in 1916.
- This revision comparison American standards from age 3 to adulthood.
- Further revisions were made in 1937 and 1960.
- Stanford- Binet is one of the most widely used tests even today.

The Concept of Mental Age

- Children taking the Binet- Simon test were assigned a score that corresponded to the age group they belonged to.
- This score indicated their “mental age”.
- Mental age referred to the average age of children who secured the same score.
- Mental age can be understood as the typical intelligence level found for people at a given chronological age.

- Mental age of a person can be different from his or her chronological age i.e., it can be above or below that.
- It could reflect whether or not a child was performing at a level at which his age mates were.
- BUT it gave rise to a problem. How could we compare people belonging to different age groups?
- Will a 20 year old with a mental age of 23 be equally intelligent as an 8 year old having a mental age of 11?

The problem becomes more serious when we are talking of comparing those below their mental age e.g. comparing an 8 year old operating at a mental age of 5, with a 20 year old operating at a mental age of 17

The Concept of Intelligence Quotient or IQ

- As a result of problems with depending merely on mental age, a solution was devised in terms of intelligent quotient, a concept whereby the chronological age of the person is also given due consideration.

Intelligence Quotient: IQ

- An indicator or measure of intelligent that considers a person's mental as well as chronological age.
- The formula for IQ:
$$\text{IQ score} = \text{MA} / \text{CA} \times 100$$

Using This Formula Means That

- If the mental and chronological age of a person is the same, then he or she will have an IQ of 100. If one is below his chronological age then the IQ will fall below 100 and vice versa.

Deviation IQ Scores

- Psychologists kept working on the concept of IQ and made amendments in the primary concept of IQ.
- Today, although the main concept of calculating IQ is still adhered to, we talk in terms of Deviation IQ scores.
- Deviation IQ scores are based upon sophisticated statistical techniques. The average score of a group is kept in mind, and the IQ of the person taking an IQ test is stated with reference to the points with which he deviates from the average.

Available Statistics Show That

- Around two-thirds or 66% of the population fall within a range of 15 IQ points from the average IQ score of 100 i.e.,
the majority of people have an IQ between 85 and 115.

The Meaning of IQ Test Scores

<u>IQ score</u>	Rating
< 70	Retarded
85	Borderline
100	Average
above 115	Superior
above 140	Gifted

IQ Tests

- Today a number of IQ tests are available.
- But before choosing a test for assessing a subject's intelligence, and considering it a trustworthy device, the psychologist has to make sure that the test is :
 - Valid
 - Reliable, and
 - Standardized

Stanford-Binet Test (4th Ed.)

- Last revised 1985
- Contains a series of test items that vary in nature according to the subject's age.
- For example a child may be asked to copy figures or answer questions about everyday life activities.
- Older subjects solve analogies, explain proverbs, or describe similarities underlying sets of words..

Administration of Stanford-Binet Test

- Individual-oral administration.
- The examiner begins from a mental level at which he finds out the subject to be.
- Items from succeeding levels are asked.
- The test ends when they reach a level where no items are successfully attempted.
- IQ is computed by examining the pattern of correct and incorrect responses of the subject.

Wechsler Adult Intelligence Scale- Revised: WAIS-R

and

- Wechsler Intelligence Scale for
children-III:WISC-III

WAIS-R and WISC-III:

- Both were developed by psychologist David Wechsler.
- The two tests consist of two major parts:
 - ✓ Verbal part/scale
 - ✓ performance or non verbal part/scale

Verbal scale

- consists of more conventional types of problems involving vocabulary definition, and comprehension of various concepts.
- The subscales include:
 - ✓ Information
 - ✓ comprehension
 - ✓ Arithmetic
 - ✓ Similarities

Performance Scale

- It involves assembling small objects and arranging pictures in a logical order.
- The subscales include:
 - ✓ Digit symbol
 - ✓ Picture completion
 - ✓ Object assembly

Administration of WAIS and WISC

- Individual administration.
- Time consuming.

Group Intelligence Tests

- Considering the time consuming nature of the administration of Stanford-Binet Test, WAIS-R, and WISC-III, psychologists have developed a number of group intelligence tests.

Cultural Biases and Intelligence Tests

- Tests used to assess people's intelligence have been frequently criticized for being biased against particular groups of people.
- Culture-fair IQ tests are developed and used for overcoming this problem. These tests do not discriminate against any minority or cultural group.

Alternative Formulations

- These include:
 - ✓ Moral intelligence
 - ✓ Social intelligence
 - ✓ Emotional intelligence

Moral Intelligence

- Given by Coles (1997) and Hass (1998)
- It is the ability to differentiate between right and wrong
- More comprehensively, it is the capacity of making right decisions that are not only beneficial for one self but to others as well

Social Intelligence

- Manifested as SQ.
- Ability to understand and deal with people; salesmen, politicians, teachers, clinicians, and religious leaders exhibit this type of intelligence
- It is also the ability to understand and deal with in own self by identifying one's thoughts, feeling, attitudes and behaviors
- Given by Hough, 2001; Riggio, Murphy, & Pirozzolo, 2002).

Emotional intelligence (EI)

- It is the type of social intelligence which is the ability to cope with one's own and other's emotions, to differentiate between them and use information for guiding one's thoughts and actions.
- Indicated by the EQ of a person.

- It includes these aspects:
- Self-awareness
- Managing emotions
- Empathy
- Handling relationships

Lecture 35

Recap

- Theories of intelligence
- Measurement of intelligence
- History of the measurement of intelligence
- The concept of intelligence quotient or IQ
- IQ tests
- Alternative formulations

Psychopathology

- Psychological illness, psychological disorders, or mental illness.
- The term is used to describe abnormal behavior.

Psychopathology

- Psychopathology is the area of study in psychology that primarily focuses upon the origin, development and manifestation of behavioral and mental disorders.
- Abnormal psychology is that branch of psychology that studies, describes, explains, and identifies abnormal behavior.

- The observable behavior and mental experiences of an individual may be indicative of a mental or psychological disorder. The overt behavior and other experiences provide cues to the development of mental or psychological disorders..
- Psychiatrists and clinical psychologists treat mental disorders.
- Besides, they are also interested in studying and conducting research on the nature and role of the events that cause these disorders e.g. past history of a person and other variables that contribute to mental illness

Historical Perspective of Psychopathology

- Human life existed, presumably, on earth even more than three million years ago, but the records of only few thousand years are available to us.
- One of the earliest traces in history, about the treatment of mental disorders date back to the stone age that was more than half a million years ago.

- Headaches, convulsive attacks, and other brain related problems were treated with stone instruments in which a hole was made in the skull of about 2 cm. This process was called “trephining”.
- Evidence is available that trephining was performed even 4000 to 5000 years ago.
- It is believed that the stone age people used to perform this procedure.

- The ancient explanations centered around the supernatural: gods, evil spirits, demons etc. It was thought that the supernatural forces caused strange and troubled behaviors, as they resided within the body of the person.

In the ancient societies the influence of evil spirits was the main focus:

- Herodotus, the ancient Greek historian, wrote about a king who was driven insane by evil spirits.
- According to the legend of Hercules, he killed his own children because evil spirits drove him mad.

- Trephining was done in order to allow the evil spirits to escape the body of a person.
- This view prevailed throughout the rich civilizations of that time such as the Egyptian, Hebrew, Greek, and Chinese civilizations.

- One of the most primitive ways of treating abnormality, and freeing the person of the “evil” possession was the use of “exorcism”.
- Various techniques were used for casting the evil spirits out of the body of the afflicted person; these included black magic, noisemaking, prayer, and the use of potions.
- In extreme case flogging and burning the patient were also used.

- This treatment were done by the “ Shaman or the medicine man”, who was regarded a very special person possessing unusual healing powers.
- In Egypt and Greece, the tasks of treating the ill were eventually taken over by the priests .
- These priests were a combination of a priest, physician, psychologist and a magician.
- The cure however still depended on magic.

- But not all ancient thinkers believed in this approach.
- In the Golden Age of Greece, temples for healing the ill were maintained.
- This was like a revolution that took place at a time when knowledge about human anatomy and physiology was very limited, and insufficient.
- During that time, Hippocrates, “ the father of modern medicine”, made his contributions in various fields.

Hippocrates

- Believed in the rational knowledge for understanding and treating the psychological disorders and denied the interventions that were used at that time for treating the abnormal.
- He that all illnesses, including mental disorders, had a natural cause and required natural treatments for curing the patient.
- He maintained that brain is the central organ for all activities including the intellectual abilities and that disorders are the result of brain pathology.

- He also believed that abnormality was caused by the imbalances of some humors or liquids within the body of a person.
- He gave theory of “ humors” that account for the basic human activity. He was of the view that the perfect health is the result of the proportionate mixture of these humors

- Maintained that four humors led to four temperaments forming one's personality. the temperaments were:
 - a) Sanguine(Cheerful and Active)
 - b) Melancholic(Sad)
 - c) Choleric(Angry and Aggressive)
 - d) Phlegmatic(Calm and Passive)
- Hippocrates maintained that epilepsy is caused by insufficient air carried by the veins to the brain and limbs.

Socrates

- For him, soul is most important it should be properly looked after.
- It is not just one faculty or any particular material entity; rather it is a broad area comprising intelligence and character, or man's conscious personality.
- He maintained that thought and reasoning are the building blocks of personal worth and happy life.
- It can be concluded that problems with the soul led to abnormal behavior.

Plato

- Believed in the humane treatment of mentally ill patients
- He was of the view that disorders developed when the conflict arises between emotions and reason
- But despite having modern thoughts, he still believed that mental disorders were partly treated by the divine powers too.

Aristotle

- He mainly followed Hippocrates' theory of mental disorders.
- Gave different ideas about many emotional states such as anger, fear, envy, hatred, courage etc.

Asclepiades

- First ever to distinguish between the acute and chronic mental disorders
- Also differentiated between delusions, hallucination and illusions.
- The movement of atoms in the body caused disease.
- He advocated the practice of baths, dieting, exercise, and massage for curing disease.

Galen

- Most outstanding physician of antiquity after Hippocrates.
- Influential contributions were made by him regarding the anatomy and nervous system of human beings as dissections were being performed by him on animals.

The following many centuries
did not see a major
breakthrough regarding the
understanding and treatment of
abnormality.

In the Middle Ages

- In the middle ages of Europe, religious thought, dominated the understanding and treatment of mental disorders.
- This period lasted from the fall of the Roman Empire in 5th century AD till the 15th century.
- It was thought that the mentally ill were possessed by demons or the devil.
- They were accused of being witches who could infect others with madness.

As a result

- The mentally ill were treated not humanely but with religious inquisition and barbaric treatment.
- However many thinkers even at that time believed that mental disorders did have a physical cause.
- They believed that the imbalance in the four basic humors, grief, and poor diet caused such illnesses.

In the Islamic World

- The mentally ill were treated humanely even at that time.
- Asylums were made for the mentally ill.
special baths, diet, medication, music, and pleasant environment were provided to the patients.

Such asylums began in 8th century AD.

the first one was established in Baghdad, and was followed by others in Cairo, Damascus and Fez.

In Europe

- The mentally ill underwent inhuman treatment for long.
- The situation prevailed even in the 17th and 18th century AD.
- They were either left to wander in wilderness, or kept isolated in institutions.
- Many hospitals in Britain, France and Italy became notorious for their callous way of treating the mentally ill.

However with growing public awareness the need for a change was felt.

- La Bicetre hospital in Paris , France, was a pioneer in this regard.
- Pussin was the superintendent of a ward for incurable mental patients.
- He released patients from shackles, and the staff was forbidden from beating them.
- This practice was continued by Philippe Pinel who became the chief physician of La Bicetre hospital.

Developments in 17th and 18th Centuries

- The period of 17th century is regarded as the “age of reason”, and that of 18th century as the “age of enlightenment”.
- Great changes as well as discoveries were made at that time in a number of fields including psychology

Franz Friedrich Anton Mesmer

(1734- 1815)

- Mesmer proposed that a power similar to magnetism existed in humans.
- This, animal magnetism, exercised a powerful influence on our body.
- He asserted that animal magnetism had a medicinal value.
- He believed that mental illnesses can be cured by maintaining and balancing the magnetic field.
- He was of the view that hand movements and their magnetic force were enough to produce dramatic changes in the ill persons.

- He introduced mesmerism, a trance like state, as a curative technique.
- Mesmerism was similar to today's hypnosis.
- He made more use of magnetism than suggestion.

The major breakthrough took
place in the later half of the
18th century

Philippe Pinel: (1745-1826)

- His contributions began with the revolutionary changes at the La Bicetre hospital in Paris .
- Believed that abnormal behavior is caused by some hereditary defects or nervous system defects.
- He was of the view that mental patients should be treated with great care as he severely condemned chaining and shackling of the mentally ill.
- Due to his efforts, La Bicetre and Salpetriere hospitals were regarded as the first modern hospitals for caring and curing the insane.

Pinel Gave the Concept of *Moral Treatment*

- He said that the mental patients should be treated with kindness and sensitivity.
- Cruelty and violence were discouraged and forbidden.

William Tuke

- Established the York Retreat in rural England in 1796.
- Here, mental patients were provided compassionate treatment.
- This retreat became a model for others to follow.

Wilhelm Griesinger:(1817-1868)

- Believed that the best way to understand mental disorders is to assume that they are caused by brain pathology.
- Firmly believed on the organic origin of mental disorders rather than the psychological ones.

Kraepelin: (1856-1926)

- Published the first system of classification of mental diseases.
- This system could help to label different disorders as psychosis and neurosis.
- Kraepelin clearly differentiated between 'dementia praecox' (now known as schizophrenia) and 'depressive psychosis'; severe mental disorders.
- These severe disorders were thought to be developed out of the organic reasons.

Jean M. Charcot: (1825- 1893)

- French neurologist.
- Charcot was most interested in observing and treating hysterical patients.
- He noticed that these patients had strange beliefs about their own bodily functions.
- He developed techniques for treating hysterical patients as well as also being able to induce hysterical symptoms in normal individuals

DEFINING ABNORMALITY

What is abnormal behavior?

- Abnormality can be defined in a number of ways.
- People identify, understand and explain abnormality according to their past experience, common information, cultural tradition, societal attitude, and/or professional knowledge.

Definitions of Abnormal Behavior

- 1. **Statistical definition:**
- People deviating from the norm are considered “abnormal”.

- BUT what if majority of people indulge into erratic behavior?
- What about the creative artists who did not go along the norm?

2. Abnormality As Deviation From the “Ideal”

- Ideal refers to the standard toward which most people strive.
- Abnormality is not striving toward the ideal.
- BUT what about those for whom the ‘ideal’ is not the ‘ideal’?
- For example a student who is a very good painter and does not want to pursue conventional education.

Lecture 36

Recap

- Psychopathology
- Historical perspective of psychopathology
- Definition of abnormality

Definitions of Abnormal Behavior

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- People deviating from the norm are considered “abnormal”.

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- Abnormality is not striving toward the ideal.
- BUT what about those for whom the ‘ideal’ is not the ‘ideal’?
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3. A Sense of Personal Discomfort Seen As Abnormality

- A person is seen as abnormal if his thoughts and behavior are a source of discomfort for him.
- Discomfort can be in the form of anxiety, distress, or guilt.

5. Inability to function effectively

- **People who can not function and perform as effectively as they ought to, are seen as abnormal.**
- **This definition includes adjusting , and adapting to the social requirements .**

6. The Legal Definition of Abnormality

- **Laws in different countries define abnormality according to their legal standards.**
- **It is primarily needed for differentiating sanity from insanity.**
- **Abnormality may be viewed as not being able to foresee and understand the consequences of the criminal act.**
- **Or it can be taken as inability to control one's own thoughts and behaviors.**
- **Or it can be the ability to see right as different from wrong.**

Perspectives on Abnormality

- Approaches to studying, describing, understanding, explaining, and predicting abnormality.
- These approaches affect the way a mental patient will be treated.

1. Medical Perspective

- **Psychological problems are caused by physiological factors.**
- **These can be the biological processes and systems, genetic factors, the nervous system and the neurotransmitters, hormonal changes, or external variables affecting the biology of a person.**

2. Psychodynamic Perspective

- **Childhood experiences are the root cause of mental disorders.**
- **Unconscious determinants are significant.**

3. Behavioral Perspective

- **Abnormal behavior is learned.**
- **Abnormality is a learned response.**
- **It results from our interaction with the external world.**

4. Cognitive Perspective

- **The factors causing mental disorders are a person's cognitions, thoughts, and beliefs.**

5. Humanistic Perspective

- **People's need to self-actualize, and their responsibility for their own actions, play a central role in abnormality behavior.**

6. Sociocultural Perspective

- **The social milieu in which one lives, the family and the people around, the society, and the culture at large are of primary importance in the onset, and later treatment, of mental illness**

Classification of Mental Disorders

- **The first classification system of mental disorders was given by Kraepelin.**
- **A number of classification systems followed afterwards.**
- **The purpose was to assist the clinicians diagnose mental disorders, as well as to determine the extent of the problem.**

Classification Systems

- **DSM- IV- TR**
- **ICD**

DSM-IV-TR

- **Diagnostic and statistical manual of mental disorders is the classification system compiled by the American Psychiatric Association.**
- **This is the most widely used classification system all over the world.**

ICD: International Classification

- **For decades, mental health professionals in Western Europe and a major part of the world used this classification system.**
- **ICD was developed by the World Health Organization.**
- **ICD is a comprehensive classification system of all kinds of diseases, including psychological or psychiatric illnesses.**

- **For a number of years ICD9 remained a popular diagnostic system.**
- **Research, in the last more than a decade, reflected that the revised and improved versions of DSM had an edge over ICD in many respects.**
- **Besides, there were no major differences as such in the two systems.**
- **Also, the need for a single universally accepted system was intensely felt.**
- **Therefore today DSM-IV-TR is recognized as a universally accepted diagnostic system.**

DSM-IV-TR

- **The first DSM was published in 1917.**
- **It originated from a project of the American Medico-Psychological Association, now known as American psychiatric Association and United States Bureau of the Census.**
- **In order to collect uniform data on hospitalized mental patients, they developed a list of 59 mental illnesses.**

- **The list was further expanded with the publication of the first DSM in 1952.**
- **The first DSM included a list of 106 mental illnesses.**
- **DSM-II was published in 1968.**
- **DSM-III was published in 1980.**
- **DSM-III-R was published in 1987.**
- **DSM-IV was published in 1994.**
- **DSM-IV was developed after a special 27-member task force of experts worked for five years.**
- **More than 1000 psychiatrists contributed and advised in deciding about the diseases and other information to be included in DSM-IV.**
- **DSM-IV-TR was published in 2000.**

- **DSM-IV-TR contains definitions of more than 200 mental disorders.**
- **These disorders are organized into 17 major categories.**

Multi Axial System of DSM-iv-RT

- **DSM-IV-R also contains five axes, or five types of information, that have to be considered in the diagnosis of a patient.**

Axes of DSM-IV-TR

- **Axis I:** Clinical disorders
- **Axis II:** Long standing problems that are frequently overlooked in the presence of disorders listed in axis I; mental retardation, and personality disorder.
- **Axis III:** General medical conditions that may be relevant to a psychological disorder.

- **Axis IV: Psychosocial or environmental problems that a person is facing.**
- **these problems may affect the diagnosis, treatment, or the course of the mental disorder.**
- **Axis V: Global assessment of functioning.**

Major Categories of Disorders in DSM-IV-R

- **Anxiety disorders**
- **Somatoform disorders**
- **Dissociative disorders**
- **Mood disorders**
- **Schizophrenia**
- **Personality disorders**
- **Sexual disorders**
- **Substance-related disorders**
- **Delirium, dementia, amnesia, and other cognitive disorders**

Anxiety Disorders

- **Disorders in which anxiety becomes an impediment in a person's routine functioning.**
- **Anxiety is a reaction to real or imagined threat that may hamper the daily functioning and results in uneasiness, worry, and apprehension.**
- **In anxiety disorders, anxiety occurs without an obvious external cause, to an extent that it affects routine functioning of the person.**

- **Stress is the part of daily routine in a person's life but the reactions to stress vary from individual to individual.**
- **Anxiety is one of the various reactions to stress.**
- **Whether or not one will develop anxiety, and to what extent, will depend on the nature of stress faced, family history, fatigue or over work, and the person's coping strategies.**

- **Major symptoms of stress include**
 - **Sleeplessness**
 - **Headaches**
 - **Twitching and trembling**
 - **Dry mouth**
 - **Memory problems**
 - **Nightmares**

- o **Irritability**
- o **Fatigue**
- o **Sweating**
- o **Muscle tension**
- o **Insomnia**

Common causes are:

- o Imagined threat**
- o Grief**
- o Physical or emotional stress**
- o Use of drugs**
- o Withdrawal from drugs**

Subcategories of Anxiety Disorders

- Generalized anxiety disorder
- Panic disorder
- Phobic disorder
- Obsessive compulsive disorder
- Post- traumatic stress disorder

Treatment can be done through:

- Finding the actual cause of anxiety
- Avoid becoming dependent on mood altering drugs
- Avoid stimulants such as caffeine, nicotine, alcohol etc
- Biofeedback and relaxation therapy
- Aerobic exercises
- Avoid the effects that have been produced due to anxiety, if anxiety is cured, the other symptoms will be resolved automatically

Generalized Anxiety Disorders

- The disorder marked by long-term, persistent, anxiety and worry.

Generalized Anxiety Disorders

- It refers to the long- term anxiety in which there is continual and exaggerated state within the person due to which he/ she is continually tense, apprehensive and in automatic nervous system arousal
- Chronic form of anxiety disorders

Causes include;

- o Hereditary causes,
- o Or this disorder begins at very early age and the revealing of the symptoms is gradual not burst

Treatment involve;

- o Medications and use of psychotherapy
- o Exposure therapy
- o Behavioral therapy and cognitive behavioral therapy

Symptoms involve;

- o People with this disorder are unable to relax,
- o Insomnia
- o Trembling,
- o Muscle tension,
- o Head aches, sweating,
- o Twitching,
- o Trembling,
- o Feel tiredness,
- o Depression etc

Panic Disorder

- Disorder in which anxiety is manifested in the form of panic attacks lasting from a few seconds to many hours.
- Panic attacks are unpredictable, resulting from vague anxiety and that may accompany physiological manifestations.
- Symptoms include:
 - Dizziness and/or fainting
 - Sweating
 - Trembling
 - Palpitation
 - Nausea

- o Chocking
- o Fear of dying
- o Fear of being out of control
- o Skin blushing or flushing
- o Chest pain and discomfort
- o Sleep disturbances
- o Agitation
- o Facial paralysis etc

Causes involve:

- o Use of drugs and stimulants
- o As a result of some incident or risk factor
- o The exact cause of panic attacks are still not known; may result due to temporal dysfunction of the brain or may have been learnt through past experiences.
- o More frequent in women than men

Prognosis: The disorder is difficult to treat and long- lasting as well, but behavioral therapies and use of drugs can minimize the symptoms

Phobias

- **Phobias are the particular, persistent, irrational and intense paralyzing fear of some objects and situations that they are unable to explain and overcome; and that may occur without any actual cause.**

b. Symptoms include:

Perspiration

- o Frustration,
- o Rapid heart beat
- o Headaches

Lecture 37

Recap

- **Definitions of abnormality**
- **Perspectives on abnormality**
- **Classification of mental disorders**
- **DSM-IV-R**
- **Anxiety disorders**
 - I. **Generalized anxiety disorder**
 - II. **Panic disorder**
 - III. **Phobias**

Causes of Phobias may include;

- o Result of some traumatic event or disaster;
- o Hereditary component
- o Prevalent equally in men and women
- o Anxiety
- o Panic attacks

Some Common Phobias

Phobias	Fear of:
Acrophobia	Height
Aerophobia	Flying
Agoraphobia	Entering public places
Claustrophobia	Closed spaces
Hydrophobia	Water
Mikrophobia	Germs
Nyctophobia	Darkness

Some Common Phobias

Phobias	Fear of:
Ophidiophobia	Snakes
Phonophobia	Speaking out loud
Pyrophobia	Fire
Thanatophobia	Death
Xenophobia	Strangers

Treatment of Phobias includes:

- o Use of behavior therapy especially behavioral- modification therapy
- o Procedure of systematic desensitization is used
- o Biofeedback is also helpful

Obsessive- Compulsive Disorder

- **Obsession** is an unwanted, recurrent and persistent thought that continuously recurs, and that can be intrusive and inappropriate
- A **compulsion** is the uncontrollable urge to perform an apparently strange and unreasonable act repeatedly.

- **Symptoms** include;
 - o Distress,
 - o Frustration,
 - o Anxiety etc

Causes include;

- o Risk factor,
- o Stereotype behaviors,
- o Brain abnormalities,
- o Unpleasant thoughts,
- o As a result of some incident etc

Prognosis: It is a chronic illnesses in which total removal of symptoms is not possible, but improvement through medication and therapy is possible

Somatoform Disorders

- A disorder in which psychological problems take the physical (somatic) form without any apparent physical cause; a state where there are physical symptoms present but no explicable medical cause.

Symptoms include;

- o Blurred vision,
 - o Dizziness,
 - o Vomiting,
 - o Difficulty in swallowing etc
 - o There are **two types** of somatoform disorders
- 1) Hypochondriasis
 - 2) Conversion disorders

1) Hypochondriasis

- Type of somatoform disorder in which the person experiences a persistent fear of illness, and is preoccupied by health concerns.
- Even minor pains and aches may be interpreted as a symptom of some serious disease.

Symptoms involve;

- o In this disorder doctor shopping is very frequent
- o Sympathy may exaggerate these complaints
- o Patient undergoes surgery and regularly takes medication
- o Patient focuses closely on normal physiological states such as rapid heart beat, sweating, palpitations etc; patient interprets it as some severe disease.
- o Minor health problems may become severe as a result of persistent stress and discomfort

2) Conversion Disorders

- Disorder in which the persons undergoes an actual, genuine and specific, physical problem and disturbance.
- The problem has a purely psychological reason and there is no biological cause involved.
- The problem manifests itself suddenly, without any prior indication.
- Unexplainable neurological symptoms appear at once when no testable cause is present.

Symptoms include;

- Partial blindness,**
- Loss of voluntary control over motor and sensory functions**
- Inability to hear and talk**
- Sudden display of emotions; and at times there is no emotion**
- Symptoms may be exaggerated by stress**
- The sufferers frequently do not show a natural concern about the symptoms.**

Causes include;

- Hereditary component, and observational learning
- A state of severe stress
- People who have other organic problems may develop conversion disorder
- A prior knowledge of the disease and symptoms is there.

Dissociative Disorder

- A disorder in which critical personality facets, that are normally integrated and working together, become separate.
- This allows stress avoidance and anxiety reduction by way of escape.
- The person uses defense mechanisms for avoiding stress and to deal with traumatic experiences
- At a time, two or more personalities ,may exist within the person

- **Symptoms** include;
- Auditory or visual illusion,
- Feeling of confusion and disorientation,
- Severe anxiety attacks,
- Suicidal attempts,
- Inflicting self-injuries

Causes involves:

- High state of stress
- **Treatment includes:**
- Psychotherapy
- Self- induced trance
- Minimize stress

Types of Dissociative Disorder

i. Dissociative Amnesia:

- A state when a selective loss of memory occurs.
- The person is unable to recall specific events often as
 - a result of extreme stress.
- Significant memory loss occurs about personal information that is not due to an organic cause.
- This disorder vanishes abruptly as it begins and rarely re-occurs.

ii. Dissociative Fugue:

- Fugue means “flight”
- Fugue is a form of amnesia.
- The sufferer takes sudden impulsive trips, at times assuming a new identity.
- Dissociative fugue includes forgetting as well as fleeing from one's home for days and weeks, also being unable to remember one's identity
- o Unconscious wandering in which the person has limited social contacts.
- o In some instances, person may take over another personality that is more sociable than the previous one

iii. Dissociative identity disorder/Multiple personality:

- o Rare disorder in which the person may take over two or more personalities that are entirely different from one another
- o The first one is usually restrained, restricted and dull but the other one is entirely different from the previous one; one's mannerisms, vocal, movements are entirely different from one another

Mood Disorders

- Psychological and affective disturbances characterized by emotional extremes that are enough to produce troubles in daily living
- The emotional response is disturbed and so strong that it disturbs everyday living.
- Mood disorders mainly include:
 - Major depression
 - Mania
 - Bipolar disorder

i. Major Depression

- Previously known as “melancholia”
- Major depression is a severe form of depression.
- Common form of mood disorders
- A disorder characterized by lack of concentration, decision- making, sociability, withdrawal from others, and a feeling of worthlessness and inadequacy.
- Depression is labeled as depressive disorder when it persists for long and hampers daily life.

Symptoms include:

- Concentration problems,
- Irritability and restlessness,
- Persistent sadness, anxious and empty mood
- Fatigue,
- Appetite changes
- Feeling of agitation
- Sleep disturbances
- Hopelessness and pessimism
- Loss of interest in activities which are pleasurable
- Suicidal thoughts

- **Causes** include;
- Hereditary cause,
- Stress,
- Chemical imbalances in the brain; the sufferer however has the belief that it is a medical illness rather than a psychological one
- Most commonly occurs in people with low self-esteem
- Women are twice as likely to develop major depression as men.

- Learning experiences may contribute to the development of depression
- Serious loss in business or some other disaster,
- Relationship problems, financial setbacks etc
- **Treatment** includes;
- Use of medication,
- Psychotherapy,
- Behavioral therapy

ii. Mania

- **Mania is the opposite state of depression.**
- **It is an extended state of intense wild elation.**

iii. Bipolar Disorder

- Bipolar disorder is a combination of depression and mania.
- The sufferer alternates between periods of extreme euphoria and elation i.e., mania, and bouts of depression.

Side Effects of Mood Disorders

- The height of elation may lead to high creative output, although it does not ensure high quality of the creative output.
- The manics are often reckless and end up with self injury.

Causes

- Psychodynamic explanation: Feeling of loss, that can be real or potential.
- Hereditary factor: These disorders appear to be running in families.
- The role of neurotransmitters: Serotonin and norepinephrine have been found to be related to these disorders.
- Alterations in the level of these chemicals have a role to play in mood disorders.

- Behavioral explanation; Lack of, or reduction in, positive reinforcement leads to mood disorders.
- Cognitive explanation: the sufferers of depression believe that they are life's losers, they are failures, inadequate, and not meant to be the 'winners' in life. They have a pessimistic view of life.
- Evolutionary psychology explanation: Considering the impact of the genetic factors, it states that depression is an adaptive response to unattainable goals.

Schizophrenia

- Schizophrenia is a category of mental disorders marked by severe distortion of reality.
- There is a deep division between the real world and the schizophrenic's world.

What makes schizophrenia different from other disorders?

- Significant decline from a previous level of functioning.
- Disturbances of thought and language

Symptoms in Schizophrenia

- Delusions
- Hallucinations
- Emotional disturbances
- Withdrawal

Delusions

- Unshakable, firm, and deeply believed in beliefs are held by the schizophrenic.
- Delusions can be about one's being grand, or being persecuted by others, or others planning against him, or one's thoughts being relayed to others who are out of physical reach.

Hallucinations and Perceptual Disorders

- The schizophrenic has sensory experiences that ordinary people do not have.
- they may hear voices, see people or objects, and/or smell things that others find to be non existent.
- The hallucinations mean reality to the schizophrenic.
- Hallucinations are usually based on the delusions.
- The sense of own body is also affected in schizophrenia.

Emotional Disturbances

- overall the schizophrenics show a flat, blank, and bland emotional response.
- Also, their emotional responses are inappropriate.

Withdrawal

- Schizophrenics live in an isolated world of their own.
- Schizophrenics withdraw from others.
- They avoid socializing.
- They are not interested in others.
- In extreme cases they are oblivious of the presence of others.

Types of Schizophrenia

- **a. Disorganized or hebephrenic type:**
- Marked by inappropriate emotion; inappropriate giggling, laughter, silliness, incoherent speech, infantile behavior, strange and at times obscene behavior.

b. Paranoid Schizophrenia

- The patient experiences delusions and hallucinations of his own greatness.
- Behavior is unpredictable, and erratic.
- Sense of judgment is lost.

c. Catatonic Schizophrenia

- Catatonic schizophrenia is marked by disturbances in the motor activity and muscular control.
- Major disturbances occur in movement.
- At times all motion stops and the patient just freezes in one position
- This frozen posture may last for hours and even days
- In some phases the patient exhibits wild, free floating, and even violent movement.

d. Undifferentiated Schizophrenia

- This variety of schizophrenia involves a combination of the major symptoms found in other varieties.
- This diagnosis is used when patients do not fit into any one of the major categories of schizophrenia.

e. Residual Schizophrenia

- Residual schizophrenia consists of minor signs of schizophrenia after a major, more serious, episode.

Lecture 38

Recap

- Anxiety disorders
- Somatoform disorders
- Dissociative disorders
- Mood disorders
- Schizophrenia
- Types of schizophrenia

Psychotherapy:

- The psychological interventions used for helping a person solve his psychological problems.
- The application of the principles, knowledge, and expertise in psychology for resolving conflicts, overcoming problems, adjusting better to the changing circumstances, coping with crises, and getting rid of mental illness.

When does one need to go to a psychotherapist?

According to Engler and Goleman (1992) the following list of symptoms can serve as a guideline for people in determining whether they need outside intervention, and whether it can be of help:

- Long-term feelings of distress that interfere with your sense of well-being, competence, and ability to function effectively in daily life activities.
- Occasions in which you experience overwhelmingly high stress, accompanied by feelings of inability to cope with the situation.
- Prolonged depression or feelings of hopelessness, particularly in the absence of a cause.

- Withdrawal from other people.
- A chronic physical problem for which no physical cause can be determined.
- A fear or phobia that prevents you from engaging in everyday activities.
- Feelings that other people are out to get you or are talking about you or plotting against you.
- Inability to interact effectively with others, preventing the development friendships and loving relationships.

Varieties of psychotherapies:

- **About 400 psychotherapies are available to us.**
- **Psychotherapists can choose any of these depending upon their theoretical orientation, as well as the nature of the problem they are dealing with.**

- **Also, depending upon their prior knowledge if any, patients and sufferers may choose a psychotherapeutic approach that they prefer to undergo.**

Psychotherapeutic Orientations:

- **Although a variety of psychotherapies are available, the primarily stem from four major psychological approaches:**
- **Psychodynamic approach**
- **Behavioral approach**
- **Cognitive approach**
- **Humanistic approach**

Who can be a psychotherapy provider?

- **Clinical Psychologists**
- **Counseling Psychologists**
- **Psychiatrists**
- **Psychoanalysts**
- **Licensed Professional Counselors or Clinical Mental Health Counselors**
- **Clinical or Psychiatric Social Workers**

Clinical Psychologists

- They are psychologists with a Ph.D. or Psychology.D. degree.
- They have to complete a postgraduate internship prior to practicing.
- They have a specialization in the assessment and treatment of psychological problems.

Counseling Psychologists

- **Counseling Psychologists have Ph.D. or Ed.D. degree.**
- **They typically provide help in solving routine-life adjustment problems.**
- **Their usual workplace is a university mental health clinic.**

Psychiatrists

- **Psychiatrists are primarily medical doctors.**
- **They have an M.D., M.B.B.S. or any other qualifying medical degree according to the law of any nation.**
- **The undergo post-graduate training in dealing with mental disorders.**
- **Being medical doctors, psychiatrists can prescribe medicine.**
- **Although there are no restrictions on who they can treat, psychiatrists generally treat severer cases of mental disorders.**

Psychoanalysts

- These professionals specialize in psychoanalysis.
- They can be medical doctors or psychologists trained in psychoanalysis.

Licensed Professional Counselors or Clinical Mental Health Counselors

- **These professionals hold a master's degree,
with a national or state certification in
therapy.**
- **They provide therapy to suffering
individuals, couples, an families.**

Clinical or Psychiatric Social Workers

- These professionals are basically master's degree holders.
- They undergo specialized training.
- As a result of their training, they can provide therapy regarding family and personal problems.

Psychotherapies based upon the psychodynamic approach:

- **These therapies are primarily based upon the psychodynamic model introduced by Freud.**
- **This approach is based upon two assumptions:**
 - **Unresolved past conflicts are the root cause of abnormal behavior.**
 - **Unacceptable unconscious impulses will enter consciousness.**

Psychoanalysis

- Introduced by Sigmund Freud.
- Therapy is based on the premise that the roots of all abnormal behavior reside as unresolved conflicts in the unconscious.
- To avoid these conflicts, people use defense mechanisms

- Psychoanalysis is an intensive, long term, psychotherapeutic procedure.
- Requires long sessions over extended periods----- may be years
- Better suited to intelligent individuals
- Involves a special relationship between the therapist and the patient

- ❑ **Target:** To explore unconscious motivation, conflicts, desires
- ❑ **Goal:** Establishing intra psychic harmony by developing awareness of the role of the id, reducing over compliance with super ego, and by strengthening the ego
- ❑ **Understanding of ‘repression’:** The therapy gives central importance to the understanding of the manner in which the person uses repression to handle conflict

Interventions used in Psychoanalysis:

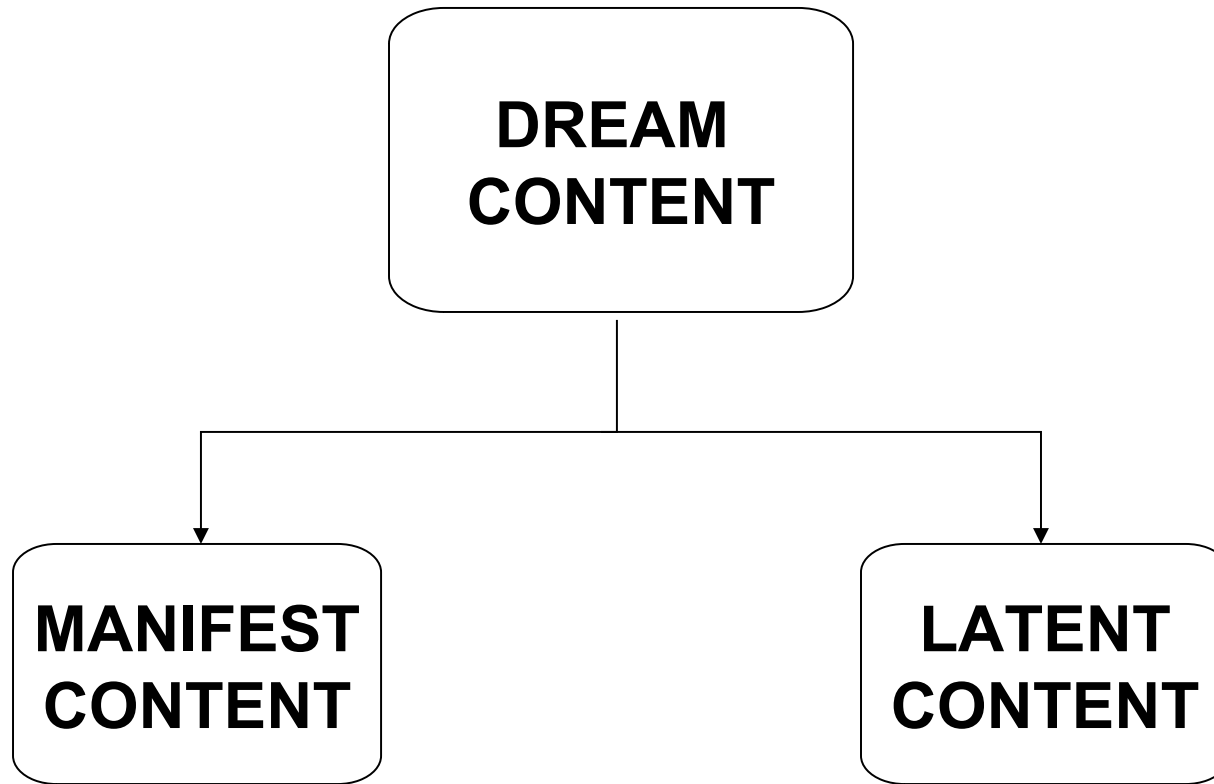
1. Free association:

- ☐ Kept in a comfortable position, the patient is asked to talk aloud and say whatever comes to his mind without considering whether or not it is relevant, rational, or sensible
- ☐ The patient is asked to reveal even the most undesirable and strong thoughts that have been repressed. This leads to emotional release, called 'catharsis'

Dreams in Freudian Approach

- **Dreams reflect unconscious needs, desires, impulses.**
- **With the help of these dream analysis, psychotherapists seeks an understanding about the unconscious motives, desires and conflicts**

- **There are two parts of dreams**



- **Manifest part is what the patient describes about the dream and its surface interpretation**

- **Latent part is the hidden part or the “ true message” within the dream**

Symbolism

- ❑ The manifest content is in a symbolic form**
- ❑ Converted into this form by the ‘dream censor, a mechanism that ensures that sleep is not disturbed by unconscious desires, and those desires are presented in a socially acceptable form**

Analysis of Resistance

- At times patient feels inhibitions, and is unable or unwilling to express some thought or feeling i.e., barriers between conscious and unconscious. The psychoanalyst aims to break down such resistances so that the patient is enabled to face the unpleasant thoughts, impulses, events

Dream Analysis:

- The therapist tries to uncover the latent content of dreams and decipher the symbolism involved
4. Analysis of Transference & Counter Transference:

□ Transference: The patient's emotional response toward the therapist, is often an indication of the patient's relationship with a person who had been the center of the conflict. It may be negative or positive

□ Counter Transference: The therapists' emotional reaction toward the patient

Criticism Against Freudian Psychodynamic Theory

- ❑ There is no scientific proof that many psychodynamic constructs , e.g. unconscious, exist**
- ❑ Psychic Determinism: Freudian approach is deterministic and leaves not much room for conscious, rational, decision making or personal will to act**
- ❑ It ignores the external variables and the environment**

- ❑ It emphasizes the early childhood experiences too much**
- ❑ Mostly criticized for its interpretation of the relationship between the genders**
- ❑ The therapy is too time consuming and therefore expensive**

Behavioristic Approach to Treatment

- ❑ A psychological approach that considers the relationship between behavior and environmental stimuli as the focus of study; observable behavior is what psychology should be studying, understanding and explaining**
- ❑ An approach that dominated psychology for most of the 20th century**

What do the Behaviorists Study?

- ☐ Observable/ overt behavior**
- ☐ Specific measurable responses**
- ☐ How particular types of behaviors are controlled by particular types of environmental stimuli**
- ☐ Method of investigation: Data typically collected under controlled laboratory conditions, employing technological assistance**

What the Behaviorists Are Not Interested in:

- ☐ Unconscious**
- ☐ Inner motivation**
- ☐ Biochemical processes**

Behavior Therapy Is More Directed Towards

- Behavioral therapists focused on unlearning the mal- adaptive patterns of behavior which people acquired through learning; and abnormal behavior would be treated by making them to relearn new experiences
- Changing individual's behavior and and make them able to function more efficiently definitely solves the problem

Classical Conditioning and Behavior Therapy

- Behavior therapy uses various classical conditioning strategies for modifying the maladaptive behavior. these important techniques are;
- Systematic desensitization
- Aversive therapy
- Flooding
- Assertion therapy

Systematic Desensitization

- A procedure used in behavior therapy in which a stimulus that generates pleasant feeling is repeatedly paired with the anxiety provoking stimulus in hope to alleviate anxiety
- The steps involved are;
- Before using this procedure, the patient has to learn muscle- relaxation techniques by the therapist

- The next step is to make hierarchy of fears_____ a list is made by increasing severity in which the stimulus that is least severe is placed first and the severe in the last i.e. the anxiety provoking stimulus
- The patient is placed in a relaxing position and is gradually exposed to the anxiety provoking stimulus, starting from the least fearful to the most_____ it is used to extinguish anxiety

- Whenever person feels anxiety, the session stopped there and asked the patient to relax
- This method proves to be very effective with people with irrational fears, anxiety, phobias etc

Aversive therapy

- Developed by Ivan Pavlov
- A technique used to unlearn the unwanted habits by associating it with the unwanted impulses
- This technique is frequently used with alcoholics, smokers and substance abusers

Observational Learning

- Learning the experiences through modeling and imitation
- Behavior therapists used modeling for teaching the patient new skills and habits and alleviate their anxieties, fears and phobias

Operant Conditioning

- Operant conditioning approaches uses the rewards and punishment strategies for modifying behaviors

Token Economy/ Token System

- The person is rewarded with some form of a token every time a desired behavior is exhibited
- The token can be play money/token or a chip representing money; it can be the silver or gold stars earned by the child; parents can give different colored paper tokens for good behavior
- After a specific number of token have been earned, they can be exchanged for something desirable

Contingency Contracting

- A written contract is held between the client and the therapist, specifying all goal-behaviors as well as consequences; parents and teachers can also use it
- the contract is followed strictly no matter if the consequences of behavior are negative, and the client may in fact dislike them; the purpose is to promote target behavior
- e.g if an over-eater fails to refrain from confectionary throughout the week, he will have to send a donation cheque for drinks in a marathon ; the cheques are prepared at the beginning of the program

Shaping

- ❑ Successive approximations of a required/ desired response are reinforced until that response is fully learnt**
- ❑ In the beginning each and every success is reinforced with a reward, no matter how small the success**
- ❑ Once the desired response is learnt the reinforcer immediately follows it , every time it happens**

- ❑ **Once learnt the behavior, in many cases, the organism may not need reinforcement since many behaviors are self reinforcing e.g. learning to play a musical instrument**

Usefulness of Behavior Therapy

- Found to be effective with 50-90% of patients especially treating phobias, anxiety disorder and other unwanted behaviors such as smoking and drug abuse
- Also helpful in the sense that it can also be employed by non-professionals too
- Cheap and economical as it directly focused on the problem to be unlearn

- Criticized also because it also give importance to the overt observable behavior and does not evaluate the inner thoughts and experiences

Cognitive Approach in Behavior Modification

- ❑ Negative and unacceptable behavior is modified through constructive strategies**
- ❑ According to this theory, person's beliefs and attitudes effect the motivation and behavior of a person**
- ❑ In order to modify the behavior, reinforcement techniques are used**
- ❑ For attaining the desired goal, realistic strategies are used with continuous feedback**

Behavior Modification

- A therapeutic/intervention strategy used for modifying behavior in such a manner that the frequency of desired behavior is increased up to the optimal level, and the frequency of undesired behavior is brought down to the minimum.....or to extinction level
- The intervention is based upon the principles of learning

Steps in Behavior Modification

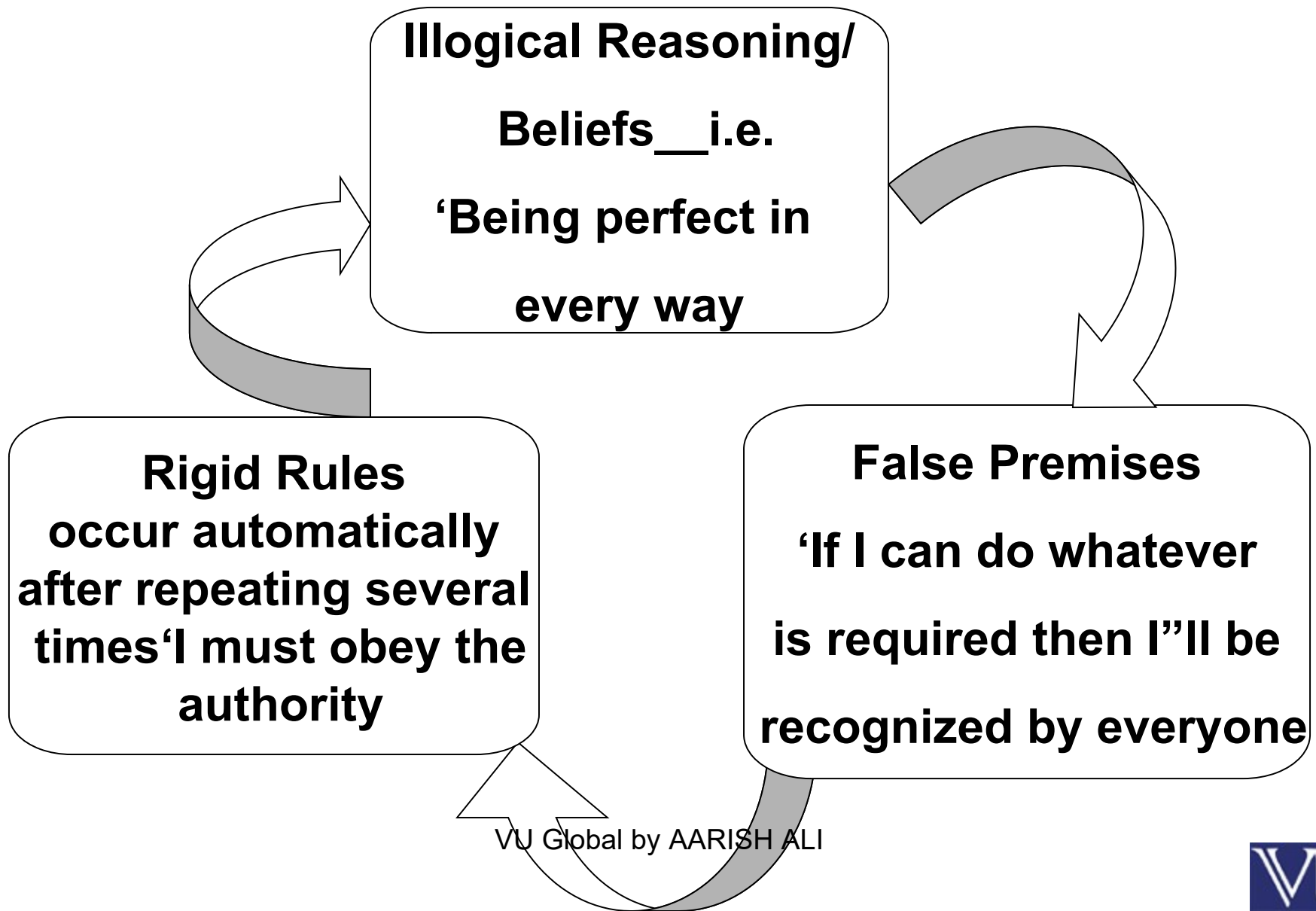
- i. Identification of goals in terms of target behavior
- ii. Recording the preliminary/background information concerning the behavior in question
- iii. Designing the intervention, issues involved and deciding its components
- iv. Implementation of the planned program as well as careful monitoring

- v. Recording the events, progress, and problems during the implementation phase
- vi. v. Evaluating the program and making alterations if required

Altering the Belief System

- ❑ Psychologists are of the view that psychological problems arise due to the way people perceive themselves in relation with the people they interact**
- ❑ Main focus of the therapist is to alter the irrational belief system of a person**

Cause of Psychological Illness



Cognitive Theory for Depression

- ❑ Aaron Beck formulated the therapy for depression patients**
- ❑ Therapist helps the depressive person to change the faulty patterns of thinking through problem- solving techniques**
- ❑ Believed that depression reoccur in depressive patients because the negative thoughts occur automatically from which they are unaware**

Four tactics are used by the therapist that unfounded depression

- i. Challenging the patient's ill beliefs**
- ii. Evaluating the cause of depression**
- iii. Attributing the cause to the environmental situation/ event not to the person's in competencies**
- iv. Finding the alternative and effective solutions for the complex problems**

Rational-Emotive Behavior Therapy

- ❑ Developed by Albert Ellis (1962, 1977)**
- ❑ Focused on altering the irrational beliefs into more acceptable way**
- ❑ Clients are forbidden to use “should”, “must”, “ought” etc**
- ❑ Confrontation techniques are used which focused on changing the attitudes through rational reasoning**

❑ Task is to protect the self worth, potential to be self-actualized by blocking the irrational thinking patterns

Lecture 39

Recap

- Psychotherapy
- Who needs psychotherapy?
- Varieties of psychotherapies
- Theoretical orientations of psychotherapies
- Providers of psychotherapies
- Psychodynamic therapies
- Behavioral Therapies

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Implosive Therapy

- Making the patient directly encounter the fear-provoking stimulus.

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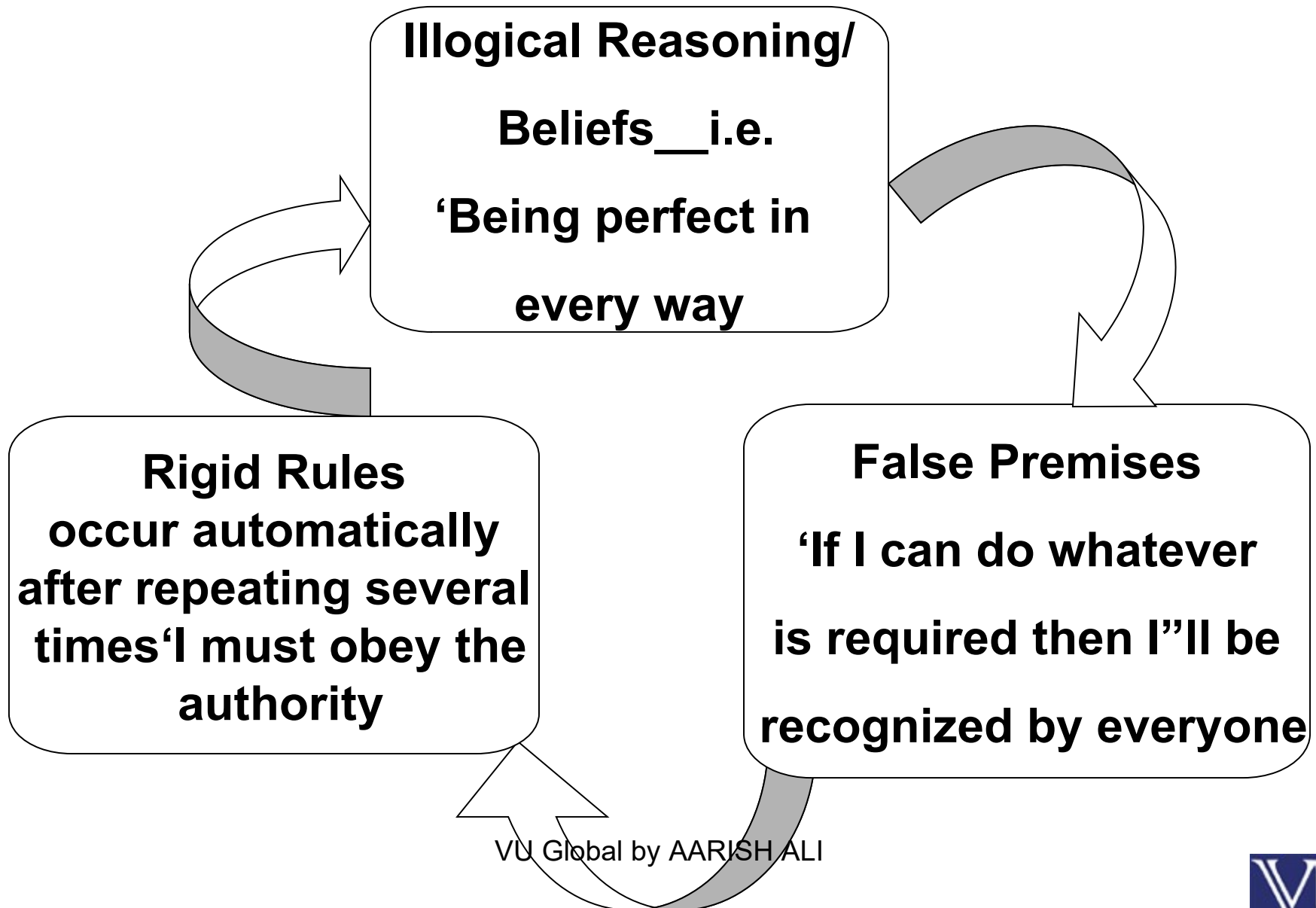
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Humanistic Therapies

Central Themes of Humanistic Approach

- ❑ Human beings are capable of shaping their own destiny**
- ❑ They can think and design their course of action and can follow it in the way they like**
- ❑ People can overcome or minimize the environmental, and intrinsic influences**
- ❑ “Here and Now” is important**
- ❑ “wholeness” or “completeness” of the personality is important rather than its separate, disintegrated, structural parts**

Humanistic approach emphasizes:

- ❑ Individual's freedom in directing his future**
- ❑ capacity for personal growth**
- ❑ intrinsic worth, and**
- ❑ potential for self-fulfillment**

Rogers' Approach

- ❑ Primarily a clinical theory, based on years of Roger's experience dealing with his clients**
- ❑ In its richness and maturity his theory matches that of Freud; a theory well thought-out and logical, having broad application**
- ❑ The theory emphasizes on a single factor “force of life” which he calls the actualizing tendency i.e. built-in motivation present in every life-form to develop its potentials to the fullest extent possible**

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To be fully functioning means experiencing:

- i. Optimal psychological adjustment**
- ii. Optimal psychological maturity**
- iii. Complete congruence (a feeling of integration when the self and the ideal self match; incongruence is a feeling of conflict or unease experienced in case of a mismatch between the two)**

Carl Roger's Psychotherapy

- ❑ Carl Rogers is best known for his contributions to therapy known as “person- centered/ Client- centered therapy/ Non- directive therapy**
- ❑ Also known as the Rogerian Therapy”**
- ❑ His main technique “Reflection”__ mirroring of emotional experiences**
- ❑ Aim of the therapy:to help a person grow and self-actualize**

- ❑ **Rogers maintained that the therapist must possess the following qualities:**
 - i. **Congruence -- genuineness, honesty with the client**
 - ii. **Empathy -- the ability to feel what the client feels**
 - iii. **Respect -- acceptance, unconditional positive regard towards the client**

Humanistic Approaches to Therapy

- ❑ The approach suggesting that people are in control of their life
- ❑ The person or the self , and personal growth and development are emphasized by the humanistic psychologists
- ❑ The humanistic approach includes a number of other theories with the same or similar orientation e.g., ‘existential’ and ‘phenomenological’ psychology

- ❑ They view that abnormal behavior results when an individual is unable to find true meaning and purpose of their life**

Central Themes of Humanistic Approach

- ❑ Human beings are capable of shaping their own destiny**
- ❑ They can think and design their course of action and can follow it in the way they like**
- ❑ People can overcome or minimize the environmental, and intrinsic influences**
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- ☐ **Intrinsic worth and**
- ☐ **Potential for self-fulfillment**

- ❑ Rogers had the person-centered approach since the ‘person’ was the main figure of importance**
- ❑ He believed that the most powerful human drive is the one to become “fully functioning”**
- ❑ Fully functioning= a person becomes all that he or she is capable of**

❑ To be fully functioning means experiencing:

- i. Optimal psychological adjustment
- ii. Optimal psychological maturity
- iii. Complete congruence (a feeling of integration when the self and the ideal self match; incongruence is a feeling of conflict or unease experienced in case of a mismatch between the two)
- iv. Complete openness to experience

Carl Roger's Psychotherapy

- ❑ Carl Rogers is best known for his contributions to therapy known as “person- centered/ Client-centered therapy/ Non- directive therapy**
- ❑ Also known as the Rogerian Therapy”**
- ❑ His main technique “Reflection”__ mirroring of emotional experiences**
- ❑ Therapy in which the therapist's role is to reflect back the patient' statement in such a manner that the patient finds solutions to his/ her problems**

- ❑ **Aim of the therapy: To help a person grow and self-actualize**

- ❑ Rogers maintained that the therapist must possess the following qualities:
 - i. Congruence -- genuineness, honesty with the client
 - ii. Empathy -- the ability to feel what the client feels
 - iii. Respect -- acceptance, unconditional positive regard towards the client

Unconditioned positive regard: Roger calls it as it aims to show acceptance and understanding on the part of therapist regardless of what the client is thinking, feeling and expressing

Other Approaches

Existential Therapy

- Contrary to humanistic approach to therapy that focuses on unique freedom and potential as positive force, existential therapy is based on the notion that when an individual becomes unable to deal with his/ her freedom, it will result in anguish, fear, and concern
- The goal of life, according to this therapy is to properly grip and use that freedom effectively with value systems one has in his/ her life

- Therapy also focuses on the individual's responsibilities one must take in order to make free choices about their lives
- Therapist job is to probe and challenge the patient's views of the world around
- In the same way, a strong bond is developed between the patient and therapist so that the patient finds comfortable while interacting; also they both are experiencing the same difficulties and apprehensions

Gestalt Therapy

- The Gestalt therapy that focuses on integrating the patient's thoughts and feelings into an integrate whole
- This can be done through by placing oneself in that position as mother or father and then his/ her own position in order to experience different parts of a conflict

- The aim of the therapy to express to whatever is frustrating and conflicting to the person such as kicking the things around when angry, yelling out while frustration etc
- This sort of activities encouraged the person to act out/ express the things that are conflicting and frustrating

Group Therapy

- A form of therapy in which people discuss problems and difficulties with the group members
- Several unrelated people work with the therapist and discuss their psychological problems with him; in some the therapist is active and directed towards the problem and sometimes the group takes an issue and determine how to precede
- People often discuss issues like smoking, alcoholism, lacking social skills

- The other members of the group provide emotional support and suggestions so that the it can also be helpful to them in similar circumstances

Family Therapy

- The therapy which involves two to three members of the family for finding solutions to problems
- Therapist considers family members as a unit in which each member serves as a contributor in order to gain an understanding of “ how” they interact with one another

- Family therapists believed that problems arise in the family due to the rigid roles and conventions exhibited by the family and the solution of their problems lie in adopting new and constructive roles and patterns of behavior

Biomedical therapies

- Medication
- Psychosurgery
- Lifestyle changes

Lecture 40

Recap

- Behavior Therapy
- Cognitive Therapy
- Humanistic approach to psychotherapy
- Other approaches
- Biomedical approach

Popular Areas of Psychology

- Psychology today is the most popular social science.
- American Psychological Association has 55 divisions.

Most popular areas of psychology today:

- Clinical psychology
- Health psychology
- Organizational psychology

Major Subfields of Psychology

- ☐ **Behavioral Neuroscience**
- ☐ **Clinical psychology**
- ☐ **Clinical Neuropsychology**
- ☐ **Cognitive Psychology**
- ☐ **Counseling Psychology**
- ☐ **Cross-cultural Psychology**
- ☐ **Developmental Psychology**
- ☐ **Educational Psychology**
- ☐ **Environmental Psychology**
- ☐ **Evolutionary Psychology**
- ☐ **Experimental psychology**

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- ☐ **Forensic Psychology**
- ☐ **Health Psychology**
- ☐ **Industrial/Organizational Psychology**
- ☐ **Personality psychology**
- ☐ **Program Evaluation**
- ☐ **Psychology of women**
- ☐ **School Psychology**
- ☐ **Social Psychology**
- ☐ **Sport psychology**

SOCIAL PSYCHOLOGY

__The branch of psychology concerned with how people's thoughts, feelings and actions are affected by others.

- How will we behave if we do not have any people around us?
- What would we be like if we had never ever come across another human being?

ATTITUDES,BEHAVIOR AND PERSUATION

➤ Attitudes:

Learned predispositions to respond in a favorable or unfavorable manner to a particular object.

➤ Attitudes can also be defined as our learned evaluations that we hold about other people, objects, events, behaviors, and beliefs.

ABC MODEL:

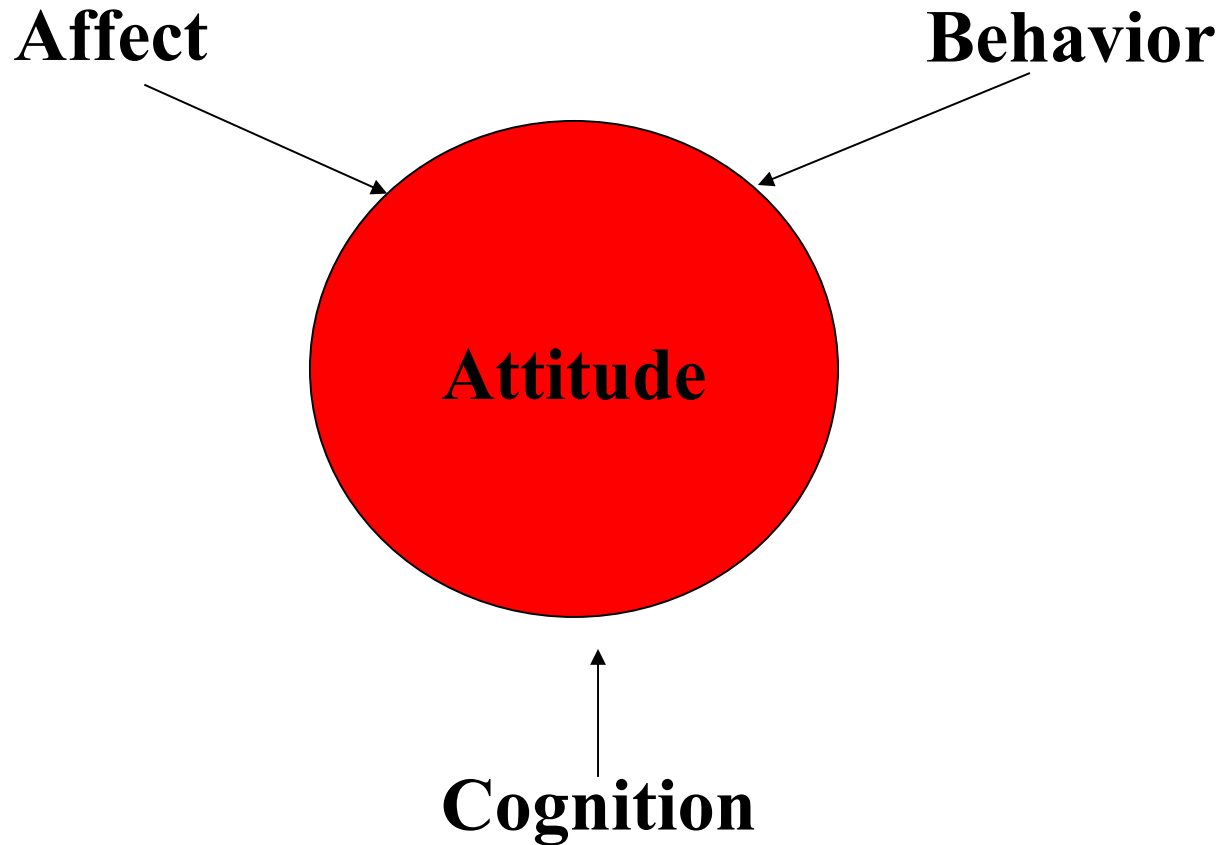
The model suggesting that an attitude has three components: affect, behavior and cognition.

Affect component: That part of an attitude encompassing how one feels about the object of one's attitude.

Behavior component: A predisposition to act in a way that is relevant to one's attitude.

Cognition component: The beliefs and thoughts held about the object of one's attitude.

Components of Attitudes



FORMING AND MAINTAINING ATTITUDES

- **Classical Conditioning and Attitudes:**

Advertisers make use of the principals of classical conditioning of attitudes by attempting to link a product they want consumers to buy with a positive feeling or event.

- **Operant Conditioning Approaches to Attitude Acquisitions:**

Attitudes that are reinforced, either verbally or nonverbally, tend to be maintained.

Vicarious Learning of Attitudes:

Learning by observing others.

For example, even if they have never met a blind person, children whose parents say that “blind people are incompetent’ may adopt such attitudes themselves.

We also learn attitudes vicariously through television, films and other media.(e.g violence).

AGENTS OF ATTITUDE FORMATION

-
- Family
- School
- Peer group
- Neighborhood
- Social Environment
- Mass Media

Persuasion

- Persuasion is the process through which people's attitudes are changed.

Factors affecting attitude change:

Message source

Characteristics of the message

Characteristics of the target audience

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1. Message Source:

- The characteristics of the attitude communicator or the deliverer of the message are important.
- Characteristics that make a difference:
 - Physical attractiveness
 - Social attractiveness
 - Expertise
 - Trustworthiness
 - Audiences believe that the communicator does not have any ulterior motive.

2. Characteristics of the Message:

- **Two-sided messages are more effective than one-sided messages.**
- **Two-sided messages are the ones in which both sides of the argument are discussed.**
- **Fear-producing messages are effective when it is accompanied by a solution to the problem as well.**
- **If the message is too fear-provoking, and without a solution too, then it may be ignored.**

3. Characteristics of the Target:

- Less intelligent people are less resistant and the more intelligent are more resistant to persuasion.**
- Persuasion is relatively easier with women when in public settings, and when they have less knowledge about the issue under question.**
- There are no gender differences in case of a change in private attitudes.**

Types of Information Processing used by the Target for Interpreting the Message:

- **Central Route Processing**
- **Peripheral Route Processing**

Central Route Processing

- **Interpretation of messages involving thoughtful consideration of the issues and arguments used to persuade.**

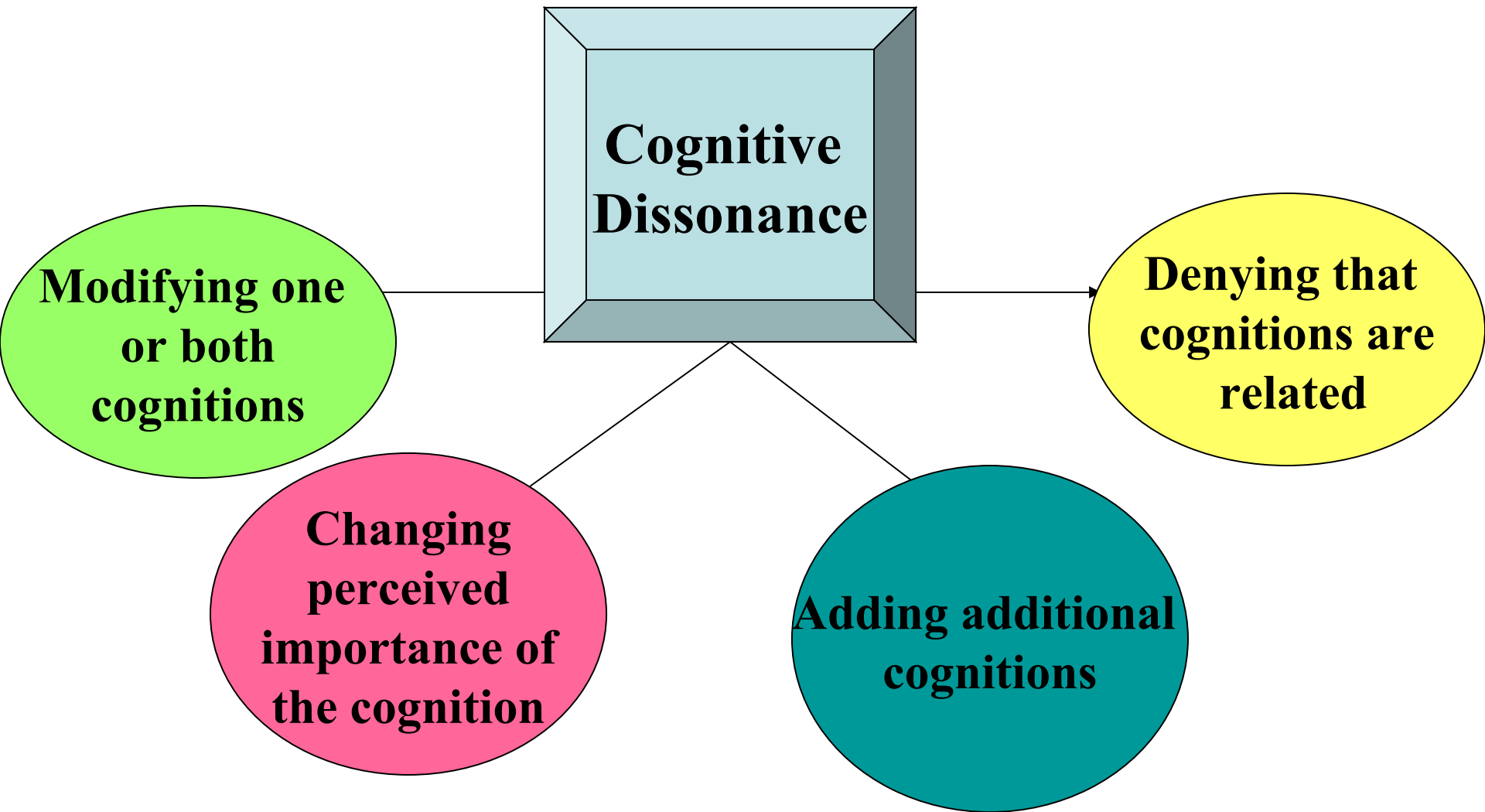
Peripheral Route Processing

- **Interpretation of messages involving consideration of the message source, and related general information, instead of considering the message itself.**

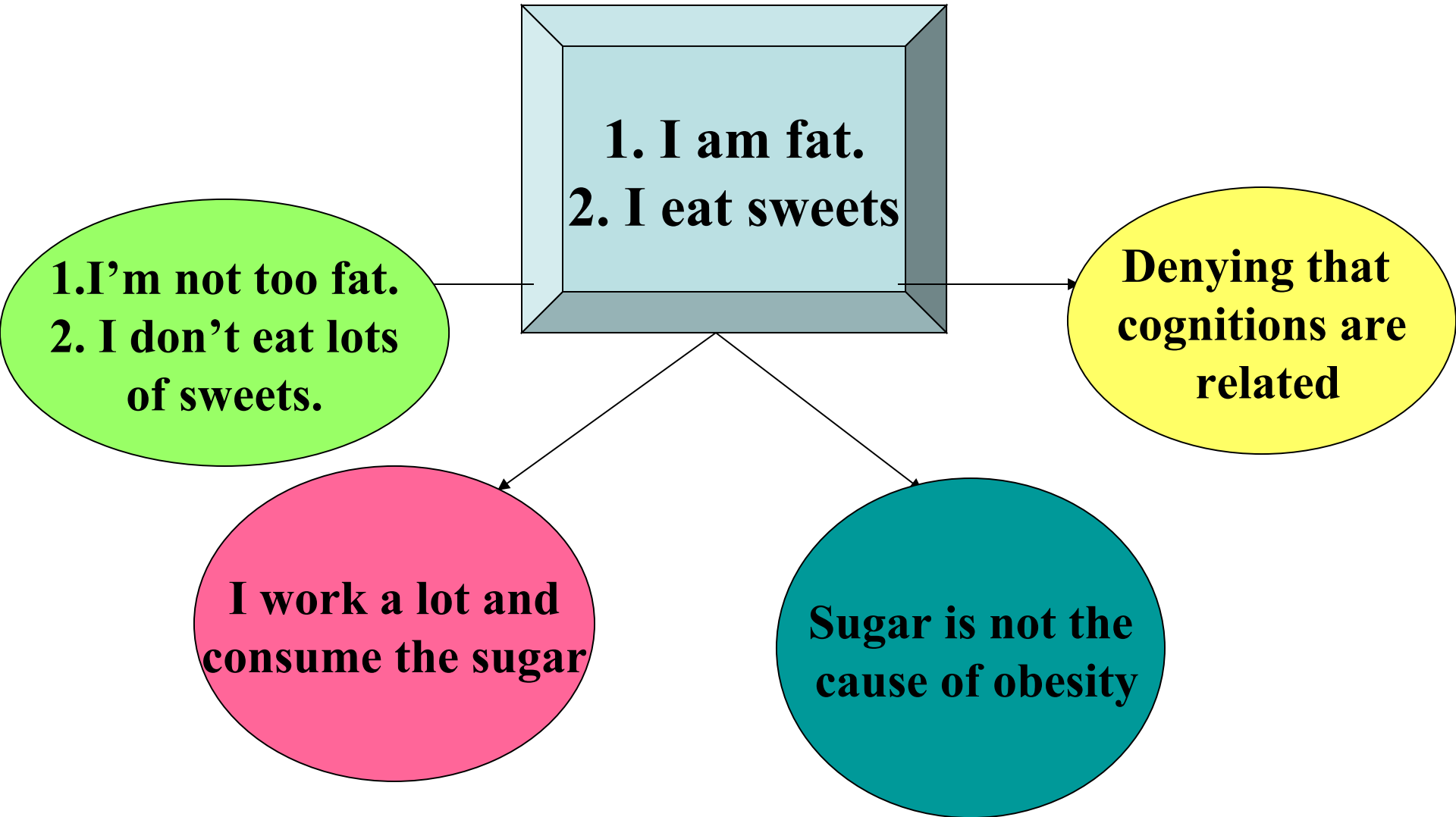
Cognitive Dissonance

- **Cognitive dissonance is a conflict experienced by a person.**
- **This conflict arises when an individual holds two contradictory cognitions i.e., attitudes or thoughts.**

Reducing Cognitive Dissonance



Reducing Cognitive Dissonance



SOCIAL COGNITION

The processes that underlie our understanding of the social world.

- We use social cognitions to understand and make sense of others and of ourselves.

- Schemas:

- Research shows that we have highly developed schemas.

Schemas are sets of cognitions about people and social experiences.

Impression formation:

The process by which an individual organizes information about another individual to form an overall impression of that person.

●Central traits:

The major traits considered in forming impressions of others.

Theories of social cognition describe how people form an overall impression of other's personality traits.

Attribution Processes

- The processes of attributing causes to other people's behavior

Attribution Theory:

- The theory of personality that explains how we decide, about the specific causes of an individual's behavior on the basis of samples of that person's behavior.

The Nature of Perceived Causes of Behavior

- **Situational causes:**

A perceived cause of behavior that is based on environmental factors.

- **Dispositional causes:**

- **A perceived cause of behavior that is based on internal traits or personality factors.**

Biases in Attribution:

1. The fundamental attribution error:

People have a tendency to attribute the behavior of other people to dispositional causes; they give less importance to the situational causes.

2. The halo effect:

- If our initial perception of a person is positive then we tend to expect that the person has other uniformly positive characteristics too.

3. Assumed similarity bias:

- We have a tendency to think that others are similar to us. This may happen even when we see someone for the first time.

Social Influence

- The process through which our behavior is affected by the actions of another individual or a group.
- Social influence can be seen in terms of:
- Conformity
- Obedience

Conformity:

- Conformity is going along with people.
- People are said to be conforming when their behavior or attitudes change as a result of a desire to follow the beliefs or standards of other people.
- The following variables may determine whether or not one will conform:
 - The characteristics of the group.
 - The situation in which the individual is responding.
 - The kind of task
 - Unanimity of the group

Obedience:

- It is a form of conforming behavior that results from the commands of others.

Compliance:

- A form of conforming behavior that results from direct social pressure.
- Different tactics are used by lay people, organizations, and salespersons to make others comply:
 - The foot-in-the-door technique
 - The-door-in-the-face technique
 - The that's-not-all technique
 - The not-so-free sample

Prejudice and Discrimination:

- Prejudice refers to the negative or positive expectations about social groups and their members.
- **Discrimination:**
 - **Negative behavior toward members of a particular group.**

Prejudice and discrimination may result from stereotypes:

- **Stereotype:**
A kind of schema in which beliefs and expectations about members of a group are held simply on the basis of their membership in that group.

Stereotypes can be positive as well as negative.

- **Ingroup-Outgroup Bias:**

The tendency to hold less favorable opinions about groups to which we do not belong , while holding more favorable opinions about to which we do belong.

- **Outgroups:**

Groups to which people feel and believe they do not belong.

Ingroups:

Groups to which people feel and believe they do belong.

- **Self-fulfilling prophecy:**
**An expectation about the occurrence of an event or
behavior that increases the likelihood that the
event or behavior will happen.**

Recap

- **Popular areas of psychology**
- **Social psychology**
- **Attitudes**
- **Persuasion**
- **Social cognition**
- **Attribution processes**
- **Social influence**
- **Prejudice and discrimination**

Health Psychology

- **The branch of psychology that focuses upon the role of psychological factors in the development and prevention of illness, in coping with disease, and in health promotion.**
- **The branch of psychology that studies the relationship between psychological variables and well being of a person.**

According to Brannon and Fiest (2000):

- **Health psychology is the branch of psychology that concerns individual behaviors and lifestyles affecting a person's physical health.**
- **Health psychology includes psychology's contribution to the enhancement of health, the prevention and treatment of disease, the identification of health risk factors, the improvement of the health care system, and the shaping of public opinion with regard to health.**

A health psychologist works in the following areas:

Enhancement of health

Prevention of disease

Treatment of disease

Identification of risk factors

Improvement of the health care system

Shaping public opinion regarding health

Understanding Health:

In order to understand the modern concept of health, we need to consider some questions:

- How would you define health?**
- Who is a healthy person?**
- Can lifestyles be healthy and unhealthy?**

According to the World Health organization (WHO):

HEALTH is “ a state of complete physical , mental , and social well – being and _not merely the absence of disease or infirmity”.

.

- The modern approach towards health is more prevention - oriented rather than treatment – oriented .**

Emergence of health psychology :

- American Psychological Association established its Division 38 , Health Psychology, in 1978 .
- This was the formal establishment of health psychology as a separate discipline.

- Facts leading to this newer definition and approach :
 - a. The leading causes of death today are no more infectious diseases, but lifestyle – related diseases;
implying that they are preventable .
 - b. The escalating costs of health care have led to the realization that these costs could be controlled ,
even reduced , by preventive strategies .

- c. The dissatisfaction with the existing medical model has been on a constant increase .**
- Dissatisfaction is primarily based on the observation that a person, even after taking proper medication and receiving full medical care may still not be feeling healthy.**
 - This implies that something else may also be contributing to one's well-being .**

d. Research on the role of psychological factors in health and illness:

- **It was observed that psychological factors have a role in physical illness.**
- **Also, psychological interventions are helpful in altering physiological conditions and symptoms.**

- ☐ **Today there is a growing consensus that health is essentially an ‘individual responsibility’.**
- ☐ **No community or state can give health BUT the required facilities.**

**NO IMPROVEMENT IN THE HEALTH CARE
SYSTEM CAN BE EFFICACIOUS UNLESS
THE CITIZENS ASSUMES RESPONSIBILITY
FOR THEIR OWN WELL – BEING .**

Different psychological theories and models have tried to explain health – related beliefs and behaviors .

The Health-Belief Model ,
The Protection Motivation Theory ,
The Theory of Planned Behavior ,
Self – Regulatory Model ,
Self – Efficacy Theory .

All explanations agree on one point i.e.,

A person's health , as well as the feeling of being healthy, depends to a large extent , on how much control does one feel over one's life , circumstances , and health .

**HEALTH or a STATE OF COMPLETE WELL-
BEING has to be earned and maintained by the
individual himself, who must accept this
responsibility**

Some Psychological Variables Affecting

Health:

HEALTH LOCUS OF CONTROL :

Who is responsible for an individual's degree of health or illness ? ...the doctors , family/ friends , fate , or the person himself ?

PERCEIVED CONTROL :

**The tendency to feel or experience responsibility about the personal health related acts and behaviors,
as well as the ability to get the desired outcomes .**

SELF PERCEPTIONS OF WELLNESS :

- The individual's personal feeling and evaluation of experiencing well-being , especially in comparison with others .
- Wellness dimensions include ; emotional , intellectual, physical , social , spiritual, and general dimensions.

- **LEARNED HELPLESSNESS :**
- **Proposed by Martin Seligman .**
- **It is a belief that one has no control over the environment .**
- **People conclude that unpleasant or aversive stimuli can not be controlled.**
- **This pattern becomes so ingrained that they do not try to remedy the aversive circumstances even if they actually can exert some influence.**

Health and Principles of Behavior Change

- Classical conditioning
- Operant conditioning
- Observational learning

Classical conditioning

- Many health-related behaviors are a result of classical conditioning:
- Fear of going to a doctor or dentist
- Fear of surgical procedures
- Fear of swallowing medicine
- Bulimia and Anorexia Nervosa: Fear of gaining weight after eating even in small amounts.

Operant conditioning

- A number of health-related behaviors depend on operant conditioning:
- Children tend to over-eat when mothers encourage them for eating.
- Teenagers may begin drug abuse or smoking when peers dare them to try a drug or a cigarette.
- Many people exercise regularly because others appreciate their physical fitness.

Observational Learning:

- Drug addiction usually results from observational learning.
- People whose peers exercise regularly tend to indulge in regular exercise.
- Children may go for surgical procedures without fear if they observe other children doing the same.

STRESS , HEALTH, and ILLNESS

- **STRESS** is the response of an organism to events that are threatening or challenging.

STRESSORS: circumstances that produce threats to our well being (e.g. death of a loved one, marriage , exams, illness).

- **UPLIFTS** : Minor positive events that make one feel good e.g. seeing an old friend , feeling healthy , completing a task .
- **DAILY HASSLES** are also a source of stress.:

THE NATURE OF STRESSORS

- **There are three general classes of events that can be seen as stressful :**

a . CATAclysmic EVENTS

b . PERSONAL STRESSORS

c . BACKGROUND STRESSORS

•a . CATAclysmic EVENTS :

- Strong stressors that occur suddenly affecting many people at once e.g., natural disasters .**
- Once they are over people can look forward to the future ,knowing that the worst is behind them.**
- But at times the victims may experience POSTTRAUMATIC STRESS DISORDER i.e., re -experiencing the original stress event and associated feelings in flashbacks or dreams.**

b . PERSONAL STRESSORS :

- **Major life events that have immediate negative consequences which generally fade with time (e.g.death of a loved one ,loss of job ,broken love affair).**

c . BACKGROUND STRESSORS:

- **Daily hassles causing minor irritations but have no long term ill effects unless they continue or are compounded by other stressful**

MANIFESTATIONS OF STRESS :

**A. Psychological: tension , anxiety ,
depression , helplessness.**

**B. Physiological: Psychosomatic disorders
i.e., medical problems caused by an
interaction of emotional , psychological ,
and physical difficulties (allergies ,
aches and pains, flu).**

Impact of Stress:

a. Direct physiological effects

b. Harmful behaviors

c. Indirect health-related behaviors

a. Direct Physiological Effects:

- High blood pressure
- Decreased functioning of the immune system
- Elevated hormonal activity
- Psychophysiological conditions

b. Harmful Behaviors:

- Increased unhealthy behaviors e.g. smoking, alcoholism, and/or drug abuse.
- Poor nutrition.
- Sleep disturbances.

c. Indirect Health-related Behaviors:

- Poor compliance with medical advice.
- Delayed medical consultation and diagnosis.
- Avoiding going to a doctor

STRESS AND ILLNESS

➤ CORONARY HEART DISEASE:

A very significant role is played by stress . Type A personality .

Type A personality pattern is closely associated with CHD as compared to Type B. Type A is characterized by competitiveness ,impatience, a tendency toward frustration , and hostility.

CANCER AND STRESS :

Research evidence supports the hypothesis that stressors have a role to play in the onset of cancer .

SMOKING:

Cigarette smoking has been found to be associated with stress which in turn is associated with heart disease , cancer , and a number of other diseases

.

Stress and Recovery From **Disease:**

- ❑ The patients' emotional response may partially determine the course of their disease .**
- ❑ A POSITIVE STATE OF MIND HAS BEEN FOUND TO BE ASSOCIATED WITH LONGEVITY OF LIFE .**

Measuring Stress:

- Major life events may be used as indicators of the stress one experienced.
- The Social Readjustment Rating Scale by Holmes and Rahe is one of the assessment tools that can indicate the stress related risk that one may be facing.

Sample Items: The Social Readjustment Rating Scale

Life event	Mean value points
Death of spouse	100
Divorce	73
Marital Separation	65
Marriage	50
Son or daughter leaving home	29
Change in residence	20

Understanding Stress:

General Adaptation Syndrome Model:

A model explaining the course of stress .

Proposed by Hans Selye.

It suggests that a person's response to stress consists of three stages:

- a. Alarm and mobilization stage : a person's initial awareness of the presence of a stressor.**
- b. Resistance stage : coping with the stressor .**
- c. Exhaustion stage: failure to adapt to a stressor leading to physical , psychological , and emotional problems .**

•COPING WITH STRESS

The efforts to control , reduce , or learn to tolerate the threats that lead to stress.

STRATEGIES FOR COPING WITH STRESS

- **Psychodynamic view:**
Defense Mechanisms:
unconscious strategies people use to reduce anxiety by concealing its source from themselves or others .
-

•COPING CAN BE :

a . Emotion-focused

b . Problem-focused

•COPING STYLES (hardiness) AND SOCIAL SUPPORT PLAY A SIGNIFICANT ROLE .

•PRACTICAL STRATEGIES for COPING with STRESS:

- 1. turn threat into challenge.
- 2 . Make a threatening situation less threatening.
- 3 . Change your goals
- 4 . Take physical action.
- 5 . Preparing for stress before it happens i.e.,
stress inoculation.

Psychological Interventions for Helping People cope with Stress:

- Relaxation techniques
- Biofeedback
- Behavior therapy
- Cognitive therapy
- Hypnosis
- Meditation

Behavioral/Psychological Factors in Major Disorders and Risky Behaviors:

- **Coronary Heart Disease**
- **Cancer**
- **Diabetes**
- **AIDS**
- **Hepatitis**
- **Accidents**
- **Obesity**
- **Eating Disorders**
- **Addictive behavior**

Health Promotion:

- Health Education

Helping the Diseased and the Dying:

Lecture 42

Recap

- Health psychology
- Definition of health
- Emergence of health psychology
- Psychological factors in health and illness
- Health and principles of behavior change
- Relationship between stress, health, and illness
- Behavioral factors in major disorders and risky behaviors
- Health promotion
- Helping the diseased and the dying

Industrial/Organizational Psychology

- **The branch of psychology that investigates the psychology of the workplace.**
- **Industrial/organizational psychologists use the scientific methods and knowledge for studying the affects, cognitions, and behaviors of people in the work settings.**

Major focus of interest:

How best to fit the right person to a given job.

How best to fit the job to the person

How best to fit the right person to a given job.

What does “doing a good job” mean?

Personnel selection:How to select people who do the job well?

Training:How to train them so that they do the job well?

Motivation:How to motivate them so that they do the job well?

How best to fit the job to the person?

Quality of work life

Job satisfaction

Worker safety

Fitting the right person to the job:

- It involves the following:
 - ☐ a. Job analysis
 - ☐ b. Personnel selection
 - ☐ c. Personnel training
 - ☐ d. Worker's motivation

a. Job analysis:

- The first step in selecting the right person for a job is to do a job analysis.
- Job analysis is to prepare a specific description of a job. It encompasses the qualities and behaviors required of a person to do the job properly.

- **It is “the systematic study of the tasks, duties, and responsibilities of a job and the knowledge, skills, and abilities needed to perform it” (Riggio,1990).**
- **The purpose of job analysis is to find the best person for the job.**
- **For this we have to be very clear about the requirements of the job.**
- **Also, these requirements have to be translated into specific measurable behaviors.**

A complete job analysis is a two-step process:

- Step 1:

Preparing a detailed description of what a person to be selected for a particular job is expected to do.

- ☐ The job analysis has to be specific.
- ☐ Instead of stating general duties it should describe actual behaviors that the person has to perform.
- ☐ In general the duties and the responsibilities have to be specified.

Step 2:

- **Determining the performance criteria needed for the proper performance of a job .**
- **The specified duties and responsibilities have to be translated into measurable personal characteristics.**
- **Here, the Industrial/Organizational psychological has to outline the exact behaviors and characteristics that a person ought to have in order to perform the job in the best possible manner.**

‘Hard’ Criteria and ‘Soft’ Criteria:

- **Hard Criteria:**

These are the objective criteria.

These criteria are obtained from the available data e.g. salary, number of units sold, number of absentees etc.

- **Soft Criteria:**

These are the subjective criteria.

Soft criteria have a personal touch and require a degree of judgment i.e., sense of humor, congeniality, creativity etc.

- For example the best student of your college may be selected on the basis of her grades, or her interaction with fellow students, or both i.e., soft as well as hard criteria.

b. Personnel Selection

- Once job analysis is complete, the next task is to select the right person for the job.
- Personnel selection includes:
 - ☐ Devising ways of selecting the best applicant.
 - ☐ Making decisions regarding retention.
 - ☐ Making decisions regarding promotion.
 - ☐ Making decisions regarding termination.

Personnel Selection: Functions of an Application Form

- **It can be a rough screening device.**
- **It can supplement, or provide cues for, interviewing.**
- **The information contained in the application form may be used as a predictor of future performance e.g. academic record and job history can be indicative of a person's ability and potential.**

Personnel Selection: **Employment Interview**

- Employment interviews can be structured or unstructured.
- Structured interviews are preferred over the unstructured interviews.
- These consist of carefully phrased, prescribed, uniform, and fixed-ordered questions for all applicants.
- Structured interviews are considered more valid than the unstructured ones.

Personnel selection: Use of Psychological Tests

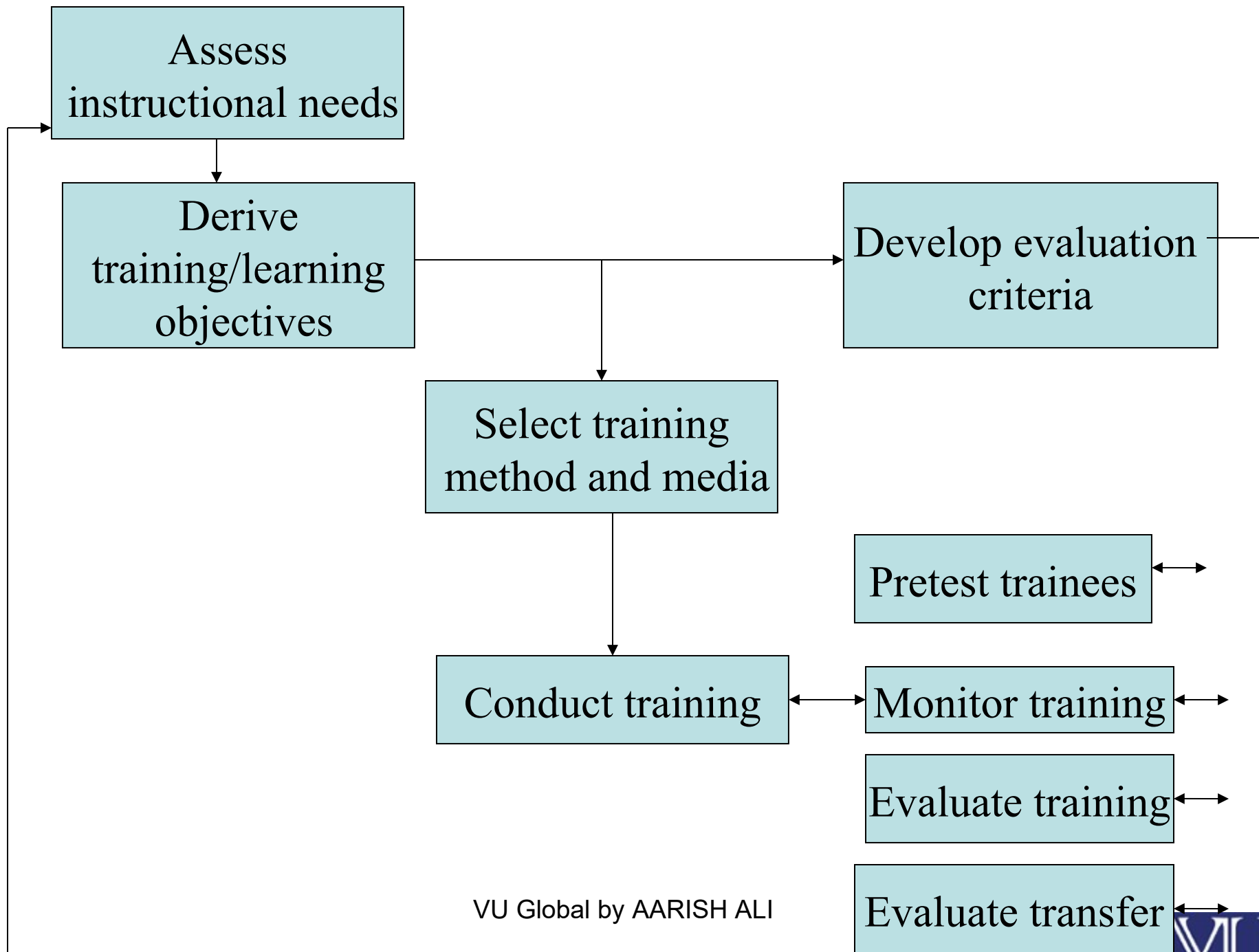
- At times the data obtained through application form and interviews may need to be supplemented by psychological assessment.
- Intelligence, ability, aptitude, achievement, or personality tests may be used.
- The tests of cognitive functioning (e.g. ability or achievement) have been found to be most useful.
- The use of I.Q tests for screening purposes is an issue of dispute.

c. Training the Selected Personnel

- Proper training is a requirement and a partial guarantee that the selected person will do the job well
- Training refers to a systematic and intentional process of altering the behaviors of employees to increase organizational effectiveness (Gerow, 1997).

Assessing Training Needs

Goldstein's (1986, 1989) System:



Evaluating training Effectiveness:

- It can be done in various ways;
 - Taking trainees' ratings.
 - Assessment by the organization i.e., measuring effectiveness with reference to training objectives.

d. Workers' Motivation:

- Workers' motivation affects efficiency and productivity of the organization.
- a team comprising unmotivated workers will not be able to attain the desired goals.

Motivation to Work

- For an organizational psychologist, what motivates a person to carry on or not his work is much dependent on three explanations. They are;
- Need theories
- Cognitive theories and
- Reinforcement theories

- **Need theories are primarily based upon Maslow's hierarchy of needs in which the most basic needs are easily fulfilled while complex needs may be difficult to meet or may remain unfulfilled.**
- **In order for higher order needs to be met, the basic and lower level needs have to be met first.**

- **When this theory relates to work, it maintains that initially workers are more concerned with salary and job security but when these requirements are met, then they move towards the more sophisticated and complex needs.**
- **In this way workers strive to fulfill their requirements by doing work**

Cognitive Theories of Work Motivation

- Theories that focus on the cognitive aspects of motivation; i.e. about the job and the work place as the motivational force
- How people think, feel, understand and expect about the job are the cognitions that affect motivation.

Expectancy Theory:

- Workers make logical choices to do what they believe will result in their attaining outcomes of highest value.

Equity Theory:

- Workers are motivated to match their inputs and outcomes with those of fellow workers in similar positions.
- This can be assumed to have a basis in vicarious learning.

Reinforcement Theories of Work Motivation

- The theory is based on the learning principles and maintains that motivation is increased or decreased by the level and type of reinforcement that is given to the workers
- Positive reinforcement increases in production or optimal production
- Punishment suppresses motivation.
- No reinforcement discourages motivation.

Equity Theory:

- Workers are motivated to match their inputs and outcomes with those of fellow workers in similar positions.
- This can be assumed to have a basis in vicarious learning.

Goal setting as a motivational technique:

- **Setting goals can motivate workers:**
 - i. **Set difficult but achievable goals.**
 - ii. **Goals should be specific and focused rather than general and vague.**
 - iii. **Feedback regarding achievement of goals should be regularly provided.**
 - iv. **Employees should be aware of the specific goals, and should accept them as reasonable.**
 - v. **Cultural concerns should be kept in mind.**

In collective cultures, where working together is practiced, involvement in goal setting is more important than in individualistic cultures.

Fitting the Job to the Person:

- Molding the job and the workplace in such a way that workers put in optimal efficiency and productivity.

Job Satisfaction:

- An attitude toward, or a collection of positive feelings about, one's job or job experience.

Factors of Job Satisfaction:

- **In job satisfaction, decision- making is very important .**
- **Usually two types of decision- making may take place:**
 - **Decentralized decision- making and**
 - **Centralized decision- making**
- **In decentralized decision- making the power extends throughout the organization**
- **In centralized decision- making power and authority rests in the hands of just few individuals at the top**

- Job satisfaction is higher in organizations following decentralized decision making.

Some Facts about job

Satisfaction:

- Younger workers tend to be more dissatisfied with their job.
- Older workers are more job dissatisfied by the end of their careers.
- There are no significant gender differences in terms of job satisfaction.
- There is a positive relationship between job satisfaction and perceived level or status of one's job.

Nature and Type of Job and Job Satisfaction

- It depends on two things
- **Job clarity:**

The rules and what is being expected of the person from the particular job are explicit to the worker.

- **Role conflicts:**

It has a negative effect on job satisfaction.

It arises when the person is unable to perform the job adequately and optimally because the roles and responsibilities are not clearly defined.

- Role conflict may make it easier to hide mistakes.
- But it also leaves little room for one's contribution being clearly identified.

Ways of Enhancing Job Satisfaction

- **Industrial and organizational psychologists are concerned with ways of enhancing job satisfaction as it is beneficial both for the organization and the workers.**
- **There can be three main approaches to affecting job satisfaction:**
 - **Changing the job**
 - **Changing the person in the job**
 - **Matching the person to the job**

Worker Safety:

- Job satisfaction can be improved if the organization is concerned about the safety and security of the workers, and the workers are also aware of the fact.
- Practical and explicit steps toward maintaining employees' protection have a positive effect.

Lecture 43

Recap

- **Industrial/Organizational psychology**
- **Focus of interest of industrial/Organizational psychology**
- **Selecting the right person for the job**
- **Worker's motivation as an issue in Industrial/Organizational psychology**
- **Fitting the job to the person**
- **Job satisfaction**
- **Ways of enhancing job satisfaction**
- **Worker's safety and job satisfaction**

Try to find answers to these questions:

- **Why do manufacturers advertise their products?**
- **Why do advertisers use models in the advertisements?**
- **Why do advertisers use cartoons rather than human models in the advertisements of children's products?**
- **Why do manufacturers introduce only one new model in a year?**
- **How will we decide what to buy if there were no advertisements?**

Consumer psychology

- The branch of psychology that studies consumers' buying behavior and the effect of advertisement on these behaviors.
- Consumer psychology focuses upon consumers' decision making and their behavior in the market place.

- The effect of advertisement on people's attitude and buying habits is an area of special interest for a consumer psychologist.
- Consumer psychology is Division 23 of APA.

Focus of interest:

What do consumer psychologists study?

- The responsive attitude of humans towards the product and service related information and experiences.
- Responses such as emotions, feelings, attitudes, beliefs and judgment as well as purchase decisions and consumption practices.

- **Information about the product and the related service information is very important.**
- **Variables such as advertisements, product packaging, package labels, coupons, consumer magazines, and word- of - mouth communication with the near ones that affect consumer behavior.**
- **Also brand loyalty, product preference, price, marketing and selling strategies are a focus of interest.**

- **The main aim or goal of consumer psychology is to describe, predict, influence, and/or explain consumer responses**

Consumer psychologist

- Consumer psychologists are the psychologists who are educated and trained in understanding consumer habits and the influence of advertisements on consumers' attitudes, thoughts and behavior.

Other Tasks of Consumer Psychologist :

- Consumer psychologists are educators, trainers, researchers as well as administrators.
- Understanding and predicting consumer behavior is not an easy task.
- Even the experienced salesmen are not able to accurately read the consumer's mind.
- THE MAIN TASK OF A CONSUMER PSYCHOLOGIST IS TO CONDUCT CONSUMER RESEARCH.

History of Consumer Psychology

- J B. Watson was the first ever prominent psychologists to apply principles of psychology in the field of advertising.
- Watson believed that psychology could not be recognized as a scientific discipline until its practical utility is proved by its application and demonstration in real life situations.

- According to Watson:
“If psychology would follow the plan I suggest, the educator, the physician, the jurist, and the businessman could utilize our data in a practical way.”
- He himself designed ads for Johnson and Johnson’s baby powder.
- In that ad, he not only targeted the emotions and anxieties of mothers, but also used the experts’ recommendations and the impact of using the product.

Focus of Interest

- The main focus of interest of consumer psychology is:
- To emphasize the consumer's problems, likes, dislikes and preferences
- Develop strategies for making ads, influencing purchasing decisions, innovative marketing, brand and product techniques

Sub- Fields of Consumer Psychology:

- I. Advertising
- II. Perception
- III. Life stages; time, experiences and preferences have a physical as well as psychological effect
- IV. Motivation
- V. Psychology of price
- VI. Market research

Important Terminology of Consumer Psychology:

- i. Consumer behavior
- ii. Personality
- iii. Sales
- iv. Marketing
- v. Product Choice
- vi. Product preference
- vii. Brand loyalty

Learning principles used for attracting consumers:

- Classical conditioning:

Advertisers use models to create a positive feeling and attraction for the product.

Operant conditioning:

Manufacturers announce prizes, prize coupons, extra amounts of product, upsized products at same price, price reductions, and lucky draws for consumers.

Observational learning:

- Advertisements show renowned people using the product under question so that potential consumers feel like following their foot steps.

Some Factors affecting product preference :

- Personality type and brand loyalty
- Peer pressure
- Product price
- Reputation of the product
- Packaging: color, wrappers, shape, size, Captions etc.
- Quality of the product
- Advertisement
- Need

Advertising Appeals:

- Advertisements are to develop a positive image about the product by using attractive models and positive feelings like happiness, excitement, curiosity, and desire.
- Two main techniques are used to make advertisements:
 - I. Soft sell appeals
 - II. Hard sell appeals

I. Soft Sell Appeal

- An approach used to selling the product in which the softer aspects of the product are promoted.
- The product is associated with an image related to the product's use.
- The soft sell delivers the message that the potential buyers will obtain or radiate a desirable image through the use of the product.
- The purpose is to attract the potential buyer through the peripheral route.
- Here the image rather than the product is sold.

Examples of the soft sell appeal;

- Advertisements promoting cosmetics talk about the beauty of the model.
- As of men's products deliver messages about the manliness of the male model, or how people are impressed by his masculinity e.g. advertisements of cigarettes.

II.Hard Sell

- The hard sell approach is the opposite of the soft sell appeal.
- It focuses on the qualities of the product itself.
- The advertisers promote the function of the product.
- Qualities like taste, smell, speed, durability, efficiency, dependability, and/or its nutritional value are emphasized.
- It is stressed as to how much improved the consumers' lives will be because of using the product being advertised.

Examples of the Hard Sell Appeal:

- Advertisements of automobiles, joggers, mechanical instruments, exercisers, metal products, air conditioners etc.

Research Findings About Advertising Appeals:

- children's products are sold through soft sell appeal.
- In case of women's products soft sell appeal is used in advertisements of cosmetics, toilet soaps, and garments.
- But in case of women's perfumes or shoes hard sell appeal is preferred.
- Advertisements of household linen and paints use the hard sell appeal.

Self- Monitoring and Buying Behaviors:

- Self monitoring has been found to be an important variable in ones buying behavior and experimenting with new products.
- Self monitoring is the tendency to consider our own behavior and changing it in order to present ourselves well in particular social situations
- High self- monitors are flexible in nature and adjust their behavior from situation to situation in an attempt to present themselves in an effective manner.
- They are concerned about the image they project and are apt to show conspicuous variations in their behavior from one social context to the next

- In contrast, low self- monitors are relatively more “fixed” in their general attitude.
- They are much less sensitive to the demands of different social settings, and their behavior is more indicative of their attitudes, values and beliefs.
- The behavior of low self- monitors is considerably more consistent across situations, as compared to that of high self- monitors

- high self monitors are affected more by advertisements in general and soft sell appeal in particular.

Psychographics: Investigating The Psychology of the Consumers

- When the manufacturer launches a product, or the advertiser plans a campaign, they have to be aware of the needs of the potential buyers.
- Research has shown that people belonging to different backgrounds and different socioeconomic classes have different buying needs and behaviors.

- It has been identified that people with different lifestyles and needs go for different varieties of products.
- Researchers have used different psychological profiles of people based upon their lifestyles to predict and target their needs.
- Psychographics is a technique for dividing people into lifestyle profiles that are related to purchasing patterns.
- Such profiles take into account characteristics as marital status, race and ethnic background, and educational level, as well as the kinds of activities that potential buyers engage in.

- One of the most popular and widely used general classification systems for consumers is VALS or Values and Lifestyles(Mitchell, 1983).
- The VALS approach, divides consumers into four major groups.

VALS Nine American Lifestyles

- **Need driven consumers include**
- Survivors
- Sustainers
- **Outer- directed consumers include**
- Belongers
- Emulators
- Achievers

- **Inner- directed consumers include**
- I- Am- Me
- Experientials
- Societally Conscious
- **Combined outer- and Inner- Directed Group**
- Integrateds

- Research findings show that members of particular subcultures will have distinct purchasing patterns.
- Different groups have significant differences in values and interests, besides the socio-economic variations.
- Awareness of these differences enables the sellers to attempt to target products and advertisements toward certain groups.

a. Need- driven consumers:

They make purchases primarily to satisfy basic, elemental needs.

- Outer- directed consumers:
- They make purchases in order to increase their own self- awareness.
- The outer- and inner- directed consumers have similar social and self- needs
- Like Maslow's hierarchy, the first few levels in the VALS hierarchy classify groups of individuals whose needs are largely related to two particular lifestyles, the SURVIVORS and the SUSTAINERS

- Both of these are small, impoverished groups whose basic preoccupation is to simply ‘ living’.
- These groups have limited material resources.
- The range of products that are advertised through advertisers is quite limited for the sustainers and the survivors.
- The manufacturers, sellers, and the advertisers are more keen to work upon the groups that have the financial means to purchase the products that they are there to sell.

- There are two other categories :

The I- Am-Me's (young, egocentric people confused about their goals in life).

- The Experientials (former I-Am- Me 's who become surer of their goals and more outward looking) and socially conscious (mature, well informed individuals with a strong interest in social issues such as politics or the environment).

Psychographic Profiles can be used for many reasons:

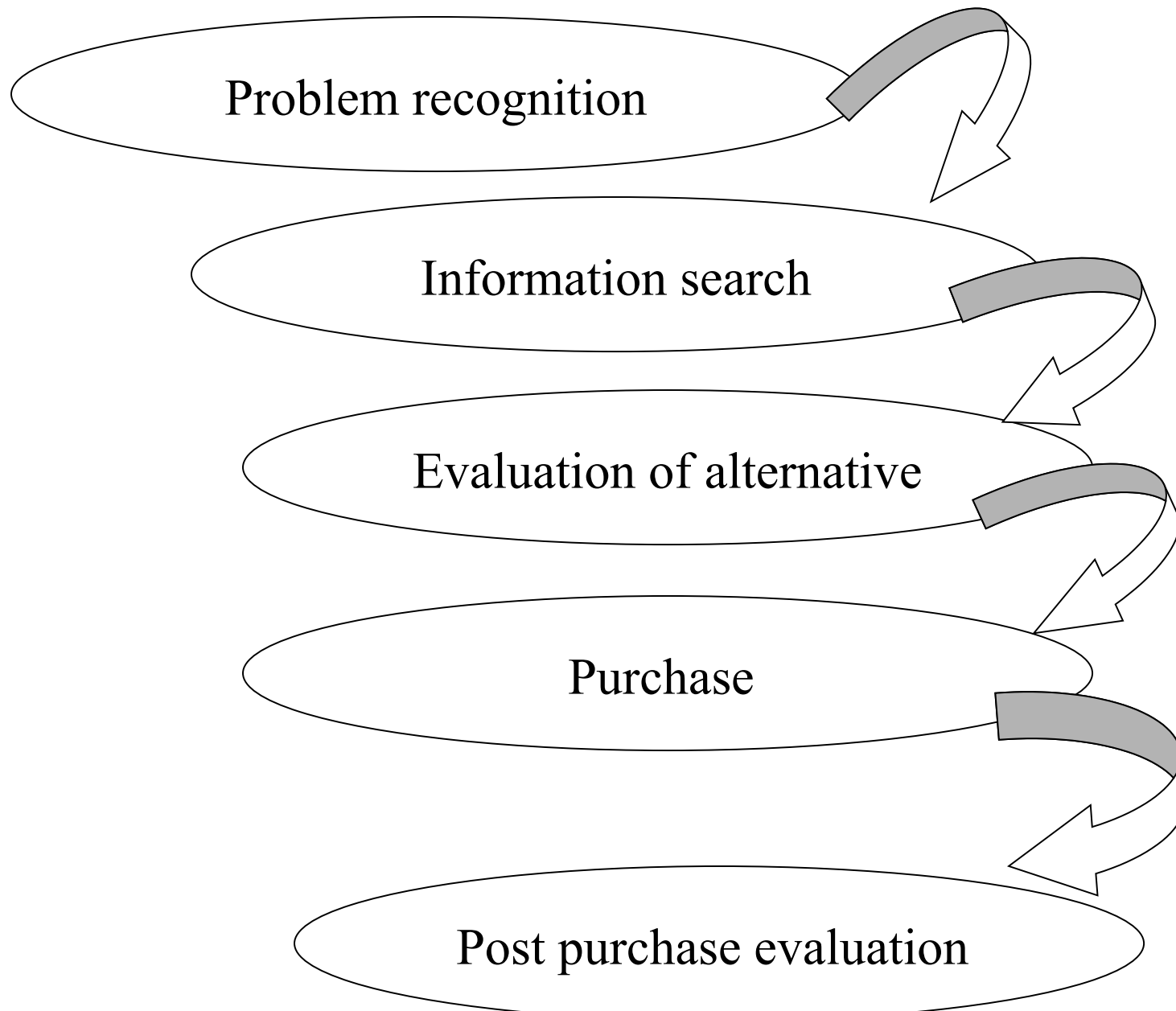
- Identifying specific needs of people coming from specific backgrounds i.e., where to sell tissue papers, and where to paste the advertising posters.
- Also, these help in deciding the medium of advertising.
- For example, in areas without electricity TV ads would be meaningless.
- Instead radio could be an apt medium since most people will be listening to a transistor.

Sub cultural Differences in Consumer Behavior

- Consumer research has revealed a lot about the understanding of differences that exist between sub groups within the general population.
- The way different groups make decisions regarding purchase are different e.g. in our culture taking loans for buying household goods is generally not considered desirable..
- Studies on sub cultural differences in consumer behavior have revealed that clear differences exist between different racial and ethnic groups.

Consumer Cognitive Processing: Decision Making in the Marketplace

- How do you decide which product to buy, which brand to stick to, and which new brand or product to buy?
- Micromarketing is a technique of targeting marketing efforts to particular buyers on a neighborhood or even individual store level.



Problem recognition

- The first process in the model is to recognize that some types of needs exists.
- In the case of the cell phone that continually breaks down, the need is to buy a new one. In the other cases, such as buying a music system , the need is not fully clear, and a certain amount of cognitive efforts is required before one can make decision.

Information Search

- Once the problem is clear, the consumer gathers relevant information through different sources.

Evaluations of Alternatives

- Different alternatives gathered after research are compared and contrasted.
- Their pros and consequence are evaluated.

Brand Beliefs

- Consumers also make decisions regarding their brand beliefs and brand loyalties.
- Brand beliefs are the assumptions held by consumers of a product, based on their knowledge of the reputation of the manufacturer.
- Prior experience with a brand also plays a role in taking a decision.

Purchase

- Finally the purchase is made.
- When purchasing a product the consumer tries to keep the risk to the minimum.
- He may seek a guarantee or assurance.

Post Purchase Evaluation

- Last stage in the decision buying process
- In this stage, we assess the decision to buy, including the choice of the product.
- Advantages and qualities are also evaluated.
- In case of an expensive choice, the consumer may experience cognitive dissonance

- Cognitive dissonance reduction strategies may be used here.
- However if the re evaluation suggests that in spite of being expensive the product was worth buying, then there may be no cognitive dissonance.

Advantages of Career in Consumer Psychology

- Challenging environment to work in, with other people
- Foresight and insight of human behavior
- Creativity and innovation
- Hard work pays off when the outcomes are satisfactory

Drawbacks of Career in Consumer Psychology

High risk burnout when the deadline is given about the product and its selling strategies

Slow start initially in terms of understanding things and computing data

Lack of co- operation___ difficult to find co operative people

Discouragement when the things do not go as desired

Decision overload when have to make certain important decisions of work completion

Lecture 44

Recap

- Consumer psychology
- Focus of interest
- History of consumer psychology
- Basic terminology and sub-fields of consumer psychology
- Advertising appeals
- Psychographic profiles
- Sub cultural differences in consumer behavior
- Consumer decision making

Have you ever thought about these issues?

- **Why don't cricketers show a consistent performance?**
- **Why do they come back to the pavilion when we expected them to make a century?**
- **why do sportsmen end up with a sprain or a pulled muscle when they had to play a crucial match?**
- **Why do spectators cheer up their favorite team and hoot the other one? Why do some players become aggressive, agitated, irritable, and even refuse to play when hooted or teased by the crowd?**

- The answers to these questions may be found in sport psychology.

Sport psychology

- Exercise and sport psychology is Division 47 of APA.
- The branch of psychology that studies, understands, describes, and predicts the impact of psychological variables on athletic and sport performance.

Sport psychology is the application of the principles, knowledge, training, and understanding of psychology for the understanding of factors affecting sport performance, with an aim to improve it, and to make the sportspersons feel stronger and more confident.

- “The application of psychological principles to sport and physical activity at all levels of skill development” (Brown an Mahoney, 1984).

History of Sport Psychology

- Although man was always interested in sports, in improving sport performance, in sport training, and in negatively affecting the performance of the opponents team, the history of scientific research is not very old.
- Research in psychological aspects of sport originated in Europe and then flourished in the U.S.

- The earliest proper sport research was reported by Norman Triplett in 1897.
- He analyzed the performance of cyclists under conditions of social facilitations.
- For this purpose he used field observation and secondary data.
- He reported from his research that the presence of other competitors could facilitate better cycling performance.

Coleman Roberts Griffith

- Known as the father of Sport psychology in North America.
- He established the first sport psychology laboratory at the University of Illinois in 1925.
- He studied the nature of psychomotor skills, motor learning, and the relationship between personality variables and physical performance.

The formative years of Sport psychology:

- The period from 1950 to 1980 is considered as the formative years of sport psychology.
- Sport psychology emerged as a separate discipline distinct from exercise physiology and motor learning.

The Subject Matter of Sport Psychology:

- Education
- Training
- Research
- Sport performance

Psychological Characteristics of Athletes:

- **One of the areas of special interest to the sport psychologists is the study of personality characteristics of athletes and sportspersons.**
- **One could predict sport performance and choose good athletes if one knew the psychological indicators of sport performance**

- one has to be clear about the different connotations of the words 'athletes' and 'sportsmen'.
- Although the two terms are used differently in everyday life vocabulary, to a sport psychologist these two do not mean much different.

Some Research Findings:

- Although not very strong empirical evidence is available on this issue, some interesting facts are available in research literature.
- As compared to the non athletes, the athletes usually score higher on tests of : assertion
- dominance
- aggression, and
- need for achievement

Athletes score lower on:

- Anxiety level
- Depression , and
- Fatigue.
- These findings stand more true when the athletes are at a high skill level.

- Athletes in some sports e.g. hockey and football are more tolerant of pain as compared to athletes in other sports e.g. bowling or golf.
- But there is some evidence suggesting that the pain tolerance may be an outcome of an athlete's success rather than a cause.

Enhancing and Maximizing Sport Performance:

- One of the major tasks of a sport psychologist is to assist the trainers, the coaches, and the team managers as well as the sportspersons themselves in raising the level of performance.
- In this regard, a major portion of research evidence pertains to the arousal level of the sportsperson.

- Sport psychologists have also worked in the following areas:
- Enhancing motivation
- Learning relaxation techniques
- Stress management
- Pain control and management
- Overcoming anxiety
- Enhancing stamina
- Improving performance

Arousal level:

- Sport psychology suggests that sport performance can be improved by manipulating arousal level of the one playing in the field.
- Arousal level has been found to be related with motivation of the sportsperson.

A certain level of arousal is good, in fact essential, for sport performance and competition.

However, too much or too little arousal has negative effects.

What is Arousal?

- Arousal is a neutral physiological phenomenon or state accompanied by:
- Increased heart rate
- Elevated Blood pressure
- Rapid respiration
- Increased metabolism
- More than usual hormone secretion

- The state of arousal is an active sympathetic nervous system (SNS) state.
- it is like a fight or flight response.

How much Arousal is Good for Sport Performance ?

- Although no standard can be fixed for it, research shows that different levels of arousal may be involved in optimal performance on different tasks.
- According to Cox (1990) :
 - Making a long putt in golf requires a low level of arousal,
 - Blocking a shot in volley ball requires a slightly higher level,
 - Making a tackle in football an even higher level, and
 - A bench press in weight lifting requires a very high level of arousal.

Arousal Level and the Role of a Sport psychologist:

- **A sport psychologist makes the sportspersons aware of, and sensitive to, their arousal level.**
- **They are trained to maintain arousal at a certain appropriate level in a manner that the task being performed remains concentrated upon.**
- **The knowledge of biofeedback is applied here.**
- **They are trained in being sensitive to arousal related indicators e.g. blood pressure, respiration rate, heart and pulse rate, muscle tension etc .**
- **There is abundant evidence available that suggests, and that has proved, that human beings can gain control over even their involuntary functions.**

Mental Practice: Helping sportspersons give “Peak Performance”:

- Sport psychologists have devised strategies for helping athletes and others in reaching and maintaining peak performance.
- Research suggests that mental practice involving “imagery” accompanied by actual physical practice is helpful in learning skills, improving and maintaining performance.

Mental Practice and Sport Performance:

- Rehearsing the task mentally, imagining one's self in the actual field.

Functions of Mental Practice:

- The task at hand is rehearsed mentally. this provides a vivid image of the scenario to occur later.
- Negative thoughts that may interfere with performance are reduced.
- The sportsperson can rehearse his/her part in a team sport .
- It helps in setting realistic goals.

The Phenomenon of Home Field Advantage:

- It is a common observation that athletes and sportspersons perform better and are more relaxed when playing in home ground.
- Psychological research does not provide a 100% solid support to this observation.
- Mixed findings are available in this regard.
- Research has shown that frenzied, yelling, screaming home town fans may raise arousal levels of the home team beyond the point of maximum efficiency.
- This reveals the negative effects of playing on the home ground.

- One possible fallout may be that the athletes feel more under pressure as they feel they are expected to perform at their best, and no one is going to forgive a mistake.

- According to Mahoney, while commenting on Olympic athletes :
“ At this level of competition the difference between two athletes is 20 percent physical and 80 percent mental”.

Mental practice can help overcome this problem.

Other strategies used by sport psychologists:

Overcoming self consciousness

- The same tactics as those used for assertiveness training are used.

Principles of learning and sport performance:

- Operant conditioning approach involving positive reinforcement are effective in raising motivation to performance at peak level..
- In learning a sport observational learning is most beneficial.
- Vicarious learning can take place through direct, live observation, or through video recordings.

Stress management and overcoming anxiety:

- Relaxation techniques
- Good nutrition
- Developing optimism through cognitive interventions
- Self Talk

REMEMBER IN PRAYERS
KEEP SMILING
SAY NO TO CURRUPTION
HELP TO OTHERS

Regards,
AARISH ALI & TEAM

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